Yatharth Hospital (YATHARTH)

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Call: Aug 2023

August 26, 2023

The Listing Department
National Stock Exchange of India Limited
Exchange Plaza, 5th Floor, Plot No. C/1
G Block, Bandra-Kurla Complex, Bandra (E)
Mumbai – 400 051
Symbol: YATHARTH
Dept. of Listing Operations
BSE Limited,
P J Towers, Dalal Street,
Mumbai -400001, India

Scrip Code: 543950

Sub: Earnings Call Transcripts

Dear Sir/Madam,

Pursuant to Regulation 46(2)(oa) of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed the transcript of Investors' conference call organized on August 21, 2023 post declaration of Financial Results (Standalone & Consolidated) for the quarter ended June 30, 2023.

The transcript is also available on the website of the Company: https://www.yatharthhospitals.com/investors in the section of Corporate Announcements.

Kindly take the same on record.

Your faithfully,
For Yatharth Hospital and Trauma Care Services Limited

Ritesh Mishra
Company Secretary & Compliance Officer

Corporate Office: HO-01, Sector - 01, Greater Noida West, 201306
CIN: L85110DL2008PLC174706

"Yatharth Hospital and Trauma Care Services Ltd
Q1 FY2024 Earnings Conference Call"

August 21, 2023

MANAGEMENT : MR. YATHARTH TYAGI – WHOLE TIME DIRECTOR
MR. DEEPAK KUMAR TYAGI – PRESIDENT , STRATEGY
& FINANCE

MR. AMIT KUMAR SINGH – GROUP CEO
CA PANKAJ PRABHAKAR – CFO

MODERATOR : MR. PRASHANT NAIR – AMBIT CAPITAL

Moderator: Ladies and gentlemen, good day and welcome to the Q1 FY2024 earnings conference call of Yatharth Hospitals, hosted by Ambit Capital. As a reminder, all participants' lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing "*" then "0" on your touchtone telephone. Please note that this conference is being recorded. I now hand the conference over to Mr. Prashant Nair from Ambit Capital. Thank you and over to you Mr Nair!
Prashant Nair: Thank you Michelle and good afternoon, everyone. I am Prashant Nair from Ambit Capital. I welcome you to the Q1 FY2024 earnings conference call of Yatharth Hospital and Trauma Care Services Limited. From the management, we have with us today, Mr. Yatharth Tyagi, Whole Time Director, Mr. Deepak Kumar Tyagi, President - Strategy and Finance, Mr. Amit Kumar Singh, Group CEO and Mr. CA Pankaj Prabhakar, CFO. I now hand over the call to Yatharth for his opening comments and to take it forward. Over to you Yatharth.
Yatharth Tyagi: Good afternoon, Yatharth Tyagi this side and a very warm welcome to all of you for the first earnings call of Yatharth Hospital and Trauma Care Services Limited, to discuss the business and financial performance of the quarter ended June 30, 2023. I have with me, Mr. Amit Kumar Singh, our Group CEO, Mr. Pankaj Prabhakar, our Group CFO and Mr. Deepak Kumar Tyagi, our President - Strategy and Finance. I hope you would have had an opportunity to go through the quarterly presentation uploaded on stock exchange websites. We are delighted to report that our listing had an overwhelming response from investors and the overall financial community. This achievement reflects the confidence and the trust that you, our shareholders have placed in our vision and potential. We are committed to maintain the highest standards of transparency, accountability, and communication as we navigate this exciting journey together. As we move forward, we will continue to leverage our strengths, innovate relentlessly, and work collaboratively to capit

alize on emerging
trends and industry dynamics.
Let me take our first earnings call as an opportunity to give you a brief overview about the company and the business model before we move to the quarter's performance and the Q&A. Yatharth Hospitals operate super specialty hospitals in the National Capital Region of Delhi and Jhansi Orchha region of Madhya Pradesh. We are among the top 10 largest private hospitals in Delhi NCR region, in terms of number of beds as of March 2023. We started our journey in 2008 with the first hospital in Greater Noida and since then, expanded our presence with hospitals in Noida and Noida Extension. In May 2022, we acquired our fourth Hospital in Jhansi Orchha in Madhya Pradesh, adding 305 beds to our capacity, aimed to expand into new geographies and improve our presence in the regional healthcare market. With this acquisition, our total bed capacity presently stands at 1,405 beds,

including 394 critical care beds. Our hospital strength lies in our strong team of 600 plus doctors, clinical excellence spanning across 30 specialties and super specialties, including our 10 Centers of Excellence.
We have achieved FY2021 to FY2023 compounded annual growth rate of 51% in revenues to become a Rs. 5,203 million revenue company. Our profitability has grown at a CAGR of 90% during this period. We are optimistic about our strong growth potential as we build upon our key strengths of diversified specialties, our advanced medical equipment and technology, strong team of medical professionals and our ongoing expansion initiatives, both organic and inorganic. We have been steadfast in our commitment to expanding our portfolio of high-value specialties. Our journey has been marked by a significant increase in the share of high-value specialties, a testament to our commitment of providing the highest quality of care and addressing the evolving healthcare needs of our patients. I am happy to

declare that our New Jhansi Orchha unit, which started operations in April 2022, has become operationally breakeven in this quarter. We are committed towards ramping up occupancy at our Jhansi Orchha unit, while continuing to grow all our hospitals through strategic initiatives and investments. We ventured into organ transplant operations, which began in December 2022 and had already yielded remarkable outcomes. The successful completion of 50 kidney transplant surgeries since the inception of the program showcases the dedication of our medical team and the state-of-the-art infra that supports them. We have received necessary approvals for liver transplant operations and expect the same to start very soon. Our oncology services will consistently push boundaries of research, treatment, and patient centric care. Our Noida Extension Hospital is taking lead by offering a full suite of oncology treatments, including radiation and PET lines for oncology treatments to start in the coming quarters. Our performance lays the groundwork for future expansion including the potential for organic and inorganic growth opportunities through strategic partnerships and acquisitions. We understand that inorganic growth is not merely about expansion, but also about identifying synergistic opportunities that amplify our strength, extend our reach, and ultimately enable us to serve more patients with exceptional care.

Our recent quarter's performance is a testament of a balanced growth approach. We have delivered a strong revenue growth of 39% Y-o-Y to Rs. 1,545 million during Q1. Our inpatient volumes improved by 14% year-on-year and our ARPOB has increased to Rs. 28,140, up 6.4% year-on-year, while Noida Extension Hospital recorde

d the highest

ARPOB of around Rs. 34,000. The OPD consultation tariffs for all of three hospitals in Delhi NCR have been revised recently by approximately 12%. In terms of key specialties, Nephrology and Urology (Renal Sciences) registered a growth of 56% year-on-year, Cardiology grew 35% year-on-year and Oncology revenues almost tripled year-on-year during the quarter. Our occupancy was 51%, against 40% in Q1 FY2023 and 49% in Q4 FY2023, with the Noida Hospital reporting highest occupancy level at 92%. Our EBITDA

was Rs. 414 million, up 61% year-on-year and EBITDA margin at 26.8%, expanding by 368bps year-on-year, while our PAT was Rs. 190 million, up 73% year-on-year. While we are focused to grow our revenue and EBITDA, with deleveraging of our balance sheet in this quarter with the proceeds of the IPO, our net profitability and overall balance sheet will emerge significantly strong in the upcoming quarters.

With this, I would now like to hand over to our moderator for question and answers.

Moderator: Thank you very much. We will now begin the question-and-answer session. Ladies and gentlemen, we will wait for a moment while the question queue assembles. We will take the first question from the line of Ankit Jaiswal from Futuregrow Spectrum. Please go ahead.

Ankit Jaiswal: Thank you for your opening statement and congratulations on gaining good percentages of Y-o-Y and Q-o-Q. I have a couple of questions to ask. As I have gone through the presentation that you have shared, I have found that at present there is no international customers in the payor mix right, so what are the company plans to acquire the international payor mix, because I guess those are one of the important drivers of getting the revenues. The second question is like in the revenue segment, I have seen that 33% revenue is coming from the medicines and others is 26%, so can you please elaborate about those two segments and one more question like the occupancy on a consolidated basis, I can see that occupancy is around 51% so what are your future plans to increase the occupancy on a consolidated basis?

Amit Kumar Singh: This is Amit Singh, Group CEO, so I would like to take your first question about the international patients. If you look at that in our previous presentation as well all these super specialties, be it transplant program or oncology or even a cardiac and even a highest neurosurgery, which we started, and the international patients started coming in our hospital. We have a very solid program for our next two years that how these international patients we will be entering into it, and we have identified the market. Our team is relentlessly working on those areas. We have identified the pockets in various countries. We are signing up with the various agreements with the government and a good number of patients flow has started coming. In fact, I will tell you the number of surgeries of kidney transplant is an example I gave you. Whatever number of 50 kidney transplant we have done it, more than 95% of these patients are international. We have done very good surgeries. We have done very good numbers in neurosurgery and other specialties. So, it is on a growing side. Yes, our commitment is the next two years to get to grow the unit, which is very closely coming up with the Jewar Airport, which is Asia's largest airport. A 200-bed facility is going to be completely inclined for our international patients, so our strategies are very much clear, and I am sure when we have our next call for the next quarter, we will give much better numbers and for next questions, I will hand over Yatharth.

Yatharth Tyagi: As far as your second and third question, you asked about the percentage of medicine, so just to clarify here this medicine represents the department of internal medicine. It is not Pharma drugs, so the Department of Internal Medicine what it means is the treatment of lifestyle diseases like diabetes, cough, cold, fever, and dengue. This is what the department comprises, which has physician doctors so the share is roughly around 33% for this quarter and as far as your question on what others as you would have seen, what others mean, so if you look at the Q1 FY2023 and Q1 FY2024 breakup of the pie chart that we have also provided in the presentation, others signify few departments and these includes pediatrics, gynecology, gastroenterology, pulmonology, oncology and organ transplant so if I want to break up for the latest quarter, pediatrics it is roughly around 4%, gynae is around 4%, gastro is around 2%, pulmo is around 4%, onco is around 3% and organ transplant is around 1%, so this is the breakup of the others. Now coming to your second question on the occupancy so occupancy, yes in the overall group level it seems 51%, but that is because two of our hospitals have recently started. The Jhansi Orchha that started complete operations in last fiscal year, it was its first year of operation and that is why the occupancy is less in the first year. Now in the latest quarter, compared on year-on-year basis, we have almost doubled our occupancy for Jhansi Orchha Hospital, and I am sure in the coming quarters both Jhansi Orchha Hospital as well as Noida Extension Hospital mature much more in occupancy and at a group level and at a consolidated level, I think we will be able to see their impact in the coming quarters on how the overall occupancy increases in that.
Ankit Jaiswal: Thank you for answering all the questions. One last question from me; is there any doctor equity participation method that you follow or what is the plan to retain the doctors?
Yatharth Tyagi: Right now, we do not have any equity that is held by our doctors. As far as retention of our doctors that is something we will consider in the coming quarters, but as of now the attrition in senior doctors that is super specialty doctors is anyways quite less in our hospital and going forward the retention would continue to be strong because of the DNB Courses that are also coming up in our hospital as well as the departments increasing, it gives doctors a good clientage and that is why they are able to stick with us for a long time but yes as far as doctor equity concerned that is something we will consider in the coming time.
Ankit Jaiswal: Thank you.
Moderator: Thank you. We will take the next question from the line of Reshab Sisodiya from Sameeksha Capital. Please go ahead.
Reshab Sisodiya: Thank you for the opportunity and congrats on a good set of numbers. I have a few questions so if I look at your ARPP like average revenue per patient so it has gone up almost 20% to 23% on a Y-o-Y basis, so how much of this would be due to change in case mix versus any price hike that we have taken?

Yatharth Tyagi: The price hike that has happened recently is more on the OPD front, so as far as the IPDs are concerned, at an overall level not much has been there to significantly contribute to the ARPOB, so it is happening because of the case mix. As you see now, we are doing high share our Oncology Department has grown on a year-on-year ba

sis. In fact, our organ
Transplant Department has recently started plus other high-end super specialties which we are able to do. In fact, neurosurgery, is another department that in terms of volume has grown as far as the Nephrology, so this is what is contributing to the increase in the ARPOB and not just the price hike.
Reshab Sisodiya: Okay understood. Because for higher transplant patients and onco and other specialties that is one of the reasons for your increase in ARPP as well?
Yatharth Tyagi: Yes.
Reshab Sisodiya: Currently, we are 50% to 51% occupancy as you mentioned and almost four-and-a-half

days of ALOS, so there at what point do you see like after reaching let us say 60% occupancies, your ALOS should start reducing?

Yatharth Tyagi: Your question on the occupancy of 30% I did not understand that. Which hospital are you taking about or are you talking about the group?

Reshab Sisodiya: I am looking at a group basis 51% and the ALOS is almost 4.6 so at what level of occupancy do you think your ALOS can start reducing after reaching 60% and 65% so post that you can have better volume as well as margins?

Amit Kumar Singh: I think see if you look at it as long as we are maintaining this ALOS 4 to 4.5 it is absolutely fine. We are with it, and we are not intending to reduce it absolutely. If you look at it, we are much into midline. Even increasing the super specialty, we will be into this because the other specialties will slightly reduce. So, anything between 4, 4.5 to 4.6 still is a very good ALOS and the management has no intent to reduce further. If we maintain this, I think it is absolutely fine with us with the higher occupancy as well.

Reshab Sisodiya: Okay sure that is helpful and one last book-keeping question, so if you could just share the census bed on each hospital basis and also the volumes for each hospital for the quarter?

Deepak Kumar Tyagi: This is Deepak Tyagi. You should see our slide that we have already provided, but if you want the occupancy, the Noida is around 90%, Greater Noida is around 66% if I am not mistaken, Noida Extension is around 36% and Jhansi is around 17%. We are already providing the detailed number of our IP volumes and OP volumes and the revenue respectively by the hospital and I request if you can have it on the presentation we have provided.

Reshab Sisodiya: Okay sure. One last thing; any capex guidance for the year and our debt repayment that we have done in the quarter, or we will be doing next?

Yatharth Tyagi: Capex, we have already stated our intentions and plans in the IPO statement. We will be doing around Rs.132 Crores of capex out of the IPO proceeds. We are working very fast to deploy this amount, so that it can fruitfully be invested back into the business; we will be doing that. What was your next question? On question on the loan repayment, yes, we have repaid our existing bank debt, so our long-term and short-term borrowings have already been done, so you will see the results probably from the coming quarters.

Reshab Sisodiya: Okay sure. Thank you. That’s it from my side.

Moderator: Thank you. We will take the next question from the line of Ankur Kumar from Alpha Capital. Please go ahead.

Ankur Kumar: Thank you for taking my question. My first question is on Greater Noida is the oldest facility, but it has lower utilization in terms of 65% so any thoughts on why it is lower than our Noida facility and any thoughts on improving that?

Yatharth Tyagi: If you look at its Greater Noida seems older, however you can do the account that in 2018 is where we expanded this hospital from 100 bedded to a 400 bedded hospital. So, the story of Greater Noid

a actually starts from 2018, so then that makes Noida Hospital because that started in 2013 with 250 beds and that what it is right now, so Noida in that aspect is an older hospital and Greater Noida started in 2018 and it is moving towards the maturity now.

Ankur Kumar: Got it and on CNBC, you were giving an interview, you are expecting 60% occupancies also and now you are also talking of possibility of 60% so do you think we can achieve that 60% in this year?

Yatharth Tyagi: The management is hands on to work towards it. Maybe it achievable at the end of this fiscal, it is to be seen. Our plan is quite strong and as you can see, we have also had a decent occupancy jump both year-on-year and quarter-on-quarter basis, so let us see how that goes for the fiscal year when this ends.

Ankur Kumar: Got it and any thoughts on new acquisition? Any target area that we are looking at and in terms of bed also anything we are specifically looking at?

Yatharth Tyagi: We are looking to expand both organic and inorganic. As far as organic expansion is concerned both our Greater Noida Hospital, we have acquired the adjacent land parcel and we are expanding that hospital from 400 bedded to around 600 bedded and Noida Extension Hospital also which is a 450 bedded, we were recently the highest bidder for the adjacent land parcel behind this hospital and we also look to expand this hospital by around 250 beds more so around 700 beds so that is the organic expansion. As far as inorganic expansion is concerned, we are actively looking out for opportunities and very soon we will be able to finalize a few opportunities. We want to and right now look in the areas that are North India so not just restrict ourselves to Delhi Market. We are looking for opportunities across Uttar Pradesh, Haryana, Punjab, and Madhya Pradesh, so that is where the acquisition should come in the coming time.

Ankur Kumar: Got it and in PBT, we are talking about our material and consumers are lesser than peers, so can you comment why is that so and on margin is the current 27% odd sustainable?

Management: Material and consumption have traditionally been lower because be it ours while the management provides deep insight into and one of our directors is very, very focused on the effusion, be it capital acquisition, or be it the operational expenditure, and that we continue that is around 18% to 19% we remain normally of our topline. We expect that same to continue in the future years to come as well. Also along with the material your question on how that contributes to higher profitability also you have to look at the fact that our manpower cost, that is both if you add the doctors and the non-doctors that is admin, is lesser that also contributes to our profitability because we have not outsourced a lot of our support services, so whether it be housekeeping, GB and all these specialty management we keep in house so we save on the service charge to the vendors as well as the service tax, so that is also contributing to our probability answering question how we are able to achieve a higher profitability.

Ankur Kumar: Got it. Thank you and I will join back in the queue.

Moderator: Thank you. We will take the next question from the line of Dhara Patwa from SMIFS Limited. Please go ahead.

Dhara Patwa: Thank you and congratulations on a good set of numbers. I have two questions. First is since 37% of our revenue comes from government business, which is quite higher as compared to the peers, so I believe there is limited scope for price hike so what could be the growth drivers to grow the ARPOB further from 30,000 and year on and second question is Noida as a locati

on has usually command higher ARPOB to the tune of 45,000 to 50,000, but we have 50% ARPOB than our peers, so can you explain this and how can we improve this product?

Yatharth Tyagi: Our ARPOB will grow and that will happen both from pricing as well as the case mix, so regardless of the percentage of the government business our cash rate as well as health insurance's rates that is the TPA, they undergo a yearly revision and that will contribute to the ARPOB, however the bigger growth will come from the case mix that you have seen on year-on-year basis. So as soon as we offer the complete spectrum of oncology services, this

ARPOB is bound to increase. Right now, we are only doing medical and surgical oncology. Our radiation oncology is about to start in the coming quarters, so once that starts only then you become complete oncology centre and patients prefer to coming to one stop solution for all oncology, so oncology coming up will definitely increase our ARPOB as well as when the contribution of international patients will rise, ARPOB will also increase from that. Already our CEO is mentioning that we are closest to the Asia's largest international airport that is about to start in Greater Noida somewhere around next year. So, we are quite strong on international operations numbers in the coming time and that will be a huge contributor to our ARPOB. Second is your question and the territory of Noida and why our ARPOB been traditionally lesser that is again if I reverse these points I said, two reasons: First when we started our chain all those years back, we intentionally kept our pricing lesser so we were around a 15% to 20% cheaper from the likes of other hospitals who were in Noida, the super specialty or big chain, and second reason for ARPOB being lesser was the case mix because we do not have robotic surgery, because we do not have oncology in high proportion and international share that is why the ARPOB was lesser, but actually if you look now and especially if you look at the case of the Noida Extension Hospital which is doing around 34,000 of ARPOB that is because the mix of super specialty services is higher in this hospital leading to a higher ARPOB and over the coming quarter when the share of inventory in medicine will come down and these super specialty services increases that will contribute largely to an increase in ARPOB.

Dhara Patwa: Okay understood. One last question, there a lot of peers which are expending in Delhi NCR Region like one such example is with Medanta coming up with 550 bed capacity in Noida and even even Max Healthcare is expanding their operations in Delhi NCR region so how do you see this competition? Do you think that ARPOB levels and occupancy levels will still be maintained post this competition?

Yatharth Tyagi: Yes, in fact we are also expanding in our existing hospitals. So, by the time as you mentioned Medanta or other hospitals that they enter Noida, two of our hospitals Greater Noida or extension are already among the top 10 largest private hospitals of Delhi NCR in terms of number of beds. With these expansion of around 600 beds and around 700 bedded hospital in Noida Extension, we will be among probably top five or top four largest hospitals in terms of more beds, so we have had this strategy of using economies of scale and we believe in the territories where we operate to be the biggest hospital in terms of number of beds and so far we are able to do that and going forward we will continue to maintain that and at the same rate of time even if more competition come these cities are growing cities. The likes of Greater Noida and Noida population is growing year-on-year s

pecifically with the coming up of the airport. These are still far from being saturated, so we feel our occupancy will continue to rise no matter how much the competition comes.

Dhara Patwa: So that is it from my side. That is helpful.

Moderator: Thank you. We will take the next question from the line of Deepak Poddar from Sapphire Capital. Please go ahead.

Deepak Poddar: Thank you very much for the opportunity. First up I just wanted to understand now in terms of acquisition you mentioned, what is the ticket size we are looking at in terms of acquisition target?

Yatharth Tyagi: We are looking on both levels. We are also going in for O&M model as well as we are looking to complete a 100% equity purchase and as far as the number of the size of these hospitals are concerned, we will be looking to acquire hospitals which are at least 300 to 400 bedded in size so we are not going for small hospitals because all our hospitals are considered big super specialty hospitals and that is what we will continue to grow further. We already have a decent amount from the primary IPO reserve for acquisition funding, so that is where the attrition will come from, and 300 to 400 bed size hospital is what we will be willing to acquire, both on O&M as well as 100% equity purchase.

Deepak Poddar: In terms of rupees growth so what is the range that we are looking to spend in the acquisition?

Deepak Kumar Tyagi: See, it all depends as Yatharth just mentioned that what is the bed size and it depends on the location so it would be difficult without having complete deed in our hand to scribe to any amount, so it is better to go by the size and then we see that what the geographical location and what kind of facility are we providing so the price range can you can imagine the real estate present so it would not be proper on our part to provide any price commitment at this point of time.

Deepak Poddar: No. What I was trying to understand more on the front what is the maximum amount that we are willing to spend on the acquisition trend?

Deepak Kumar Tyagi: If you ask the per bed cost, all to all it would be anywhere between Rs. 60 lakhs to Rs. 75 lakh or Rs. 80 lakh kind of pricing we are expecting but that is a very wide range I am providing as I said that it all depends on the size, the location, the facility, and all this.

Deepak Poddar: Understood so may be about Rs. 200 Crores to Rs. 300 Crores is the maximum that we might look at in terms of acquisition side?

Yatharth Tyagi: Yes if it is a 100% equity purchase.

Deepak Poddar: On 100% equity understood fair enough. We have paid this Rs.250 Crores to Rs.260 Crore’s debt right that we had on our balance sheet out of the IPO proceeds of Rs.500 Crores?

Deepak Kumar Tyagi: Yes, we have already done that.

Deepak Poddar: So, the Rs.6 Crore’s interest cost that used to come will not come now henceforth?

Deepak Kumar Tyagi: Yes. It will not.

Deepak Poddar: Okay fair enough and I was going through your presentation as well as your remarks, we are focusing towards more high-value specialties right and even the full suite of oncology treatment and we do expect a higher occupancy in Noida extension as well as in Jhansi

Hospital, so that effectively means a better fixed cost absorption? Ideally what is the margin aspiration we would have right because currently only we are doing around 26% to 27% kind of an EBITDA margi

n. Now all these three things happening so our EBITDA margin aspiration of potential should be much higher right?

Deepak Kumar Tyagi: You are so right, but I am not giving any guidance to you because what our EBITDA margin in the future were to come. We are 26% in this quarter. We expect because we already mentioned that Jhansi has become operational EBITDA positive in this quarter, though marginally. With the better revenue line coming on in Jhansi and contribution coming from all the hospitals, this EBITDA margin would go up decently in the recent quarters to come and I would say that by March 2024 you would see better results than what we are showing now.

Deepak Poddar: But I mean aspirationally 30% is that something we can target?

Deepak Kumar Tyagi: I would not scribe to that way forward. We have our plans. We believe in growing steadfast, so I am not getting any 30% or any percentage to you right now.

Deepak Poddar: Understood. Fair enough. In terms of terms of expansion, you mentioned in a couple of expansion in Noida as well as Noida Extension and by when we are looking to complete that?

Yatharth Tyagi: That is in Greater Noida Hospital and not the Extension Hospital. So, I think very soon we are about to start the construction and roughly it would take three years for the complete hospital to start. Two to three years roughly we will see in that. Greater Noida will start much faster and Noida Extension will start few months later after that.

Deepak Poddar: Right and then my final query is on your revenue. Last three years we have seen a very good CAGR in terms of more than 50% so any sort of CAGR we might look at for the next two to three years; any outlook that you would look to provide?

Yatharth Tyagi: Again, we will not be providing any specific guidance on the exact number, but I think as we would have seen from the Q1 results, we will continue to grow in a very good fashion and that is what all the targets are.

Deepak Poddar: That is great. Okay that is very helpful. That is about it from my side. All the very best. Thank you.

Moderator: Thank you. We will take the next question from the line of Hiten Boricha from Sequent Investments. Please go ahead.

Hiten Boricha: Thank you for the opportunity. Actually, you gave some numbers related to capex of Rs.132 Crores, which we are going to spend on to acquire hospitals. Can you please repeat it, I just missed the number what is the exact number of beds we are adding?

Yatharth Tyagi: This capex that we have mentioned of around Rs.130 Crores. this is the capex that will go into buying of medical equipments for all of four hospitals. This is not the capex that will be utilized for the expansion of the bed capacity of Greater Noida and Noida Extension, so around Rs.133 Crores is what we have raised from the IPO, which will be used to buy new medical equipments for example we are adding oncology machines like PET scans, we are buying surgical robots, we are also buying orthopedic robots, so that is what this capex comprises of.

Hiten Boricha: Okay thanks for that and you also mentioned related to additions of our beds into hospitals, so what are we spending there?

Deepak Kumar Tyagi: We are expanding at Greater Noida from 400 bed to 600 bed that is our plan. At Noida Extension, we have been declared highest bidder we will be expanding our capacity from 450 to 700 beds, but the cost of these expansion is not there in the IPO money. This we will be making out of our internal accruals and the money we are going to save from the debt service, which otherwise would have gone, now we are saving because of the repayment of debts.

Hiten Boricha: This is eve

ntually going to be spending over the period of three years, right?

Yatharth Tyagi: Yes, that will be, so that we will be accumulating our cash and spending it accordingly.

Hiten Boricha: Okay sir and one quickly bookkeeping keeping question. What is our current cash and debt as of today because I heard you have repaid all the debt so there is no debt so what is the cash in the book now?

Deepak Kumar Tyagi: Cash if you ask me immediately, I need to check but roughly in the range of today sitting here, it will be the range around Rs.5 Crores after paying and I am not including the IPO money, which we have otherwise lying in our bank; all I am talking about the current account of the company out of which Rs.255 Crores debt we had in June, so out of that Rs.245 Crores we have paid so there is hardly any debt left very small amounts of debt would have been left now. The IPO money for the inner acquisition like general corporate purpose and unidentified acquisition all these money is lying separate. We have not touched them as of now.

Hiten Boricha: Can you quantify that?

Yatharth Tyagi: It could be around Rs. 65 Crores of identified and unidentified acquisition around Rs. 140 Crores to Rs. 145 Crores of GC lying there and then some amount lying for the expense account to the tune of around Rs. 15 Crores to Rs. 20 Crores, which we will be utilizing post meeting our all-IPO expenses, which we are making as of now.

Hiten Boricha: Thank you.

Moderator: Thank you. We will take the next question from the line of Amit Jeswani from Stallion Asset. Please go ahead.

Amit Jeswani: Good afternoon. I just want to understand what would be the ROIC, or the payback period on the Rs. 132 Crores capex that we are doing that is my first question?

Deepak Kumar Tyagi: Did you ask about ROC.

Amit Jeswani: Yes, it is, okay? You can say anything?

Deepak Kumar Tyagi: Okay, see we are looking it would be difficult to make out the separate ROI for it. I would be better call it that what is going to be payback of these thing. I expect the payback for this investment should be in the range of around two to three years because this is one thing it is not maintenance capex, it is enhanced capex, but major most of it is going on to the oncology or around the Rs.60 Crores to Rs.65 Crores of the amount going into oncology and robotic surgery and all these things so I expect that overall investment of Rs.132 Crores will be paid back in two to three years' time.

Amit Jeswani: Got it so that would mean about Rs.40 Crores to Rs.50 Crores a year? Just trying to understand, our profits were about Rs.66 Crores last year. The debt we paid Rs.24 Crores as interest so that would mean that our pretax profit was Rs.88 Crores last year and now you will not have to pay any interest of course. We might get some other income so we will have Rs.24 Crores added to the PBT level, and you are saying we will make about extra Rs.45 Crores to Rs.50 Crores extra per year on the Rs. 132 Crores equipment investment did I guess that right?

Deepak Kumar Tyagi: You are right on the analysis part, but I mentioned that it would be payback period, so it may not materialize anything, and everything is 2024. It could come in 2025 and 2026 in a phased manner as well, so it would come in the form of higher ARPOB, higher EBITDA, higher revenue billings coming in, and yes resulting into the better PAT, which at this point of time we are at around 12%, so I am not giving any guidance, but you can make out that these things coming in better PAT is expected.

Amit Jeswani: Got it and this does not include the new capex for the existing hospitals that we are increasing our bed capacity, so what kind of capex should we expect there and so that we are already at very high-capacity utilization in one of our hospitals at 80% to 90% so should we expect that capex to start this year itself?

Deepak Kumar Tyagi: Yes, capex is two facets Amit. One is that what do capex we have sort in the IPO so that is around Rs. 132 Crores. A good lot of portions would be utilized in this fiscal itself and there are some portions, basically Jhansi, the radiation set will be utilized in the coming year. The second set of capex, which will be in the seeded kind of things which Yatharth just mentioned that it would be in the form of expansion at our current hospital so that would be coming and as far as the ticket size of the new capacity, which we are adding I had already indicated that it would be in the range of Rs. 60 lakhs to Rs. 75 lakh per bed.

Amit Jeswani: You said you have increased the prices of surgeries and operations by 12% and that was done after this quarter?

Yatharth Tyagi: No just to correct you there. The 12% increase was the OPD prices that is the consultation, outpatient department consultation and that was somewhere at the start of this quarter.

Amit Jeswani: Got it. Our goal as I am understanding would be whatever cash we generate we would keep reinvesting across a cluster-based approach in North India and with every passing year our average revenue per occupied bed should keep increasing from assuming we are at 26,000 last year? That number do you have a target in mind that in the next two to three years this number should be 35,000 to 40,000 something like that?

Yatharth Tyagi: You will see our ARPOB will increase, but we have not taken a specific target as to how much. That is something I cannot share. However, in Noida Extension Hospital if you see the ARPOB is already 34,000, so we expect the ARPOB here to increase sharply post the Oncology Services and yes that will overall group our ARPOB tend to rise in the coming years.

Amit Jeswani: I know you are not giving future-looking statements, but this 27% EBITDA margin is a sustainable number and there will not be any large deviations to that? It probably will go higher, but at least this number is a sustainable number, right?

Yatharth Tyagi: Yes, see and we increased it to go a bit higher and it will continue to sustain because our growth is coming from also from our organically that is the existing hospitals will grow in occupancy so EBITDA margin will be sustainable.

Amit Jeswani: Got it and how do you retain doctors? What is that thing that works best for Yatharth to retain doctors?

Yatharth Tyagi: The promoters Dr Ajay Kumar and Dr Kapil Kumar; they themselves are medical professional from so this understanding and thought process of a medical profession is really embedded in our management and doctors relate to a management a lot. We are not a corporate that give targets and specific numbers of doctors that this is how much is expected from you. We let them practice in a medical, ethical and a very transparent way and also if you see the growth in terms of our IPD growth volumes that have happened over the course of last two to three years, the doctors who join their hospitals they have also grown a lot in terms of their patient clientage because of the clients that came to Yatharth, so they are able to be with us for a long time because our doctors are also growing not

just
the hospitals and that is why they are with us for a long time and we also engage them in a lot of internal programs like various conferences as well as patient engagement so that is why the doctors are involved in every decision making. If not that they are just involved in the patient care, then if some medical equipment is to be finalized, if a new department infra has to be change, we really involve the doctor, so we really empower them and that is why they have a bigger say at Yatharth Hospital and that is why they are with us for a long time.

Amit Jeswani: My best wishes to you, Yatharth and the team. Thank you so much. I will be in touch.

Moderator: Thank you. We will take the next question from the line of Aashil Shah from CJ Shah. Please go ahead.

Aashil Shah: Good morning a couple of questions here. What would be the margins that we hit in the Noida Hospital?

Deepak Kumar Tyagi: We are not giving any specific guidance hospital wise.

Aashil Shah: The existing margins at Noida Hospital?

Deepak Kumar Tyagi: Noida and Greater Noida put together you can say I am not giving any specific number, but that would be the range of around 28% so this answer I am giving. Does this answer you?

Aashil Shah: Yes, that definitely helps, and this Noida and Greater Noida does not include Noida Extension?

Deepak Kumar Tyagi: No Noida Extension is a different hospital.

Aashil Shah: Okay. Do we feel that the other hospitals, which are your Jhansi and Noida extension can come on a standalone basis come closer to these margins or is there some difference in payer mix and case mix, which is why they might be inherently lower margin even after?

Deepak Kumar Tyagi: The payer mix is not different, but definitely there would be a case mix so the margins at the respective hospital would be different. We expect Noida Extension being niche on robotic, oncology and organ transplant, we will be doing far better in the coming years to come and so is the Greater Noida. Noida traditionally had been doing and it would contribute to do that. Jhansi is a different price segment. It is a new hospital, so marginally on a margin side, it will come to its side, so that is the case basically.

Aashil Shah: Got it so as on today without giving specific numbers, would Noida extension be a higher margin than Greater Noida already or that is eventually?
Deepak Kumar Tyagi: At this point of time, our Noida Region Hospital is almost at par. In the future we expect that Noida extension will contribute better.
Aashil Shah: Got it. Fair enough and compared to our Medanta, Max and the other guys who are expanding there, our ARPOB is lower just wanted to know for the similar kind of treatment let us say for some oncology-related treatment or some surgical treatment our cost of treatment is similar to theirs or is it lower in general?
Yatharth Tyagi: It is a bit lower. If I put a rough estimation, it would be around 15% to 20% cheaper from the likes of the hospital that you just mentioned.
Aashil Shah: Got it and do we expect to keep like let us say that their pricing goes down? Do we expect to keep matching it or by continuing to be 15% to 20% lower or how do we look at that?
Yatharth Tyagi: Regardless of what our competitive prices are, our existing prices will see a yearly increment of a few BPS or percentages and that is what we will be focusing on.
Aashil Shah: Got it and last question on the Jhansi as well as Noida Extension part as we go up in occupancy are we also looking to have more doctors and nurses and the variable costs will increase or have you already done that employee how many, however we need we have them ready already?
Yat

harth Tyagi: Majority of the cost has already been incurred and it is already there. When the occupancy increases some only a few handful of nurses are required and just in doctors if I talk about so mostly the resident medical officers that are required. The specialists and the super specialist doctors are already there and already hired, so we do not see much incurrence of cost when the occupancy increases in these hospitals.

Aashil Shah: One question I missed. Any reason for the margin to be going down before IPO; it went down by a couple of basis points each year? What was the reason for that?
Yatharth Tyagi: Those were the COVID years. In COVID, the margins were higher because of we had a lot of fixed cost was less and they were high patient volumes, so that is the reason for the margins high at that time but even post that just 1% or 2% post COVID it did and it has been maintained now so these we consider a good healthy margins and that is what has been typically the case with the company for very long and since the inception of the company we have always been EBITDA positive. We have always maintained healthy margins and that is what the case is.
Aashil Shah: Understood. Thanks a lot for taking out the time. This is very helpful.
Moderator: Thank you. We will take the next question from the line of Hussain Bharuchwala from Carnelian Asset Management. Please go ahead.
Hussain Bharuchwala: Good set of numbers and congratulations for that. Just one question, I wanted to understand, this Jhansi Orchha where we have 8% occupancy currently do you plan to ramp up so how you expect the ramp up to happen. Eventually expect the utilization to be at higher 50% utilization and how are you planning to do? Is there something that we have some thoughts on that?
Yatharth Tyagi: Jhansi Orchha is a very important hospital for us. The reason it will not take that much time for occupancy to ramp up. You said three to four years. We expect that to be much, much faster than that. Typically, when a hospital starts, its first year of operation it goes into getting all the licenses including the legal licenses and then when that happens and then that is where you get all the empanelment including the healthcare insurances including the government empanelment. So that is where the first year went for this hospital and now, I think a lot of these empanelment are already there and yet we are awaiting to tweak the empanelment, so as soon as this empanelment happen, we expect that a major contribution should come as far as the occupancy is concerned. Also, another reason for we expect the occupancy to ramp up soon there. It is now all the departments are completed and all the doctors so in the first year especially in the hospital, which is around 450 kilometers away from Delhi NCR region, it takes time to get all the super specialty doctors, but as of today now most of the departments are complete with all the doctors so that will also contribute in the coming quarters for a good occupancy contribution, so the occupancy should wrap up very soon because of these two key factors.
Hussain Bharuchwala: Okay got it. Two more question you have already answered. Thank you.

Moderator: Thank you. We will take the next question from the line of Manoj Shroff from Kivha Advisors. Please go ahead.
Manoj Shroff: I want to know about the attrition. Can you update us on the fewer doc

tor attrition and the overall doctor's attrition and the other staff attrition as well and can you give us comparison? I think it was there in the prospectus. Can you share what is it now and in plans how will we bring it down?
Deepak Kumar Tyagi: On efficient part Manoj, the most important thing is that our key doctors, the doctors who basically drive our volumes and business, that attrition has already come down below 9%, that is one part. With this DNB program coming in, we expect that our other junior doctor's attrition will also come down because that will be providing them long-term engagement. The other important attrition, which worries us is the nursing attrition. Nursing industry wide it is a problem, but now we have with the engagement coming so this number is also coming down steadfast. These are the important numbers which really affects our business. As far as other attrition in the terms of our core employees is high, but because they are not that we are not recognizing their contribution. They are important but this over a period of time will come down with the better brand equity and better job prospects we are going to create.
Manoj Shroff: If I can get just go on this project in January 2022 in the July prospectus 2022, we said that our overall doctor attrition was 65.7%. Can we share how much is it now?
Deepak Kumar Tyagi: As I just mentioned that how our key doctors are retiring so that is below 10%. As far as other doctors, the 65% of what we had stated in our DrHP was inclusive of both our key doctors and junior doctors, so junior doctor's attrition as I mentioned that with the DNB program coming in, which we have already started at Noida Extension and Greater Noida and soon it will start at Noida as well for which the formalities are on the way. This attrition will also come down. I am hoping, not putting any figure because that is not in my hand, but that will come substantially lower than what we had stated in the DrHP at that point of time because that was the time when COVID was also stopping us and doctors and other medical and other medical professionals were in demand so this year that is basically fiscal 2024, we are normal year from that attrition point of view and that will come down substantially.
Manoj Shroff: So mostly right now say Q1 or say the last couple of quarters it is yet at 65%?
Deepak Kumar Tyagi: I am not putting any figures because we had not computed that way, so I am not putting any ticket because the figures are not with me as of now. We might publish it in the Q2 earnings, but not for Q1 at this point in time.

Manoj Shroff: Got it and one thing in the Medanta and Max Hospital, as you said our pricing was 15% to 20% lower, but there ARPOB is a 60,000 and 70,000 and our ARPOB in Noida is 34,000, so the gap seems much higher for any other reason again?
Yatharth Tyagi: Yes, that is because of a change mix and now we are maturing towards the hospital, which in the coming time will contribute more from super specialty and we do not have international clientage so far in the tunes of what they have yet we are starting to grow now on our international clientage as well as robotic surgeries. So, these three things combined a big reason for our ARPOB being lesser but in coming years once we start all these three aspects and they contribute into bigger volumes in overall and our ARPOB will be somewhere closer.
Manoj Shroff: Got it. That is fantastic. The margins you mentioned even at such a low ARPOB comparatively our margins are in fact much better or equal to them? You mentioned one of the reasons was I think support staff, etc., anything other strong reasons which y

ou can share?
Deepak Kumar Tyagi: The margins are basically some total of how we effectively use our manpower, that is basically our doctors, because that goes into the specialty charges and then it comes to employee cost and then if you see our low consumption of material also, so they are also lower as comparatively, so these are the factors basically we are very hands on, on anything and everything. Mr. Amit is saying or for that matter Dr Ajay and Dr Kapil. Anybody and everybody put his effort to see that every cent what we are spending is utilized better and

that contributes to our margins and that is the reason that our ARPOB, which is a bit lower, I would say substantially lower when we see as to the likes of the top notches, are still better than what the industry peer shows.

Manoj Shroff: One last question, the nearest hospital to our catchment area in the branded national brand I believe is Fortis if I am not wrong, is that right?

Yatharth Tyagi: Yes, that is right. Fortis is there is Noida. Fortis is also there in Greater Noida.

Manoj Shroff: Right and how much is our pricing lower to that Fortis there? Will it be in the same 15% to 20%?

Yatharth Tyagi: Yes, roughly in that tune what we have mentioned.

Manoj Shroff: The Fortis margins are also significantly lower than ours, so I was just wondering how do we win and do much better than them in occupancy there?

Yatharth Tyagi: Can you repeat your first part of the question. Your voice was a bit lesser.

Manoj Shroff: My question was how did you manage to get more occupancy versus Fortis in that particular area? Anything basically how is the brand better?

Yatharth Tyagi: Two to three key factors on why patients in our locality are choosing us over the national brand. First the key factor is infrastructure. Two of our hospitals that are the greater Noida which we renovated in 2018 and Noida Extension but that started in 2019, these are pretty brand new hospitals, which are at par with any India's health infra is concerned, whether it be a room or whether it be the patient waiting area, so the patients get that a price which is 15% to 20% cheaper so that is A. B is in the territories for example Greater Noida and for example in Noida Extension, we really had an early mover advantage, so our brand is very strong in these areas and also if you look at the overall Noida and Greater Noida territory, people perceive Yatharth as a brand with around 1,000 beds in this territory. So, we are the sort of the biggest players and that is why we are able to cater to lot of large volume. Our key doctors are the ones that have really become brands in this area and people are coming and choosing us and another big thing happened with us is during COVID, ours was one of the biggest private COVID players in the region. The patient experience that the patients had during COVID plus word of mouth that spread, post-COVID it has really, really helped us and it has really helped in the post COVID numbers that you see now people have seen our latest infrastructure, they have seen a really good patient care experience. The word of mouth has really spread across this region, which is helping us to attract even higher volumes of patients.

Manoj Shroff: Thank you so much for patiently answering. Thank you so much and all the best.

Moderator: Thank you. Ladies and gentlemen; this will be the last question for today which is from the line of Ankit Jaiswal from Futuregrow Spectrum. Please go ahead.

Ankit Jaiswal: Thank you for taking my question again. I have one major question regarding the hiring process of key doctors? Like other hospitals have a plan of hiring the doctors from t

heir
attached or medical colleges so what is the exact hiring process or your thought process?

Yatharth Tyagi: Well, we have had this strategy that we do get doctors from the likes of Max, Fortis, Apollo who have recently joined us and we have intentionally not targeted their head of departments. We have targeted their number twos that is the subheads that means the associate directors, who get a chance to become HODs and directors at our hospital and they are equally skilled, and they are equally experienced, and they are actually much more hungry and they are the ones who are actually driving the volumes in those hospitals. So, they join us because here they get the chance to become the head of the departments where in their older hospitals that will take them much longer time and that is why they are able to join us and be with us for a long time. Now we are getting a lot of leading doctors from all these hospitals and that is where we get our doctors and also our own doctors who have

been with us for a long, long time, they have also grown into a big brand now, they have also become senior and leading doctors of the city. That is where our doctor’s pool is coming from.

Ankit Jaiswal: One more last question; will this call transcript will be available on the company’s website?

Yatharth Tyagi: Sorry, what is available on the company's website?

Ankit Jaiswal: This call transcript?
Yatharth Tyagi: Yes, it will be available on our website.
Ankit Jaiswal: Okay thank you.
Moderator: Thank you. Ladies and gentlemen, that was the last question for today. With that we conclude today's conference call. On behalf of Ambit Capital, we thank you for joining us and you may now disconnect your lines. Thank you.
Management: Thank you very much from the company side as well. Thank you very much.
Moderator: Thank you members of the management.
(This document has been edited for readability purpose)
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Call: Nov 2023

November 16, 2023

The Listing Department
National Stock Exchange of India Limited
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G Block, Bandra-Kurla Complex, Bandra (E)
Mumbai – 400 051
Symbol: YATHARTH
Dept. of Listing Operations
BSE Limited,
P J Towers, Dalal Street,
Mumbai -400001, India

Scrip Code: 543950

Sub: Earnings Call Transcripts

Dear Sir/Madam,

Pursuant to Regulation 46(2)(oa) of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed the transcript of Investors' conference call organized on November 8, 2023 post declaration of Financial Results (Standalone & Consolidated) for the quarter/half year ended September 30, 2023.

The transcript is also available on the website of the Company: https://www.yatharthhospitals.com/investors in the section of Corporate Announcements.

Kindly take the same on record.

Your faithfully,
For Yatharth Hospital and Trauma Care Services Limited

Ritesh Mishra
Company Secretary & Compliance Officer
Mem. No. A51166
Enclosed as above.

Corporate Office: HO -01, Sector - 01, Greater Noida West, 201306
CIN: L85110DL2008PLC174706

"Yatharth Hospitals and Trauma Ca re Services Ltd
Q2 & H1 FY2024 Earnings Conference Call "

November 8, 2023

MANAGEMENT : MR. YATHARTH TYAGI – WHOLE TIME DIRECTOR

MR. AMIT KUMAR SINGH – GROUP CEO
CA PANKAJ PRABHAKAR – CHIEF FINANCIAL OFFICER
MR. DEEPAK KUMAR TYAGI – PRESIDENT , STRA TEGY
& FINAN CE

MODERATOR : MR. PARAM DESAI – PRABHUDAS LILLADHER

 Moderator: Ladies and gentlemen, good day and welcome to Yatharth Hospital's Q2FY24 Earnings
Conference Call hosted by Prabhudas Lilladher. As a reminder, all participant lines will be in the listen only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you n eed assistance during the conference call, please signal operator by pr essing * then 0 on your touc htone phone. Please note that this conference is being recorded. I now hand the conference over to Mr. Param from Prabhudas Lilladher. Thank you and over to you, Mr. Param.

Param Desai : Yeah. Thank you Malcoln, and good morning everyone. On behalf of Prabhudas Lillad her, we welcome you to the Q2FY24 Earnings Conference Call of Yatharth Hospital & Trauma Care Services Limited. From the management , we have w ith us today , Mr. Yatharth Tyagi , Whole -time Director, Mr. Amit Singh , Grou p CEO, Mr. Deepak Tyagi , President Strat egy & Finance and Mr. Pankaj , CFO.

I now hand over the call to Yatharth for his opening comments and to take it forward. Over to you, Yat harth.

Yatharth Tyagi: Hi, Good Morning. This side Yatharth Tyagi , Whole -time Director at Yatharth Hospital & Trauma Care Servic es Limited. A very warm welcome to you all for the Earnings Conference Call for the quarter and half year ended 30th September 2023.

I have with me Mr. Ami t Kumar Singh, our Group CEO, Mr. Pankaj Prabhaka r, our CFO, and Mr. Deepa k Kumar Tyagi, our President of Strategy & Finance. Let me draw your attention to the fact that on this call our discussions will include certain forward -looking statements which are predictions, projections or other estimates abou t future events. These estimates reflect management's current expectations about future performance of the company. Please note that these estimates involve several risks and unce rtainties that could cause our actual results to differ materially from what is expressed or implied.

We have uploaded our presentation on the exchange and the company website , and I hope you all might have received and had an opportunity to go through i t.

Since this is our second earnings call post our listing on the stock exch anges , I would like to give a brief background on our company quickly. Yatharth Hospital operates super specialty hospitals in the national capital region of Delhi and Jhansi -Orch ha region of Madhya Pradesh. We are among the top 10 largest private hospital s in Delhi -NCR region in terms of number of beds. We started our journey in 2008 with the first hospital in Greater Noida and since then expanded our presence with hospitals in N oida and Noida Extension. In 2022 , we acquired our fourth hospital in Jhansi -Orchha in Madhya Pradesh with the aim of expanding

 into new geographies. Our total bed capacity presently stands at 1405 beds , including 394 critical care beds.

I am pleased to an nounce that we have delivered a stellar performance thi

s quarter. Our revenue has experienced a remarkable 34% year -over-year growth , reaching ■1 ,713 million , while our Profits after Tax have surged by an impressive 70% compared to the same period last year and 45% compared to the immediately preceding quarter.

We have successfully co mpleted our IPO during the month of August and subse quently your company has received the primary proceeds of ■610 crores which has boosted our capital base and helped us turn Net Debt free during the quarter.

Our ongoing commitment revolves around divers ifying a range of medical specialties and introducin g new ones across all our hospitals. Q2 witnessed a double -digit growth across

most of our specialties. Our Nephrology & urology , and Neuroscience departments have grown by 40 -45% year -on-year. Orthoped ics and Cardiology have grown by 20 -25% year -on-year. While our Gastroenterology, Pulmonology and Oncology department revenue has more than doubled compared to the last year.
Our commitment continues to remain towards enhancing our suite of Oncology ser vices and create a comprehensive one -stop destination f or all cancer related treatments. The Radiation Oncology machines has arrived at our Noida Extension hospital, and we expect it to be commissioned by January 2024 , seamlessly integrating the Radiation O ncology line within our Oncology Center. We are in the process of ordering equipment for robotic surgeries which is expected to arrive in the coming quarter. Thus, we are well on track to offer robotic service by the coming quarter and a comprehensive suit e of Oncology treatm ents by the fourth quarter of this fiscal year.
As we are progressing, we have concluded around 100 kidney transplants so far and we are constantly expanding our organ transplant and medical tourism business which is poised to drive substantial growth and contribute to an improvement in average revenue per occupied bed in the upcoming quarters. We are continuously evaluating opportunities, both organic inorganic, and strategic partnerships and remain committed to expand our bed capaci ty in the coming years . We aim to double our bed capaci ty over the next 3 -4 years via mix of both greenfield and brownfield expansion. We understand that inorganic growth is not merely about expansion but also about identifying synergistic opportunities th at amplify our strengt hs, extend our reach and ultimate ly enable us to serve more patients with exceptional care.
 At our current capacity, we have seen a remarkable improvement in our utilization. So, the Noida and Greater Noida hospital report ed highest occupancy levels of 96% and 73% respectively during the quarter. Our other hospital at Noida Extension has received an occupancy level of 45% during quarter compared to 28% in Q2 FY23, while our Jhansi Orchha Hospital is now at 20% compared to 5% in Q2FY23. In line with our Noida and Great Noida Hospital, we ex pect our Noida Extension hospital to reach optimum utilization level s by FY2025. Thus, we have already acquired a land parcel adjacent to our Great Noida hospital and declared L1 bidder for a land adjacent t o our Noida Extension hospital , which should supp ort our organic expansion plans in these hospitals.
Ladies and gentlemen, regarding the recent income tax search operation at our company from 19th-22nd October 2023, I would like to address you that the s earch operation yielded no significant preliminary findings of unaccounted cash or incriminating transactions. The company is duly replying to the queries of the tax authorities and does not foresee any impact on the operating and financial performance of the company in the coming quarters.
I would like to now hand over the

call to our Chief Financial Officer, Mr. Pankaj Prabhakar,
for the financial update for the quarter.
Pankaj Prabhakar : Good Morning everyone. I am happy to announce tha t your company has reported another quarter of strong performance. We have recorded our highest ever qua rterly revenue at ■1,713 million , reporting to a robust growth of 34% year -over-year and 11% quarter -over-quarter , with growth across specialties and improvement in occupancies across our hospitals. Our Inpatient revenue have grown by 37% year -over-year du ring the quarter. Our EBITDA for the quarter was ■456 million, up by 36% year -over-year and 10% quarter -over-quarter. Our EBITDA margins for the quarter expanded by 40 basis points to 26.6% , with improved operating leverage and efficiencies across all our hospitals.
A significant development in this quarter is that Yatharth Hospital has become Net Debt free as of September’2023 -end, by utilizing around ■2 ,450 million proceeds raised during the IPO to repay majority of our existing debt burden . For our comp any, maintaining a debt-free status is not just a goal but it is a core part of our financial strategy and philosophy. Operating without the burden of debt will allow us to focus on sustainable growth, innovation and long -term success. In line with our deb t reduction, our finance cost has reduced by 42% year -over-year and by 50% quarter -over-quarter to ■29 million during the quarter. Full impact of debt reduction on finance cost will be further visible during Q3.
Consequently, we have been able to grow our profits considerably by 70% year -over-year and 45% quarter -over-quarter to ■276 million during the quarter.

 As far H1 performance is considered, our revenue has grown by 36% year -over-year to ■3,258 million. Our EBITDA for the half year was ■870 millio n, up by 47% year -over-year, while our EBITDA margin for the half year expanded by 192 basi s points to 26.7%. Our Profit after Tax for the half year were up by 71% Y oY to ■466 million.

Our Balance Sheet has strengthened significantly during the quarter with pr imarily capital infusion of ■6 ,100 million from the IPO. Our net worth has increased to ■8,065 million by September -end compared to ■1 ,830 million as of March'2023 -end. As on September -end, your company stands net debt free , with net cash position of ■3 ,109 million , which we strive to invest towards our ongoing and planned organic and inorg anic expansion as stipulated in our IPO objectives.

Overall, our recent quarter performance is a testament of our balanced growth approach. I would like to emphas ize on t he fact that while our focus remains to multifold our revenue and EBITDA in the comin g quarters and years, with the de leveraging of our Balance Sheet , our net profitability and overall Balance Sheet will continue to emerge significantly strong in the up coming quarters.

With this, I would like to hand over to Moderator for Question and Ans wers. Thank you.

Moderator: Thank you very much. We will now begin the Question -and-Answer session. Anyone who wishes to ask a question may press * and 1 on their touch stone telephone. If you wish to remove yourself from the question queue, you may press * and 2. Participants are requested to use handsets while asking a question. Ladies and gentlemen, we will wait for a moment while the question queue assembles.

The fir st question is from the line of Amit Thawani, who's an individual investor. Please go ahead.

Amit Thawani : Hi Everybody and congratulations on a fantastic set of numbers. My first question is, last full year , the government business was 37% of our total r evenue, can you tell me what that number is for H1 and for Q2?

Deepak

Kumar Tya gi: Well, Amit, thanks for the question. You know, government business is significant to us, and it is in the range of around 36 -37% and still going. So, we are maintaini ng the same thing. Good part is th at the growth is coming from all the sectors. You know, be it from the corporate sector , or be it from the insurance or the government business. But the best part is that, you know, as Mr. CFO has mentioned that we are goin g for a balanced growth and so our revenue is coming from all the quarters of the company.

 Amit Thawani : Okay -okay, thank you. Coming back to this same question, can you divide the government business? Just broad percentages as to how much is coming from C GHS, how much is coming from PM Jan Arogya an d how much is maybe coming from ESIC? Are there so me broad numbers there?

Deepak Kumar Tyagi : No, we don't have any business coming from PM Jan Arogya. You know, very small amount it comes at Jhansi , but that's very, very negligible when it comes under the percentage terms. Frankly speaking, I haven't di stinguished that what is coming from CGHS or ESI C. If it is so important, we'll get back to you.

Amit Thawani : Okay. Sorry, I'm a little bit of an amateur in this and so I'm just going to ask very basic question. So, the budget that the govern ment allocate s for CGHS and ESIC, are they one combined budget or is it a separate budget for the three? And can you give us like what has been the growth in the allocation of this expenses over last year by the budget? Do you have those numbers by any cha nce?

Amit Kum ar Singh : If you look at ECHS, CGHS and ESI, all those have a separate budget. So, however it is all controlled by the government authorities. All three have a separ ate entity who takes care of it. So, I think, in last couple of years as such there's no is sues in terms of, you know, releasing the funds. So, the frequency they're maintaining that is good.

Amit Thawani : Okay -okay. Secondly, I mean, we were in the proces s of getting approvals or empanel ment rather of our Jhansi Hospital, can you give us the pr ogress there? What is th e status of empanelment there?

Amit Kumar S ingh : Jhansi, if you look at it, the team is working very hard in terms of getting the empanelment . First is the insurance sector, so whatever the insurance and TPA operating in India as of now, I think 60 -70% already it has gotten e mpaneled now. The focus is to do couple of

PSUs like BHEL, Railways and others which is having a significant number of p opulations residing over there. So, already, I mean, we have reached on the last leg of the confirmations from those authorities. I thin k next quarter you will see those numbers start coming. Rest others, the corporate s, whichever corporates are available, our team is working there. I think a couple of them already we got it and it's in the proce ss and next month or something, I think, we'l l have a significant number of those empanelment s.

Amit Thaw ani: Okay. So, basically, we have received 70% of the empa nelment that we had intended to take?

Amit Kumar Singh : No, approximately, I would say that i nsurance cycle side , it's around 30 -32 insurance companies , private a nd GICs working/operational. So, I think more than 50% -60% we have received and in the coming mo nths I think more is going to come.

Amit Thawani : Okay -okay, got it. Our ARPOB is roughly ■ 28,000, can you divide that ARPOB into how much is government ARPOB and how much is non -government ARPOB?

Deepak Kuma

r T yagi: So, you know, so far, we have not disti nguished. I told you that, you know, we are putti ng up the system. In the last call also you were there , I distinctly remember, and I had mentioned that we are putting up the system where we will be putting up ARPOB not only from the business segment but b y the specialties also. But at this point in time , because this is mid of the year, so this will take som e time. I am sure that by Q4 kind of thing…Q3, again, I'll not be promising that we will be publishing ARPOB from different segment and specialties , but definitely by Q4 we will be having this kind of information.

Yatharth Tyagi : Yeah, Amit, just to add on this, so if we talk about broad figures, so roughly I would say the ARPOB, let's say from a cash patient to an insurance patient, for insurance patient would be roughly around 15% lesser than the cas h patient. And, similarly, the government patients are usually 10% -15% lesser than the insurance business. So, I think you can drive at an overall roughly ARPOB from there.

Amit Thawani : Excellent, excellent. I was just looking for that. Thank you. Thank y ou so much.

Moderator : Thank you. The next question is from the line of Dhara Patwa from SMIFS Lim ited. Please go ahead.

Dhara Patwa : Good Morning. First, congratulations on good set of numbers. So, first ques tion is, we will be starting Oncology related r obotic services in our Noida Extension Hospital, so how much difference is there in the prices of traditional surgeries and the robotic ones, first, and do we have pay per use model for this robotic equipment or we will be buying it fully and capitalizing it?

Amit Kumar Si ngh: So, see, if you look at robotic surgery, it comes within premium because overall industry if you see whic hever center has got a robotic surgery, so I won't say it as any specific number because that varies disease to disease, I mean, treatment to treatment what percentage is to be charged. But, I think, roughly it is close to ■50,000 -■1 lakh depending on what sur gery is being performed on the robot specifically, be it a transplant surgery or a general surgery. So, this is the range and the premium every hospital charges. I think our intent is the same.

Yatharth Tyagi: And as far as your question on the pa y per use model or be it paying upfront, so we would be paying upfront for the robot bec ause it's also included in our Capex plans fo r the medical equipment that we have raised from the proceeds of the primary IPO. And actually , that will also help us to negotiate better on the consumabl e that we have to get from them on the price of the consu mables per surgery. So, we will be paying up front for the robot, and we will be negotiating on the pay for surgery consumables.

Dhara Patwa : Okay, thank you. Second question was, cost of consumables has increased considerably in this quarter by 56% and 28% sequentially. So , what was the reason for t hat?

Yatharth Tyagi : So, as we mentioned the overall strategies, we are doing much more super specialties now and treatments like, let's say cancer. So , oncology has a lot of chemotherap y procedures which includes h igh end consumables and drugs as well as expensive medicines. So , this is why when we are moving towards super specialized treatments, is where the cost of these

consumables, especially medicines, are increased in those treatmen ts.
Deepak Kumar Tyagi: Just to add. You know, Dhara what happens is it depends on what kind of specialties do we undertake. Now, consumab le for the quarter is around 21%. Yes, it was 18%, but that's the range it moves, anywhere between 17% to 20%, 21%. So , it's nothing alarming . We no te this, as Mr. Yatharth just mentioned , that

oncology is picking up , so cost of those consumables have gone up. So , this is the reason. But there is nothing to be worried about this cost coming in.
Moderator: The next question is from the line of Prerana Amanna from PNARS Partnership. Please g o ahead.
Prerana : Yeah. Hi Sir, congratulations on a good set of numbers. So , my first question is like, why has the receivable increased drastically?
Deepak Kumar Tyagi : Thanks for this observation. See, a good part of our business comes from the go vernme nt segment, 37 - 38% kind of thing. And off late what is happening is that the government is trying to shift from system. So, because of this change over, there is some confusion there and some processing challenges there. So that' s why there is a payment d elay. And this is a major reason why the receivables have gone up. We are pretty much controlled on the insurance and the corporate side. But since our government sector is a significant business, so that's why the receiva bles hav e gone. We expect that thi s dust should settle down anytime soon.
Amit Kumar Sin gh: This is a very temporary thin g. Earlier the payments were being you know, managed by the UPI, now it's moving towards the NHS. So , I think that's the reason which pr obably you
 know…. but this is jus t temporary thing , for the last quarter and that is for across, wherever whosoever hospital doing know government business is facing this. So , we have nothing to worry about it.
Prerana : Okay, Sir. Sir , my next question is a bit lengthy. First, I wanted to underst and was why are we having a huge payer mix coming from government , if I have to compare it to any other hospitals, that is one. The second is, are we going to see this payer mix change in the coming years, moving more towards the insurance and the self-paye r, since the revenue coming from there will be a bit higher, right? And it can improve our ARPOB also. And since we are having this 37% coming from government, how is the pay cycle here? Because if, let's say a self -payer is paying, you will get t he money, whatever the treatment money is there before he is discharged from the hospital. So , what is the pay cycle here and how it affects our working capital? And the last is on the price hike. I believe when you talk about the self -care, you can take a price hik e again, increasing our ARPOB. Similarly, even with insurance, I believe every hospital has different contracts and according to the contract gets revised, the price hike is better there too. So , what is with the pric e hike here in the government ? Like, wer e you able to take any price hike, let's say, in this year, like in this H1? And if you have taken, what is the difference in price hike between all the three segments?
Yatharth Tyagi: So, coming back to your first ha lf of the question is, first of all, it's important to understand that we do government business on our own discretion and on our own voluntary sa ke. We are not bounded by the government to do these government patients because the land that we have constru cted the hospitals, it's som ething that we have purchas ed ourselves, it is not provided by the government on any subsidized rate. So , we don't have to reserve any beds for the EWS patients from the government. So , the government business that we do is basica lly broadly divided into c ategories like CGHS, ECHS and ESI. And you know, we live in an area, let's say our Greater Noida hospital is just adjacent to one of the Asia's largest ECHS society. ECHS is for army retired people , they have this ECHS card. So , these are the people that c ome to our hospital. These are the people that are living in our community. Just because they ha

ve government card doesn't mean that these patients are sent to us directly by the government. These patients are living across near ou r hospitals and they volu ntarily choose us. Similarly, CGHS, Noida is full of lot of bureaucratic population who are living in our vicinity. So that's why it's there. And on the question front that going forward, yes, right now we have capacity to utilize. So, we don't want to sto p this business because it's ad ding to our overall good business. So, I think going forward, yes, let's say when we have a capacity where we have to pick and choose which category to choose, then we do see cash and insurance segment increasing. And also wi th the coming years , I think our insurance segment is increasing already. So , I think going forward, that's what I see the

 trend there is. As far as the second half of the question, I think for the price revision of the government , I think Mr. Amit can tak e that question.

Amit Kumar Singh : The first part, we don't see such a problem. We're taking the government business as long as it's not impacting us. But coming back to your second question about insurance. Yes, year -on-year the rates we negotiate with th e government every two years. Even this ye ar, we have negotiated, and impact has started coming very recently. Our No ida Hospital is due for a rate revision. I think already process is done, very soon we're going to have that, probably by end of this mont h, we'll have that rate revision. So yes, slowly the gaps are bridging whatever we have , we have whatever the 20% price gap from any big hospital. So, this is going to be year -on-year. You see the rate coming in will be much bette r off in terms of the rate s.

Prerana : Okay. Sir, could you put a number as to what has been t he price hike, let's say for the cash segment, that is the self -payer segment versus insurance versus that. I mean, could you put an exact number? Is it possible?

Amit Kumar Singh : It's dif ficult to remember because see, the negotiations happen on the you know, it is not only as in a blanket percentage, right? It happens with the specialties , procedures, everything. So , it's just not only easy process, it's a very le ngthy process , where we s ee what all the surgeries and procedures are happening there, and accordingly that pe rcentage varies. So, it's very difficult to tell you that one percentage number, but it's a good percentage double digit increase in terms of the SO Cs.

Prerana : Okay, sir. Thank you so much for the detailed explanation, I really appreciate it. My last question is, could you provide us with a guidance for H2 FY24, H2 of this year or you know, FY24, FY25 where you see the top line and the revenue guidan ce basically. And now that we are moving more towards specialty segments, and also, we are increasing our oncology , oncology I believe has gone from 1% to 3%, and even the organ transplant, also the number of organ transplants that we are doing, the surger ies also have increased . So, do we see a margin expansion happening from here? I believe right now our margin expa nsion is 26%. Do we see a further expansion happening because of more specialty? And international, the medical tourism also, I believe we are about to start. So , because of that, are we going to see any changes?

Yatharth Tyagi : As far as your forward -looking guidance, I wouldn't put an exact number, but yes, we do see quarter -on-quarter growth as far as top line is concerned for the coming quart ers and the years. Now moving towards your second question on the expected increase in the margins due to the increase in the sup er specialties. Yes, we do expect when our super specialty increases, just not the margins, b

ut also the ARPOB, so we are expec ting good ARPOB

 increa se from super specialties and especially the transplant programs and the international patients. Our margin s, we expect growth in that because of the existing facilities, the organic growth we are doing because there's still capacity utilization left in al l our, except Noida hospital which is running quite high on utilization, our both Greater Noida and Noida extension and we still have capacity utilized. So that's where we feel margins could increase. And lastly, your question on the international business . I think Mr. Singh can take that.

Amit Kumar Singh : Actually , if you look at it, in the last quarter, if you remember when w e were discussing and also, we just started and then a good number of patients from the Bangladesh and the SAA RC countries were com ing, this quarter also we have made a significant improvement as patients started coming from Fiji, Cambodia, other SAAR C count ries. In fact, to be very honest, we have taken the initiatives for where we are signing agreement with a couple of CIS countries hospitals where our doctors will go over there, and we'll work on their capability and capacity increase and certain surg eries can be performed over there and those complicated surgeries which cannot be done over there can be done in our place. So, I thin k that strategies are in place. Very soon you will see a good number. We are significantly growing in this. I think last y ear we had a couple of crores, last quarte r.

Deepak Kumar Tyagi: The operational part , which Mr. Amit Singh was talking about , the beg inning has been made. The last quarter it was a beginning, we did good numbers , around two crores of business we have done from international. The prospects are looking good because n ow therapists there, the team of doctors and ever y infrastructure is avai lable. The word has spread in the community. So, we expect that this number would grow significantly in Q3 & Q4 and going forward it would be a good contribution to our overall mix.
Prerana : Okay Sir. And Sir, I remember in the la st con call you were plann ing to construct a hospital near some Jewar airport mainly for this only, so you can attract even more medical tourism. Any updates on that?
Yatharth Tyagi : So, what you're talking about is the expansion plan in already existing hospi tal in Greater Noida. So, Great Noida Hospital is currently 400 in bed. We have acquired the adjacent land parcel there to expand it to 600 beds, and that is the existing hospital of ours which is quite close to the Jewar airport. So going forward, that's what those bed s would add on those international tourism and medical tourism lines.
Prerana : Okay, sir. Thank you very much. I really appreciate you all answering my questions in detail. Thank you very much and all the best for the coming quarters.
 Moderator: Thank you. The nex t questio n is from the line of Prashant Nair from Ambit Capital. Please go ahead.
Prashant Nair : Hi, Good Afternoon. Just can you give a ball -park sense of the profitability at your individual hospitals, especially in Noida exte nsion and Jhansi -Orchha, which are relatively new, ind ividually?
Deepak Kumar Tyagi : Prashant , this is again you know, profitability if you ask me , it would be a ball -park kind of thing. I'll not be giving very specific numbers . But if you ask me , it would be in the range of around 2 8-29% at Noida. So, in the range of Greater Noida and little shared lesser in Noida

extension. So , it would be around 25 -26% at No ida extension. J hansi is just marginally profitable ; on EBITDA side I'm talking about . It d id Rs. 90 lakhs of EBITDA at a turn over of around Rs. 8 crores. So, this is how overall things are coming up. This is, again, a plan that this is in line with t hat where we have to publish the information on the segments and the individual hospital profitability. So, that is in the bigger p lan. I owe this to my investors and let's hope that by Q4 we will be having this better numbers .
Prashant Nair : Sure. And secondly, I have a follow up question on the working capital. So, receivables you mentioned there is some transition at the government level which has led to this increase. Where do you expect, say, receivables days to stabilize at? W hat level approximately, once we are past this transition?
Deepak Kumar Tyag i: Because things are still in place and yes, D r. Ajay and Mr. Amit Singh are tr ying their best and pushing it hard to have this figure come down in the Q3 itself. I am very hopeful that if not in Q3, then by Q4, it would be coming down gradually. I expect that our days should be in the range of around 85 to 95, anywhere in between by the end of this year.
Prashant Nair : Okay, thank you. That's it from my side.
Moderator : Thank you. The next question is from the line of Sagar Doshi from Intuit Investment. Please go ahead.
Sagar Doshi : Hello. So, basical ly, I just wanted to ask regardin g the occupancy level. We are at 51% currently, and you are g uided for around 60% by Q4. So , any changes in that? And what can I expect in the next financial year? Just a rough growth or ramp up in the Jhansi Hospital. So , something on the occupancy part?
Yatharth Tyagi : So, I think we are well on track to the expected occupancy that w e have forecasted for the coming quarters, and I don't see any change to those forecasts. And yes, this quarter did have a significant rise in oc cupancy due to also a lot of d engue patients that was spread
 across Delhi NCR. But I feel also going forward in the quarters, we are well on track with respect to our forecast for the expected occupancies. And as far as Jhansi is concerned, already it is r amping up. And as we mentioned earlier in the Q&A, that a lot of empanelment , including the health insurance and gove rnment panels, are well on track. So ,

I think that will be a key attribute to that occupancy ramping up in the coming quarters.
Sagar Doshi : So, any number that you woul d give for next financial year?
Yatharth Tyagi: No, I think not an exact number on that, but we do expect increase in th e occupancy quarter -on-quarter basis.
Saga r Doshi: Okay. Also , one more thing is the average revenue per bed decreased marginally quarte r-on-quarter. So, any specific reason for it or how should we look into it?
Yatharth Tyagi : Yeah, ARPOB is increasing because of change in the case mix. As we were mentioning that we are moving towards a hospital which is doing high on super specialties. S o, as we see a rise in oncology treatments and transplant program so that's the reason why increase is there on quarter -on-quarter. And I do expect this ARPOB increase to continue much further in the coming quarters.
Deepak Kumar Tyagi : Just to add to this , and important would be to see the Q4 movements when we start this oncology treatment. So , what I expect with this PET and radiation line things to come up, revenue wi ll go up to that extent, our ALOS would come down and ARPOB wo uld increase, and that wou ld give the impetus for profitability improvements. But I'm not giving any future numbers, so allow me that. But this is the general trend, and this is a fair match on t

he ground that how it functions .
Sagar Doshi : Okay, got it. L ast one. In one of the que stions , you said that we are looking at doubling the bed capacity in three to four years. So , when can we expect commissioning of the first phase or the first increase of bed, let's say, first 100 -200 beds etc.? So , when can that commissioning be , a rough idea of the timeline?
Yatharth Tyagi : Yeah, so when we say doubling, that also includes the expansion of our existing facilities , that's the Noida Extension and Greater Noida Hospital. So that construction would t ake two to three ye ars. But before that al so, we do expect beds to add up. That will be through the mode of acquisitions of new hospitals that we are looking to do. And I think as far as the plan is concerned for the remaining two quarters, we would be loo king to complete one acquisition of a hosp ital. So , I think before the end of this fiscal year is we do expect our first increase in the capacity to come up through the means of an acquisition of a hospital in the North India region.
 Sagar Doshi : Okay. So , any organic expansio n would take around t wo to three years.
Yatharth Tyagi : Yes, if we are constructing, that's what it takes usually to provide capacity of 200 to 250 beds.
Deepak Kumar Tyagi : Sagar it is basically because it's a Greenfield, it would be about what Mr. Yathar th was alluding it to that the Brownfield or Greenfield will take that much of time. But if it comes by the O&M route or if we take over any existing hospital, that would be quicker. So , if you see Brownfield would be coming in the two to three years' time. So that' s around 45 0 bed we are talking about. The 950 bed would be coming the other way and it can happen quickly.
Sagar Doshi : Thank You. That was helpful.
Moderator : Thank You. The next question is from the line of Vivek Gautam from GS Investments. Please go ahead.
Vivek Gautam : Any concerns on the op erating cash flows and receivables from our side and any plans of reducing dependency on government business, number one. Number two, any acquisition plan and how is like Jhansi -Orchha Hospital and how is t hat performing, sir? Thank you.
Yatharth Tyagi : So as far as, you know, the government b usiness and that is concerned, the receivables, we've already addressed that in our previous questions. Now I'll come to your second question on the acquisition front. Yes, as was mentione d that the plan is that you know, before the end of the fiscal year to complete one acquisition for hospital. We are looking for, you know, acquisition which is roughly in the tune of around 150 beds, at least, somewhat around from 150 beds to 250 beds. Th at is what we're looking for right now. And we're looking at the ter ritories of Delhi NCR, Haryana, Uttar Pradesh, Madhya Pradesh. So, that is, you know, the plan on the acquisition part is concerned and we are open for both O&M model as well as equity pur chase.
Then your question on Jhansi -Orchha is concerned. I think yes that hospital is quite well, as mentioned, we are getting the empanel ment done there and the occupancy has also increased on not just the year -on-year basis, but also quarter -on-quarte r basis. So, I think the Jhansi -Orchha hospital is well on track.
Vivek Ga utam: And how is our catchment area in terms of the location of our hospitals , specially Noida extension, and how is the population growth and what is the occupancy level and how muc h

can it go up to?
 Amit Kumar Singh : Se

e if you look at it, if we come to the Noida extension areas whi ch is the enters into the Ghaziabad region. So, and it's an excellent connectivity through the Meerut Delhi highway, this goes to, you know, the Meerut , Modi Nagar, Hapur those regions to entire Ghaziabad, that is our good catchment area and Noida extensio n itself is now getting densely populated because there's lots of you know, the bigger sites are coming. So, this is in a way if you lo ok at it, Gautam B uddha Nagar , if you have any idea about this region, this is an area where, you know, it's coming up ver y, very fast. So, I think the coming years, you'll see a good number of people moving from those areas, Delhi and other places to this s ide of it. So, t hat is the Noida extension and then I think earlier also we have spoken about our advantage as far as ge ography is concerned. So Greater Noida has an excellent connectivity towards, you know, till Agra , Mathura, those regions, you know, tha t's one region b ecause of the Yamuna Expressway and Noida has an edge of Delhi and Faridabad region, so I think we hav e a geographical advantage and a good catchment area.

Vivek Gautam : What about competition intensity with coming up of the new Yashoda Hos pital and the bi g units coming up and next purchase and other thing and how are we planning to cope up with that? Thank you.

Amit Kumar Singh : See if yo u look at it, I think, the competition about any areas is growing and having a potential competition is bound to come, b ut I think we are not scared of those things. In fact, that could be a good thing that this area will become a hub for good healthcare. So, we have our own advantages, we have our own strengths, and being the oldest and having the range of services what we are providing is, I think, as of now others whosoever will come will take some time, till that time we will be very much placed as in biggest in this region. So, I think that is not our concern, we are focusing on our own strengths. You kn ow, the specialt y, super specialt y which we are still not having, we are working on to have those things. So, nothing to worry about it as such.

Vivek Ga utam : And s ir, how is the opportunity size and which will increase usage of medical insurance. it augur s well for us ve rsus the self -paying patients.

Amit Kumar Singh : Can you kindly repeat that question your voice is cracking.

Vivek Gautam : Yeah, my question is about the opportunity size for us, in terms of basically, due to the COVID and the lack of medic al insurance, pe ople were suffering a lot and as we say that, between basically if you stay in ICU for 15 -20 days, you come from middle class, you can go to the lower class if you don't have the ins urance. So, how is the insurance penetration level happeni ng as per your e xperience and how does that augur well for our future?
 Amit Kumar Singh : See if you look at it the good part about COVID and ye s people got worried about their healthcare. And that's what did you see the data shows that the last three years if you see the kind of the insurance penetration has gone. The insurance companies growing very fast. And the more and more tier two and tier three cities' people are having their insurance. So, I think, that's a by increasing this perc entage any healthca re facility will benefit because people will demand, you know, the quality healthcare and that is they will move from those agents to and also very important quality health care. And so, we are on the same line and I'm sure not only we , other healthcare ser vices also goin g to benefit of it.

Vivek Gautam : Thank you.

Mod erator : Thank you. The next question is from the line of Naman Bhansali, from Perpetuity V

entures LLP. Please go ahead.

Naman B hansa li: Thank you for the opportunity. First question is in Noida hospital, thei r occupancy are at 96% this quarter. Is it on the 450 beds capacity, which we have or is it on some other number because we haven't heard such level of occupancy in any other hospitals case.

Yatharth Tyagi : Yeah, so I think, you know, you've got that point , correct. And that occupancy is not on 450 but on smaller beds of 250 bed. So that's why, you know, we're able to maintain an

occupancy of 90% above in that hospital , and you know, for a hospital of, let's say, 450
beds or beyond that, it w ill be difficul t to reach that occupancy, so Greate r Noida hospital is
around 250 beds.
Naman Bhansali : But in optimally any hospital can do at a max of what range?
Yatharth Tyagi : So, I think depends on hospital to hospitals, but let's say, for our hospit als of let's sa y
around 400 beds or 450 beds that is Greater Noida extension. So, Greate r Noida is touching
around 70% out there. We do feel that we can still cater to a much more occupancy to
around I would say somewhere between 75 to 80%. We would consid er booked an
optimum utilization there.
Naman Bhans ali: Got it. Thank you. Second question is you had men tioned that Brownfield take 2 years to
build and greenfield around 3 -4 yea rs. Sir, how is the breakeven time period in such
functions, in brownfield vs a greenfield?
Deepak Kumar Tyagi : The breakeven for the Brownfield would not be an issue because first of all, you know, the
second cost of these hospitals, many kind of thing so, the base of the cost will be less
breakeven should come, I'm talking ab out the overall fac ilities. If we talk about overall
facility because we are still having a margin of around 28 -29% coming in from Greater
 Noida . So, you know, they would not be a performance down for the hospital per se , but
when it comes down to the you k now the setup bein g created that the 200 or 250 beds
which are coming because good lot of its cost , the fixed cost will be shared by so I expect
that on an individual basis , the payback should be anywhere between three and kind of
thing.
Naman Bhansa li: Okay , I understood. Lastly, you had mentioned that your gross margin had fall due to the
increase in consumer expenses for new high -end specialty which you're doing . So just want
to understand that in terms of ARPOB, why is it not reflecting that if you are do ing some
high-end specialty in a particular quarter , the ARPOB would have moved up on Q oQ basis
till date. So, marginally the ARPOB would decline by 2% on Q oQ basis. So, can you just
help me understand that when does the A RPOB eventually fade out in terms o f these high -
end specialties?
Deepak Kumar Tyagi : First of all, you know there is a slight increase in the you know, in the percentage term the
consumables cost around 21% , which is around 18 to 19% in the last quarter. Important is
that it did not retain t he EBITDA. In fact, the EDITDA on Q oQ basis has gone up. The
numbers I'm not sure it is there, but it has gone up from Q1 to Q2. So, this is there. So first
of all, I want to dispel that EBITDA has come down because of the increase in the cost.
What was th e second quest ion, sorry I didn't write it.
Yatharth Tyagi: Coming to your question on the part that you know even the high specialty where we are
moving , Jhansi is having a lower ARPOB even with high specialty becau se in Jhansi
Orchha hospital the prices are also signi ficantly lesser compared to the Delhi NCR. So yes,
we are moving towards you know basic specialties there but then also the ARPOB will be
always less in that territory due to the pricing strategy that we have taken in the Orc

hha
hospital.
Nam an Bhansal i: Got it understood and on a big picture basis , over the next three to five years , the ARPOB
can grow at what range.
Yatharth Tyagi : We do expect the ARPOB to grow quarter -to-quarter. I wouldn't give exact figure on that,
and you know we are well on track for that.
Naman Bhansali : Thank you.
Moderator : Thank you. The next question is from the line of Prashant Nair from Ambit Capital. Please
go ahead.
Prashant Nair : Hello, my question has been answered. Thanks.
 Moderator : Thank you. The next questio n is from th e line of Amit Thawani, who's an
individual
investor. Ple ase go ahead.
Amit Tha wani : Hi, yeah, just a follow up question. We are almost halfway through the December quarter.
Can you give us the occupancy of Jhansi and Noida extension for the hal f quarter?

Yatharth Tyagi : For the half quarter, yes, I think that's what we have included in our ea rnings presentation. But, you know, as far as the quarter occupancy is there, it is available there.
Amit Thawani : No, no, this quarter, this December quart er?
Deepak Kumar Tyagi : Amit, you know, taking occupancy, see first of all, yes, we are publishing, we will continue to publish occupanc y on a quarterly basis. But the occupancy has to be seen on a longer term. Now, we are almost a month down into this qua rter. First of all, the occupancy levels are almost same as it was in the month of September this is one thing, but giving a number specific that what that occupancy would be it will be too early to know, call. I request if you can wait for the quarter to end process matter.
Amit Thawani : But just to get a directional sense , we are highe r?
Deepak Kumar Tyagi : We are pretty much there. The occupancy levels continue to be good. So, that's the subjective answer I can give.
Yatharth Tyagi : Also, just to add on this, tradi tionally, but you know, we do expect occupancy to remain you know, in the positive direction, but traditionally in Q3, usually in Delhi NCR occupancy tends to be always lesser given to that the winter is coming up and you know, due to the fog in creasing. S o, both are at OPD level and both at IPD level, you see this is considering the Delhi NCR , the coolest period. But, you know, we should be on our track even afte r that.
Amit Thawani : Exactly, that's why I was asking because Q2 and Q4 are generall y the best quarters and Q3 is not really the best quarter. So that's why I was asking can you still expect an improved occupancy?
Yatharth Tyagi : So, I mean, I will say, for, let's say, things like OPD o r elective surgeries, these tend to be lesser in Q3, and as far as occupancy is concerned, I think overall, it should be on the similar track.
Amit Thawani : Okay, thank you, sir. Thank you.
 Moderator : Thank you. The next question is from the line of Sumit Gupta from Motilal Oswal. Please go ahead.
Sumit Gupt a: Yeah. S o, I have two questions, the first one the margin front. So, if it is possible, can you quantify how much margin do we expect over the next three to fou r years? And the second point is the incremental growth in the overall EBITDA, which hospitals or which facilities would likely contribute more from here on and the third question is regarding how much Capex we plan to deploy over the [inaudible].
Deepak Kumar Tyagi : So first of all, you know, I will dismiss that, you know, what kind of margins we'll be having in the years to come. I'm not giving any futuristic direction. Very subjective answer we can give

, we are at 26% plus EBITDA and these EBITDA margins are expected to grow, which in itiatives of Mr. Yatharth and Mr. Amit just mentioned, it's basically the induction of international business coming in, oncology coming in, more and more higher specialties are coming in. So, these would add to two things. The ARPOB would increase and ARP OB would significantly then drive it, for the margins to increase. But if you ask me, yes, we have our internal plans, but I'm not supposed to you know, disclose at this point of time to the investor community , sorry for that. So, this is one thing.
Now sec ond question was about our Cap ex plan. For the next three years, we ha ve a Cap ex plans of around 800 C r coming in, which is basically for the addition of these 1 ,400 more 1,450 beds coming in. This plan may, go here and there depending on if more beds come on the O&M side , then the CapEx would be lesser. But if it goes on mo re on a greenfield side, it would be higher but at this point of time, we have this kind of thing. So, that is the case, and we are sufficiently funded for this expansion. We don't expect a long -term basis, we would need any loan capital to fund these proj ects.
Sumit Gupta : The follow up question was on the incremental growth of the EBITDA. So which facility would contribute more towards it?
Deepak Kumar Ty agi: Well, all the facilities are contributing but if you see the AR POB, the ARPOB would settle, then I wou ld say that Noida extension is to be seen. Did it answer you?
Sumit Gupta : Okay, thank you.
Moderator : Thank you. The next question is from the line of Prashant Nair from Ambit Capital. Please go ahead.

Corporate Office: HO -01, Sector - 01, Greater Noida West, 201306
CIN: L85110DL2008PLC174706 Prashant Nair : I just had a question on the tax rate. So, in the first half the tax rate is higher than what it
was in fiscal 23. So, is this the level we should look at for the full year and going forward or
will it settle down?
Deepak Kumar Tyagi : No, you know, Prashant what happens that you know, mid way, because auditors generally
tend to be a little conservative. I would say that you know, because we will be best of the
tax position coming in from our two companies, that is Yatharth hospital having Noida and
Greater Noida hospital and AKS which is ba sically Operating Noida extension hospital ,
there, the tax rate, because the MAT and everything has been utilized , so, it will be better
than the last year, I expect that the tax rate will be in the range from 25% for these two
hospitals. As far as the Jha nsi is concerned, Jhansi is because it is h aving the accumulated
losses. So, we will go for the traditional tax percentage. Overall, you know, this is basically
because by March '24 the position would be clear, I expect the overall tax burden should
come do wn in the percentage terms . There on 30%, i f I'm not mistaken, for the first half,
you know, just a minute. I can check the numbers is around 33% , 30% plus kind of thing at
this point of time, I expect that it would be in the range of around 26 to 28% anywhere in
between.
Prashant Nair : Yeah, thank you.
Moderator : Thank you. The next questi on is from the line of Tanmay from Mirae Asset. Please go
ahead.
Tanma y: One questio n on the Noida Extension hospital. If I look at the average revenue per patient,
I'm just dividing the overall top line might no t be the right calculation, but dividing the
overall top line by the inpatient revenue that we have. It has gone up significantl y, let's say
from H1 FY 23 to H1 FY 24. What could be the reason? Is it purely case mix or any
particular reason , that you would want to highlight? I think it's lower than the other
hospitals in the Noida extension part. Yeah.
Yatharth Ty agi: Yeah. Sir, just yeah. So, the reas

on for increase is case mix is there. And it is also not just
the Case mix. There's a lot of new treatments that we have started here which are high end
value, which we were not doing earlier. So, you know, procedures like kidney transpl ant
and bone marrow transplant, you know, we recently started around a year back. So, this is
why the sharp increase is there. And as far as other specialties are concerned, these are also
growing much more in the proportion to specialties which are at a l ower end of the ARPOB
spectrum. So, that is why it's there. And also, this is a hospital where we are getting the
oncology mach ine, which is already reached and will start functioning from January
onwar ds. And robots are also being planned in this hospital . So, this is the reason why the

Corporate Office: HO -01, Sector - 01, Greater Noida West, 201306
CIN: L85110DL2008PLC174706 ARPOB has increased as well, and we do expect much more increase quarter on quarter at the ARP OB of this hospital.
Tanmay : So, this specialty is also increasing , I mean the ALOS nu mber looks lower than the other
hospital, t he other hospitals that we have. So, that is the right trend to look at or how should
we look at it I mean.
Yatharth Tyagi : So, I think lower , so actually lower ALOS sort of represents a higher share of super
specialties. So, let' s say if 3 to 4 days of AL OS is there for super specialties. And if there
are more patients of general specialties, let's say for internal medicine or oth er procedures,
that's where the ALOS tends to be higher.
Tanmay: Okay, thanks .
Moderator : Thank you, ladies and gentlemen. Due t o time c onstraints, that was the last question. I now
hand over the call to the management for the closing comments. Please go a head, sir.
Yatharth Tyagi : Yeah, I think I would just like to thank all the participants who attended our earnings call,
and we look forward to deliverin g on the numbers and the promises that we have kept. And
we are well on track to, you know, do that in the coming quarters. Thank you, everybody,
again for participating. Thank you.
Moderator : Thank you on behalf of Prabhudas Lilla dher. That concludes this conference. Thank you for
joining us. And you may now disconnect your lines.
(This document has been edited for readability purpose)
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Call: Nov 2023

Corporate Office: HO -01, Sector - 01, Greater Noida West, 201306
CIN: L85110DL2008PLC174706

"Yatharth Hospitals and Trauma Ca re Services Ltd
Q2 & H1 FY2024 Earnings Conference Call "

November 8, 2023

MANAGEMENT : MR. YATHARTH TYAGI – WHOLE TIME DIRECTOR
MR. AMIT KUMAR SINGH – GROUP CEO
CA PANKAJ PRABHAKAR – CHIEF FINANCIAL OFFICER
MR. DEEPAK KUMAR TYAGI – PRESIDENT , STRA TEGY
& FINAN CE

MODERATOR : MR. PARAM DESAI – PRABHUDAS LILLADHER

Corporate Office: HO -01, Sector - 01, Greater Noida West, 201306
CIN: L85110DL2008PLC174706 Moderator: Ladies and gentlemen, good day and welcome to Yatharth Hospital's Q2FY24 Earnings
Conference Call hosted by Prabhudas Lilladher. As a reminder, all participant lines will be in the listen only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you n eed assistance during the conference call, please signal operator by pr essing * then 0 on your touc htone phone. Please note that this conference is being recorded. I now hand the conference over to Mr. Param from Prabhudas Lilladher. Thank you and over to you, Mr. Param.

Param Desai : Yeah. Thank you Malcoln, and good morning everyone. On behalf of Prabhudas Lillad her, we welcome you to the Q2FY24 Earnings Conference Call of Yatharth Hospital & Trauma Care Services Limited. From the management , we have w ith us today , Mr. Yatharth Tyagi , Whole -time Director, Mr. Amit Singh , Grou p CEO, Mr. Deepak Tyagi , President Strat egy & Finance and Mr. Pankaj , CFO.

I now hand over the call to Yatharth for his opening comments and to take it forward. Over to you, Yat harth.

Yatharth Tyagi: Hi, Good Morning. This side Yatharth Tyagi , Whole -time Director at Yatharth Hospital & Trauma Care Servic es Limited. A very warm welcome to you all for the Earnings Conference Call for the quarter and half year ended 30th September 2023.

I have with me Mr. Ami t Kumar Singh, our Group CEO, Mr. Pankaj Prabhaka r, our CFO, and Mr. Deepa k Kumar Tyagi, our President of Strategy & Finance. Let me draw your attention to the fact that on this call our discussions will include certain forward -looking statements which are predictions, projections or other estimates abou t future events. These estimates reflect management's current expectations about future performance of the company. Please note that these estimates involve several risks and unce rtainties that could cause our actual results to differ materially from what is expressed or implied.

We have uploaded our presentation on the exchange and the company website , and I hope you all might have received and had an opportunity to go through i t.
Since this is our second earnings call post our listing on the stock exch anges , I would like to give a brief background on our company quickly. Yatharth Hospital operates super specialty hospitals in the national capital region of Delhi and Jhansi -Orch ha region of Madhya Pradesh. We are among the top 10 largest private hospital s in Delhi -NCR region in terms of number of beds. We started our journey in 2008 with the first hospital in Greater Noida and since then expanded our presence with hospitals in N oida and Noida Extension. In 2022 , we acquired our fourth hospital in Jhansi -Orchha in Madhya Pradesh with the aim of expanding

into new geographies. Our total bed capacity presently stands at 1405 beds , including 394 critical care beds.
I am pleased to an nounce that we have delivered a stellar performance thi

s quarter. Our
revenue has experienced a remarkable 34% year -over-year growth , reaching ■1 ,713 million , while our Profits after Tax have surged by an impressive 70% compared to the same period last year and 45% compared to the immediately preceding quarter.
We have successfully co mpleted our IPO during the month of August and subse quently your company has received the primary proceeds of ■610 crores which has boosted our capital base and helped us turn Net Debt free during the quarter.
Our ongoing commitment revolves around divers ifying a range of medical specialties and introducin g new ones across all our hospitals. Q2 witnessed a double -digit growth across most of our specialties. Our Nephrology & urology , and Neuroscience departments have grown by 40 -45% year -on-year. Orthoped ics and Cardiology have grown by 20 -25% year -on-year. While our Gastroenterology, Pulmonology and Oncology department revenue has more than doubled compared to the last year.
Our commitment continues to remain towards enhancing our suite of Oncology ser vices and create a comprehensive one -stop destination f or all cancer related treatments. The Radiation Oncology machines has arrived at our Noida Extension hospital, and we expect it to be commissioned by January 2024 , seamlessly integrating the Radiation O ncology line within our Oncology Center. We are in the process of ordering equipment for robotic surgeries which is expected to arrive in the coming quarter. Thus, we are well on track to offer robotic service by the coming quarter and a comprehensive suit e of Oncology treatm ents by the fourth quarter of this fiscal year.
As we are progressing, we have concluded around 100 kidney transplants so far and we are constantly expanding our organ transplant and medical tourism business which is poised to drive substantial growth and contribute to an improvement in average revenue per occupied bed in the upcoming quarters. We are continuously evaluating opportunities, both organic inorganic, and strategic partnerships and remain committed to expand our bed capaci ty in the coming years . We aim to double our bed capaci ty over the next 3 -4 years via mix of both greenfield and brownfield expansion. We understand that inorganic growth is not merely about expansion but also about identifying synergistic opportunities th at amplify our strengt hs, extend our reach and ultimate ly enable us to serve more patients with exceptional care.

At our current capacity, we have seen a remarkable improvement in our utilization. So, the Noida and Greater Noida hospital report ed highest occupancy levels of 96% and 73% respectively during the quarter. Our other hospital at Noida Extension has received an occupancy level of 45% during quarter compared to 28% in Q2 FY23, while our Jhansi Orchha Hospital is now at 20% compared to 5% in Q2FY23. In line with our Noida and Great Noida Hospital, we ex pect our Noida Extension hospital to reach optimum utilization level s by FY2025. Thus, we have already acquired a land parcel adjacent to our Great Noida hospital and declared L1 bidder for a land adjacent t o our Noida Extension hospital , which should supp ort our organic expansion plans in these hospitals.
Ladies and gentlemen, regarding the recent income tax search operation at our company from 19th-22nd October 2023, I would like to address you that the s earch operation yielded

no significant preliminary findings of unaccounted cash or incriminating transactions. The company is duly replying to the queries of the tax authorities and does not foresee any impact on the operating and financial performance of the company in the coming quarters. I would like to now hand over the

call to our Chief Financial Officer, Mr. Pankaj Prabhakar, for the financial update for the quarter.

Pankaj Prabhakar : Good Morning everyone. I am happy to announce tha t your company has reported another quarter of strong performance. We have recorded our highest ever qua rterly revenue at ■1,713 million , reporting to a robust growth of 34% year -over-year and 11% quarter -over-quarter , with growth across specialties and improvement in occupancies across our hospitals. Our Inpatient revenue have grown by 37% year -over-year du ring the quarter. Our EBITDA for the quarter was ■456 million, up by 36% year -over-year and 10% quarter -over-quarter. Our EBITDA margins for the quarter expanded by 40 basis points to 26.6% , with improved operating leverage and efficiencies across all our hospitals.

A significant development in this quarter is that Yatharth Hospital has become Net Debt free as of September'2023 -end, by utilizing around ■2 ,450 million proceeds raised during the IPO to repay majority of our existing debt burden . For our comp any, maintaining a debt-free status is not just a goal but it is a core part of our financial strategy and philosophy. Operating without the burden of debt will allow us to focus on sustainable growth, innovation and long -term success. In line with our deb t reduction, our finance cost has reduced by 42% year -over-year and by 50% quarter -over-quarter to ■29 million during the quarter. Full impact of debt reduction on finance cost will be further visible during Q3. Consequently, we have been able to grow our profits considerably by 70% year -over-year and 45% quarter -over-quarter to ■276 million during the quarter.

 As far H1 performance is considered, our revenue has grown by 36% year -over-year to ■3,258 million. Our EBITDA for the half year was ■870 millio n, up by 47% year -over-year, while our EBITDA margin for the half year expanded by 192 basi s points to 26.7%. Our Profit after Tax for the half year were up by 71% Y oY to ■466 million.

Our Balance Sheet has strengthened significantly during the quarter with pr imarily capital infusion of ■6 ,100 million from the IPO. Our net worth has increased to ■8,065 million by September -end compared to ■1 ,830 million as of March'2023 -end. As on September -end, your company stands net debt free , with net cash position of ■3 ,109 million , which we strive to invest towards our ongoing and planned organic and inorg anic expansion as stipulated in our IPO objectives.

Overall, our recent quarter performance is a testament of our balanced growth approach. I would like to emphas ize on t he fact that while our focus remains to multifold our revenue and EBITDA in the comin g quarters and years, with the de leveraging of our Balance Sheet , our net profitability and overall Balance Sheet will continue to emerge significantly strong in the up coming quarters.

With this, I would like to hand over to Moderator for Question and Ans wers. Thank you.

Moderator: Thank you very much. We will now begin the Question -and-Answer session. Anyone who wishes to ask a question may press * and 1 on their touch stone telephone. If you wish to remove yourself from the question queue, you may press * and 2. Participants are requested to use handsets while asking a question. Ladies and gentlemen, we will wait for a moment while the question queue assembles.

The fir st question is from the line of Amit Thawani, who's an individual investor. Please go ahead.

Amit Thawani : Hi Everybody and congratulations on a fantastic set of numbers. My first question is, last full year , the government business was 37% of our total r evenue, can you tell me what that number is for H1 and for Q2?

Deepak

Kumar Tya gi: Well, Amit, thanks for the question. You know, government business is significant to us, and it is in the range of around 36 -37% and still going. So, we are maintaini ng the same thing. Good part is th at the growth is coming from all the sectors. You know, be it from the corporate sector , or be it from the insurance or the government business. But the best part is that, you know, as Mr. CFO has mentioned that we are goin g for a balanced growth and so our revenue is coming from all the quarters of the company.

CIN: L85110DL2008PLC174706 Amit Thawani : Okay -okay, thank you. Coming back to this same question, can you divide the government
business? Just broad percentages as to how much is coming from C GHS, how much is
coming from PM Jan Arogya an d how much is maybe coming from ESIC? Are there so me
broad numbers there?
Deepak Kumar Tyagi : No, we don't have any business coming from PM Jan Arogya. You know, very small
amount it comes at Jhansi , but that's very, very negligible when it comes under the
percentage terms. Frankly speaking, I haven't di stinguished that what is coming from CGHS
or ESI C. If it is so important, we'll get back to you.
Amit Thawani : Okay. Sorry, I'm a little bit of an amateur in this and so I'm just going to ask very basic
question. So, the budget that the govern ment allocate s for CGHS and ESIC, are they one
combined budget or is it a separate budget for the three? And can you give us like what has
been the growth in the allocation of this expenses over last year by the budget? Do you have
those numbers by any cha nce?
Amit Kum ar Singh : If you look at ECHS, CGHS and ESI, all those have a separate budget. So, however it is all
controlled by the government authorities. All three have a separ ate entity who takes care of
it. So, I think, in last couple of years as such there's no is sues in terms of, you know,
releasing the funds. So, the frequency they're maintaining that is good.
Amit Thawani : Okay -okay. Secondly, I mean, we were in the proces s of getting approvals or empanel ment
rather of our Jhansi Hospital, can you give us the pr ogress there? What is th e status of
empanelment there?
Amit Kumar S ingh : Jhansi, if you look at it, the team is working very hard in terms of getting the empanelment .
First is the insurance sector, so whatever the insurance and TPA operating in India as of
now, I think 60 -70% already it has gotten e mpaneled now. The focus is to do couple of
PSUs like BHEL, Railways and others which is having a significant number of p opulations
residing over there. So, already, I mean, we have reached on the last leg of the
confirmations from those authorities. I thin k next quarter you will see those numbers start
coming. Rest others, the corporate s, whichever corporates are available, our team is
working there. I think a couple of them already we got it and it's in the proce ss and next
month or something, I think, we'l l have a significant number of those empanelment s.
Amit Thaw ani: Okay. So, basically, we have received 70% of the empa nelment that we had intended to
take?

CIN: L85110DL2008PLC174706 Amit Kumar Singh : No, approximately, I would say that i nsurance cycle side , it's around 30 -32 insurance
companies , private a nd GICs working/operational. So, I think more than 50% -60% we have
received and in the coming mo nths I think more is going to come.
Amit Thawani : Okay -okay, got it. Our ARPOB is roughly ■ 28,000, can you divide that ARPOB into how
much is government ARPOB and how much is non -government ARPOB?
Deepak Kuma

r T yagi: So, you know, so far, we have not disti nguished. I told you that, you know, we are putti ng
up the system. In the last call also you were there , I distinctly remember, and I had
mentioned that we are putting up the system where we will be putting up ARPOB not only
from the business segment but b y the specialties also. But at this point in time , because this
is mid of the year, so this will take som e time. I am sure that by Q4 kind of thing…Q3,
again, I'll not be promising that we will be publishing ARPOB from different segment and
specialties , but definitely by Q4 we will be having this kind of information.
Yatharth Tyagi : Yeah, Amit, just to add on this, so if we talk about broad figures, so roughly I would say the
ARPOB, let's say from a cash patient to an insurance patient, for insurance patient would be
roughly around 15% lesser than the cas h patient. And, similarly, the government patients
are usually 10% -15% lesser than the insurance business. So, I think you can drive at an
overall roughly ARPOB from there.
Amit Thawani : Excellent, excellent. I was just looking for that. Thank you. Thank y ou so much.
Moderator : Thank you. The next question is from the line of Dhara Patwa from SMIFS Lim ited. Please
go ahead.
Dhara Patwa : Good Morning. First, congratulations on good set of numbers. So, first ques tion is, we will
be starting Oncology related r obotic services in our Noida Extension Hospital, so how much

difference is there in the prices of traditional surgeries and the robotic ones, first, and do we
have pay per use model for this robotic equipment or we will be buying it fully and
capitalizing it?
Amit Kumar Si ngh: So, see, if you look at robotic surgery, it comes within premium because overall industry if
you see whic hever center has got a robotic surgery, so I won't say it as any specific number
because that varies disease to disease, I mean, treatment to treatment what percentage is to
be charged. But, I think, roughly it is close to ■50,000 -■1 lakh depending on what sur gery is
being performed on the robot specifically, be it a transplant surgery or a general surgery.
So, this is the range and the premium every hospital charges. I think our intent is the same.
 Yatharth Tyagi: And as far as your question on the pa y per use model or be it paying
upfront, so we would
be paying upfront for the robot bec ause it's also included in our Capex plans fo r the medical
equipment that we have raised from the proceeds of the primary IPO. And actually , that will
also help us to negotiate better on the consumabl e that we have to get from them on the
price of the consu mables per surgery. So, we will be paying up front for the robot, and we
will be negotiating on the pay for surgery consumables.
Dhara Patwa : Okay, thank you. Second question was, cost of consumables has increased considerably in
this quarter by 56% and 28% sequentially. So , what was the reason for t hat?
Yatharth Tyagi : So, as we mentioned the overall strategies, we are doing much more super specialties now
and treatments like, let's say cancer. So , oncology has a lot of chemotherap y procedures
which includes h igh end consumables and drugs as well as expensive medicines. So , this is
why when we are moving towards super specialized treatments, is where the cost of these
consumables, especially medicines, are increased in those treatmen ts.
Deepak Kumar Tyagi: Just to add. You know, Dhara what happens is it depends on what kind of specialties do we
undertake. Now, consumab le for the quarter is around 21%. Yes, it was 18%, but that's the
range it moves, anywhere between 17% to 20%, 21%. So , it's nothing alarming . We no te
this, as Mr. Yatharth just mentioned , that

oncology is picking up , so cost of those
consumables have gone up. So , this is the reason. But there is nothing to be worried about
this cost coming in.
Moderator: The next question is from the line of Prerana Amanna from PNARS Partnership. Please g o
ahead.
Prerana : Yeah. Hi Sir, congratulations on a good set of numbers. So , my first question is like, why
has the receivable increased drastically?
Deepak Kumar Tyagi : Thanks for this observation. See, a good part of our business comes from the go vernme nt
segment, 37 - 38% kind of thing. And off late what is happening is that the government is
trying to shift from system. So, because of this change over, there is some confusion there
and some processing challenges there. So that' s why there is a payment d elay. And this is a
major reason why the receivables have gone up. We are pretty much controlled on the
insurance and the corporate side. But since our government sector is a significant business,
so that's why the receiva bles hav e gone. We expect that thi s dust should settle down
anytime soon.
Amit Kumar Sin gh: This is a very temporary thin g. Earlier the payments were being you know, managed by the
UPI, now it's moving towards the NHS. So , I think that's the reason which pr obably you
 know…. but this is jus t temporary thing , for the last quarter and that is for across, wherever
whosoever hospital doing know government business is facing this. So , we have nothing to
worry about it.
Prerana : Okay, Sir. Sir , my next question is a bit lengthy. First, I wanted to underst and was why are
we having a huge payer mix coming from government , if I have to compare it to any other
hospitals, that is one. The second is, are we going to see this payer mix change in the
coming years, moving more towards the insurance and the self-paye r, since the revenue
coming from there will be a bit higher, right? And it can improve our ARPOB also. And
since we are having this 37% coming from government, how is the pay cycle here? Because
if, let's say a self -payer is paying, you will get t he money, whatever the treatment money is

there before he is discharged from the hospital. So , what is the pay cycle here and how it affects our working capital? And the last is on the price hike. I believe when you talk about the self -care, you can take a price hik e again, increasing our ARPOB. Similarly, even with insurance, I believe every hospital has different contracts and according to the contract gets revised, the price hike is better there too. So , what is with the pric e hike here in the government ? Like, wer e you able to take any price hike, let's say, in this year, like in this H1? And if you have taken, what is the difference in price hike between all the three segments?

Yatharth Tyagi: So, coming back to your first ha lf of the question is, first of all, it's important to understand that we do government business on our own discretion and on our own voluntary sa ke. We are not bounded by the government to do these government patients because the land that we have constru cted the hospitals, it's som ething that we have purchas ed ourselves, it is not provided by the government on any subsidized rate. So , we don't have to reserve any beds for the EWS patients from the government. So , the government business that we do is basica lly broadly divided into c ategories like CGHS, ECHS and ESI. And you know, we live in an area, let's say our Greater Noida hospital is just adjacent to one of the Asia's largest ECHS society. ECHS is for army retired people , they have this ECHS card. So , these are the people that c ome to our hospital. These are the people that are living in our community. Just because they ha

ve government card doesn't mean that these patients are sent to us directly by the government. These patients are living across near ou r hospitals and they volu ntarily choose us. Similarly, CGHS, Noida is full of lot of bureaucratic population who are living in our vicinity. So that's why it's there. And on the question front that going forward, yes, right now we have capacity to utilize. So, we don't want to sto p this business because it's ad ding to our overall good business. So, I think going forward, yes, let's say when we have a capacity where we have to pick and choose which category to choose, then we do see cash and insurance segment increasing. And also wi th the coming years , I think our insurance segment is increasing already. So , I think going forward, that's what I see the

 trend there is. As far as the second half of the question, I think for the price revision of the government , I think Mr. Amit can tak e that question.

Amit Kumar Singh : The first part, we don't see such a problem. We're taking the government business as long as it's not impacting us. But coming back to your second question about insurance. Yes, year -on-year the rates we negotiate with th e government every two years. Even this ye ar, we have negotiated, and impact has started coming very recently. Our No ida Hospital is due for a rate revision. I think already process is done, very soon we're going to have that, probably by end of this mont h, we'll have that rate revision. So yes, slowly the gaps are bridging whatever we have , we have whatever the 20% price gap from any big hospital. So, this is going to be year -on-year. You see the rate coming in will be much bette r off in terms of the rate s.

Prerana : Okay. Sir, could you put a number as to what has been t he price hike, let's say for the cash segment, that is the self -payer segment versus insurance versus that. I mean, could you put an exact number? Is it possible?

Amit Kumar Singh : It's dif ficult to remember because see, the negotiations happen on the you know, it is not only as in a blanket percentage, right? It happens with the specialties , procedures, everything. So , it's just not only easy process, it's a very le ngthy process , where we s ee what all the surgeries and procedures are happening there, and accordingly that pe rcentage varies. So, it's very difficult to tell you that one percentage number, but it's a good percentage double digit increase in terms of the SO Cs.

Prerana : Okay, sir. Thank you so much for the detailed explanation, I really appreciate it. My last question is, could you provide us with a guidance for H2 FY24, H2 of this year or you know, FY24, FY25 where you see the top line and the revenue guidan ce basically. And now that we are moving more towards specialty segments, and also, we are increasing our oncology , oncology I believe has gone from 1% to 3%, and even the organ transplant, also the number of organ transplants that we are doing, the surger ies also have increased . So, do we see a margin expansion happening from here? I believe right now our margin expa nsion is 26%. Do we see a further expansion happening because of more specialty? And international, the medical tourism also, I believe we are about to start. So , because of that, are we going to see any changes?

Yatharth Tyagi : As far as your forward -looking guidance, I wouldn't put an exact number, but yes, we do see quarter -on-quarter growth as far as top line is concerned for the coming quart ers and the years. Now moving towards your second question on the expected increase in the margins due to the increase in the sup er specialties. Yes, we do expect when our super specialty increases, just not the margins, b

ut also the ARPOB, so we are expec ting good ARPOB

 increa se from super specialties and especially the transplant programs and the international patients. Our margin s, we expect growth in that because of the existing facilities, the organic growth we are doing because there's still capacity utilization left in al l our, except Noida hospital which is running quite high on utilization, our both Greater Noida and Noida extension and we still have capacity utilized. So that's where we feel margins could increase. And lastly, your question on the international business . I think Mr. Singh can take that.

Amit Kumar Singh : Actually , if you look at it, in the last quarter, if you remember when w e were discussing and also, we just started and then a good number of patients from the Bangladesh and the SAA RC countries were com ing, this quarter also we have made a significant improvement as patients started coming from Fiji, Cambodia, other SAAR C count ries. In fact, to be very honest, we have taken the initiatives for where we are signing agreement with a couple of CIS countries hospitals where our doctors will go over there, and we'll work on their capability and capacity increase and certain surg eries can be performed over there and those complicated surgeries which cannot be done over there can be done in our place. So, I thin k that strategies are in place. Very soon you will see a good number. We are significantly growing in this. I think last y ear we had a couple of crores, last quarte r.

Deepak Kumar Tyagi: The operational part , which Mr. Amit Singh was talking about , the beg inning has been made. The last quarter it was a beginning, we did good numbers , around two crores of business we have done from international. The prospects are looking good because n ow therapists there, the team of doctors and ever y infrastructure is avai lable. The word has spread in the community. So, we expect that this number would grow significantly in Q3 & Q4 and going forward it would be a good contribution to our overall mix.

Prerana : Okay Sir. And Sir, I remember in the la st con call you were plann ing to construct a hospital near some Jewar airport mainly for this only, so you can attract even more medical tourism. Any updates on that?

Yatharth Tyagi : So, what you're talking about is the expansion plan in already existing hospi tal in Greater Noida. So, Great Noida Hospital is currently 400 in bed. We have acquired the adjacent land parcel there to expand it to 600 beds, and that is the existing hospital of ours which is quite close to the Jewar airport. So going forward, that's what those bed s would add on those international tourism and medical tourism lines.

Prerana : Okay, sir. Thank you very much. I really appreciate you all answering my questions in detail. Thank you very much and all the best for the coming quarters.

 Moderator: Thank you. The nex t questio n is from the line of Prashant Nair from Ambit Capital. Please go ahead.

Prashant Nair : Hi, Good Afternoon. Just can you give a ball -park sense of the profitability at your individual hospitals, especially in Noida exte nsion and Jhansi -Orchha, which are relatively new, ind ividually?

Deepak Kumar Tyagi : Prashant , this is again you know, profitability if you ask me , it would be a ball -park kind of thing. I'll not be giving very specific numbers . But if you ask me , it would be in the range of around 2 8-29% at Noida. So, in the range of Greater Noida and little shared lesser in Noida

extension. So , it would be around 25 -26% at No ida extension. J hansi is just marginally profitable ; on EBITDA side I'm talking about . It d id Rs. 90 lakhs of EBITDA at a turn over of around Rs. 8 crores. So, this is how overall things are coming up. This is, again, a plan that this is in line with t hat where we have to publish the information on the segments and the individual hospital profitability. So, that is in the bigger p lan. I owe this to my investors and let's hope that by Q4 we will be having this better numbers .

Prashant Nair : Sure. And secondly, I have a follow up question on the working capital. So, receivables you mentioned there is some transition at the government level which has led to this increase. Where do you expect, say, receivables days to stabilize at? W hat level approximately, once we are past this transition?
Deepak Kumar Tyag i: Because things are still in place and yes, D r. Ajay and Mr. Amit Singh are tr ying their best and pushing it hard to have this figure come down in the Q3 itself. I am very hopeful that if not in Q3, then by Q4, it would be coming down gradually. I expect that our days should be in the range of around 85 to 95, anywhere in between by the end of this year.
Prashant Nair : Okay, thank you. That's it from my side.
Moderator : Thank you. The next question is from the line of Sagar Doshi from Intuit Investment. Please go ahead.
Sagar Doshi : Hello. So, basical ly, I just wanted to ask regardin g the occupancy level. We are at 51% currently, and you are g uided for around 60% by Q4. So , any changes in that? And what can I expect in the next financial year? Just a rough growth or ramp up in the Jhansi Hospital. So , something on the occupancy part?
Yatharth Tyagi : So, I think we are well on track to the expected occupancy that w e have forecasted for the coming quarters, and I don't see any change to those forecasts. And yes, this quarter did have a significant rise in oc cupancy due to also a lot of d engue patients that was spread
 across Delhi NCR. But I feel also going forward in the quarters, we are well on track with respect to our forecast for the expected occupancies. And as far as Jhansi is concerned, already it is r amping up. And as we mentioned earlier in the Q&A, that a lot of empanelment , including the health insurance and gove rnment panels, are well on track. So , I think that will be a key attribute to that occupancy ramping up in the coming quarters.
Sagar Doshi : So, any number that you woul d give for next financial year?
Yatharth Tyagi: No, I think not an exact number on that, but we do expect increase in th e occupancy quarter -on-quarter basis.
Saga r Doshi: Okay. Also , one more thing is the average revenue per bed decreased marginally quarte r-on-quarter. So, any specific reason for it or how should we look into it?
Yatharth Tyagi : Yeah, ARPOB is increasing because of change in the case mix. As we were mentioning that we are moving towards a hospital which is doing high on super specialties. S o, as we see a rise in oncology treatments and transplant program so that's the reason why increase is there on quarter -on-quarter. And I do expect this ARPOB increase to continue much further in the coming quarters.
Deepak Kumar Tyagi : Just to add to this , and important would be to see the Q4 movements when we start this oncology treatment. So , what I expect with this PET and radiation line things to come up, revenue wi ll go up to that extent, our ALOS would come down and ARPOB wo uld increase, and that wou ld give the impetus for profitability improvements. But I'm not giving any future numbers, so allow me that. But this is the general trend, and this is a fair match on t

he ground that how it functions .
Sagar Doshi : Okay, got it. L ast one. In one of the que stions , you said that we are looking at doubling the bed capacity in three to four years. So , when can we expect commissioning of the first phase or the first increase of bed, let's say, first 100 -200 beds etc.? So , when can that commissioning be , a rough idea of the timeline?
Yatharth Tyagi : Yeah, so when we say doubling, that also includes the expansion of our existing facilities , that's the Noida Extension and Greater Noida Hospital. So that construction would t ake two to three ye ars. But before that al so, we do expect beds to add up. That will be through the mode of acquisitions of new hospitals that we are looking to do. And I think as far as the plan is concerned for the remaining two quarters, we would be loo king to complete one acquisition of a hosp ital. So , I think before the end of this fiscal year is we do expect our first increase in the capacity to come up through the means of an acquisition of a hospital in the North India region.
 Sagar Doshi : Okay. So , any organic expansio n would take around t wo to three years.
Yatharth Tyagi : Yes, if we are constructing, that's what it takes usually to provide capacity of 200 to 250

beds.
Deepak Kumar Tyagi : Sagar it is basically because it's a Greenfield, it would be about what Mr. Yathar th was alluding it to that the Brownfield or Greenfield will take that much of time. But if it comes by the O&M route or if we take over any existing hospital, that would be quicker. So , if you see Brownfield would be coming in the two to three years' time. So that' s around 45 0 bed we are talking about. The 950 bed would be coming the other way and it can happen quickly.
Sagar Doshi : Thank You. That was helpful.
Moderator : Thank You. The next question is from the line of Vivek Gautam from GS Investments. Please go ahead.
Vivek Gautam : Any concerns on the op erating cash flows and receivables from our side and any plans of reducing dependency on government business, number one. Number two, any acquisition plan and how is like Jhansi -Orchha Hospital and how is t hat performing, sir? Thank you.
Yatharth Tyagi : So as far as, you know, the government b usiness and that is concerned, the receivables, we've already addressed that in our previous questions. Now I'll come to your second question on the acquisition front. Yes, as was mentione d that the plan is that you know, before the end of the fiscal year to complete one acquisition for hospital. We are looking for, you know, acquisition which is roughly in the tune of around 150 beds, at least, somewhat around from 150 beds to 250 beds. Th at is what we're looking for right now. And we're looking at the ter ritories of Delhi NCR, Haryana, Uttar Pradesh, Madhya Pradesh. So, that is, you know, the plan on the acquisition part is concerned and we are open for both O&M model as well as equity pur chase.
Then your question on Jhansi -Orchha is concerned. I think yes that hospital is quite well, as mentioned, we are getting the empanel ment done there and the occupancy has also increased on not just the year -on-year basis, but also quarter -on-quarte r basis. So, I think the Jhansi -Orchha hospital is well on track.
Vivek Ga utam: And how is our catchment area in terms of the location of our hospitals , specially Noida extension, and how is the population growth and what is the occupancy level and how muc h can it go up to?
 Amit Kumar Singh : Se

e if you look at it, if we come to the Noida extension areas whi ch is the enters into the Ghaziabad region. So, and it's an excellent connectivity through the Meerut Delhi highway, this goes to, you know, the Meerut , Modi Nagar, Hapur those regions to entire Ghaziabad, that is our good catchment area and Noida extensio n itself is now getting densely populated because there's lots of you know, the bigger sites are coming. So, this is in a way if you lo ok at it, Gautam B uddha Nagar , if you have any idea about this region, this is an area where, you know, it's coming up ver y, very fast. So, I think the coming years, you'll see a good number of people moving from those areas, Delhi and other places to this s ide of it. So, t hat is the Noida extension and then I think earlier also we have spoken about our advantage as far as ge ography is concerned. So Greater Noida has an excellent connectivity towards, you know, till Agra , Mathura, those regions, you know, tha t's one region b ecause of the Yamuna Expressway and Noida has an edge of Delhi and Faridabad region, so I think we hav e a geographical advantage and a good catchment area.
Vivek Gautam : What about competition intensity with coming up of the new Yashoda Hos pital and the bi g units coming up and next purchase and other thing and how are we planning to cope up with that? Thank you.
Amit Kumar Singh : See if yo u look at it, I think, the competition about any areas is growing and having a potential competition is bound to come, b ut I think we are not scared of those things. In fact, that could be a good thing that this area will become a hub for good healthcare. So, we have our own advantages, we have our own strengths, and being the oldest and having the range of services what we are providing is, I think, as of now others whosoever will come will take some time, till that time we will be very much placed as in biggest in this region. So, I think that is not our concern, we are focusing on our own strengths. You kn ow, the specialt y, super specialt y which we are still not having, we are working on to have those things. So, nothing to worry about it as such.
Vivek Ga utam : And s ir, how is the opportunity size and which will increase usage of medical insurance. it augur s well for us ve rsus the self -paying patients.
Amit Kumar Singh : Can you kindly repeat that question your voice is cracking.
Vivek Gautam : Yeah, my question is about the opportunity size for us, in terms of basically, due to the COVID and the lack of medic al insurance, pe ople were suffering a lot and as we say that,

between basically if you stay in ICU for 15 -20 days, you come from middle class, you can
go to the lower class if you don't have the ins urance. So, how is the insurance penetration
level happeni ng as per your e xperience and how does that augur well for our future?
 Amit Kumar Singh : See if you look at it the good part about COVID and ye s people got worried about their
healthcare. And that's what did you see the data shows that the last three years if you see the
kind of the insurance penetration has gone. The insurance companies growing very fast.
And the more and more tier two and tier three cities' people are having their insurance. So, I
think, that's a by increasing this perc entage any healthca re facility will benefit because
people will demand, you know, the quality healthcare and that is they will move from those
agents to and also very important quality health care. And so, we are on the same line and
I'm sure not only we , other healthcare ser vices also goin g to benefit of it.
Vivek Gautam : Thank you.
Mod erator : Thank you. The next question is from the line of Naman Bhansali, from Perpetuity V

entures
LLP. Please go ahead.
Naman B hansa li: Thank you for the opportunity. First question is in Noida hospital, thei r occupancy are at
96% this quarter. Is it on the 450 beds capacity, which we have or is it on some other
number because we haven't heard such level of occupancy in any other hospitals case.
Yatharth Tyagi : Yeah, so I think, you know, you've got that point , correct. And that occupancy is not on 450
but on smaller beds of 250 bed. So that's why, you know, we're able to maintain an
occupancy of 90% above in that hospital , and you know, for a hospital of, let's say, 450
beds or beyond that, it w ill be difficul t to reach that occupancy, so Greate r Noida hospital is
around 250 beds.
Naman Bhansali : But in optimally any hospital can do at a max of what range?
Yatharth Tyagi : So, I think depends on hospital to hospitals, but let's say, for our hospit als of let's sa y
around 400 beds or 450 beds that is Greater Noida extension. So, Greate r Noida is touching
around 70% out there. We do feel that we can still cater to a much more occupancy to
around I would say somewhere between 75 to 80%. We would consid er booked an
optimum utilization there.
Naman Bhans ali: Got it. Thank you. Second question is you had men tioned that Brownfield take 2 years to
build and greenfield around 3 -4 yea rs. Sir, how is the breakeven time period in such
functions, in brownfield vs a greenfield?
Deepak Kumar Tyagi : The breakeven for the Brownfield would not be an issue because first of all, you know, the
second cost of these hospitals, many kind of thing so, the base of the cost will be less
breakeven should come, I'm talking ab out the overall fac ilities. If we talk about overall
facility because we are still having a margin of around 28 -29% coming in from Greater
 Noida . So, you know, they would not be a performance down for the hospital per se , but
when it comes down to the you k now the setup bein g created that the 200 or 250 beds
which are coming because good lot of its cost , the fixed cost will be shared by so I expect
that on an individual basis , the payback should be anywhere between three and kind of
thing.
Naman Bhansa li: Okay , I understood. Lastly, you had mentioned that your gross margin had fall due to the
increase in consumer expenses for new high -end specialty which you're doing . So just want
to understand that in terms of ARPOB, why is it not reflecting that if you are do ing some
high-end specialty in a particular quarter , the ARPOB would have moved up on Q oQ basis
till date. So, marginally the ARPOB would decline by 2% on Q oQ basis. So, can you just
help me understand that when does the A RPOB eventually fade out in terms o f these high -
end specialties?
Deepak Kumar Tyagi : First of all, you know there is a slight increase in the you know, in the percentage term the
consumables cost around 21% , which is around 18 to 19% in the last quarter. Important is
that it did not retain t he EBITDA. In fact, the EDITDA on Q oQ basis has gone up. The
numbers I'm not sure it is there, but it has gone up from Q1 to Q2. So, this is there. So first
of all, I want to dispel that EBITDA has come down because of the increase in the cost.

What was th e second quest ion, sorry I didn't write it.
Yatharth Tyagi: Coming to your question on the part that you know even the high specialty where we are moving , Jhansi is having a lower ARPOB even with high specialty becau se in Jhansi Orchha hospital the prices are also signi ficantly lesser compared to the Delhi NCR. So yes, we are moving towards you know basic specialties there but then also the ARPOB will be always less in that territory due to the pricing strategy that we have taken in the Orc

hha
hospital.
Nam an Bhansal i: Got it understood and on a big picture basis , over the next three to five years , the ARPOB can grow at what range.
Yatharth Tyagi : We do expect the ARPOB to grow quarter -to-quarter. I wouldn't give exact figure on that, and you know we are well on track for that.
Naman Bhansali : Thank you.
Moderator : Thank you. The next question is from the line of Prashant Nair from Ambit Capital. Please go ahead.
Prashant Nair : Hello, my question has been answered. Thanks.
 Moderator : Thank you. The next questio n is from th e line of Amit Thawani, who's an individual investor. Ple ase go ahead.
Amit Tha wani : Hi, yeah, just a follow up question. We are almost halfway through the December quarter. Can you give us the occupancy of Jhansi and Noida extension for the hal f quarter?
Yatharth Tyagi : For the half quarter, yes, I think that's what we have included in our ea rnings presentation. But, you know, as far as the quarter occupancy is there, it is available there.
Amit Thawani : No, no, this quarter, this December quart er?
Deepak Kumar Tyagi : Amit, you know, taking occupancy, see first of all, yes, we are publishing, we will continue to publish occupanc y on a quarterly basis. But the occupancy has to be seen on a longer term. Now, we are almost a month down into this qua rter. First of all, the occupancy levels are almost same as it was in the month of September this is one thing, but giving a number specific that what that occupancy would be it will be too early to know, call. I request if you can wait for the quarter to end process matter.
Amit Thawani : But just to get a directional sense , we are highe r?
Deepak Kumar Tyagi : We are pretty much there. The occupancy levels continue to be good. So, that's the subjective answer I can give.
Yatharth Tyagi : Also, just to add on this, tradi tionally, but you know, we do expect occupancy to remain you know, in the positive direction, but traditionally in Q3, usually in Delhi NCR occupancy tends to be always lesser given to that the winter is coming up and you know, due to the fog in creasing. S o, both are at OPD level and both at IPD level, you see this is considering the Delhi NCR , the coolest period. But, you know, we should be on our track even afte r that.
Amit Thawani : Exactly, that's why I was asking because Q2 and Q4 are generall y the best quarters and Q3 is not really the best quarter. So that's why I was asking can you still expect an improved occupancy?
Yatharth Tyagi : So, I mean, I will say, for, let's say, things like OPD o r elective surgeries, these tend to be lesser in Q3, and as far as occupancy is concerned, I think overall, it should be on the similar track.
Amit Thawani : Okay, thank you, sir. Thank you.
 Moderator : Thank you. The next question is from the line of Sumit Gupta from Motilal Oswal. Please go ahead.
Sumit Gupt a: Yeah. S o, I have two questions, the first one the margin front. So, if it is possible, can you quantify how much margin do we expect over the next three to fou r years? And the second point is the incremental growth in the overall EBITDA, which hospitals or which facilities would likely contribute more from here on and the third question is regarding how much Capex we plan to deploy over the [inaudible].

Deepak Kumar Tyagi : So first of all, you know, I will dismiss that, you know, what kind of margins we'll be having in the years to come. I'm not giving any futuristic direction. Very subjective answer we can give

, we are at 26% plus EBITDA and these EBITDA margins are expected to grow, which in itiatives of Mr. Yatharth and Mr. Amit just mentioned, it's basically the induction of international business coming in, oncology coming in, more and more higher specialties are coming in. So, these would add to two things. The ARPOB would increase and ARP OB would significantly then drive it, for the margins to increase. But if you ask me, yes, we have our internal plans, but I'm not supposed to you know, disclose at this point of time to the investor community , sorry for that. So, this is one thing.
Now sec ond question was about our Cap ex plan. For the next three years, we ha ve a Cap ex plans of around 800 C r coming in, which is basically for the addition of these 1 ,400 more 1,450 beds coming in. This plan may, go here and there depending on if more beds come on the O&M side , then the CapEx would be lesser. But if it goes on mo re on a greenfield side, it would be higher but at this point of time, we have this kind of thing. So, that is the case, and we are sufficiently funded for this expansion. We don't expect a long -term basis, we would need any loan capital to fund these proj ects.
Sumit Gupta : The follow up question was on the incremental growth of the EBITDA. So which facility would contribute more towards it?
Deepak Kumar Ty agi: Well, all the facilities are contributing but if you see the AR POB, the ARPOB would settle, then I wou ld say that Noida extension is to be seen. Did it answer you?
Sumit Gupta : Okay, thank you.
Moderator : Thank you. The next question is from the line of Prashant Nair from Ambit Capital. Please go ahead.
 Prashant Nair : I just had a question on the tax rate. So, in the first half the tax rate is higher than what it was in fiscal 23. So, is this the level we should look at for the full year and going forward or will it settle down?
Deepak Kumar Tyagi : No, you know, Prashant what happens that you know, mid way, because auditors generally tend to be a little conservative. I would say that you know, because we will be best of the tax position coming in from our two companies, that is Yatharth hospital having Noida and Greater Noida hospital and AKS which is ba sically Operating Noida extension hospital , there, the tax rate, because the MAT and everything has been utilized , so, it will be better than the last year, I expect that the tax rate will be in the range from 25% for these two hospitals. As far as the Jha nsi is concerned, Jhansi is because it is h aving the accumulated losses. So, we will go for the traditional tax percentage. Overall, you know, this is basically because by March '24 the position would be clear, I expect the overall tax burden should come do wn in the percentage terms . There on 30%, i f I'm not mistaken, for the first half, you know, just a minute. I can check the numbers is around 33% , 30% plus kind of thing at this point of time, I expect that it would be in the range of around 26 to 28% anywhere in between.
Prashant Nair : Yeah, thank you.
Moderator : Thank you. The next questi on is from the line of Tanmay from Mirae Asset. Please go ahead.
Tanma y: One questio n on the Noida Extension hospital. If I look at the average revenue per patient, I'm just dividing the overall top line might no t be the right calculation, but dividing the overall top line by the inpatient revenue that we have. It has gone up significantl y, let's say from H1 FY 23 to H1 FY 24. What could be the reason? Is it purely case mix or any particular reason , that you would want to highlight? I think it's lower than the other hospitals in the Noida extension part. Yeah.
Yatharth Ty agi: Yeah. Sir, just yeah. So, the reas

on for increase is case mix is there. And it is also not just the Case mix. There's a lot of new treatments that we have started here which are high end value, which we were not doing earlier. So, you know, procedures like kidney transpl ant and bone marrow transplant, you know, we recently started around a year back. So, this is why the sharp increase is there. And as far as other specialties are concerned, these are also growing much more in the proportion to specialties which are at a l ower end of the ARPOB spectrum. So, that is why it's there. And also, this is a hospital where we are getting the oncology mach ine, which is already reached and will start functioning from January onwar ds. And robots are also being planned in this hospital . So, this is the reason why the

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CIN: L85110DL2008PLC174706 ARPOB has increased as well, and we do expect much more increase quarter on quarter at the ARP OB of this hospital.

Tanmay : So, this specialty is also increasing , I mean the ALOS nu mber looks lower than the other hospital, t he other hospitals that we have. So, that is the right trend to look at or how should we look at it I mean.

Yatharth Tyagi : So, I think lower , so actually lower ALOS sort of represents a higher share of super specialties. So, let' s say if 3 to 4 days of AL OS is there for super specialties. And if there are more patients of general specialties, let's say for internal medicine or oth er procedures, that's where the ALOS tends to be higher.

Tanmay: Okay, thanks .

Moderator : Thank you, ladies and gentlemen. Due t o time c onstraints, that was the last question. I now hand over the call to the management for the closing comments. Please go a head, sir.

Yatharth Tyagi : Yeah, I think I would just like to thank all the participants who attended our earnings call, and we look forward to deliverin g on the numbers and the promises that we have kept. And we are well on track to, you know, do that in the coming quarters. Thank you, everybody, again for participating. Thank you.

Moderator : Thank you on behalf of Prabhudas Lilla dher. That concludes this conference. Thank you for joining us. And you may now disconnect your lines.

(This document has been edited for readability purpose)

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Call: Feb 2024

February 19, 2024

The Listing Department
National Stock Exchange of India Limited
Exchange Plaza, 5th Floor, Plot No. C/1
G Block, Bandra-Kurla Complex, Bandra (E)
Mumbai – 400 051
Symbol: YATHARTH
Dept. of Listing Operations
BSE Limited,
P J Towers, Dalal Street,
Mumbai -400001, India

Scrip Code: 543950

Sub: Earnings Call Transcripts

Dear Sir/Madam,

Pursuant to Regulation 46(2)(oa) of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed the transcript of Investors' conference call organized on February 12, 2024 post declaration of Financial Results (Standalone & Consolidated) for the quarter/nine months ended December 31, 2023.

The transcript is also available on the website of the Company: https://www.yatharthhospitals.com/investors in the section of Corporate Announcements.

Kindly take the same on record.

Your faithfully,
For Yatharth Hospital and Trauma Care Services Limited

Ritesh Mishra
Company Secretary & Compliance Officer
Mem. No. A51166
Enclosed as above.

"Yatharth Hospitals and Trauma Care Services Limited
3QFY24 Earnings Conference Call"
February 12, 2024

MANAGEMENT : MR. YATHARTH TYAGI – WHOLE-TIME DIRECTOR – YATHARTH HOSPITALS AND TRAUMA CARE SERVICES LIMITED
MR. AMIT KUMAR SINGH – GROUP CHIEF EXECUTIVE OFFICER – YATHARTH HOSPITALS AND TRAUMA CARE SERVICES LIMITED
MR. PANKAJ PRABHAKAR – CHIEF FINANCIAL OFFICER – YATHARTH HOSPITALS AND TRAUMA CARE SERVICES LIMITED
MR. DEEPAK TYAGI – PRESIDENT STRATEGY AND FINANCE – YATHARTH HOSPITALS AND TRAUMA CARE SERVICES LIMITED

MODERATOR : MR. PRASHANT NAIR - AMBIT CAPITAL

Moderator: Ladies and gentlemen, good day and welcome to Yatharth Hospital's 3QFY24 Earnings Conference Call hosted by Ambit Capital. As a reminder all participants line will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touchtone phone. Please note that this conference has been recorded.
I now hand the conference over to Mr. Prashant Nair from Ambit Capital. Thank you and over to you sir.
Prashant Nair: Thank you Manuja. Good morning and welcome to the 3QFY24 earnings call of Yatharth Hospital and Trauma Care Services. Today we have with us from management: Mr. Yatharth Tyagi, Whole-Time Director: Mr. Amit Kumar Singh, Group CEO; Mr. Pankaj Prabhakar, CFO; and Mr. Deepak Tyagi, President Strategy and Finance.
I would now like to hand the call over to Mr. Yatharth Tyagi for his opening remarks. Over to you Yatharth.
Yatharth Tyagi: Good morning and a very warm welcome to Yatharth Hospital's and Trauma Care Services Limited Earnings Conference Call for the quarter and nine months ended 31st December 2023.
Let me draw your attention to the fact that on this call our discussions will include certain forward-looking statements which are predictions, projections or other estimates about future events. These estimates reflect management's current expectations about future performance of the company.
Please note that these estimates involve several risks and uncertainties that could cause our actual results to differ materially from what is expressed or implied. We have uploaded our presentation on the exchange and the company website and I hope you all might have received and have had an opportunity to go through it. This quarter was full of dynamism and yet I'm happy to announce that we have ended this quarter with a stellar performance.
Our revenue has experienced a 21% year-on-year growth reaching INR1,670 million while a profit after tax has surged by 39% compared to the same period last year. As part of our ongoing commitment to diversifying and expanding our range of medical specialties across all our hospitals I am delighted to share some noteworthy achievements from quarter 3. Our company introduced robotic surgeries marked by the installation of Da Vinci X surgical robot and Stryker's Mako orthopaedic robot at our Noida extension hospital.
We have indeed taken the lead in conducting greater Noida regions first robotic surgery ever. Furthermore during the quarter we expanded our transplant services to include liver transplants. These initiatives underscore our dedication to delivering cutting-edge healthcare solutions and improving patient outcomes.
In summary the quarter has been marked by impressive double-digit growth across most of our specialties. Our gastroenterology and pulmonology revenues surged by 50% plus year-on-year while our nephrology and urology and cardiology grew 30% year-on-year. Additionally our oncology revenues surged by more than tenfold on a year-on-year basis.

As a result of these strategic initiatives, the share of internal medicine in our revenue mix has decreased to 26% down from 33% previously, and thus our overall ARPOB reached INR29,367 up by 14% year-on-year and 7% quarter-on-quarter. Among all our hospitals, Noida extension registered the highest share of revenue coming from super specialties at around 60% and thus highest ARPOB levels of INR34,287 during the quarter. Our commitment remains to enhancing our specialty mix, expanding our suite of oncology services and creating a comprehensive one-stop destination for all cancer related treatments.
The radiation oncology line

has already arrived in our Noida extension hospital, and we are
expected and set to commence operations in March 2024, seamlessly integrating it with a oncology center. As we advance, our focus remains on expanding our medical tourism business anticipated to fuel significant growth and enhance the average revenue per occupied bed ARPOB in the coming quarters.
Ladies and gentlemen as outlined in our IPO plans we express our intent to vigorously explore opportunities including both organic and inorganic and strategic partnerships to bolster our bed capacity in the forthcoming years. Today I am delighted to announce that we have signed a definitive agreement to acquire Asian Fidelis Hospital, a 200 bedded super specialty hospital located in Faridabad Delhi NCR. This hospital was built three years ago on a 1.25 acre land. It

is equipped with all the latest facilities and currently operates with a bed capacity of 175 beds expandable to 200 to 225 beds. The acquisition involves the 100% equity purchase of Pristine Infracon Private Limited at a consideration of INR1,160 million.
The transaction is expected to be completed by March 2024. The hospital is ready to be operationalized immediately as all necessary approvals, accreditations and empanelment with all major health care insurance companies and TPAs are in place and hence we are confident that this hospital will commence generating revenue from day one. We understand that inorganic growth is not merely about expansion but also about identifying synergistic opportunities that amplify our strengths, extend our reach and ultimately enable us to serve more patients with exceptional care.
And thus this acquisition fits well with the company's stated approach of expanding its presence in North India region and further enhances Yatharth's positioning in Delhi NCR market.
At our current capacities, Noida and Greater Noida Hospital reported highest occupancy levels of 90% and 68% respectively during the nine-month period. Our other hospital at Noida Extension has achieved an occupancy level of 42% during the same period in line with our Noida and Greater Noida Hospital. We expect our Noida Extension Hospital to reach optimum utilization levels by financial year 2025.
Thus we have already acquired a land parcel adjacent to our Greater Noida Hospital and have also been allotted land adjacent to our Noida Extension Hospital in the last quarter. With this land we aim to take ground field expansion towards increasing our Greater Noida bed capacity to 600 from 400 presently and Noida Extension bed capacity to 700 from 450 presently. Overall we remain committed in our aim to double our bed capacity over around next three years via a mix of both green field and brown field expansion.

Further our focus remains on continuing our growth trajectory, creating value for our shareholders and advancing our strategic initiatives all while maintaining the highest ethical standards. Your trust and support throughout this period have been invaluable and we are more determined than ever to propel our company towards success and sustainable growth.
I would now like to hand over the call to our Chief Financial Officer Mr. Pankaj Prabhakar for the financial updates for the quarter.
Pankaj Prabhakar: Thank you Yatharth. Good morning everyone. I am happy to announce another robust set of results for the quarter ended 31st December 2023. Our quarterly revenue stood at INR1668 million recording a robust growth of 21% year over year with improvement witnessed across all our specialties.
Our in-patient revenue were up by 22% Y-o-Y during the quarter. Our company reported an EBITDA of INR464 million for the quarter up 29% year over year and 2% quarter over quarter.
We are pleased to report a significant increase in our margins for the quarter.
Our EBITDA margin expanded b

y 160 basis points year over year and 123 basis points quarter
over quarter to 27.8% reflecting our continued focus on operational efficiency and adaptability of our business model in navigating market challenges and capitalizing on opportunities. Our profits grow by 39% year over year and 7% quarter over quarter to INR295 million primarily attributable to lower finance cost during the reporting period. As mentioned earlier the full impact of debt reduction is starting to kick in with our finance cost reduced by 97% year over year and 94% quarter over quarter during the quarter.
This significant reduction in finance cost underscores our prudent financial management strategies which have effectively optimized our capital structure and reduced our borrowing expenses. As a result of this and flow through of improved EBITDA, our tax margin expanded by 220 basis point year over year and 154 basis point quarter over quarter to 17.7%. Our revenue per operating base for the quarter improved to 29,309 up by 14% and 6% quarter over quarter. The improvement was driven by a consistent shift towards higher complexity procedures and was witnessed across specialties with nephrology and urology, pediatrics and oncology being key drivers. Bed occupancy for the quarter has stood at 52%.
Let me also briefly touch on the consolidated nine months FY24 result numbers of the company. Our revenue grew by 31% year-over-year to INR4,926 million. Our EBITDA for nine months was INR1,334 million up by 40% year-over-year while our EBITDA margin expanded by a solid 177 points to arrive at 27.1%. Our profit after tax went up by 57% year-over-year to 761 million and tax margin came in at 15.5% expanding by 259 basis points.
Our average revenue per operating beds for the nine months is INR28,316 up 8% year-over-year. The improvement was witnessed across specialties with gastroenterology and pulmonary growing by over 50% plus, nephrology and urology, cardiology by 30% plus and oncology revenue growing over 10x on a year-on-year basis.

Bed occupancy has stood at 53% versus 44% in the corresponding previous nine months. Our balance sheet continues to be a strong with net cap position up to 2,814 million which we strive

to invest towards our ongoing and planned organic and inorganic expansion as stipulated in our IPO objects. In essence our earning momentum remains robust and we anticipate this trend to persist in the future.
I express this with confidence as our growth drivers are clearly outlined in our strategic plans.
With this I would like to hand it over to the moderator for question-and-answer. Thank you.
Moderator: Thank you very much. We will now begin the question-and-answer session. The first question is from the line of Hussain Bharuchwala from Carnelian Asset Management. Please go ahead.
Hussain Bharuchwala: My first question I just wanted to understand in regards to the acquisition. So this acquisition that we have done so can you give us some details as to is it a pre-revenue acquisition or the hospital is completely ready and basically all the approvals as you said are in place.
So is it currently running with all the staff and everything or we have to set up staff, we have to do the marketing and the ramp up will happen gradually. Also tell us, wanted to update as to what are the other hospitals in the region as to which are the peer competition and whose market share that we were able to grab. So if you can give us some details on that front we will be very grateful. Thank you.
Yatharth Tyagi: So this hospital was built you know three years back. It was operational earlier. It was being run by the previous promoter and then for last two three months this hospital has been paused. But as I said that all the empanelments and all the licenses are available.
So the ramp up will happen very quick in this hospita

l. Yes the revenue will start from zero onward but we expect it to catch faster given the fact that all the healthcare insurances and some of the government empanelments are already in place. This hospital is in Faridabad and if you look at the micro market in Faridabad this hospital is actually located in Greater Faridabad and there are, Faridabad is a big healthcare market.
The drainage is not just from Faridabad but also from the nearby cities including you know the likes of Mathura, lot of patients come there and that whole belt of Haryana which is catering and draining into the Faridabad. So yes there are other hospitals also.
I wouldn't like to take names of that but yes there are big hospitals in the vicinity of around beds of 400 beds, 300 beds, 200 beds as well and this is already a very fertile and well-established healthcare market of Faridabad and we hope to reap the benefits of the same.
Hussain Bharuchwala: So just a follow-up question on the same. So any reason why the previous promoter exited and they actually sold and the hospital got closed down? If you can just give us some details on that front?
Yatharth Tyagi: So the chain, so this was earlier owned by a corporate chain of hospitals that also have a private equity investor with them. This hospital was actually in, this specific hospital was 50% owned by them roughly and 50% by another real estate person.

So I think there was some dispute amongst both of them and this is why the hospital was paused and it is a great locality. There are a lot of societies, urban societies coming up and already in that area with lot of high population and that is why we expect the same to ramp up.
Amit Kumar Singh: But I think our meeting with all those, both the promoters are very amicably well done. So there is no further any disputes will come into our picture.
Deepak Tyagi: So, okay, whatever their differences, though they have been settled and you know we are going to get a very clean, you know 100% equity.
Hussain Bharuchwala: Okay and so when by when we'll be able to start ramping up and when we start seeing the revenue from which quarter we can see this happening?
Yatharth Tyagi: I think we are planning to launch the hospital from April 2024. So from the first day of the new financial year, we should be hoping to get the revenue from this hospital.
Hussain Bharuchwala: Got it. And just secondly, on the quarter-on-quarter revenue when we see there is a slight dip in the revenue, I understand that there was a dengue event which happened in the last quarter. So I think as a result there was higher occupancy. But any specific reason that due to the revenue quarter-on-quarter there was a slight dip, if you can explain that front?
Yatharth Tyagi: So I think see that is an, I mean if you look at the health hospital industry, that is industry-wise trend, that is nothing specific to us. But yes, being in North India, specific to three reasons that we did see was, you know, this quarter had lot of festivals and holidays. Diwali was there, Chhat

Pooja was there. So that does, lot of people in the nearby areas, they go back to their hometown. So the OPDs usually dip in this quarter. The second reason was, you know, by late November and December, winter comes in North India, lot of fog is there and due to lower visibility, usually traditionally also we have seen lesser OPD volumes in this quarter. And I think that's in line with the industry.
Hussain Bharuchwala: Got it, got it. That was the only question. I will get back in the queue. Thank you.
Moderator: Thank you very much. The next question is from the line of Nirvana Laha from Badrinath Family Office. Please go ahead.
Nirvana Laha: Thanks for the opportunity and congratulations on a strong set of results. So I have a few questions, basic questions around your hospital. So can you tel

l us if there are any government commitments across your hospital to provide free of cost outpatient or inpatient services? Because these typically happen when you are allotted land, etcetera? So if you can talk a little bit about that.
Amit Kumar Singh: No, absolutely not. So there is no any, you know, we are bound from the government side to provide any free these things. All four properties of ours is absolutely owned by us. So there is no compulsion whatsoever.

Yatharth Tyagi: Yes, the lands that we have our hospitals on, we have purchased these lands from the local authorities. And it has not been allotted from the government at any subsidy rates. We have paid the market rates.
And this is why, at least in the region that we operate our hospitals in, we don't have any liability to treat any government patients at a reduced price.
Nirvana Laha: Got it. And that's true also for the new land that you are expanding in Noida, right?
Yatharth Tyagi: Yes, yes, yes.
Nirvana Laha: Okay, thanks. So coming to your receivables and payer mix, so I think right now you are at 27% insurance and 37% government selling payments. So in terms of your internal benchmarks, how is this looking like? Where would you like to be in terms of government insurance and cash? And how does it compare with competition?
Yatharth Tyagi: So I think we are roughly one third, one third, one third in across all three segments, that is cash, insurances and corporates as well as government. One or the other quarter, one is 5% up and down. And that that happens. But overall, I think we are on the similar line.
And that is something you know, right now, we have capacity to utilize. So you know, we do that. There's no specific discussion that you know, one share should be bigger than the other. But yes, we do feel that over the course of a few years, when you know, the share of both cash and insurance and corporate patients should increase up given the fact of the increasing, you know, identity of a brand and the infrastructure coming up. And I think that will happen over the course of period.
Nirvana Laha: Okay, any number or aspirational target three years out that you would like to have?
Yatharth Tyagi: No, there's no specific or internal targets also that we have kept for that.
Nirvana Laha: Okay, got it. And can you tell us what out of your receivables right now, how much is government linked in terms of a number right now?
Deepak Tyagi: Say again.
Nirvana Laha: Out of your receivables, can you tell us how much is government linked to the number on total receivables? And out of that, how much is the government linked to the house? And what's the average revenue for the government payment?
Deepak Tyagi: You know, you know, whatever the debts we have, as Mr. Tyagi just mentioned, that we have around 33%-35% of the revenue coming in from the government. Yes. But you know, if you see the debtor inventory, you've taken a very potent question.
Around 70% of our total debt is from the, in spite of the fact that it's around 35% of the total business, around 75% of the debt remains with the government authorities. So we are working on that. You know, we have systems in place, but government debt is high and it's nothing to

ask. It's basically, you know, people who are working with the government in the market. This is the same story with them.
Nirvana Laha: Right. We are aware. That's why the question. Thanks for sharing the presentation. So coming to your medical tourism, you know, you said that one of the hospitals you want to make it a center for medical tourism. So can you tell us which geographies you're targeting and what kind

of marketing or patient acquisition strategies you're already doing? And do you already have a
team? Ho

w big is it? What's the focus right now?
Amit Kumar Singh: So, then in the last two quarters, earning call, what we had mentioned earlier, so we have set up altogether different complete separate international marketing team. And the team is progressing very well. In fact, to be very honest, I'll not take a number, but I'll tell you first quarter versus this quarter number has gone significantly.
But yes, not that which you can tell you that this is a percentage in percentage. But yes, internally, this number has probably doubled in terms of the revenue. The couple of areas which we have focused deliberately, because one is that if you look at the many patients come from the Bangladesh region, which is another, you know, very easier market for us.
So we are targeting that market. We have a kind of agreement with the CIS countries. Fiji, there was another country recently we are focusing on the Iraq because lots of as we have started transplant program.
So good number of transplant patients come from the Iraq government. And we are doing a significant progress on our African market. So these are the four or five markets, which we are working on.
And I think I'm very happy to see the progress and probably next couple of quarters, you will see a significant numbers, which you would able to tell you in a contribution. Plus, airport next year is going to come probably by the end of this year probably. So that is where we are also hopeful. So our planning accordingly and that is what this 200 beds, the greater Noida and Noida extension, we are planning on the similar line of it.
Nirvana Laha: Got it. And in terms of margin, do you think medical tourism offers the same kind of EBITDA that you are currently delivering or will it be EBITDA accretive?
Amit Kumar Singh: So, yes, I mean, the international market is always give you much better, you know, the ARPOB that we have, you know, impact on the EBITDA margin. So there is well known fact, but it varies, which country, which where you're working, what kind of deal you are there. But yes, it is it has a significant increase in ARPOB in a procedure wise.
Nirvana Laha: Okay, I have one more question. Can I ask now? Should I come back in the queue?
Amit Kumar Singh: Please go ahead.

Nirvana Laha: So, from our conversation, we have understood that, you know, right now, ortho, cardio, these departments are sort of becoming, you know, basic necessity for any good hospital. They are no longer differentiators. That's what we have understood.
So, do you think that is true? And if that is true, then which department are you focusing on as centers of excellence to sort of differentiate yourself? Because Delhi NTR is a pretty crowded, you know, market. So your hospitals will be known by which centers of excellence? How is your thinking there?
Yatharth Tyagi: So I think we are, going towards a share now where our super specialties is increasing. So definitely, as the brand grows, the more high-end treatments and more life-saving treatments and, life-saving risky surgery patients opt for that is always a good benchmark of, how hospital reputation and brand is growing. And that is the similar of how our cases of super specialities mix is growing. In fact, as I mentioned in my commentary about the Noida Extension Hospital, our share is already coming from 60% from the super specialities and this is increasing in the latest months also.
And I think as far as creating a different and a sort of a niche in the market where we present and what we are doing in terms of different specialties is, you know, that is in terms of robotic surgeries. So during the quarter, we started our robotic surgeries, we installed the Da Vinci X and we installed the Mako Orthopedic Robot.
So even if, you know, ortho and general surgeries are important for any basic and good hospit

al,
but even in then what infrastructure and technologies you have also plays a role. So that's why our having these robots in these specialties is really, you know, differentiating us in the markets we are present and that is the route we have taken.
Nirvana Laha: Got it. Apart from robotics, do you think any other department you are trying to develop based on which the reputation of which you think patients will get attracted to your path?
Yatharth Tyagi: Yes, so our whole transplant program, if I say, so, you know, bone marrow transplant, kidney transplant and liver transplant, you know, these, if you can see if these transplants are happening at any hospital, they require the highest most standards of, you know, infection control in terms of quality, in terms of the medical expertise present.
So patients do understand that if these level of high end transplants are happening, then that

means all the other specialties have to be up to the mark in order to support these transplants, whether it be the lab services, radiology services, blood bank services. So developing high end transplant program is what, you know, we are developing as well. And oncology definitely, you know, starting radiation oncology very soon will be, you know, completing this whole suite of services.

Nirvana Laha: Got it. Got it. Thank you so much. I'll get back to you.

Moderator: Thank you very much. The next question is from the line of Dhara Patwa from SMIFS Limited. Please go ahead.

Dhara Patwa: Thank you for the opportunity and congratulations on a good set of numbers. So I just had one question regarding acquisition of Fidelis. Since currently it's a zero revenue hospital, so by when can we expect a bigger breakeven for this hospital? And will we be having radiation in oncology unit from day one or we would be adding it later in the year?

Yatharth Tyagi: So Fidelis hospital, we are not planning a radiation oncology unit as of now, because we are quite content with the other specialties and we feel the other specialties would be a much key attractor for this hospital rather than just radiation oncology.

So we will be having medical oncology, we will be having surgical oncology there, but not radiation oncology there. And traditionally, if you talk about EBITDA breakeven, all our hospitals tend to break even around in two years' time. So I think this hospital being in Delhi NCR market should also be no different to that.

Dhara Patwa: So two years -- within two years, I guess we could see EBITDA breakeven and thereafter a positive growth trajectory, right?

Yatharth Tyagi: Yes, I think that's correct.

Dhara Patwa: Yes. That's it from my side.

Moderator: Thank you very much. The next question is from the line of Ankur Kumar from Alpha Capital. Please go ahead.

Ankur Kumar: Hello, sir. Thank you for taking my question. So my first question is, in terms of if I look at our IPD and OPD volumes, they're down in nine months as well as in 3Q year-on-year, while our revenues have grown. So basically, the length of stay has increased, but volumes have been going down. So any comment on that, please?

Yatharth Tyagi: See, if you look at it, two things, a, in terms of revenue, we have grown, but yes, the volume, it's a degrowth, marginally. The reason being, and this is a scenario of probably all hospitals in Delhi, NCR, or Northern India, particularly, if you see it, because this time, I think winter has extended.

Earlier, you know, the end of January, the winter gets over, but this time, even in February, we are sitting. So I think it's a good healthy month has increased probably one more month as extended. So that's the reason we would say. Otherwise, there's no other specific reason.

Ankur Kumar: Sorry, sir, nine months, also, it's degrown.

Yatharth Tyagi: Yes, that's what we're coming to now. And also, see, last

year, in this specific region we are in,
there was a flu season. So there were a lot of -- not just Dengue was definitely higher last year, but even other flus, like there were a lot of pediatric patients with flu and other flu symptoms are there. So that was the seasonal thing. But even then, our revenue has grown because of the increase in the ARPOB leading to higher revenue. And it is bound to grow much further ahead.

Amit Kumar Singh: It's not cause of worry, absolutely.

Ankur Kumar: Got it, sir. And sir, on Faridabad Hospital side, you said EBITDA breakeven in two years. So for next year, what kind of losses at EBITDA level we will have to bear on for that?

Deepak Tyagi: Good question. The thing is that the two years we have said, now, first of all, you will see that the overall size, this is a 116 total deal, where the capitalization is not that heavy, in spite of the fact that all the specialties we will be running. So we will not -- we are not going to have a debt finance out of it.

So it's a complete -- because we do have 650 million staying with us for the acquisition and rest we will be doing from the general corporate reserve. So there will not be a net debt, I'm talking, because we still have around INR284 crores of cash lying with us. It could be a temporary arrangement, you know, we do something.

Now, second thing come on the depreciation side. So depreciation is not that heavy. We are expecting, because it's a mature market, the revenue will be coming very quickly. I'll not be able to give, because that's still with a discussion with the management that what kind of revenue are we expecting in '24 and '25?

My expectation I'm saying that in the second year, we will be having a sizable turnover. That's a starting '25-'26, we will be having a sizable turnover. In '24-'25 also, we will not be having much losses coming out of it. So EBITDA, it will be -- EBITDA loss we are talking, it would not be much of drag on the overall financials.

And with the momentum we are getting from the established hospital, our EBITDA margins will remain solid, the way we are presenting as of now.

Amit Kumar Singh: And one thing important, we'll have to see. This hospital, it's already doing good revenue. It's just that three months pause. So that's advantage we will definitely going to get it.

Ankur Kumar: Got it, sir. And last question is on that IT raid that happened, anything came out on that front?

Yatharth Tyagi: No, I think, as we already stated earlier, there's been no finding as of now in that, and we are complying with the authorities to settle that case very soon. And as and when there's any update on that, even more, we will exchange -- we will inform the exchanges.

Ankur Kumar: Got it. Thank you and all the best.

Moderator: Thank you very much. The next question is from the line of Aashil Shah from CJ Investment Group. Please go ahead.

Aashil Shah: Good morning, sir. Congrats on a good set of numbers. I had a couple of questions. One was, what is the current level of doctor attrition? And what are we doing to control that? Because I believe that was a little bit difficult point earlier on.

Amit Kumar Singh: The doctor attrition is, again, as we have said earlier, very much controlled, lesser than a 10% of the consultant level. And in the last quarter what we talked about that we started a couple of in-house programs like a DNB and diploma courses. So there is no attrition on even an RMO, which we call it as a floating doctor. So it is very much under control.

Yatharth Tyagi: 10% attrition is, as we said, in the superficiality doctors. But overall attrition also in terms of doctors has also significantly come down, as mentioned at the start of the DNB programs.

Aashil Shah: Got it. And s

o what is the importance of having super speciality doctors in terms of drawing the patients in for surgery? How would you view it in terms of importance as compared to other hooks to draw in the patient?

Amit Kumar Singh: So I think the super speciality is like -- general superficiality is like you're doing -- you're getting admitted for anything, it's like internal medicine; cold, cough, fever, surgery, the gynecology problem, but super speciality is when you have cardiac, neuro, transplant program or something. So very high end procedures.

You need to have a super speciality because -- and then superficiality will add -- in business perspective, you'll see this -- all this super speciality procedure has a high pricing and then they'll have a good ARPOB and finally it will add in EBITDA. So I think this has to be a good combination because, and this is all interlinked. If a patient who comes in for any speciality he will be going to the physicians, and physicians feel that you have a problem with some kind of cardiac. So you will have to go to the cardiology department which are very specialized and there are lots of you know very specialized test for that. So super speciality has a significant listing and any good set of hospital having 300, 400 beds have to have a good super speciality.

Aashil Shah: Okay. And sir -- sorry, go ahead.

Amit Kumar Singh: No, I hope it answers your query.

Aashil Shah: Sure. Yes, it does. Second question I had was on the receivable side. What is our current amount of receivables?

Deepak Tyagi: Well, receivables, as I said that, it was answered in the previous question also because we have sizable business coming from government side. So, receivables have really gone up. It's around 100 days as of now, because government is a little sluggish in payment. The good fact is that the government has created their information system where we can see that our debts are with sovereign guarantee. So, debt has slightly gone up in this quarter.

Aashil Shah: Got it. So it's 100 days on the government revenue part.

Deepak Tyagi: I'm talking about the overall, sir.

Aashil Shah: Okay. 100 days.

Deepak Tyagi: I'm talking about the overall.

Aashil Shah: Understood. And Jhansi quarter-on-quarter, the occupancy has not increased considerably. It's only a 1% increase, although we are barely 20% occupancy. So just wanted to understand, was there any one-off and what are our plans to ramp, how are our plans to ramp that up faster?

Yatharth Tyagi: So Jhansi is doing pretty well in terms of both ARPOB. It has also increased there. And occupancy, as I said, overall, in the quarter was down for the whole group. So, nothing specific to Jhansi. As we mentioned about the seasonal dip in this quarter across healthcare industry. So that was also the impact on Jhansi.

But other than the seasonal dip, the overall other KPIs at Jhansi are really improving and we are quite happy with the progress there. In fact, we have, you know, placed a new management team there, including the marketing head, and we are quite happy of the working of the latest quarter from there.

Aashil Shah: Got it. Thank you, sir.

Moderator: Thank you very much. The next question is from the line of Prerana Amanna from Pnars Partnership. Please go ahead.

Prerana Amanna: Yes. Hello. Can you guys hear me?

Yatharth Tyagi: Yes.

Prerana Amanna: Yes. So, first of all, congratulations on a good set of numbers. So, last quarter we spoke about empanelment. We are looking for empanelment from railways and insurance for Jhansi hospital.

What has been an update on that? And also, the land case. I understand for the land case, we have got a stay order, but could you throw some more light on that?

And I think our receivable also last quarter was up and we were talking about how the -- there was a shift in the payment s

tructure. So I believe from the previous, I think the query you answered for one of the analysts, I believe that hasn't changed much.

Yatharth Tyagi: So, about the Jhansi hospital government panels, we have received now more empanelments in the latest quarter and we have recently received ECHS, which is the the ex-army people empanelment and Jhansi area is a hub for Army Cantt. So we have received this empanelment in the latest quarter. We have also received more healthcare insurances in Jhansi and this is the result why there, you know, we are seeing a good increase there as well.

And about the fact that on the receivables, I think Mr. Deepak can take that one.

Deepak Tyagi: I think I answered it twice, Ms. Prerana. And I reiterate that debtors have slightly gone up in this quarter, all because of the government thing coming in, government setup. But our receivables from others, that is the corporates and insurance, that's pretty much under control and coming down over a period of time.

Yatharth Tyagi: And just lastly, taking your question on the Jhansi land case. So as we have already mentioned earlier, we have received the official stay on that letter from the Jabalpur High Court of Madhya Pradesh. And the hospital is working as normal. There is no impact, both at the operational or the financial front, because of that. And as we said, we have received the letter from the High Court for the stay of that order, of that earlier order.

Prerana Amanna: Okay, sir. Okay, thank you. So, my last question is, if you look at the tax rate, I think in December '22, we paid around INR4 crores and now we are paying INR14 crores. Although I

understand the tax rate is somewhere around 30 percentage. So, did we -- that 12% tax we paid last year, was it like a one-off only for that year, last year, December '22?

Deepak Tyagi: First of all, correction. What the tax cost you were saying, ma'am, it's the ultimate tax cost coming in. It's not an outflow completely. First of all, what P&L you were saying is the tax cost, not the payout. Because what happens, ma'am, that when we receive money with insurance or government, they usually deduct our 10%. So, our deduction of tax is sufficient. So, it doesn't go cash out of our bank.

Now, second question is about the impact. Now, impact is basically that the, three operational companies we have, that is Yatharth, AKS Medical Resource Center and RamRaja. So RamRaja, there is no tax, because we have tons of accumulated losses coming in, and we will be not paying any tax, except out of that.

Rest of the two companies, this is the conservative approach we have taken up that this would be the next kind of thing. I expect that with the unfolding of things, we will see little reduction in the next quarter on the tax cost coming in. Very conservatively, we have posted these figures.

Prerana Amanna: Okay, sir. Thank you so much for the detailed reply. Just one last question is, I don't know if I heard it right from the previous questions you answered. You told that it will take two years for the new hospital, that is the Asian Fidelis, to break even the EBITDA margin, but our current EBITDA margin will be solid, like it won't affect. Did I hear that right?

Deepak Tyagi: You heard it very absolutely and very right, ma'am. Because, see, you have to see the scale of operations. Now, scale of operations, as I mentioned, and as Mr. Yatharth Tyagi mentioned, that our flagship on ARPOB and on EBITDA side is being overtaken by Noida Extension hospital where we are having around 35,000 of ARPOB and very close to 30% of EBITDA coming in. There the volume and the revenue ramp-up is coming in, and so is from Greater Noida, and so is from Noida. So, this EBITDA, what we are seeing as of now, will be driven by these hospitals. Where the new hospital, even if making slightly -- s

light losses on EBITDA side, would be
compensated by these hospitals, and we will be maintaining these EBITDA margins, expected to maintain these margins in future.

Prerana Amanna: Okay, sir. Thank you so much for answering all the questions, and wish you a great luck again for the next quarter. Thank you.

Moderator: Thank you very much. The next question is from the line of Siddharth Agarwal from Aasn Capital. Please go ahead.

Siddharth Agarwal: Hi. Good afternoon, everyone, and thank you for giving me the opportunity, and congratulations for a great set of numbers in what is seemingly a weak quarter. So, my first question is around a process of Faridabad acquisition. Could you please elaborate a little bit what kind of ARPOB can we expect in this particular micro-market where we have this hospital? Will it be similar to our Noida hospital?

Yatharth Tyagi: So, yes, I mean, in fact, I would say the ARPOB in that region is definitely because it's part of Delhi NCR. So, the ARPOB would be similar to what the greater Noida regions have. But also, I do feel that it will be more than our facility to talk about at Noida hospital, because in Faridabad, especially in Fidelis Hospital, we will have a good portion coming from a superficiality is there, which will be starting very soon.

So, somewhere between, I would say, between our Noida Extension and Greater Noida hospital should be roughly the ARPOB for the Faridabad hospital.

Siddharth Agarwal: And you also mentioned that we expect close to optimum utilization for our Noida Extension hospital in 2025. So, do you think current occupancy is closer to 42%? And where can we reach there in the next two years? If you can give some trajectory of expected utilization in both greater Noida and Noida for the next two to three years.

Yatharth Tyagi: Yes. So, as I said that with the radiation oncology coming up from the March, so we do feel that the occupancy will shoot up there. And also if you look at the existing occupancy of the coming month, existing month, so it is already higher than the last quarter average. And we do feel that we are well on our path towards optimal utilization in that.

And similarly, on the greater Noida. Greater Noida will, we feel that which will reach the optimization bit faster than there. And that is why we have bought the adjacent land passes in both of these two hospitals where the capacity will be utilized.

Siddharth Agarwal: So, when do we plan to kick-start the downfield expansion here? And any estimate of the capex that will be required to expand to 700 and 600 beds?

Yatharth Tyagi: Yes. So, I mean, the capex amount we have already mentioned in our previous calls. We do expect both greater Noida hospital and Noida extension expansion, the capex per bed should be around, you know 70 lakhs for that, roughly. And with the expansion plans, we are going to start very soon now.

As I mentioned that this time was very cold winters here in Delhi NCR. So, I think now with the weather improving, the construction will kick-start very soon.

Siddharth Agarwal: Great. And today between Greater Noida and Noida Extension, both of them have ability to create, treat or support critical diseases and tertiary diseases. So, where do you see, I mean, the difference between their ARPOB convert at some point or especially with ongoing investments in the Greater Noida as well? So, how does this trajectory look like to you?

Yatharth Tyagi: I think both greater Noida and Noida extension are seeing good increase in the ARPOB. And this is to credit to the fact of the increase in the super specialties. In fact, if you look at it from quarter-on-quarter also, our ARPOB has significantly increased. Yes, year-on-year we have increased very high, but even quarter-on-quarter, there was significant increase in the ARPOB. This is in line with our approa

ch to build much more on the super specialty front.

And, of course, in the radiation oncology starting in Noida extension, the ARPOB will increase much more further in Noida extension and in greater Noida, because greater Noida hospital is

very close to the Jewar airport which is going to start at the end of this year. So, international patient share will be quite high in greater Noida hospital. So, I think that will be a big driver for the ARPOB increase in greater Noida also in the coming years.

Siddharth Agarwal: Great. Thanks a lot, Yatharth and wish you all the best to the entire team. Thank you.

Moderator: Thank you very much. The next question is from the line of Priyank Parekh from Abakkus Asset Managers LLP. Please go ahead.

Priyank Parekh: Yes. So, I have a very basic question on our business. So, sizable revenue of our business comes from the internal medicine. Can you throw light on what exactly this specialty is and what are the growth drivers?

Yatharth Tyagi: So, internal medicine consists of the department of internal medicine, which comprises of communicative and non-communicative infectious diseases and general lifestyle diseases like cough, cold, fever, even dengue and hypertension. This is what is treated in this branch. And it is the physicians, the physician doctors who are part of the department of internal medicine. And it was high earlier in our brand and now, as I also mentioned earlier, it is decreasing in our revenue share because our other case mix of super specialties are increasing. So, I think the department of internal medicine is a basic necessity for any hospital to have. And initially, when patients come, it is usually it's where the patient is identified.

Okay, if they have a cardiac problem, if they have a gastro problem, and then they are referred to different departments. This is how internal medicine functions. And with the course of as already I mentioned, our reduction in the revenue -- our percentage term is going down and our percentage of other super specialties are increasing.

Priyank Parekh: Okay, got it. So, in terms of competition, we must be competing with local dispensaries and secondary care hospitals for this internal medicine rather than the large teams. Is it the right way to see it?

Amit Kumar Singh: You need to understand one thing, even a patient is coming in internal medicine department, but internal medicine department should be supported with a very high end other department radiology, high end pathology, and then other super specialties in the same. So, patient's preferences will always be to the place where all those super specialties, apart from specialties, super specialties are there.

So, that is what as such there is no competition from the local nursing homes and other but yes, when a patient chooses -- they prefer to go to the specialty first. And then from there, there is a, diverting to the super specialty depends on their issues. So, there is as such, there is no competition with this smaller unit.

Yatharth Tyagi: Even in this department, our competition would be from other corporate teams that are listed and present in our geography.

Priyank Parekh: Okay. It's just they are not specifically classifying this department separately. That's what we are…Okay. Okay. Got it. Thank you.

Moderator: Thank you very much. The next question is from the line of Amit Thawani, an Individual Investor. Please go ahead.

Amit Thawani: My first question is, can you tell me what kind of, we did about 52% occupancy in December quarter. Can you tell me what was the occupancy in January, February?

Yatharth Tyagi: No, I mean, the January, February occupancy will reveal in the results of the coming quarters. But definitely, I mean, the occupancy is

bound to increase in this latest quarter as compared to the December quarter.

Amit Thawani: Correct. I understand that. But it is materially higher. I mean, is it?

Amit Kumar Singh: Yes, Amit. It's materially higher. As I said, it was a winter effect. So, as winter is slowly going off, I think the occupancy is bound to increase.

Amit Thawani: Okay. Okay. And any guidance if you can give on margins? Are the current margins sustainable or are the margins also going to go higher?

Deepak Tyagi: So, Amit, as I mentioned that definitely for this year we are almost on close. Just one and a half months left. So, expect that the similar kind of margin we would be, we should be ending as well. In the future years also, I don't want to get into and give any guidance on the future years. But knowing the trends and the way we are increasing, we are very, we are very hopeful and we are very much certain that the current margins what we are putting up should be sustainable in the future as well.

Amit Thawani: Got it. Got it. And my last question is, are we breakeven yet on Jhansi Orchha or can you tell us what the margins on Jhansi Orchha are right now?

Deepak Tyagi: Jhansi Orchha the operation is very small and it's not much significant, but Jhansi is very significant in our overall strategy. We are EBITDA positive there. I think we are around 6% to 7% EBITDA positive there, Y-T-D basis.

Amit Thawani: Excellent. Excellent. Excellent. Thank you, very much.

Moderator: Thank you very much. The next question is from the line of Naman from Perpetuity Ventures, LLP. Please go ahead.
Naman: Thank you. Thank you for the opportunity, sir. Just one question. When we see our operational beds versus bed capacity, we have 1,405 beds capacity which we report. So what could be the operational bed number in it? Because I think one of the issues is the 90% efficiency, which you do on one of the Noida hospitals is on operational beds. So what would be the operational bed versus the bed capacity number?

Yatharth Tyagi: All our 1,405 beds are operational beds so that's, that. And even at Noida, the operational bed is 250 beds, out of which the census bed would be around 210 and 215 approximately. And on that we have 90% occupancy.
Naman: Okay. And for the remaining hospital, what would be the census bed?
Deepak Tyagi: I think that is information we can share with you. I think it's already, it's a detailed information and available on the website also. If you want it can be answered specifically, but otherwise it's available in public.
Naman: Got it. Thank you.
Moderator: Thank you very much. The next question is from the line of Nirvana Laha from Badrinath Family Office. Please go ahead.
Nirvana Laha: Thanks for giving me another opportunity, I wanted to understand how the decision for Jhansi was taken and what factors were considered before you decided to open up in Jhansi. Because even Western UP, I think is a very big market, right? It is like Agra, Bareilly, Moradabad, etcetera, Pathari. And how did we decide on Jhansi?
Yatharth Tyagi: I mean, Western UP is also something which excites us and we will also consider any opportunity that comes up in Western UP. And when it comes to Jhansi Orchha, which is technically in Madhya Pradesh, Orchha is a town in Madhya Pradesh. When we inquired about the property that was available to acquire there and the nearby area and the demographics, we felt that there's a huge catchment that was coming all the way to Delhi NCR because there are not big hospitals around that area.
In fact, our Jhansi Orchha Hospital would be probably the biggest super-speciality hospital even up till the kilometer radius of even up to Gwalior and the whole Bundelkhand area. And there's a huge potential there and this is why we went there and we are well in line with the strategy to expand across Nort

h Indian Delhi NCR markets.
Nirvana Laha: Okay. So, in terms of ramp-up in another one year, where do you see the ramp-up being for Jhansi Orchha and maybe what kind of EBITDA margins that you already are projecting?
Amit Kumar Singh: I'm not going to give you a specific number, but if you see them quarter-on-quarter, these two and all those KPIs are increasing. It's even a quarter two to quarter three, it has increased.
Yatharth Tyagi: Yes, and at Jhansi, we do feel that current occupancy when in the coming quarter and the coming year would be much and would be materialistically higher and we do feel even the margins there will improve over time.
Nirvana Laha: Right. Because, correct me if I'm wrong, in like tier two, tier three, maybe I'm not sure what Orchha is, but in those kind of towns, when a known name goes in, typically, isn't it the case that the hospital is able to do 50% kind of capacity utilization within like two years?

Yatharth Tyagi: Yes, I mean, that's probably a right estimation, but I think we would be reaching that capacity utilization, probably from now until maybe a year and a half, for sure, will be roughly around that occupancy level. Much earlier, I think, roughly a year, yes.
Nirvana Laha: Okay. And one more question, overall across your hospitals, what is the OPD to IPD conversion rate?
Amit Kumar Singh: So, typically, if you look at it, in any hospitals, OPD versus IPD, anything between 5% to 7%, 8% is a good conversion rate. So, this is what I think is there in our setup as well.
Nirvana Laha: Okay. In Delhi NCR, isn't it, isn't the number like much higher, like 15% and 15% in other hospitals?
Amit Kumar Singh: Answering in a very one sentence would be difficult. It totally depends on where you are, what all the services you are offering. If it's more of the super-specialty, this percentage definitely increases.
So, any hospital is doing much more super- specialty business, this OPD to IPD conversion is always higher. So, there is no specific answer to it, but yes, this is what the range, what I told

you is about the range it goes. But if you do more super- specialty, this percentage increases.
Nirvana Laha: Got it. So, in your Noida Extension, I think you do the most super- specialty, right? And that's why you're planning medical tourism and organ transplant, etc.
So, can you give us the figure for that hospital?
Amit Kumar Singh: Yes, in Noida Extension Hospital, yes, we do maximum super- specialty business. So, there the percentage would be much better. But similarly, I mean, in the near future, if you see other hospitals also, if you see the numbers are much better in greater Noida and particularly now, what are expectations for Faridabad as well.
Yatharth Tyagi: In Noida Extension, we are doing roughly around 60% of the business from super-specialties and this is increasing significantly even in the coming quarter and we are well in line for that.
Nirvana Laha: Right. So, is it possible to give the OPD-IPD conversion rate for that particular hospital?
Amit Kumar Singh: We'll let you know. I mean, I think we'll request our team to forward it to you.
Nirvana Laha: Okay. All right. Thanks and all the best.
Moderator: Thank you very much. The next question is from the line of Ashil Shah from CJ Investment Group. Please go ahead.
Ashil Shah: Hi, thanks again for the questions. Wanted to know what is the eventual occupancy, ARPOB and EBITDA margins we can get to in Jhansi and how long we feel we'll take to get to that?
Yatharth Tyagi: So, I think difficult to exact state of figure, but I just mentioned just two questions earlier. We do feel that more than 50% of the capacity utilization in Jhansi can be reached roughly around

one year. And there have been mu

ltiple strategies that we are doing there. When it comes to
ARPOB, yes, ARPOB will be definitely much lesser than the Delhi-NCR market there.
But even if we look at the ARPOB that is there currently, we are increasing that quarter-on-quarter basis and that ARPOB is also bound to increase further as we go forward.
Ashil Shah: And EBITDA margin? Asking because it's a little bit of a different market compared to all the other hospitals we have, including Faridabad. So, just wanted to understand where we can get that.
Deepak Tyagi: Ashish, I mentioned that for this quarter line, Jhansi is already EBITDA positive proposition. So, at this level, we have EBITDA positive. Yes, ramp up will come because our cost structure is very variable there. So, with the top line coming in, the margins are expected to increase.
Ashil Shah: Got it. And the last question is on the overall, currently we're at around 52%-55% occupancy across all the hospitals. So, just wanted to know at, we have this plan to double bed capacity.
Wanted to know the thoughts behind why we are choosing to expand the beds so much?
Yatharth Tyagi: So, the occupancy that you mentioned is at the group level and individual hospitals have a different occupancy. So, when we talk about the greater Noida hospital, the occupancy there is, closer to 65%-70% in between.
Yes, Q3 was a dip in occupancy overall, but I'm talking just in general. So, I think we do feel that, in a couple of years' time, this hospital will reach optimized utilization there. Similar to, like we've proven with the Noida Hospital in the past.
So, that's where the capacity, the reason to expand, brownfield expansion in this hospital exists.
And also in Noida Extension Hospital, as we mentioned that, with the radiation oncology coming up, in March, we do feel that, probably in two and a half years, this hospital will also reach optimization, capacity optimization, and that is why we are adding beds. And also, it is not just in line with the, the occupancy and the bed capacity.
When we add any new infrastructure to a hospital, we get it up to date with the latest infra possible. And it is very important to constantly upgrade your infrastructure, your rooms, your patient waiting areas, your OPD areas. So, and we have so far been a very, high bed capacity hospitals in the region we operate in.
And in the future also, with the competition coming in, we would like to continue this highest bed capacity in the region, and this is why also we are, upgrading our hospitals.
Ashil Shah: Okay. Got it. Thank you.
Moderator: Thank you very much. In the interest of time, that will be the last question. I now hand the conference over to management for closing comments.

Amit Kumar Singh: Thank you very much. Thanks for your faith and trust on us. I think we'll keep on delivering good results. Q4, we expect it will be much better as, every hospital. We are also doing great next one and a half months. I'm sure let's meet up after Q4 results out.

Thank you very much.
Yatharth Tyagi: Thank you, everyone.
Moderator: On behalf of Ambit Capital, that concludes this conference. Thank you for joining us and you may now disconnect your lines. Thank you.

Call: May 2024

May 31, 2024

The Listing Department
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Sub: Earnings Call Transcripts

Dear Sir/Madam,

Pursuant to Regulation 46(2)(oa) of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed the transcript of Investors' conference call organized on May 24, 2024 post declaration of Financial Results (Standalone & Consolidated) for the quarter/financial year ended March 31, 2024.

The transcript is also available on the website of the Company: https://www.yatharthhospitals.com/investors in the section of Corporate Announcements.

Kindly take the same on record.

Your faithfully,
For Yatharth Hospital and Trauma Care Services Limited

Ritesh Mishra
Company Secretary & Compliance Officer
Mem. No. A51166
Enclosed as above.

Corporate Office: HO-01, Sector - 01, Greater Noida West, 201306
CIN: L85110DL2008PLC174706

"Yatharth Hospitals and Trauma Care Services Limited
Q4 FY24 Earnings Conference Call"

May 24, 2024

MANAGEMENT : MR. YATHARTH TYAGI – WHOLE-TIME DIRECTOR, YATHARTH HOSPITALS AND TRAUMA CARE SERVICES LIMITED
MR. AMIT KUMAR SINGH – GROUP CEO, YATHARTH HOSPITALS AND TRAUMA CARE SERVICES LIMITED
MR. PANKAJ PRABHAKAR – CFO, YATHARTH HOSPITALS AND TRAUMA CARE SERVICES LIMITED

MR. DEEPAK TYAGI – PRESIDENT , STRATEGY AND FINANCE, YATHARTH HOSPITALS AND TRAUMA CARE SERVICES LIMITED
MR. NEERAJ VINAYAK – HEAD STRATEGY AND INVESTOR RELATIONS , YATHARTH HOSPITALS AND TRAUMA CARE SERVICES LIMITED
MODERATOR : MS. DHARA – SMIFS LIMITED

Moderator: Ladies and gentlemen, good day and welcome to Yatharth Hospitals Q4 and FY24 Earnings Conference Call hosted by SMIFS Limited.

As a reminder, all participants line will be in listen only mode and there will be an opportunity for you to ask question after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing "*" then "0" on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Ms. Dhara from SMIFS Limited. Thank you and over to you, ma'am.

Dhara: Thank you. Good afternoon, everyone. I, Dhara, from SMIFS Limited welcome you all to the Quarter 4 Earnings Conference Call of Yatharth Hospitals.

From the Management side, we have with us Mr. Yatharth Tyagi – Whole-Time Director; Mr. Amit Kumar Singh - the Group CEO; Mr. Pankaj Prabhakar - the Group CFO; Mr. Deepak Tyagi - President, Strategy and Finance and Mr. Neeraj Vinayak - Head Strategy and Investor Relations.

I now hand over the call to Mr. Yatharth for his opening comments and to take it forward. Over to you, Mr. Yatharth.

Yatharth Tyagi: Good afternoon and a very warm welcome to Yatharth Hospitals and Trauma Care Services Limited Earnings Conference Call for the quarter and full year ended March 31, 2024.

I have with me Mr. Amit Kumar Singh - our Group CEO; Mr. Pankaj Prabhakar - our CFO, Mr. Deepak Kumar Tyagi - our President of Strategy and Finance and Mr. Neeraj Vinayak, who has recently joined as the Head of Strategy and Investor Relations. We have uploaded our presentation on the exchange and the Company Website, and I hope you all might have received and have had an opportunity to go through it.

In an ever-evolving landscape of healthcare, Yatharth Hospitals have stood resilient and showcased unwavering commitment to excellence. Throughout the Fiscal Year, our team's dedication, strategic vision and operational efficiency have propelled us forward, enabling us to surpass expectations. Given the swift pace of modern lifestyle, patients increasingly prioritized treatments that ensure safety and swift recovery. I am happy to share that robotic surgeries have now seamlessly integrated into our standard medical procedures, with around 115 robotic surgeries already completed till date. We have indeed taken the lead in conducting the first robotic surgery in the Greater Noida region. Till date, we have installed 3 Da Vinci X Surgical Robots and one Stryker Orthopedic robots across our five hospitals.

This quarter so has performed some remarkable lifesaving medical procedures including abdominal aortic aneurysm repair, EVAR for an African international patient. I am also proud of the 7-hour neurosurgery on a young child for removal of a rare brain tumor expanding 2 kg for 18 excruciating months. Yatharth Hospitals have an unwavering commitment in upholding the most rigorous corporate governance standards, thereby ensuring utmost transparency and accountability to all valued stakeholders. Thus, I would like to highlight that our Company is in the advanced stages of finalizing a prominent big six audit firm and will be appointing them as statutory auditors in the coming time.

Speaking on the performance:

During Q4 FY24, we invite the state-of-the-art Radiation Oncology and Nuclear Medicine Center at our Noida Extension Facility, marking a significant milestone and redefining cancer care services in the region. Equipped with cutting edge technology and comprehensive facilities, including a linear accelerator, Elekta ver

sa HD, brachytherapy, and PET CT scan
capabilities, we aim to provide comprehensive cancer care services under one roof to our patients. With oncology now contributing nearly 10% to our revenue stream in Noida Extension Hospital, I anticipate significant growth in the current Fiscal Year, the establishment of cancer radiation line. With addition to this, robotic surgeries, and expansion of organ transplant procedures, I am confident of a case mix shift towards more specialty treatments which we have already displayed and should further drive higher ARPOB.

With Asia's largest International Airport, the Jewar Airport, expected to commence this year, we have already set up a dedicated team of international marketing professionals to expand their medical tourism business for the group. To cater to these international patients, our Noida Extension facility has undergone some renovations including the creation of a dedicated international patient floor and a lounge designed to provide a comfortable and welcoming environment. Furthermore, we are strategically developing the infrastructure of the additional 200 beds at our Greater Noida location, the focus on catering to international patients. Our team has been organizing facilitator meets and our doctors are constantly going and doing OPDs in countries like Africa, Iraq, and CIS.

I am glad to announce that Yatharth Hospitals have achieved industry-leading growth in revenue and profitability during the quarter and full year ended March 31, 2024. We have delivered stellar results with our revenues surging by 29%, reaching Rs. 6,705 million in financial year 24. This growth trajectory was complemented by a 35% increase in EBITDA reaching Rs. 1,799 million with the margin expansion of 113 basis points to 26.8%, further highlighting our balanced approach towards growth and profitability. Our profit after tax surged by 74% to Rs. 1,145 million during this period. In the Fiscal Year ending March 31, 2024, Yatharth Hospitals experienced notable advancements. Our expansion strategy encompassing both organic and inorganic approaches, saw significant milestone in February

2024 with the execution of a definitive agreement of the complete acquisition of Asian Fidelis Hospital in Faridabad, Haryana, now rebranded as Yatharth Hospitals, Faridabad.

This strategic move with the purchase consideration of INR 1,160 million underscores our commitment to growth through inorganic mode and is in line with the commitment to acquiring one hospital each year through Financial ‘24 to ‘26. This share purchase agreement was completed on March 28th, 2024, and the hospital was operationalized on May 12, 2024, thus expanding our presence in North India region, particularly strengthening our presence in Delhi NCR markets. Throughout our hospital network, we have observed a consistent uptick in occupancy rates, achieving an overall level of 54% for the Fiscal Year and Noida Hospital stands out with an impressive occupancy rate of 89%.

Furthermore, our Greater Noida facility has seen an increase in occupancy to 67% now. We anticipate this facility to achieve optimum utilization in this fiscal. Our Noida Extension has been growing rapidly with current utilization reaching 44% versus 31% in the previous year and this facility will be reaching occupancy level by the next year. Furthermore, our Jhansi Hospital is progressing well with an occupancy rate of 34% in Q4 2024. As we continue to ramp up operations and enhance the services offering, we expect to see continued growth in occupancy at our Jhansi Hospital.

In summary:

The quarter has been marked by impressive double-digit growth across most of our specialties and super specialties. As part of a strategic realignmen

t towards Super Specialty treatments,
we had observed a steady decrease in our share of internal medicine and a positive impact on consolidated ARPOB levels by increase in the high-end Super Specialty departments. Among all hospitals, Noida Extension Hospital registered the highest share of revenue coming from Super Specialties and thus witnessing the highest ARPOB level of Rs. 35,074 during the quarter.

I would like to hand over the call to CFO – Mr. Pankaj Prabhakar for the financial updates for the quarter. Over to you, Mr. Pankaj.

Pankaj Prabhakar: Thank you. Good afternoon, everyone. I am pleased to announce that Yatharth Hospital has achieved industry-leading growth in revenue and profitability during the quarter and full year ended March 31st, 2024.

Let me take you all through the quarterly performance first, followed by our annual performance:

During the quarter, Yatharth Hospitals achieved a revenue of Rs. 1,777.5 million, reflecting a substantial Y-o-Y growth of 24% driven by an improvement across most of our specialties and a higher ARPOB level. Our inpatient revenues were up by 23% Y-o-Y while outpatient

revenue witnessed an improvement of 26% Y-o-Y during the quarter. Our Company reported EBITDA of Rs. 465 million for the quarter, up 21% Y-o-Y while our margin stood at 26.2%. Our profit after tax exhibited remarkable growth of 121% Y-o-Y and 30% quarter-over-quarter to reach Rs. 383 million primarily attributable to lower finance cost during the reporting period.

As mentioned earlier, we are witnessing the full impact of debt reduction which has reduced our finance costs by 93% Y-o-Y. During the quarter, we have secured a debt of approximately 800 million to facilitate the transfer of pending debt liabilities associated with the Asian Fidelis Hospital onto our books. This strategic move allows us to enhance cash efficiency for allocation towards other growth initiatives. That said, we remain committed to prudent financial management strategies and optimize our capital structure for future endeavors. As a result of our strategic initiative, our PAT margin expanded significantly by 953 basis points Y-o-Y and 390 basis point quarter-over-quarter, reaching an impressive 21.6%.

Our average revenue per operating bed, ARPOB for the quarter, was 29, 266, up by 6% year-over-year. The improvement was driven by a consistent shift towards higher complexity procedures observed across various specialties with gastroenterology, pulmonology, orthopedics and oncology emerging as key drivers of growth. Furthermore, our bed occupancy for the quarter reached 57%, making a significant improvement compared to 49% in Quarter 4 FY23 indicating enhanced operational efficiency and digitalization of our facilities.

Let me now briefly touch upon the consolidated full year numbers of the Company:

Our revenue grew by 29% Y-o-Y to reach Rs. 6,705 million. Our inpatient revenue demonstrated a solid 30% Y-o-Y growth while outpatient revenue witnessed an increase of 20% Y-o-Y. Our EBITDA for the full year remains at Rs. 1,779 million, up by 35% year-over-year with margin expanding by 113 basis points to 26.8%. Our profit after tax increased by 74% Y-o-Y to Rs. 1,145 million and PAT margin stood at 17.1% expanding by 443 basis points. With our constant endeavor to shift towards more Super Specialty treatment, our ARPOB for the full year is Rs. 28,571, an increase of 8% year-over-year.

Our Noida Extension Hospital delivered the highest ARPOB of Rs. 33,994 for the year witnessing an improvement of 12% Y-o-Y. Debt occupancy for FY24 stood at 54% versus 45% for FY23. During the quarter, the revenue contribution from government business remained on a higher side at around 40% of our topline, which c

oupled with a rise in
outstanding receivable from government entities amidst election resulted in high receivable during the year end. We are in touch with the department and expect receivables to come down significantly by the coming quarter. Our balance sheet continues to be strong with a net cap position of Rs. 1,556 million. We remain dedicated to deploy these funds strategically to drive further growth and value creation. Our returns indicator remained healthy with ROCE at 32% and RoE at 22% despite an expansion in our capital base. As we progress, we are well

positioned to capitalize on the opportunities ahead and drive sustained value creation for our investors. Thank you for your trust and continued support.

With this, I would like to hand it over to the moderator for Q&A.

Moderator: Thank you very much. We will now begin the question-and-answer session. The first question is from the line of Pujan Shah from Molecule Ventures. Please go ahead.

Pujan Shah: Sir, my first question would be on the Jhansi-Orchha Hospital, just wanted to know in terms of ARPOB, if you are looking on a Y-o-Y basis, we have seen a degrowth of 30% while occupancy has improved at 32%. Just wanted to know what are we are visioning about this hospital and how we have been evolving and what our expectation limits to for next 1-2 years?

Yatharth Tyagi: Jhansi-Orchha Hospital, the occupancy has shown good growth and if we talk about ARPOB, ARPOB is increasing because some of the government scheme business that we are doing in this hospital that we recently got aggregated with. So, in the last earnings call, we mentioned that some departments like ECHS, Railways and Ayushman have been onboarded in Jhansi-

Orchha Hospital and with the occupancy of those government schemes increasing, ARPOB in those departments tend to be lesser. That is why there is a dip in the ARPOB, but overall, we are happy with the increase in the IPD volumes there and we feel that in the coming quarters this hospital in terms of occupancy will rapidly increase also.
Pujan Shah: So, any broad idea about what ARPOB we can achieve over this specific facility and what we can guide about in next 1-2 years for this facility?
Yatharth Tyagi: So, I think ARPOB should be on an increasing trend in a year to couple of years in this facility, the reason being that now as we have these government panels on board, now it is also giving us the chance to fully operationalize our Critical Care and ICU beds and now all the doctors there are complete. So, we feel that in the coming quarters as our volume of critical care patients in Jhansi increases, the ARPOB should increase much further.
Amit Kumar Singh: And also, just to add if you see the super specialties businesses are growing in Jhansi. So, this year much more numbers will achieve in terms of Super Specialty which is going to add in our ARPOB, so anything by this financial year, I think you will see a good ARPOB by the second or third quarter probably.
Pujan Shah: And on the commentary, we have said that we will try to add one hospital or one facility each year by FY26. So, that is what let us suppose if we conclude we can say that we will add two more facilities in the upcoming two financial year. So, any broad idea on what our CAPEX would be and what cost per bed we are eyeing in terms of and third, which area we have been looking to for acquisition of new facility?

Yatharth Tyagi: So, we are looking in the areas of primarily North India and that too if I talk about, outsi

de of
Delhi NCR market and Uttar Pradesh, Madhya Pradesh, Haryana, these are the areas where there is still a lot of potential we feel as the population is growing, urbanization is happening in these areas, there is a huge scope for good or Super Specialty hospitals and there are few hospitals that we are eyeing for acquisition in these areas. As far as the CAPEX per bed is concerned, see if you look at it recently, we acquired the hospital in Faridabad, and it is when we expanded and we add medical equipments to it. So, we are paying roughly around Rs. 150 crores for a bed capacity of close to 200 beds, so coming to the CAPEX for bed of 60 lakhs to 70 lakhs roughly. So, this is what we feel would be a similar CAPEX per bed if we want to acquire an existing facility. But then obviously, there are some options that are available on O&M on lease model that is something also, we are exploring.
Pujan Shah: So, just wanted to get a brief idea on the new acquisition of the 200 beds of Faridabad. So, if we consider the total cost is Rs. 116 crores plus the machinery expense, which we have spent. So, on Rs. 116 crores, it seems a bit cheap compared to when you are looking at the similar facilities in the same area, so we get a very good deal about this thing, so just wanted to know how was the hospitalized structure well before we have been acquired to and what we need to be get ramped up on and or more than Rs. 150 crores we need to spend anything else for this facility to be keep running on?
Yatharth Tyagi: No, I think that is the total amount, but I don't think Rs. 150 crores is the right amount that we feel for overall CAPEX plus the purchase of this hospital. And primarily that involves the robots that we are putting up. In fact, last week itself this hospital has been commenced operations and we have already placed the Surgical Da Vinci X Robot there. So, most of the CAPEX is being done there and some little is pending, and we feel that this hospital should ramp up very well. Faridabad in itself being a huge hospital market, the drainage from the surrounding areas including Mathura, Palwal, Sohna is huge in Faridabad. So, we feel that it is a good catchment area plus this hospital already have healthcare insurances and some of the government empanelments from day one. So, that is really helping us to ramp up the occupancy in this hospital in the coming quarters. And another point is the doctors that are available in Faridabad and the doctors who have already joined us in this hospital, when they perform operations this month itself, these are some star doctors that we feel have a good clientage along with them. So, we are very optimistic about this hospital, and it should ramp up in occupancy very soon.
Moderator: Thank you. The next question is from the line of Deepak Singh from Barclays. Please go ahead.
Deepak Singh: So, I just wanted to ask like for the next 3-5 years, can we expect this revenue like 40% which you are growing for last 40%. So, can we expect for the next 3-5 years?

Yatharth Tyagi: See, we have had a revenue growth of 29% year-on-year basis and EBITDA percentage of 35% growth year-on-year basis. We feel quite confident in sustaining this growth for the years to come. We have occupancy ramp up yet remaining. We have ARPOB that is growing on even quarter-on-quarter. So, we are quite confident of maintaining the growth margins that we have shown in the last few years in the years ahead to come also.

Deepak Singh: Are you planning to open a hospital in Bihar somewhere there or you are targeting only Delhi NCR and Haryana?

Yatharth Tyagi: So, we are looking at North India, so that includes Uttar Pradesh and on the border comes Bihar. If there is a good

opportunity there, it does sort of come within our territory of expansion. We will look into it. We have some leading doctors who belong to those regions. So, if there is good opportunity available there for acquisition that is something we will look out for.

Moderator: Thank you. The next question is from the line of Sumit Gupta from Centrum Broking Limited. Please go ahead.

Sumit Gupta: Sir, I just want to understand about the receivables part, so it is short by a lot this year, so just want to understand a bit on that?

Yatharth Tyagi: Yes, receivable has gone bit up and the reason being that as Mr. Pankaj mentioned in the commentary that there has been increase in the government business, so earlier it was close to one-third and in the latest quarter and now for the fiscal it has gone to roughly very close to 40%. So, with the government business increasing and also some delay from the segments that we receive money in, that is the reason receivables have gone up and finally if you look at it, these are three government schemes. So, one is ESI, second is ECHS and third is CGHS. Primarily, the debtors that we have are in these 3 government schemes and yes, maybe because of the elections here, maybe because of the coincidence in Quarter 4 the payments have been slow from the government side. But we feel as the government has displayed in the past that in the coming quarters this should normalize very soon. So, that is the reason for the receivables going up.

Sumit Gupta: So, just want to understand what the payables over the next 3-5 years. How do you see that panning out, government scheme coming down or insurance catching up? So, just want to understand on payer mix?

Yatharth Tyagi: See, we have also mentioned this earlier that we are focusing much more on increasing our ARPOB, focusing on the payer mix and optimizing that is not something we are thinking right now as long as our ARPOB is increasing and another reason why the government business has increased recently is because of oncology starting. We have started radiation oncology and so that has also led to increase in our medical and surgical volume. And oncology has good rates

in CGHS and government. So, this is why it is okay to take government business when it comes to oncology and high-end super specialties. So, as long as we are increasing business in those super specialties like oncology, like transplant and ARPOB is increasing, we are fine with the payer mix, but yes, we also do expect, our healthcare insurances and cash patient segment to increase at a faster rate given the brand recognition among this area and given the leading star doctors that are now in the pipeline to join us from the coming quarter onwards.

Sumit Gupta: Overall, if I compare OCF to EBITDA conversion with the peers, it is lower with respect to the open like big players in the Delhi NCR region only, so just want to understand on this part why the EBITDA to OCF conversion lower?

Deepak Tyagi: Well, one thing, because the cash accumulation is low one reason, Mr. Tyagi just explained to you because there is a significant investment going on the debtor's side. But if you are talking about the margins, the margins are pretty decent. Year-on-year basis, we are increasing, we are close to 27% now, which is around 200 basis points from last year. So, yes, your question is right that how the EBITDA is turning into the cash accumulation, so that is basically because in this year, not a happy situation, we are not liking it, but Mr. Amit Singh and Mr. Tyagi are working very hard to have these amounts realized as early as possible, but major most investment of our operating cash flows are getting on to the debtor side and that is why you are

seeing that the cash accumulation is slow.
Sumit Gupta: And sir, over the next 3-4 years or at least 2-3 years, how much EBITDA margin expansion do you see?
Yatharth Tyagi: EBITDA margins, we are quite good in terms of EBITDA margins are concerned and we expect the margins to be sustainable because even though new hospitals are coming up, but there is still lot of occupancy growth that is going to happen in our existing hospitals of Greater Noida and Noida Extension. So, overall, EBITDA margins are quite sustainable even in the next 3-4 years that we have currently.
Sumit Gupta: So, the last question on the ARPOB, so basically you are saying you are not focusing on improving the payer mix, it will just be on the basis of case mix as well as introduction of new robotic surgery. So, that will lead to growth in ARPOB, is it fine?
Amit Kumar Singh: Yes, it is absolutely fine. So, as long as it is not hitting us badly, see if you look at it, we have a bed capacity and then month-on-month, our super specialties are growing. So, I think we are okay with this current mix as of now.
Moderator: Thank you. The next question is from the line of Tushar Manudhane from Motilal Oswal Financial Service. Please go ahead.

Tushar Manudhane: Sir, just on the Noida Hospital per se, we are now almost at 89% occupancy and even the Greater Noida we are at like 67% occupancy, so any scope to add more in terms of ICU beds so that we are able to have better realizations in these hospitals?
Amit Kumar Singh: So, first, we will take the Noida as it is 89% of occupancy, so first thing is that there is no bed capacity available that we can add and there is no space where you can increase our ICU bed or any other this thing. The only thing, Noida will give us the growth is about changing the specialty and revenue mix. So, if you look at it, even IPD revenue and OPD revenue, the per person realization has increased and that is what we were working on. Coming to Greater Noida, yes, we are as Yatharth earlier said it in his commentary, that 200 beds we are adding in Greater Noida. So, as of now Greater Noida is still 67% of occupancy, still around 12% to 15% occupancy is there, left inventory is there and then adding 200 beds would give us good flexibility to add more, critical and Super Specialty beds.
Tushar Manudhane: So, considering this and in fact even Noida extension has the scope to expand the bed capacity. So, if you have to factor this then how much of CAPEX we will do in FY25 and FY26?
Yatharth Tyagi: So, the CAPEX that we have taken for the expansion of our existing Brownfields because it is not a Greenfield expansion, it is Brownfield. So, we feel 60 lakhs to 65 lakhs per bed CAPEX is required in terms of our both Noida Extension and Greater Noida Hospital coming up. So, if 65 lakh approximately CAPEX per bed for 450 beds so that is there and then yes, again for acquisition, as I mentioned earlier that CAPEX would be somewhere around 70 lakhs per bed that we are looking out for depends upon the number of beds of acquisitions that we are going to add.
Tushar Manudhane: And lastly, just sir, what kind of valuations if at all you acquire, first of all, on the strategy in terms of acquiring like in terms of earlier cases, you have done acquisition where the hospitals were sort of built but were available at a very good or a compelling valuation. So, how do you think of acquiring subsequently now in the coming days or now you will have to pay premium?
Yatharth Tyagi: So far, we have acquired at a fairly reasonable valuation. And when I said that somewhere around 60 to 70 lakhs per bed CAPEX is what we look at while at looking to acquire a hospital, so that sort of gives the fair value estima

tion, but see it is for us also depends a lot on the area. And what is the catchment? It is just not that we are looking at a certain valuation that is what propels us to do an acquisition, it has to be fit in the overall strategy and we have always had this strategy. We have been present in the area which is growing, where the population is growing and which is catering to outskirt and nearby huge drainage areas. So, that is also key and this is why Faridabad we feel as a huge drainage, this area of Greater Faridabad is coming up with a lot of societies population increasing there. So, there are other factors that also goes into when we decided acquisition, not just the valuation and we feel if those factors are catering to play, we can always look to increase and decrease certain valuation depending upon the geography that the hospital is located.

Moderator: Thank you. The next question is from the line of Dhruv Bhatia from AUM Fund. Please go ahead.

Dhruv Bhatia: Sir, congratulations on a good set of numbers on the P&L, but the balance sheet doesn't look too good because if I analyze between 23 and 24, your sales have gone up by Rs. 150 crores, but your debtors have gone up by Rs. 120 crores, so the operating cash flow after working capital changes is 0, though you have a very healthy EBITDA, in terms of cash collection is almost negative from operations after working capital. So, I am not understanding that even if I take 33% of your last year's sales going to the government where you have a receivable issue and this year it has increased to 40% that the sales to government have increased by Rs. 120 crores and the debtors have also increased by Rs. 120 crores. So, basically for your incremental government sales, you have not collected at all. Is that the way to understand it?

Yatharth Tyagi: No, definitely there has been an increase in the government business and some delay from those payment side. That is one aspect and also the other, the total debtors we have that also includes some private insurances also, some TPAs. So, overall is what you see is yes, a minority would be in the private insurances side, a majority would be on the government side, but that is how it is coming up.

Dhruv Bhatia: Yes, but sir, the P&L is looking very good, but on the operating cash flow side, the performance has been pretty poor given that almost the entire incremental sales, Rs. 120 crores has become debtors, can you provide us some sort of aging for these debtors, they are due for how long, your Rs. 220 crores odd of debtors?

Yatharth Tyagi: See the total debtors we have, as I earlier mentioned, these are primarily from three government schemes, ESI, CGHS and ECHS. These three government schemes have a UTI portal which is an online website where each details of each patient is visible with the exact amount and on the UTI portal, there is also mechanism where you can see at what stage the government is checking the bill and what stage the funds are about to be released. So, there is usually Stage-1, Stage-2, Stage-3. All these three stages are available on the UTI website for our debtors to see. And we see that with the oncology business increasing that is and compared with the slow payment from the government, it has increased, but it is only a Quarter 4 reflection which is reflecting in the overall yearend balance sheet. As we proceed further with the new financial year coming up with quarter 1, quarter 2, these debtors have significantly come down as always does with these three government schemes plus with the additional of cash and insurance revenue increasing. We are quite confident in the coming quarters, the revenue that is being generated in terms of the cash flow wo

uld be at an optimized and at a good level.

Deepak Tyagi: Dhruv, good observation, but numbers are to be seen. See, you are saying that there is an increase from Rs. 108 close to Rs. 227 crores, but you have to appreciate that in the same time the revenues also grown up by 30%. So, even if you take the same thing that number should have been anywhere between 145-146 kind of thing. So, if you deduct that it would be in the range of, I am not saying that the debtors have not gone up but saying that the entire revenue generation has gone into that, that is not correct. So, there is a question that what is this Rs. 80 crores kind of thing is coming and as Mr. Tyagi mentioned that there is a slackness from the government side and it happens. At times when the government comes, and they release the fund and immediately the debtor inventory comes down. We are cognizant of the facts and we are basically putting it across very clearly that we are working very hard and we are trying to bring this down and there is a significant effort on our topline side, on our profitability side and the same impetus we are putting onto the debtors collection as well. But the debtors’ increase is to be seen in the light of the increase in the revenue as well. It is not in absolute numbers only.

Dhruv Bhatia: No, sir. I have looked at that Rs. 150 crores sales have increased, and debtors have increased by Rs. 120 crores. I am looking at the increase as well and that is why operating cash flow after working capital changes is negative?

Yatharth Tyagi: Yes and as we said that in the coming quarters, operating cash flow will increase as these debtors will come down and we are in contacts with those departments. So, you will see a significant change in the cash flow from coming quarters ahead.

Moderator: Thank you. The next question is from the line of Prerana Amanna from PNA Capital. Please go ahead.

Prerana Amanna: So, my first question is what has been the payer mix for FY24?
Yatharth Tyagi: So, the payer mix, as we mentioned that the government category is very close to 40% now in terms of the total topline and the remaining is then equally divided between cash and TPA segment. So, somewhere around let us say 30%-33% for both cash segment as well as the private insurance segment.
Prerana Amanna: And my last question is I think last quarter we saw that the Supreme Court was talking about standardization of rates in private hospitals. So, how do you see this like is it going to benefit us or how do you see this if and so if this law is implemented, how do you see this going forward?
Amit Kumar Singh: So, this is already, the body of the private health segment, AHPI and Net Health has already made the representations and I think recently if you have heard that the Supreme Court had the observation about the PIL filed by Ophthalmic Society of India, then said that it is not practically possible to have same rate for across the super specialties and across this thing. So,

it is a matter going on and we are pretty confident that observation will not be applicable across same rate for across this thing, so we are not worried as such.
Moderator: Thank you. The next question is from the line of Pratik Poddar from Bandhan Mutual Fund. Please go ahead.
Pratik Poddar: Just one question, when I look at ARPOB for Noida Extension on a Y-o-Y basis or let us say even on Q-o-Q basis, the expansion is not material versus the case mix where oncology has become 10%. So, can you just double check and help

me understand as to why that has happened because oncology, I would have thought would be at least 2x of the base ARPOB and to that extent the numbers look slightly muted at least in the Noida Extension side?
Yatharth Tyagi: No, if you look at it, last year the Noida Extension ARPOB was close to Rs. 30,000 and if you look at Q4 for this year, that is very close to Rs. 36,000. So, that is an increase of roughly more than 5000 in that. Yes, in the overall year, the ARPOB looks a bit less than that, but if you look at the Q4 that is where the significant volume of oncology has come up. So, this is why we feel confident and now with the new fiscal and new quarters coming up, this will be much more visible and an increase of Rs. 5,000 of an ARPOB is primarily because of the shift in the case mix.
Pratik Poddar: But when I look at it sequentially, right, it has just gone up from 34,000 to 35,000 and you are saying that in Q4 we have seen a material ramp up of oncology. So, in that case, the ARPOB should have increased further, right, isn't it? And even on a Y-o-Y basis, if I see exit to exit which is Q4 FY23 to Q4 FY24, the number is 32,173 to 35,000. So, it is just a Rs. 3,000 increase, so slightly puzzling over here?
Yatharth Tyagi: No, we feel that this amount of increase, it is significant when it comes to these treatments and then you have to also see in line that when these treatments have recently started the volumes that they have to generate, so medical and surgical oncology is what we were doing earlier right. And that is what the government business is increasing, but when it comes to radiation oncology, that is what boost up ARPOB quite a lot because it is mostly on an OPD basis. That radiation line has just started in Q4. So, once those volumes increase, that is where you also see much more impact, but the impact that we still feel if you compare it to the group average, what we are doing there and this. So, I think we are on the right track as far as the ARPOB increase in extension is concerned and I am quite confident that in the coming quarters this ARPOB in Noida Extension will even touch Rs. 40,000.
Pratik Poddar: The second question is with insurers, have there been any change in card rates? Have you gone on for any increase in FY25?
Amit Kumar Singh: Yes. So, for one of the hospitals, because it is once freeze, there is just two years, the rates are frozen. So, this year we had a negotiation for our Noida Hospital. I think three months back we

did that. So, the impact we will see and next year we will have Greater Noida and Noida Extension.
Pratik Poddar: How much was the increase for Noida Hospital, sir?

Amit Kumar Singh: Totality, I think it is difficult to say because it is 135 odd procedures are being discussed and got the rate. But I think anything between, it will have impact of 7%-8% kind of impact will come and because that there is an annual revision of the rates as well, every hospital does it. So, we have also had some corrections of our own SOC, which will also have some impact.
Pratik Poddar: And just the last question on the debtors, I think it has been asked multiple number of times. Is this slightly seasonal because of elections that you are seeing an increase, or this is a year-end phenomena or how should we think about the increase in debtors? Is this something which will always happen that in Quarter 4 you will see a rise in debtors and then it liquidates in quarter 1, quarter 2 that is the way to think about it, as long as their payer mix remains at 40% from government?
Yatharth Tyagi: No, I think even at a payer mix of 40%, the debtors should come down much lesser and it could, see, as we said in the commentary as well that electio

ns could have an impact on this, but quarter-on-quarter it really depends upon when the government is getting funds. Sometimes, in March itself, some funds are released, sometimes in April. So, it is a very subjective thing the government payout that is supposed to be, but we are pretty sure that this level of debtors that are currently there should significantly reduce in the coming quarters.
Pratik Poddar: And since 45 days have been passed, can we assume that there has been some liquidation to the numbers which have been disclosed as of 30%?
Yatharth Tyagi: Definitely, there has been the reduction in that, but because we can talk about the full fiscal, but in this and that is why I am saying in decrease, you will see in the coming quarter is based on that same thing.
Moderator: Thank you. The next question is from the line of Jayesh Shah from OHM Portfolio Equi Research. Please go ahead.
Jayesh Shah: I think the first question is again on the debtor's issue. What are generally the payment terms for government and insurance?
Amit Kumar Singh: So, for the government, usually anything between 90 and 120 days and that is kind of the agreement, right and for insurance goes around 45 to 60 days. So, this is what on a paper, right, but we have seen it, particularly the government side if you see, once they will start to give you payment, at one go there is so many backlog gets cleared. So, sometimes it is 2-3 months, 4 months delay. We have experienced this in previous years as well. So, I don't think so next

coming two quarters, I think you will see a drop in these numbers right. But yes, I think we are keeping eye on it and everything is working to see this how to introduce this number.
Jayesh Shah: So, are you able to confirm to us how much has got liquidated till now which is 20th May out of Rs. 220 crores?
Yatharth Tyagi: That is something I think would not be right to share on a public call like this. But we know when we are saying that in the Q1 of the new fiscal, there will be significant reduction in the debtor. So, that is yes, there are some equations that have already started happening.
Jayesh Shah: I would request this is a very important key number, so if you share publicly, I will appreciate. Otherwise, I would like to get this offline?
Yatharth Tyagi: We will reach out to you.
Jayesh Shah: Secondly, out of this Rs. 220 crores, how much of debtors are more than 6 months?
Deepak Tyagi: Well for that, often I am not remembering, but yes, because the accounts are being done, so we will be uploading our full balance sheet. We will share that offline with you and then we can speak out later.
Jayesh Shah: Because of this debtor's level that you have on the government business, what is the price difference between what you sell on cash or insurance versus government?
Yatharth Tyagi: So, usually, typically the cash rates are, so let us say private insurance rates are somewhere close to 15% to 20% lesser than the cash rates and then even the government rates from the insurances would be 10% to 15% lesser. So, roughly around, I would say 25%-30% lesser is what government rates are compared to a cash rate. But then, of course that also depends upon the case mix. So, as we said, the reason why we are okay with doing this business now is because our Super Specialty, especially oncology when that increase. In oncology, the rate difference is not 30%, it is much lesser than that. So, that is how the government business works.
Jayesh Shah: And in your own targets, over next one or two years, this government business, will it continue to be between 30%-40% or it should come down?
Yatharth Tyagi: It will come down. See, today we have occupancy to ramp up right, we

can take it, but one if
given a choice between a cash and insurance patient and if you have restriction on the number of beds, then priorities always given to other segments.

Neeraj Vinayak: And Jayesh I would add. So, as of now, we have got negligible share of international business as of now. And we have set up a dedicated team, so there should be traction in that which should automatically bring up the cash category business and reduce the government share.

And we are pretty confident we have hired the best team in that department. So, you will see those results flowing in the quarterly numbers.

Jayesh Shah: Lastly, what is the annual rate hike that you would normally get, is it as per inflation, higher than inflation as to 9%-10%? Because you just mentioned some annual increase that you get in your hospital?

Amit Kumar Singh: So, the cash patient if you see is like as I said it earlier. So, every year there are corrections on the schedule of charges which you do depends on the services and the kind of utilization and also the various competitive analysis, what you do. But I think roughly, so it has various factors when we change it, just not only the inflation. So, I think roughly I would say around 6%-7% can make an impact on the cash patient, 6% to 8% kind of.

Jayesh Shah: And on insurance and government, do you get annual increase or how the rates are being done?

Amit Kumar Singh: Insurance, it totally depends on, there is one to one negotiation happens with GIPSA who does negotiation on your current card rate and your previous GIPSA rate right. So, keeping those two rates in mind and then you have a negotiation, and you try to get, by showing good volumes and others. So, this is how the negotiation happens.

Jayesh Shah: Are the rates fixed for 2-3 years for insurance?

Amit Kumar Singh: Two years, for a GIPSA, for a government insurance Company, for private insurance totally depends on one-to-one negotiation, be it one year or two year. Totally depends on your discussions with them.

Moderator: Thank you. The next question is from the line of Rohan Advant from Prad Capital. Please go ahead.

Rohan Advant: Just one question on the margins. If I look at EBITDA margins sequentially, they have come down a bit in spite of a good sequential pickup in the revenue and that is largely led by, I think the increase in consumables. So, what has caused this? Is this the payer mix that has caused it or something else?

Yatharth Tyagi: So, the margins is a point here and there is not that of a significant difference, but yet, yes there has been increase in the material consumption consumables that is because now we are, as we are shifting towards more super specialized treatments like transplant program, oncology, cardiac, so more consumable cost is required for these super specialized treatment and that is one of the reasons for just a minute dip in EBITDA margin, but we are confident that we still, feel that these EBITDA margins are at an optimized level and these should be sustainable in the quarters ahead.

Rohan Advant: So, next year, we will have the new 200 bedded hospital that we have acquired coming up. So, there could be some upfront cost there, but in spite of that, this 27% kind of EBITDA margins for FY25 you think are sustainable?

Yatharth Tyagi: Yes, quite sustainable because the existing other hospitals that we still have the occupancy to ramp up, there the EBITDA margins will grow. So, in the overall consolidate level group, the margins would be sust

ainable.

Moderator: Thank you. The next question is from the line of Madhav Agarwal from SG Investments. Please go ahead.

Madhav Agarwal: Could you give some guidance on how we are seeing the Asian Fidelis to ramp up during the current year and the next?

Yatharth Tyagi: So, we feel that the ramp up should be very soon there in this hospital that we have just started this month itself, in the month of May and all the doctors are already recruited there. We are just waiting for the cardiac machine Cath lab because that is something it was not having earlier as well as an MRI. So, that is why, we did an additional CAPEX of close to Rs. 25 crores, so only those two machines are remaining to reach the hospital and then the full cardiac as well as the radiology department will be ready there. So, we feel that because the health insurance is already there, some star doctors are there, this hospital should ramp up very soon and we are quite confident about the geographic area about where the hospital is located.

Madhav Agarwal: What kind of occupancy are we targeting for FY25?

Yatharth Tyagi: See, I think the hospital where it is, we already have a strong brand presence there. Faridabad is one area where we are also quite present in Delhi NCR market. So, some of our doctors are from there, some of the patients were already coming to our Noida Hospital so Faridabad. So, we feel the occupancy there should ramp up very soon. Let us say what we feel is what Jhansi is able to do in a couple of years in occupancy, we feel that Faridabad should be there much more earlier than that probably in a year. So, we are quite confident of occupancy at Faridabad Hospital.

Madhav Agarwal: Also could you tell me what has been the improvement in our international patient volume?

Yatharth Tyagi: International patient volume has been increasing given the fact that we have done around 150 kidney transplants and in last year and a half and 90% of those kidney transplants are international patients. We are now also targeting international liver transplant and international bone marrow transplant. There is already infrastructure has been renovated to cater to those patients. There has been new professional marketing team for international patients that have been recently joined us from Delhi NCR Hospital. So, we feel that in the years to come international patients should be contributing close to a double-digit contribution in our overall revenue and we are confident at this time. That is what we are targeting the times ahead.

Madhav Agarwal: What kind of steps are we taking to change our payer mix since we have a significant share of the government business around 40%? What could be our aspiration like 3-5 years down the line?

Amit Kumar Singh: As Yartharth said that these numbers as a government business, it is around contributing 35% to 40% as of now because we have a bed capacity there. So, as these, bed capacity, is scarcity going to come and more and more Super Specialty. See if you look at it what we have said that at the Noida Extension Hospital, the story of around 20-24 months only, and that is the kind of Super Specialty if you see. So, word of mouth is spreading very rapidly that catchment area has grown really far. So, we expect that as we move into the Tier-2 and Tier-3 cities and patients coming from those regions and those patients are mainly if you see this for cash or TPA segment. So, in fact our upcountry marketing and the kind of couple of OPD centers, we are setting up in various parts of the country. So, we believe that this segment is bound to grow and then these two segments will grow and the third segment automatically will go down. So, as of now it is not significantly impac

ting and that is what I think you see this number.

Madhav Agarwal: So, my last question would be, as you mentioned that we are planning to add a hospital every year for a couple of years going forward, how do we plan to fund those, are we looking to take up some debt as well or are we focusing on internal accruals to fund this?

Yatharth Tyagi: So, it will be a mix of both as our finance cost has gone down significantly. So, the internal accruals are more now. So, I think we will be able to fund it mix of that as well as the debt. But I think overall, we still feel because there is a strong cash position also at the balance sheet. So, overall net debt, we will still be net debt free even with the certain little bit of debt.

Moderator: Thank you. The last question is from the line of Sunil from Veekay Investments. Please go ahead.

Sunil: I had a question, in your financial statements, there is a footnote mentioned wherein income tax has frozen Rs. 70 crores of our amount and created a lien on the banks for that specified amount. I would like to know that when was this done and going forward won't it affect our growth since our capital is going to be blocked and for how long do you think would it be blocked? That is the first question.

Deepak Tyagi: Yes. So, we had already informed the Stock Exchange in October 2023, we had the people coming from income tax. So, they found that because we were through with the IPO money and a lot of cash we had been having which was basically for the specified purposes. So, the zeal of things, they had kept this money. Now this money is with us. They have not taken it away. It is still with us. So, it is not that they have taken this money away. Now your second question that when are they going to settle it? We wanted to settle it, we had done our things with them, but interestingly, this investigation team said that they don't have any assessment right. Government procedure, we can't help it. So, we are already with the next stage now on

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the suspend thing and we have been told that because not much of the things were there. So, very quickly, they haven't given the timelines that when they are going to have lost it, but we expect that in this financial year, that thing should be behind our back and this block money will be given back to us. So, at this point in time, it is basically frozen in our account. The account is still we are having, not that they have taken the money out.

Sunil: So, basically the account is frozen, the money lies with you, but you cannot use it?

Deepak Tyagi: We are not allowed to use that much of money. Beyond that we can, pending the full assessment by the income tax.

Sunil: So, would that put stress on the working capital cycle because Rs. 70 crores is.

Deepak Tyagi: No, see the thing is, if you see our overall cash position, we do have much more lying in our cash position. So, working capital is not an issue. Important is that we should settle the queries of income tax as early as possible and bring this money back into our system.

Sunil: And sir, I am aware of the October disclosure you all had made about the income tax checked your premises, but I would appreciate if you could have also put up a disclosure regarding the Rs. 70 crores freeze because, this just came into the financial statements now because it has been quite a few months since income tax?

Deepak Tyagi: It happened post our disclosure so we were not aware that they have done it. We got to know it later. So, because now we were finalizing the account, we thought that it would be that we should apprise our investor in the full light.

Sunil: But you feel that it should be done in this financial year?

Deepak Tyagi: Yes, pretty certain we wanted to

in fact the agencies we have who had done it, they said very clearly that they don't have any assessment power. So, that is why they cannot do the assessment. Otherwise we wanted to close the assessment in this fiscal and have this money back into our channels.

Sunil: So, they have not found anything suspicious you are saying, but they have done it as a procedure. They have just frozen this amount as a procedure.

Deepak Tyagi: Yes.

Sunil: And my second question will be on Faridabad. The OPEX cost will be higher for the next few quarters on this since you must be hiring or must have hired and will it affect the margin in the nearby quarters?

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Yatharth Tyagi: See, any new hospital starting does require a certain investment to be done until it is ramped up. So, as I said that, yes, even though we do feel that margins should be dragged a bit from Faridabad, but overall because the other hospitals are growing, at the consolidated level, it will be sustainable.

Sunil: So, it won't really materially affect your margins going forward?

Yatharth Tyagi: Yes.

Moderator: Thank you. As there are no further questions, I would now like to hand the conference over to the management for closing comments.

Yatharth Tyagi: I thank you to SMIFS and thank you all the people for your questions. We look forward to your next earning call and if any questions are there, you can reach out to us individually and offline and we will be more happy to answer. Thank you.

Moderator: On behalf of SMIFS Limited, that concludes this conference. Thank you for joining us and you may now disconnect your lines.

(This document has been edited for readability purpose)

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Call: Aug 2024

August 22, 2024

The Listing Department
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G Block, Bandra-Kurla Complex, Bandra (E)
Mumbai – 400 051
Symbol: YATHARTH
Dept. of Listing Operations
BSE Limited,
P J Towers, Dalal Street,
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Scrip Code: 543950

Sub: Earnings Call Transcripts

Dear Sir/Madam,

Pursuant to Regulation 46(2)(oa) of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed the transcript of Investors' conference call organized on August 14, 2024 post declaration of Financial Results (Standalone & Consolidated) for the quarter ended June 30, 2024.

The transcript is also available on the website of the Company: https://www.yatharthhospitals.com/investors in the section of Corporate Announcements.

Kindly take the same on record.

Your faithfully,
For Yatharth Hospital and Trauma Care Services Limited

Ritesh Mishra
Company Secretary & Compliance Officer
Mem. No. A51166
Enclosed as above.

Corporate Office: HO-01, Sector - 01, Greater Noida West, 201306
CIN: L85110DL2008PLC174706

"Yatharth Hospitals and Trauma Care Services Limited
Q1 FY '25 Earnings Conference Call"
August 14, 2024

MANAGEMENT : MR. YATHARTH TYAGI – WHOLE-TIME DIRECTOR
M R. AMIT KUMAR SINGH – GROUP CHIEF EXECUTIVE OFFICER
M R. NITIN GUPTA – PRESIDENT , FINANCE AND CHIEF OPERATING OFFICER
M R. PANKAJ PRABHAKAR – CHIEF FINANCIAL OFFICER
M R. NEERAJ VINAYAK – HEAD STRATEGY AND INVESTOR RELATIONS
M R. SONU GOYAL – GROUP FINANCE CONTROLLER

Moderator: Ladies and gentlemen, good day and welcome to the Yatharth Hospital Q1 FY25 Earnings Conference call hosted by Elara Securities Pvt Ltd. Let me draw your attention to the fact that on this call our discussion will include certain forward-looking statements which are predictions, projections, or other estimates about future events. These estimates reflect management's current expectations about the future performance of the company.

Please note that these estimates involve several risks and uncertainties that could cause our actual results to differ materially from what is expressed or implied. As a reminder, all participants' lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touch-tone phone.

Please note that this conference is being recorded. I now hand the conference over to Mr. Kashish Thakur from Elara Securities Pvt Ltd. Thank you and over to you, sir.

Kashish Thakur: Thank you, Neha. Good morning, everyone. I, Kashish Thakur from Elara Capital, welcome you all to Quarter One Earnings Conference Call of Yatharth Hospitals.

From the management side, we have with us today, Mr. Yatharth Tyagi, Whole-Time Director, Mr. Amit Kumar Singh, the Group CEO, Mr. Pankaj Prabhakar, the CFO, Mr. Nitin Gupta, President, Finance and Chief Operating Officer, Mr. Neeraj Vinayak, Head Strategy and Investor Relations Team, and Mr. Sonu Goyal, Group Finance Controller. I now hand over the call to Mr. Yatharth for his opening comments and to take it forward. Over to you, Mr. Yatharth.

Yatharth Tyagi: Good morning and a very warm welcome to Yatharth Hospitals and Trauma Care Services Limited Earnings Conference Call for the quarter ended June 30, 2024. I have with me Mr. Amit Kumar Singh, our Group CEO, Mr. Pankaj Prabhakar, our CFO, Mr. Nitin Gupta, our President, Finance and Chief Operating Officer, Mr. Neeraj Vinayak, our Head Strategy and Investor Relations, and Mr. Sonu Goyal, Group Finance Controller. We have uploaded our presentation on the exchanges and the company website, and I hope you all might have received and have had an opportunity to go through it.

Well, it gives me immense happiness as we deliver yet another quarter of exceptional medical excellence and high-end super specialty treatments at Yatharth Hospitals. Today also marks a special milestone as we celebrate one year of our public journey. Over the past one year, we have reached new heights, consistently delivering outstanding outcomes for our patients, investors, and all stakeholders. Over this journey, we have observed a shift in our brand positioning, an increase in high-end procedures and super specialty treatments across our hospitals. Each of our facilities is now equipped with state-of-the-art infrastructure, including the latest in robotic technology such as Da Vinci Surgical Robots and Stryker's MAKO Orthopaedic Robots and Cuvis Orthopaedic Robots.

It is with great pleasure that I highlight some of the key clinical achievements by our hospitals this quarter. We have recently performed epilepsy surgery on an Iraqi patient, where we removed part of the right temporal lobe of the brain. This quarter, we also performed EVAAR on a Liberian patient. EVAAR is endovascular aortic aneurysms repair without incision. This was

done endovascularly in our Cath lab. The mortality of such aneurysms is usually very high, but we were able to discharge the patient within seven da

ys of the operation. This quarter was also marked by we removing the biggest Atrial myxoma from left atrium. The size of myxoma was biggest and is yet to be reported in international journals.

The growing Yatharth brand is now able to attract renowned doctors and starlets in their fields across NCR regions. To name a few, this quarter, recently we welcomed Dr. Ajay Bhalla as Director of Department of Gastroenterology, a renowned name in Noida region. Similarly, Dr. Pawan Singh joined us recently as Director of Hematology and Bone Marrow Transplant. He is a very renowned oncologist, who has performed more than 1,000 bone marrow transplants across his career in NCR. Similarly, Dr. Bal Krisha also recently joined us in our new hospital in Faridabad, who is bringing with him a reputation as one of the most esteemed surgeons in the Faridabad region. These doctors, these additions, along with certain new additions in our doctor team, are further enhancing our brand and advancing in our centers of excellence.

We also continue to professionalize our management and foster our finance team with addition of three key management personnels this quarter to strengthen our finance function and steadfast our expansion strategy. Our clear strategy focuses on expansion and expanding our footprints across North India, ensuring we continue to lead and innovate in healthcare sector. We have also recently signed an NDA with a leading big six auditing firm, whose team is now actively working with us to ensure streamlining of our finance operations and have already started performing a management audit in our premises. These measures underscore our commitment to robust governance and sustainable growth, reinforcing our dedication to creating a long-term value for our stakeholders.

Now coming to Q1 FY25 performance, I am glad to announce that Yatharth Hospitals have achieved industry-leading growth in revenue and profitability this quarter. We have delivered stellar results, with our revenue rising by 37% year-on-year, while our profit after tax growing by 60% year-on-year.

A key highlight of this quarter is the seamless integration and commencement of operations at our new acquired Faridabad Hospital, which began in May 2024. The hospital was established three years ago with modern infrastructure and has NABH accreditation and is empaneled with all major insurance providers and TPAs. Henceforth, within the first full month of its operations, the hospital achieved an occupancy of 13% in the month of June 2024 and an ARPOB of INR 28,000. We are actively looking to enhance the hospital's capabilities by introducing state-of-the-art services and cutting-edge robotics to deliver superior healthcare solutions to the residents of Faridabad and its surrounding areas. The strategic foresight behind acquisitions highlights our commitment to integrating and optimizing assets, contributing meaningfully to our financial performance.

The quarter also earmarked a significant improvement in occupancy across all our network of hospitals. The occupancy level of our Noida Extension and Jhansi Orchha Hospital improved significantly to 61% and 45% respectively, compared to 37% and 17% in Q1 last year. Despite the addition of 200 beds at Faridabad, our overall occupancy levels of the group reached 61%,

compared to 51% in Q1 last year. As we advance our operations and expand our service offerings, we expect to see a continual increase in occupancy at all our facilities. As we see optimization of our established hospitals in Greater Noida and Noida Extension hospitals, we are all set to start construction at the land parcels acquired adjacent to these hospitals to further expand our bed capacity in this region in the coming 2-3 years.

Our state-o

f-the-art Oncology Center has also begun well, and our organ transplant program and robotics at our hospitals are now driving a shift in our specialty mix and increasing our ARPOB, as well as overall occupancy levels. A strategic shift towards super-specialty treatments has now yielded a shift of revenue contribution from internal medicine to other specialties, with Oncology now contributing 10% to groups' revenues and about 17% for Noida Extension Hospital alone. Consequently, our ARPOB has been on an improving trajectory quarter-over-quarter, crossing INR 30,000 mark in Q1. Notably, our Noida Extension Hospital has led the way with its super-specialty contribution of more than 70%, leading to the highest ARPOB in the group of INR 37,325 this quarter.

Finally coming to debtors, I am pleased to inform you that out of INR 227 crores outstanding as of March 2024, we are able to recover INR 80 crores. Given the growing scale of operations and the net outstanding positions, we are even able to see an improvement in the debtor days, by debtor days coming down to roughly by around 5 days. This trend is on an expected line and reducing debtor days of around 5 days in a quarter is a good improvement and we feel confident on continuing this trend for the coming year and we are expecting that by the end of this year, our debtor days will be close to around 100 days if we continue on this trend.

With this, I would now like to hand over the call to Mr. Pankaj Prabhakar, CFO, for the financial update for the quarter.

Pankaj Prabhakar: Good morning, everyone. I am delighted to say that Yatharth Hospital has once again achieved industry-leading growth in both revenue and profitability for the quarter ending June 30, 2024.

During the quarter, Yatharth Hospital achieved a revenue of INR 2,118 million, reflecting a substantial Y-o-Y growth of 37%, driven by an improvement across occupancy as well as

ARPOB. Notably, our Noida Extension Hospital registered a stellar 81% Y-o-Y increase in revenue, contributing 38% to our topline. Our inpatient volume was up by 28% to 15,235 this quarter, while our inpatient revenue increased by a healthy 38% Y-o-Y during the quarter. Our growth was accelerated by an improvement in our ARPOB by around 9%, standing at INR 30,551 in quarter 1. In addition to the improvement in our case mix, our focused effort towards optimizing our payor mix, as well as improved price negotiations with the private insurance companies, with enhancement of our brand recall, are all adding to this growth.

With the operating leverage, our EBITDA for the quarter improved by 30% Y-o-Y to INR537 million. However, our EBITDA margin this quarter was at 25.3% compared to 26%-27% last year. The minor blip in EBITDA margin for this quarter was on account of operational losses at our Faridabad unit, which became operational in mid-May. Adjusting for Faridabad hospital, our EBITDA margin would have been in the range of around 27.3% this quarter. As we move in subsequent quarters, we are confident that the pace of operational losses at Faridabad will

come down, and we expect marginal loss at this unit by the end of this fiscal, and hopeful to achieve a breakeven within 18 months from operationalization. Nevertheless, our occupancy and ARPOB level at our Faridabad unit encourages us on the potential contribution this facility can make in the overall group's number.

Our finance costs, though came down Y-o-Y, have increased quarter over quarter due to finance costs link to around INR 800 million raised to facilitate the transfer of pending debt liabilities associated with Faridabad hospital acquisition in our books. This deliberate move, however, allows us to enhance capital efficiency for allocation towards other growth initia

tives that may
come in future.

In line with operating profit, our profit before tax grew up by 46% Y-o-Y at INR 430 million, despite an increase in depreciation expense this quarter with Faridabad acquisition, while our profit after tax were up by 60% Y-o-Y at INR 304 million. Sequentially, our PAT margin has come down this quarter due to Faridabad losses and the conservative tax provisioning taken during the initial quarter of the year. However, our group average tax rate should settle at around 25% for this fiscal and hence, we expect this to normalize in the coming quarters.

With a more strengthened finance function and strategic initiatives such as close monitoring of our receivables, reconsideration of doctor targets to collections compared to revenue earlier, we have seen a meaningful recovery in our receivables this quarter. We are confident that when we come back next quarter with our mid-year numbers, we would be in a much better position with respect to our receivables and cash flows. Overall, our balance sheet continues to be strong with a cash position of INR 1,826 million. We remain dedicated to deploying these funds strategically to drive further growth and value creation.

Thank you. With this, I would like to hand over to the moderator for question and answers.

Moderator: Thank you very much. The first question is from the line of Pujan Shah from Molecule Ventures. Please go ahead.

Pujan Shah: So, first question would be on the, as we have mentioned in the PPT, that we are targeting around 2,800 to 3,000 beds in FY '28. So, almost we are doubling our bed capacity. So, can you just mention, as you have already mentioned in the concall, that you have been expanding at your land parcel, which is available near to your hospital. So, could you just help me with how, which is the facility, where the facility has been extended and what number of beds could be accommodated over there? And rest, how we will be targeting the expansion in terms of internal accruals or debt? Can you just give me a broad break up on that part?

Yatharth Tyagi: Yes. So, you know, as you mentioned, there are two adjacent land parcels available in our Noida Extension and Greater Noida Hospital. The Noida Extension land parcel, which is adjacent to the hospital, which is actually behind the hospital, that's where we are adding 250 beds. And Greater Noida land parcel, which is just on the right side of the hospital building, is where we are adding 200 beds. Both these 200 + 250 beds will be available for us to use in 2.5 years, roughly. And that's how we will be adding at these land parcels. Along with that, as you mentioned that, you know, there is much more bed capacity that we are looking to target in the next three years. The rest of the additions to the bed capacity will happen by, you know, means of mergers and acquisitions. And we are already in the pipeline for certain acquisitions. If you look at our journey over the last two fiscals, we have acquired two hospitals - one was in Jhansi-Orchha, and one was in Faridabad. And for this fiscal, we already have plans. These expansion plans are primarily located in the market of North India. So, you know, Delhi - NCR, Uttar Pradesh, Punjab, you know, Bihar. And these are the areas that we want to be at least for the next capacity expansion of around 3,000 beds. As far as the funding of these hospitals are concerned, as our CFO was mentioning, there's, you know, cash available with us today on the books also. Plus, there's also, you know, scope for certain debt, if required at all, we can still take. So, third is from the internal accruals, because our finance cost has significantly reduced after the IPO. So, internal accruals position will be strong in the coming years. So,

by the means
of these three channels is where we will be looking to fund our expansion of the bed capacity.
Pujan Shah: Okay but continuing with the same portion. So, let's suppose this facility, 450 beds will come in 2.5 years, which is greenfield. And ultimately, we will be acquiring. So, even though, if we look at the cash flow generation versus what we can expand it towards the bed capacity, it seems to be more aggressive on that part. So, are we confident enough to achieve 3,000 bed capacity in FY '28? Because the target seems to be a bit stretched?
Yatharth Tyagi: So, you know, we have already started on that journey. When we were 1400 beds post IPO, we acquired Faridabad Hospital at 200 beds. And moreover, the acquisitions I was mentioning also includes O&M model, you know, operation and management model, where the outflow in terms of the asset heavy won't be there. So, you know, that will allow us, you know, we are in talks right now for a 400 bed hospital that is available on O&M. And the capital outflow there would be not much. So, I think, yes, we are quite confident. And it's that we have seen certain assets, which are quality assets that we feel will add value to our group. And that's why we feel that this total bed capacity is possible in the coming years.
Pujan Shah: Okay. And my second question would be on the Faridabad. So, if you look at the payor mix, how the payor mix has been shifted in the earlier when we have acquired the hospital and currently now how we have been looking to change in the mix of payors. So, how we have been looking at the Faridabad hospital and eventually as we also started robotic surgery and specialty surgery over there. What occupancy? Obviously, it would take 18 months to break even and then we will get operating leverage benefits. But how we are looking in this hospital operating ARPOB, occupancy and in terms of payor mix?
Yatharth Tyagi: You see, Faridabad has shown very promising initial numbers. In the Q1, it has just been operational for one and a half month. But, you know, post that also what we've seen the latest numbers, it has shown very good potential. Again, reason being because that is a huge healthcare market. There are a lot of drainage in the nearby villages and there are a lot of societies that are coming around in and around where this hospital is located. So, you know, the initial numbers that you've seen in this quarter, you've already with the opening of the hospital, we're seeing an ARPOB of INR 28,000 without, you know, even the specialty services yet being matured. So, we are quite confident and good, you know, change that we have seen here and in our brand positioning also is from the day one itself, we are going ahead with star doctors in Faridabad.

You know, as I mentioned in my commentary earlier, there's a leading surgeon in the region who has joined us and much more, you know, other specialties including the critical care head, neurosurgery, you know, physicians. We have gone with starlets of those areas which are helping us drive their own patient clientele that is coming to us. You know, and so far, as we see around 85% of that hospital revenue is purely from private insurance and self-payor. So, there's a very good case mix with a very good ARPOB that we've seen in the initial numbers and going forward, we are, you know, sure because of these, you know, leading clinicians who would be driving their own clientele plus the hospital brand slowly growing with the, you know, coming quarters ahead. This will be showing a very good case mix as well as high ARPOB for us.
Pujan Shah: Okay. Got it. And sir, in our facility, we have shown occupancy of roughly around, I think, 86% in the Noida but we've seen that the average length of

stay has been increased by a bit of 5.66.
So, how we have been planning to reduce and improve the upper border?
Yatharth Tyagi: See, I think Noida being our old hospital, you know, started operations in 2013. We feel that that hospital with the limited infrastructure and the small scale that is running on and there's no, you

know, potential to increase that hospital because there's no adjacent land parcel available there. We feel that the hospital is doing fine. We don't want to trigger and change much in terms of ARPOB or the case mix in that hospital. It is our Greater Noida as well as the Noida extension hospitals that have started in 2018 and 2019 which are big infrastructures, bigger land parcels and even expansion scope available there. This is where we are expecting a huge growth in terms of our, you know, ARPOB and in terms of occupancy. And if you see on that front, Noida Extension Hospital in just last one fiscal has grown over 80% yearly and contributed to now around 38% in terms of our overall group. So, these are the hospitals that we are feeling the increase in our ARPOB as well as the case mix.

Pujan Shah: Got it. And one question, just to clarify, on finance, I just missed out the CFO comment. So, what would be the comment on the finance cost? It has been, like there was some saying that it has been increased and after that I missed out the part.

Pankaj Prabhakar: So, we have debt of approximately INR 80 crores in Faridabad acquisition. So, it would be on around 8.5% per year. So, that would be around INR 6 to INR 7 crores on average in a year that would be loaded during the financial year.

Pujan Shah: Okay. Got it. Thanks for answering my all the questions. Let me get back in the queue.

Moderator: Thank you. The next question is from the line of Runit Kapoor from Elara Capital. Please go ahead.

Runit Kapoor: I have two questions. So can you just state what is your current international patient revenue and what is your aspiration regarding that? And also can you give your hospital-wise breakup of the inpatient volumes because as on last quarter you are not disclosing that?

Amit Kumar Singh: So, as far as international patient the number is still, I mean, in the single digit, but I think we are on a very good track. And let me tell you, it's like this quarter probably the highest number of the revenue which is achieved so far from the international patients. And I think we have a

good pipeline for the future. So probably in the coming quarters, you will see a much better numbers as far as international is concerned. What is your next question? What was your next question?

Runit Kapoor: Regarding the inpatient volumes, hello, can you hear me?

Amit Kumar Singh: Yes. So, our IPD and OPD, both numbers have increased significantly. I think that is already there in our presentation. I think IPD has grown from.

Ronit Kapoor: No, I'm asking about the hospital inpatient volumes like you disclosed as on last quarter?

Pankaj Prabhakar: So, during the quarter total IPD numbers is 15,235. That includes for Faridabad, Greater Noida and Noida, all hospitals. Total number is this.

Ronit Kapoor: No, I'm asking you the individual numbers like is it possible to share that?

Pankaj Prabhakar: Yes, we are sharing to you. So for Jhansi we have a total number of 2,785. For Greater Noida, our number is IPD number is 4,555. For Noida, it is 2,967. For Extension I mean, Noida Extension 4,752 and for Faridabad the total number is 176. So only in 40-day operational period Faridabad has kicked off 176 total numbers.

Runit Kapoor: Okay, thank you. That's it from my side.

Moderator: Thank you. The next question is from the line of Nandini Maheshwari from Carnelian Asset Management. Please go ahead.

Nandini Mahesh

wari: So I have two questions. First is what is the ratio of government business? Is it coming down? And the second question is how is the oncology business shaping up?

Yatharth Tyagi: Yes, so oncology business is shaping up quite well. Oncology is now contributing close to 10% in our overall hospital's revenue. And in Noida Extension alone that is where we have the complete oncology setup. This is where it is contributing to 17% and we feel that 17% contribution in oncology is a very good number. We are even hopeful that this contribution will go up. As I was mentioning, some leading clinicians in oncology have now recently joined us including bone marrow transplant which comes under Hemat oncology. We feel that even in Noida Extension, this number of oncology alone can cross 22% as far as the contribution is concerned.

Coming to your first question about the payor mix. So, government business, yes, it is on the lines of going down, as automatically the self-payor and private insurance and international business increases. This government business will go down. And we feel that we have reached maximum capping in terms of the overall contribution of government business which was 40% last year and we feel that the business will come down as the other segments increase from here in terms of the overall contribution is concerned.

Nandini Maheshwari: Could you give me your number how much was it by the end of this quarter?

Yatharth Tyagi: It is close to 37%.
Nandini Maheshwari: And could you also just repeat what you said about the auditor's appointment? I missed that part?
Yatharth Tyagi: Yes. So we have signed an NDA with the top six audit firms of the world who have been working in our premises throughout this result declaration. And going forward also we are quite confident that certain processes are required for any leading auditor firm and certain timelines are there for them to come on board. So in the coming quarters, we are hopeful that we will be meeting those timelines and will be announcing their appointment in the coming ahead time.
Nandini Maheshwari: All right. Thank you.
Moderator: Thank you. The next question is from the line of Aman Vishwakarma from Philip Capital. Please go ahead.
Aman Vishwakarma: Thank you for the opportunity. I had a question on your Faridabad acquisition, right? So, do we have a number in case of what is the occupancy we are seeing by the end of this year?
Nitin Gupta: Presently, we are having occupancy nearly around 13% and we are going with a good pace.
Yatharth Tyagi: So as far as Faridabad is concerned, I think at the end of the year is where we are expecting an occupancy of close to 30% and this will be if we feel that if that's the number achievable, then it will be quite good. Right now, the occupancy there is 10% because it has just started. So we feel 30% at the end of this year would be a good occupancy. And see because the good part is that it already has private insurance which are already empanelled from day one. So otherwise, it would have taken a lot of time to get those empanelment. So we feel close to 30% would be a good number at the end of this year.
Aman Vishwakarma: And secondly, on your Jhansi-Orchha Hospital, the ARPOBs are on the lower end. So is it due to the seasonality or what exactly is happening there?
Amit Kumar Singh: So if you look at it as Jhansi and if you follow the last two, three, four quarters, if you see, I mean, we started ARPOB was better because the high concentration of the cash and insurance patients, mainly the cash. As we have stated earlier, there's more and more empanelment happening from the government and other the PSUs and others. So there ARPOBs are definitely elevated. So I think we believe, but you see that

the occupancy has increased significantly and that is what is in the expected line. So initially since we had no empanelment. So the cash percentage has been much more and that is what ARPOB was better, but now they are dragged down just because of the other government and PSU business. So that is as per the expected line.
Aman Vishwakarma: And just lastly on your foreign patient business, I'm sorry if this has been answered earlier, but would you be able to give us a share of the revenue from foreign patients?
Amit Kumar Singh: Not a significant number. So I would love to give you numbers when we receive a significant mark, but yes this is in a single digit as of now. But as I said it's more important about the processes. Believe me, we have a significant pipeline, and the coming quarters will be fantastic as far as international business is concerned for the Yatharth Group. As the new international

airport is going to come, it's going to be operationalized by the end of this financial year, this year, I believe. So we have that plan and the processes are placed accordingly.
Aman Vishwakarma: And just lastly, a follow up on that I mean what are we doing in terms of marketing ourselves as branded player in the foreign business?
Amit Kumar Singh: So if you look at it, there are the multi-disciplinary approach which one should have. One, you have to focus on the patients who are already floating in this region. That's one part of it. Second, set up your own spokes over those countries where there are more patients coming in Delhi NCR particularly. So, and then third is about another channel is about the overseas insurance companies. So we are working on all those three, four channels and I think we'll be doing good in the coming future.
Aman Vishwakarma: Okay. Thank you. That answers my question.
Moderator: Thank you. The next question is from Prashant Nair from Ambit Capital. Please go ahead.
Prashant Nair: Hi, good morning. Just a couple of questions. Firstly, can you provide an update on by when do you expect the Brownfield addition at Noida Extension and Greater Noida to be available in terms of the beds to be commissioned?

Yatharth Tyagi: Good morning, Prashant. I think the expansion capacity at both Noida Extension and Greater Noida would be available from - Greater Noida Hospital would be available from two years from now. So probably two years down in August is when we expect it. As far as Noida Extension is concerned it would be 2.5 years from now is when it's expected.
Prashant Nair: So around mid-2026 and early-2027. That's the general time frame?
Yatharth Tyagi: Yes.
Prashant Nair: And can you also is the capital cost of the capex for these projects, can you just give us an update on what you intend to invest?
Yatharth Tyagi: So I think this will be done through internal accruals. We expect the capex per bed because these are sort of a Brownfield expansion, the capex per bed in these two expansions would be close to INR 65 lakh. So I think that's where we feel that from internal accruals is something which we can easily do for these two expansions.
Prashant Nair: And just last question from my side. In the past, you have talked about one inorganic deal every year. So is that still an aspiration? And if yes, then which parts of the Delhi NCR region would you largely be targeting?
Yatharth Tyagi: It is definitely a strategy, and this is something which we have already proven in the last two years. So as I said, a couple of years back Jhansi-Orchha was there. Last year, Faridabad was there and this year we are actively in high advanced stages of talks with, in fact, two of the hospitals. So this is primarily as I mentioned earlier, Delhi NCR and Uttar Pradesh is where we are also seeing.

Also, Rajasthan is some territory that we have identified a hospital that is

available there for an O&M model. So this is why we feel, of course, in these three regions in the next coming years, including this year, one acquisition should be happening each year.
Prashant Nair: Okay. Thank you. That’s it from me.
Moderator: Thank you. The next question is from the line of Priyank Parekh from Abakkus Asset Management LLP. Please go ahead.
Priyank Parekh: So you mentioned about your receivables going - you record some 80cr. So what is your receivable amount as of now?
Yatharth Tyagi: Receivable amount, even though with the increased scale of business in the quarter 1, it's closer similarly to the amount that it was at the closing of last fiscal. So even though the business has increased, but we were able to bring debtors down of close by five days. So it is somewhat similar to what it was last year, but as you can see our revenues have increased even with that status.
Priyank Parekh: So is it fair to take the number same as last as of March 24?
Yatharth Tyagi: It's closer to that.
Priyank Parekh: Okay. Got it. Sir, you mentioned that adjusting to the Faridabad loss you would be having higher margin. So what is the percentage loss in Faridabad as of now?
Yatharth Tyagi: Say that question again.
Priyank Parekh: Faridabad. So I am mentioning what is the EBITDA margin of Faridabad as of now?

Sonu Goyal: It is 2% dilution in the current quarter.
If we remove the Faridabad EBITDA loss, there is a dilution of 2% EBITDA in the overall consolidated numbers.
Priyank Parekh: Okay, that way I'll work it. And sir, Jhansi ARPOB has been quite volatile. You mentioned the change in the payor mix as a reason, but what is the sustainable ARPOB you are seeing for that hospital?
Amit Kumar Singh: You see, I just said earlier that more and more payor mix is the government and PSUs because these are the majority business in that region. So we have to have those business and that is what. So reasonably I think anything has been landing from, I mean, currently, I think it's INR 10,000, but we are expecting a change of the clinical mix because more and more Super Speciality who will be playing the role there. So I think it will be closer to anything between 15 to 18 kind of that will be ideal for a patient in a Jhansi.
Yatharth Tyagi: Along with the payor mix, also the rates you have to see are quite less compared to the NCR hospitals. So even the lower pricing there plays a role in the lower ARPOB along with the difference in the pay.

Amit Kumar Singh: See, Jhansi is moreover the volume business and that is what you see currently the occupancy is 45% and at present, I think the hospital is fully packed for this. I mean, that's the last couple of days I can tell you. So it's going with an expected line and of course more and more like NABH is there. Then NABL and others licensing going to be there. So we can't say if it is like NABH hospital, you come under 10% additional rate. So I think and then having more and more penetration of the insurance business. So these are the lines which we still expecting close to INR 15,000 to INR 18,000 ARPOB. I think it will be a good.
Priyank Parekh: Okay. So with that ARPOB and it being more volume driven, we must be having quite a lower EBITDA there. So what is the broad range of EBITDA we are running there?
Sonu Goyal: We are getting EBITDA of 10%. Currently, we are having EBITDA of 10%-10.2% is the current EBITDA.
Priyank Parekh: Okay. And at peak maturity, like 70%, what should be the EBIT

DA?
Sonu Goyal: It will definitely go 25% to 26%. Another same trajectory as the consol number outstanding.
Priyank Parekh: Okay. Despite having the lower ARPOB.
Sonu Goyal: Basically, as we just rightly said, the number of patients has been increased. So the July and August more are trending more than 325 patients in the overall hospital. Although our pricing is low, NABL certification under process. So these all will jack up the overall ARPOB.
Yatharth Tyagi: And also see the reason why EBITDA margins would be similar within the occupancy because even the expenses in Jhansi are not that much. So, let's say, the manpower cost is also lesser there compared to NCR. The total cost is a bit lesser there compared to NCR. This is what will help us achieve the EBITDA margins that Sonu was talking about.
Priyank Parekh: Okay, sir. And sir, last question on my side.
Moderator: Sorry to interrupt sir. I request you to come back for follow-up questions.
Priyank Parekh: One last question, please. So what is the development on our 71 Cr seized by IT department? You were saying that we are seeing quite a good sign in last quarter. But what is the development there?
Yatharth Tyagi: No amount as of today is seized at all. Every amount has been released already. So that's the status of that.
Priyank Parekh: Okay. So now that case is gone, right?
Yatharth Tyagi: It takes time. But now there is no amount that is seized, and the report has been sent to the center and all the amount has been released.
Moderator: The next question is from the line of Prateek Poddar from Bandhan Mutual Fund. Please go ahead.

Prateek Poddar: Yes. One question. Could you just help me understand what was the drag on your P&L because of Faridabad integration this quarter?
Sonu Goyal: Like in P&L is around 2%. In the EBITDA, 2% of sales. So around INR3.6 crores is the EBITDA loss in Faridabad. That dilutes the overall 1.8% of EBITDA.
Prateek Poddar: And secondly, sir, can you help me understand the entire payor mix? You called out government is 37. Is that it?
Yatharth Tyagi: Yes. And the remaining is -- so, the cash, that is the self-payor and private insurance are equally divided among the rest. We are contributing and we are including international sales in self-payor and cash only.
Moderator: The next question is from the line of Kashish Thakur from Elara Securities Private Limited. Please go ahead.
Kashish Thakur: I have two questions. My first question is on Faridabad facility. Just wanted to know by when can we expect this facility to break even?
Yatharth Tyagi: I think within 18 months is the fair assumption for it to be breakeven.
Kashish Thakur: Understood. And the second question is on Jhansi facility. Just wanted to know how is the competitive scenario in the area, like how many unorganized players are present in Jhansi and like what kind of major patients or whether it is more kind of insurance patients or cash patients do we witness in this region?
Yatharth Tyagi: So there are a lot of unorganized players, but as far as the super specialty hospital of this scale is concerned, I wouldn't say there is a hospital even up till 200-kilometer radius from this hospital, even up till Gwalior, ours would be the largest hospital. As far as the payor mix of that region is concerned, no, there are not many private insurance patients there. Private insurance

penetration is still not there much, but self-pay as well as lot of Ayushman card scheme holders are there. That is the government business. As well, you know, that area is a big hub for both army as well as railway. So there are a lot of patients holding these cards which contribute to the government business there.
Moderator: The next question is from the line of Prerana A

manna from PNA Ventures. Please go ahead.
Prerana Amanna: Congratulations on a good set of numbers, sir. I have two questions. I think you would have already covered it. First one is, what is the debtor days as of Q1 FY '25 and the last Q1?
Yatharth Tyagi: Debtor days now are five days lesser than what it was for the full fiscal year last year. And we are quite confident that with the same trend, reducing five debtor days in a quarter is a good effort made by the new finance team that has joined. We feel that if we continue this similar trend for the whole fiscal this year, we will be somewhere close to 100 days.
Prerana Amanna: 100 days. Okay. Sir, the other question was regarding this IT raid that had happened. I think it happened almost a year now. And you were talking about it to the previous participant that the

money that was frozen has been released. So can we use this right now, this money for our operations or is it still frozen? And there is also this other additional INR 80 lakhs, right, or 8 million that is there, that has also been seized in operational cash. What about that?
Yatharth Tyagi: So it has been released, but for utilization in the operations, there is still time in that. But these are the normal protocols that happens after every investigation. And these things, they do take time, but there is no seizement of any amount within our banks. And it has not impacted any of the business and the operations or the financial working of the company.
Prerana Amanna: But, sir, what about that 8 million that was there in the footnotes of the income statement? Like, when did this happen? Because if it had happened at the same time, why was it not informed to the exchange or even last quarter when we were discussing about it?
Yatharth Tyagi: It was mentioned there in the notes. And as far as that amount is also concerned, it has been reduced now. And see, these investigations, they are informed to us also at a certain time. It is not that a similar amount is there from the beginning. Each time, there is a different amount that gets there. So I think as far as now is concerned, that amount is significantly reduced, and it has not been seized now.
Moderator: The next question is from the line of Nirali Shah from Ashika Stock Broking. Please go ahead.
Nirali Shah: Most of my questions have already been answered, but a few quick ones. On the margin front, it seems to be a bit on the lower side due to the operational losses at Faridabad unit, as Pankaj sir mentioned in the opening remarks. Is there anything you would like to mention regarding how we should look at it from a two to three years' time span on the margin numbers?
Yatharth Tyagi: Faridabad margins would be, I think, in three years' line, would be somewhere close to -- somewhere above 20% in terms of the margins are concerned. And the good part about this location and this unit would be we are expecting a good ARPOB. Yes, 28,000 is just the initial ARPOB, but we feel even the ARPOB here will be somewhere closer to what our Greater Noida and the Noida Extension are doing. We have already started robotic surgeries there. We are already even doing surgical oncology. In the last month itself, we did more than 100 operations in this hospital. So I think we are quite happy with the progress and the margins should be above 20% in the coming years.
Nirali Shah: And then the whole of the business, not just Faridabad, but the whole consolidated level?
Yatharth Tyagi: Overall business, I think margins would be sustainable. I think similar to what we are having in the overall margins are concerned because even though there would be certain drag until a few years from the new acquisitions, but our existing hospitals, that is the Noida Extension, the Grea

ter Noida will be continued to rise, would be somewhere sustainable, 0.5% up and down, but quite sustainable.
Nirali Shah: Can we take as 25% to be a sustainable margin?
Yatharth Tyagi: A bit close to that.

Nirali Shah: Okay, understood. And just one more, while Faridabad Hospital has shown a stellar performance just in the 1.5 month view, and Jhansi-Orchha Unit, so even a previous participant mentioned about the drag of ARPOB, is there any plan that we have to mitigate or maybe to do away with the government and the PSU patients that we have or trying to leave it to the industry?
Amit Kumar Singh: See, if you look at it as a totality, if you talk about the group, see, earlier we had that inventory level. So that is what we were taking this government business. But now, as you see the numbers, I think the occupancy is more than 60% at a group level. So we have reached a particular volume where, now we can do the fine-tuning of our mix. So that is what this business, government business, is already, as Yatharth mentioned before, I think we are already clipping those business, the government business. So in the future, you see insurance and cash business will have a significant, numbers than this. So it will automatically reduce. And as you know, these businesses are not compulsory for us to take. I mean, these are all we can choose.
Nirali Shah: Right. So aspiration of having an ARPOB of, say, 15,000, 18,000, would this be achievable in our business?
Yatharth Tyagi: Yes, it will be achievable in the coming years in Jhansi-Orchha.
Nirali Shah: Lastly, just a quick bookkeeping question. So this quarter depreciation was around 114 million. Should this be considered as a new milestone and implemented as we acquire new hospitals?
Pankaj Prabhakar: Can you repeat the question, please?
Nirali Shah: Yes. Quarterly depreciation was 114 million. And just wanted to know that should this be considered as the new base going forward and as we acquire new hospitals, it keeps on increasing.
Sonu Goyal: Yes. So there is an addition of one facility during this quarter, okay, around 14th of May. So because of that, there is a further addition in capex. We are moving towards niche working, niche facilities. That too by adding the robotics in all other hospitals. Overall, capex increased in this quarter around INR 60 crores. That directly correlated with the medical equipment's. Major part comes in the medical equipment's. So that's the now is a trend. We keep on seeing this somewhere else.
Yatharth Tyagi: And also, the depreciation and the new medical equipment's did not just happen in the new unit. Even in our existing hospitals, there was a lot of capex spend that was done, along with the robotics and other equipment as far as neurosurgery and certain cardiac surgeries are concerned. So yes, I think that was depreciation was there.
Nirali Shah: That's right. Thank you.
Moderator: Thank you. The next question is from the line of Sanchit Chawla from Subhkam Ventures. Please go ahead.
Sanchit Chawla: The question has already been answered. Thank you.

Moderator: Thank you. The next question is from the line of Siddharth from Caprice Investments. Please go ahead.
Siddharth: So I've got a couple of questions, sir. So could you tell me, more about the case mix at Jhansi Hospital? Because I see that, the ARPOB is quite low there. So I want to I want to understand, the case mix over there.
Yatharth Tyagi: So in Jhansi Hospital, the government share is more than the group average. And yes, that is why the ARPOB is low. But as I mentioned that

also the pricing there is low. The case mix is different there. So these are the key factors contributing to lower ARPOB there. But in the coming years, as we mentioned in the call earlier, we expect increase in the ARPOB due to the change in the case mix and increase in the super specialty treatments there. So we are also planning a medical and surgical oncology to start in Jhansi. We are also planning CTVS department, that is the cardiac surgery to start in Jhansi. These are the key factors which will drive ARPOB there in the coming years.
Siddharth: And my second question is, sir, at this number, we should be comfortably reaching INR 1000 crores this year in revenue, right? So is it safe to assume?
Yatharth Tyagi: Close to that.
Siddharth: Okay. So excluding for EBITDA loss at Faridabad hospital, our EBITDA margin should, I mean, given the fact that, our ARPOB is, constantly increasing, so we should have posted, a better EBITDA margin, right?
Yatharth Tyagi: Yes. If we remove the EBITDA loss in Faridabad, our EBITDA margins would have for all the existing facility except Faridabad, if there is an improvement or there is a status quo, we will maintain. So there will not be less than 26% or 26.5% EBITDA percentage at the overall level. Subsequent new additions.

Siddharth: So thank you and congratulations for the great set of numbers. Yes.
Moderator: Thank you. The next question is from the line of Urmi Khania from Robo Capital. Please go ahead.
Urmi Khania: Sir, so you mentioned for Greater Noida and Noida extension, 2 to 2.5 years for the capacity addition. So do you see to add 100 beds operational in the first year and the rest in the second year? And how would be the capacity utilization of these additional beds in the first year and how do you expect them to ramp up?
Yatharth Tyagi: So in two years, Greater Noida will be at the same time ready with 200 beds and within 2.5 years, Noida extension will be ready with 250 beds at the same time. So this will happen from the go. All capacity would be added simultaneously. When we add these capacities, we feel that it would take on these specific beds that have been added because it's already a running hospital. We are quite confident that within two years post that both these capacity utilization would be above 50% on these beds.

Urmi Khania: Okay. And another question is, how do you see the effective tax for full year FY '25 and FY '26?
Pankaj Prabhakar: So we are expecting 25.16% at full year taxes.
Sonu Goyal: So the tax is coming around 30% because of the losses in Faridabad. If we remove that losses of Faridabad it's actually coming 25.16%. So effective tax rate more or less it's trending to be 25% to 27%.
Urmi Khania: Okay. And for the Noida Hospital, do you see any growth there or expected to be flat or in the similar range currently?
Amit Kumar Singh: Noida has reached the peak. So yes, whatever the little growth is going to happen only because of the clinical mix. So even if you see the single digit growth that's because of the shift in the clinical mix. So I think it is very much in line.
Urmi Khania: Okay. That's it from our side. Thank you.
Amit Kumar Singh: And one more thing is about what the natural course of changing the year-round ….. So that is what the rule of charge is when you change. And then there's some negotiation with the insurance company or something. So those will play a role in increasing the ARPOB.
Urmi Khania: Okay.
Moderator: Thank you. The last question is from the line of Pujan Shah from Molecule Ventures. Please go ahead.
Pujan Shah: Hi, sir. On the broader side, I just wanted to ask. There is news that there is a capping going on and there is a discussion on the capping on the price. So is there any acti

ve outcome?
Yatharth Tyagi: Yes. So I think some quarters back there was a PIL in the court about that healthcare prices should be reserved for all the hospitals and prices should be similar and regulated. I think since then even the center has told that it would be difficult to do certain capping as well as standardization of the rate. So that's the progress on that.
Amit Kumar Singh: But whatever the private hospitals the body has already made their representation. So then nothing has come out. And we are very hopeful that nothing should come out on those lines.
Pujan Shah: Okay. Got it.
Amit Kumar Singh: I mean, we don't want to comment more because the matter is already in the court.
Pujan Shah: Okay. Thank you so much.
Moderator: Thank you. Ladies and gentlemen, we'll take this as the last question. I now hand the conference over to the management for closing comments.
Yatharth Tyagi: I thank you, Elara Securities for organizing this concall. And thank you for all the participants who have participated. I hope we were able to answer all your queries. Thank you.

Moderator: Thank you. On behalf of Elara Securities Private Limited that concludes this conference. Thank you for joining us. And you may now disconnect your lines. Thank you.

(This document has been edited for readability purpose)

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Call: Nov 2024

November 13, 2024

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Sub: Earnings Call Transcripts Q2FY25

Dear Sir/Madam,

Pursuant to Regulation 46(2)(oa) of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed the transcript of Investors' conference call organized on November 8, 2024 post declaration of Financial Results (Standalone & Consolidated) for the quarter/half year ended September 30, 2024.

The transcript is also available on the website of the Company: https://www.yatharthhospitals.com/investors in the section of Corporate Announcements.

Kindly take the same on record.

Your faithfully,
For Yatharth Hospital and Trauma Care Services Limited

Ritesh Mishra
Company Secretary & Compliance Officer
Mem. No. A51166
Enclosed as above.

Corporate Office: HO-01, Sector - 01, Greater Noida West, 201306
CIN: L85110DL2008PLC174706

"Yatharth Hospitals and Trauma Care Services Limited
Q2 & H1 FY'25 Earnings Conference Call"

November 08, 2024

MANAGEMENT : MR. YATHARTH TYAGI – WHOLE-TIME DIRECTOR
MR. AMIT KUMAR SINGH – GROUP CHIEF EXECUTIVE OFFICER
MR. NITIN GUPTA – CHIEF OPERATING OFFICER & PRESIDENT , FINANCE
MR. PANKAJ PRABHAKAR – CHIEF FINANCIAL OFFICER
MR. NEERAJ VINAYAK – HEAD, STRATEGY & INVESTOR RELATIONS
MR. SONU GOYAL – FINANCIAL CONTROLLER

Moderator: Ladies and gentlemen, good day and welcome to Q2 & H1 FY'25 Earnings Conference Call of Yatharth Hospitals and Trauma Care Services Limited hosted by PhillipCapital.

As a reminder, all participant line will be in listen-only mode and there will be an opportunity for you to ask question after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing "*" then "0" on your touchtone phone. Please note that this conference is being recorded.

Let me draw your attention to the fact that on this call, our discussions will include certain forward-looking statements, which are predictions, projections or other estimates about future events. These estimates reflect management's current expectations about the future performance of the company. Please note that these estimates involve several risks and uncertainties that cause our actual results to differ materially from what is expressed or implied.

I now hand the conference to Mr. Aman from PhillipCapital (India) Private Limited. Thank you and over to you, sir.

Aman: Thank you, Sejal. Good morning, everyone. On behalf of PhillipCapital Private Client Group, I welcome you all to the Q2 and H1 FY'25 Earnings Conference Call of Yatharth Hospitals and Trauma Care Services.

From the Management we have, Mr. Yatharth Tyagi – Whole-time Director, Mr. Amit Kumar Singh – Group Chief Executive Officer, Mr. Nitin Gupta – President, Finance and Chief Operating Officer, Mr. Pankaj Prabhakar – Chief Financial Officer, Mr. Neeraj Vinayak – Head of Strategy and Investor Relations, and Mr. Sonu Goyal – Group Financial Controller.

I now hand over the conference to Mr. Yatharth Tyagi for his Opening Remarks and we will then open the floor for question-and-answer session. Over to you, Mr. Tyagi.

Yatharth Tyagi: Good afternoon and a very warm welcome to Yatharth Hospitals and Trauma Care Services Limited Earning Conference Call for the Quarter ended September 30, 2024.

We have uploaded our presentation on the exchange and the Company Website, and I hope you all might have received and have had an opportunity to go through it. I am pleased to report that Yatharth Hospitals have achieved exceptional growth in revenue and profit, surpassing 30% YoY in H1 FY'25. This performance underscores our commitment to high impact growth strategies, operational excellence and a continued focus on advancing healthcare delivery. Our expanding network of super specialty services alongside the successful integration of the new greater Faridabad Hospital has been instrumental in driving this momentum. The Yatharth brand is now attracting leading doctors and specialists across NCR region, reinforcing our position as a trusted healthcare provider.

Our commitment to clinical excellence is demonstrated by several noteworthy cases this quarter, showcasing the advanced medical capabilities at Yatharth Hospitals. Some of them are

1. We successfully treated an elderly patient with a life threatening 8 centimeter left atrial myxoma through a complex surgery involving meticulous excision of the tumor through an open-heart procedure. Advanced cardiovascular techniques were employed to ensure the safe removal of the tumor while preserving critical heart structures, such as the mitral valves and atrial walls, resulting in a full recovery of the patient.
2. In another instance this quarter, a rare gastroenterology procedure was performed on two male patients with severe acid reflux by successfully executing Anti-Reflux Mucosal Ablation (ARMA) procedures, achie

ving discharge within just 24 hours of admission.
3. Our team also performed a non-surgical colonoscopic removal of a giant rectal polyp in an 82-year-old patient with no complication and a symptom-free recovery within 24 hours benefiting from procedure that minimizes the risk of sepsis and avoided external incisions.
4. In another scenario, an international patient with a recurrent Retroperitoneal Liposarcoma underwent a complex surgery to preserve the right kidney while completely removing a large 4 cm tumor.

These cases reflect the high standard of care and cutting-edge treatments available at Yatharth

Hospitals.
I'm also proud to announce that our Noida Extension Hospital has recently been accredited by the Joint Commission International (JCI), making it the first hospital in Uttar Pradesh to receive this prestigious recognition. We are also among the few hospitals to achieve JCI accreditation in a first attempt across India. This accomplishment highlights our commitment to providing healthcare of the highest international standards and reinforces our position as a leader in quality healthcare delivery.
Aligned with the vision to be a leading healthcare provider in North India, we are excited to announce our recent successful bid for a well-established hospital in Model Town, New Delhi offering all the high super-specialty services marking our entry into the attractive market of Delhi.
The acquisition is under the SARFAESI Act 2002 through e-auction at an estimated cost of approx. Rs.160 crores. The hospital has an expandable bed capacity of 300-plus beds.
We also recently entered in a strategic collaboration agreement to acquire a majority stake in another 400-bedded hospital in Faridabad, Haryana for approximately 91 crores. This is our second acquisition in Faridabad following a Greater Faridabad acquisition in March 2024. Built on approximately two acres of land, the hospital has completed its structure and is expected to be operationalized by the start of the next fiscal. The company plans to invest an additional Rs.100 crores towards equipping the hospital with latest medical equipment's with all high-end super-specialties including full suite of oncology services and robotics. This acquisition enhances Yatharth

Hospital's position as the leading healthcare provider with the largest private bed capacity now in Faridabad region and a critical step forward towards strengthening our reach in the NCR region.
As we continue to scale our operations, we are committed to maintaining the highest standards of governance and transparency. Aligned to this, we have appointed Deloitte as our internal auditor, ensuring rigorous oversight of our process and reinforcing our commitment to best practices in governance and operational integrity.
Overall, we are encouraged by the progress made in H1 FY'25 and are excited to build on this momentum in the coming quarters.
Our focus on patient-centric, high-quality healthcare combined with strategic acquisitions and investments in medical technologies will drive long-term value for our stakeholders. We remain committed to delivering exceptional care, expanding our network and strengthening our relationship in the healthcare sector.
I now hand over the call to Mr. Nitin Gupta for a Financial Update.
Nitin Gupta: Thank you, Yatharth, and good afternoon, everyone.
I am delighted to share that Yatharth Hospital has delivered industry-leading growth in both revenues and the profitability for the quarter and the half year ending September 30, 2024.
Let me first take you all through the Results for this Quarter:
During the quarter, Yatharth Hospital has achieved a revenue of Rs.2,178 million, reflecting a substantial growth of 27% YoY and a growth of 3% QoQ driven by t

he growth in our occupancy and
ARPOB. Notably, our Noida Extension Hospital registered the highest growth with the remarkable 53% YoY increase in revenues now contributing 37% of our top line. Our inpatient volumes were up by 32% while our outpatient volumes were up by 16% YoY in this quarter.
We have also made significant strides in advancement in our tertiary and the quaternary care services. Earlier this year, we introduced our new radiation PET line at our Noida Extension Hospital and Robotics across our Noida Extension, Greater Noida and the Greater Faridabad Hospital. These investments have significantly boosted our oncology revenue with oncology now contributing 20% to the Noida Extension revenue and 11% to the group's overall revenue, a four-fold increase from the last year. The shift towards the high value super-specialty services have driven a notable 11% YoY increase in our ARPOB, which now stands at 30,641 for the Q2 FY'25. Notably, Noida Extension achieved an ARPOB of 38,136 driven by its 70% contribution from the super-specialty services, reflecting our focus on the high-end healthcare solutions.

Our EBITDA for this quarter improved by 20% YoY to Rs.546 million; however, our EBITDA margin this quarter were at 25.1% compared to 26.6% last year. The reduction in the EBITDA

margins for the quarter was on account of the operational losses at our Greater Faridabad unit, which became operational in mid-May this year. Adjusting for the Faridabad Hospital, EBITDA margin would have been in the range of 26% to 27% for the quarter. Within five months of the operation, our Greater Faridabad Hospital now contributes to around 4% of our top line and thus we remain optimistic about contributing to the Greater Faridabad facility in our overall Group's revenue.
Overall, our profit grew by 12% YoY and 2% QoQ at Rs.310 million, a relatively slower profit growth recorded due to an increase in the depreciation expenses in the quarter with the Greater Faridabad acquisition.
Now, coming to the Half Year Numbers:
Our revenue grew by 32% YoY to Rs.4,296 million. Our inpatient volumes were up by 30% while our outpatient volumes were up by 12% YoY in H1. Our overall group occupancy also increased to 61% in H1 FY'25 compared to 54% same period last year. Notably, our Noida Extension and the Jhansi Orchha have shown impressive occupancy growth reaching to 60% and 47% respectively compared to 41% and 19% in the same period last year. Our Greater Faridabad Hospital operational since May '24 has also delivered solid occupancy growth.
Our EBITDA for the half year stood at 1,083 million, up by 25% YoY while EBITDA margin stood at 25.2%. Our profit for the half year stood at 613 million at 32% YoY. Our balance sheet continue to remain strong with the net profit position at Rs.1,541 million.
Our company's credit rating has been recently updated by CRISIL at A- with a Stable outlook, reflecting an adequate safety and the low credit risk of our financial instruments.
Our return on the capital employed post the Faridabad acquisition came at 23%. Our substantial efforts to reduce our debtors by optimizing our payer mix has paid off with the working capital days at the end of September '24, reducing to 104 days compared to 112 days in March '24 despite high growth in H1.
Looking ahead, we remain confident to not only sustain the growth momentum in revenue and the profitability, but also towards capitalizing opportunities and to achieve operational excellence and expanding our reach in North India.
Thank you. With this, I would like to hand over to the Moderator for the questions-and-answers.
Moderator: We will now begin the question-and-answer session. The first question is from the line of Pujan Shah from Molecule Ventures.

Please go ahead.

Pujan Shah: Sir, first question would be on the Delhi acquisition what we have done from the SARFAESI Act. Just wanted to know like what amount we will be trying to deploy for the restructuring and trying to implement for better structure in the coming years? And the second question would be on the thing which is related to is it like what could be the ARPOB possibility in this specific hospital and what we are eyeing to specific in this thing?
Yatharth Tyagi: Good afternoon. So, the Delhi Model Town hospital that we have got through the SARFAESI Act, we will be paying around, if you add the expenses, 160 crores will go to the Union Bank through which we have won the bid for this hospital, around 25% has already been paid from the company's book to the bank, and the rest would be paid very soon. Above and beyond this 160 crores, we will spend a bit more on the upgradation of certain infrastructure as well as upgrading the medical equipment's. So, we feel that infra updation and medical equipment should be costing somewhere around 60 crores to 70 crores more. So, overall, we see an outlay of 220 crores for this hospital. The hospital is located in a very prime area of Model Town in Delhi with huge residential population residing across the area. There are a lot of renowned doctors also practicing in the nearby hospitals there and we feel that it has huge potential there. The expandable bed capacity that this hospital can have is up to 300 beds. As far as your question on the ARPOB is concerned, we feel the ARPOB in this hospital will be more than the ARPOB of our Noida hospitals because in Delhi the pricing is bit higher compared to Noida and Faridabad and also we will be deploying super-specialties from day one in this hospital and in the vicinity, the ARPOB is also higher of other hospitals. So, we feel around an ARPOB of somewhere close to 40,000 is easily achievable in this hospital.
Pujan Shah: Sir, you correctly said the expandable capacity is 300 beds. So, what would be the current capacity as of now?
Yatharth Tyagi: It would be close to 150 to 170 beds.
Pujan Shah: My second question would be on the acquisition of Faridabad Hospital. Currently, we have been investing 91 crores for 60% of the stake as per the enterprise value what we have been deriving at and additionally we will be investing 100 crores. So, how we have been planning to like, stake would be majority of ours after that 100% of the acquisition, then we will invest 100 crores or like we will be keep investing on a 60%-40% basis?

Yatharth Tyagi: So, we will keep on investing on 60-40% basis. 60% is the acquisition cost that we were on 90 crores plus more will be spent. As I was mentioning for upgrading Delhi Hospital, same will be spent in Faridabad to finish the structure, a certain minor work is still left and all the medical equipment's needs to be put in. Here, the medical equipment cost will be bit more than the Delhi hospital because this Faridabad new hospital will also have the complete oncology machines, so it will have a radiation oncology, PET scans, basic therapy. So, similar outlay of the cost including the acquisition as the medical equipment's, almost totaling would be similar to the Delhi Hospital cost and in both these two hospitals we are trying to operationalize from the first day of the new financial year. We might

do a soft launch earlier, we might start OPDs earlier, but as far as the complete hospital launch is concerned, that's what we are eyeing.

Pujan Shah: All the CAPEX infusion would be via internal ac

cruals, or we are planning some of the debt also?

Yatharth Tyagi: I think we have all the options open to us and with time we will consider how to go about that.

Moderator: The next question is from the line of Mohit Mehra from Guardian Capital. Please go ahead.

Mohit Mehra: This Delhi Hospital it was set up in 2019, right? How come you get to operationalize like Q1 FY'26? I don't understand the long time period.

Yatharth Tyagi: So, this hospital is currently non-operational for the last almost 1.5 years. This hospital was earlier being run by a family of doctors who had got the hospital, and they were running it from the land that forming a trust, but they faced some financial difficulties, so they couldn't continue the hospital and the hospital went to the bank through whom they would have extended the loan to the previous promoter and bank then further resold this hospital in an online auction. So, the hospital has been technically not operational for the last 1.5 years. So, that's why we would be upgrading it, making it operational, installing new medical equipment's, recruiting doctors. That's why we plan to start it at the new fiscal.

Mohit Mehra: The land is also Yatharth for both the newly acquired hospitals or will rent be need to paid?

Yatharth Tyagi: No. So, the complete land structure and the buildings belong to us for both these hospitals. As it's a SARFAESI deal, the Delhi Hospital now, so the land and the structure and the building belong to Yatharth Hospital.

Mohit Mehra: And what caused this sharp rise in employee expenses?

Yatharth Tyagi: Our thought process has been last 1 to 1.5 years is to get a leading and renowned doctors across NCR, because also we have started certain new therapeutic areas which we were not doing earlier, so new doctors were onboarded and leading doctors, HODs from the hospitals have joined, Yatharth hospitals are currently also joining. So, this could have led to a bit of an increase in the expense in the doctor cost.

Mohit Mehra: But will this exert downward pressure on the margins, or do you expect the revenue will more than compensate for the higher OPEX?

Yatharth Tyagi: I think the quarter-on-quarter margins should be sustainable. So, that's what we are targeting.

Moderator: The next question is from the line of Priyank Parekh from Abakkus Asset Managers LLP. Please go ahead.

Priyank Parekh: I wanted to ask on the drop in occupancy in our Noida and Noida Extension Hospital. Of course, the ARPOB has grown but how should we read it with the new trend?

Amit Singh: So, the drop is mainly on the government business and as we had mentioned earlier, what we do is in a very controlled manner in this government business. So, as the occupancy of Noida and Greater Noida was inching towards optimal utilization, we thought let's have some kind of sanitization on this business and that's what you see, but the impact here is that the self-paying patient contribution has increased. So, in the next coming quarters you will see the impact of this action. So, this is totally deliberate and there is no other factor involved.

Yatharth Tyagi: And also, the occupancy hardly overall is 1% to 2% dip. So, that's not a major occupancy dip. In fact, if you see our IPD volumes have increased and that's the main aspect. And as Mr. Amit mentioned, what is more important to us now is the cash segment and the insurance segment has increased and the government segment has decreased, which has only led to this decrease in the 1% or 2% dip.

Priyank Parekh: Yes, but there is a sharp drop in Noida Hospital. It was running around 90%, now it has come to

80%.
Amit Singh: So, this is exactly what I said. See, if you look at where the headway for the Noida Hospital to grow is, we

can't grow in terms of number of beds. That's the limitation we have. So, we thought to work on the payer mix and the case mix. And that's what we did, and you can see the impact on my ALOS, which earlier used to be higher because of this government business. Now this quarter it's bit lesser. As I said it's totally in a controlled way. We know exactly what we are doing and you will see the impact in the coming quarters.
Priyank Parekh: So, this level of occupancy will remain for upcoming years, right?
Yatharth Tyagi: It will increase, but now the incremental increase will be in the cash and the private insurance segment. This is why it might seem reduction now because the government business has reduced now and every incremental now will be at a higher ARPOB. So, that's how the revenue will increase in an optimized, utilized hospital.
Priyank Parekh: What is our EBITDA loss in Faridabad Hospital?
Sonu Goyal: So, as we started Faridabad in the month of May, so in the month of May and June, it is basically a half quarter and Q2 is a complete quarter of three months. So, there is a loss, but if you see the loss has been reduced from Q1 versus Q2, Q1 loss is around 256% negative and Q2 losses coming around 28% negative. Overall, we are on a better trend and the losses are reducing.
Priyank Parekh: This 700 crores of QIP that we are planning, so how we are planning it to utilize? So, first thing that for our recent acquisition in Model Town and Faridabad, I understand we are going to spend around

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CIN: L85110DL2008PLC174706
400-450 crores. Additional above that how we are planning to spend it specifically when we have 250 crores of cash in our balance sheet as of now?
Yatharth Tyagi: See, we have a strong cash position, and we also have a strong leverage to take bank loans, right. So, for us, the company can easily have the option to utilize the cash we have, and we can always take more bank debt as and when required. But at the same point of time we are also keeping an option open because as of now these two are just not the only hospitals that Yatharth group will be looking to add in the course of the next two to three. More acquisitions that will be coming up in the next two to three years. So, either we utilize the complete cash and take a bank debt today and complete these two acquisitions that we have done in Faridabad and Delhi, but that would mean that in the years forward that would restrict our growth into being aggressive in terms of the other aspects that we are currently evaluating. So, we are also keeping an option open where today we can raise funds for just not these two hospitals, but also certain more aspects. So, in the future for next two to three years if certain more bigger opportunity comes for acquisitions or Greenfield, then we can take bank debt that time also. So, it would continue to sustain the growth of the number of beds that we are doing. So, we are keeping our options open as far as that is concerned.
Priyank Parekh: If I ask you in other way, apart from this acquisition, what sort of planned CAPEX we have for our existing structure?
Yatharth Tyagi: For existing structure, the planned CAPEX we are already doing and we plan to do from an internal accrual. So, the Brownfield expansion of the Greater Noida Hospital and the Noida Extension Hospital where we are adding 200 beds and 250 beds each. This will be happening through our internal accruals and we see a CAPEX of around 60 to 70 lakhs per bed for these 450 beds addition that will be happening in two to three years' time. This is already being planned from internal accruals and I would like to also appraise you that the Greater Noida brownfield expansion has just started. So, we are already on track with that. Any other CAPEX that would be done

other than these organic expansions would be done through the other funds.
Moderator: The next question is from the line of Tushar from Motilal Oswal Financial Services. Please go ahead.
Tushar: Sir, this was mainly to do with Faridabad. So, we already just started this 200 bed Faridabad Hospital and now we are sort of getting this another which will overall expand the bed capacity to say like 600. So, just to know your thought process as far as Faridabad as the market is concerned where we're getting so bullish in terms of ARPOB or in terms of the demand? And given the backdrop that I guess there's a big 2,000 bed Amma Hospital also there, I mean of course it's more from a social angle point of view rather than the profit-making perspective. But just your thoughts on Faridabad market?
Yatharth Tyagi: The reason why we have added one more hospital in Faridabad which is a 400 bed and it's a much bigger hospital. First of all, the 200 bedded hospital is in Greater Faridabad region and then this 400-

bed hospital is in the old town and main city of Faridabad. So, technically there would be 15 to 20 kilometers gap between two hospitals. And if you see at the history of the company, that's what we did in Noida; we first started the small hospital in Greater Noida, then we started in Noida, then we went to Noida Extension. So, we feel that once you become a big player in a city, your brand is able to attract much more patients and doctors. Being a small player of just 200 beds in Faridabad, it's not that easy to compete with other hospitals which have 500-600 beds or 300 beds in that region. But now because we will have the capacity of total 600 beds, we'll be almost one of the largest private player in that region. So, that will be able to sort of benefit us in terms of our brand positioning is concerned. And another reason you see is that even in terms of star doctors, the same cost can be shared between two hospitals. We can actually leverage our profitability and expenses and divide them the same for two hospitals. This is what we even did when we started hospitals in Greater Noida and Noida. So, it sorts of benefits us in terms of managing our expenses is concerned. Obviously, the third thing is your brand positioning is very important. So, now with 200 beds, it's not that easy to attract the leading doctors of Faridabad. But today as soon as we have announced the 400-bed hospital, even a 200-bed hospital will now benefit because same star doctors are now willing to join us and work in both the hospitals as head of departments. And we feel that Faridabad in general is a good hospital market, it is an ever-expanding market, there's a road which is even connecting the Jewar Greater Noida Airport to Faridabad. So, even that city will benefit from the upcoming airport that is being planned in Greater Noida. And if you look at the competitive landscape in Faridabad, I think everybody is doing well there. So, we feel this city which is actually an even an older city compared to Noida and Gurgaon is now undergoing a massive expansion. There's a huge population drainage that comes from the nearby villages of Haryana, including Palwal, Sona, even Agra, Mathura highway connects to that region. That's why we feel that it was a no brainer decision for us to take even the doctors who had joined us at the 200 bedded hospital. You should see that there's a hospital which is 400 bed available we should definitely look at it and it is actually from the common consensus that we did the study, we felt that it is the perfect choice for acquisition.

Tushar: This is proposed to 400, but currently what is the capacity?

Yatharth Tyagi: It will be starting as a 400 bedded hospital because it is a new hospital that is starting u

p, the structure
is now complete, some more months it would require for the interiors to happen. So, we will start it as 350 to 400 bed operationalized in that hospital.

Amit Singh: But we'll go in a phased manner definitely by looking at the occupancy and other.

Tushar: So, broadly, what kind of ARPOB can be expected, now with this 400 bed where you can attract clinical talent as well as super-specialty compared to say 200 bed earlier?

Yatharth Tyagi: So, I think we feel that the ARPOB that our Noida Extension hospital is having which is one of the highest ARPOB contributor to our overall group, which is having close to an ARPOB of Rs.38,000. We feel the bigger Faridabad hospital can at least start off with a similar ARPOB and can even go

beyond because we are not just planning to start super-specialty services, we will be starting liver transplant, bone marrow transplant. So, it will become a center of excellence for the whole region.

Moderator: The next question is from the line of Virat from SkyRidge Wealth Management. Please go ahead.

Virat: Just need some better understanding regarding New Delhi hospital. You mentioned it currently has a capacity of 150 to 170 beds, and you will be spending close to 220 to 230 crores for the hospital including upgradation. So, will you need to spend extra to get to the 300 mark or will the 60 to 70 crores for upgradation will be enough for the expansion to reach 300?

Yatharth Tyagi: Yes, it will be enough for expansion. So, when we say 220 crores total outlay, that includes the 160 crores acquisition cost plus around 60 crores would be spent. See, the upgradation of two, three floors don't cost much and that's all what will be required. This two, three floors need to be constructed, the rest of the medical expense is almost same for 170 to 300 beds. So, the cost is including in that.

Virat: Just some question on your internal rate of return requirements and payback period for which you acquire the hospitals for, what's your internal target and also your maintenance CAPEX figure would be helpful?

Nitin Gupta: So, the payback period, we presume that it maybe by two years probably we kind of average period we perceive.

Yatharth Tyagi: Two years is the breakeven period that we are looking at and I think three to three and a half would

be the fair payback period is what we're looking at. Maintenance CAPEX wouldn't be that high because it's a newer hospital. So, all the machines and equipment's will be new. I think so maintenance CAPEX yearly will not be more than 15 crores.
Moderator: The next question is from the line of Sumit Gupta from Centrum Broking. Please go ahead.
Sumit Gupta: First question is on the overall margin. So, like previously you used to guide around 27% margin. So, now going forward can we expect it to be around 25% margin over the next two to three years?
Yatharth Tyagi: So, we have always talked about sustaining our EBITDA margins close to 25%-26%. And if you look at even with an increase in the doctor expenses QoQ, even with the increase in the business of cash and private insurance segment, international business is increasing, our share of super-specialty business is increasing even then our margins are almost if you say from last quarter it is close to that. It's not a variation much. So, we feel that the margins that we have now of 25.5% to 26% should be sustained.
Sumit Gupta: And how much the mature hospitals would be making?

Yatharth Tyagi: Mature hospitals would be making, if I talk about, Noida hospital, the margins there should be very close

to 29%, if I talk about other hospitals, so somewhere around 28% to 29% is our mature hospitals would be making.
Sumit Gupta: Another question is on Jhansi Hospital. So, why has the ARPOB declined to like nearly 12,500 versus if I talk about FY'23 or FY'24 it was at around 18,000? And is there any further room to grow for this ARPOB?
Amit Singh: First of all, if we take a look at Jhansi, it's margin are as expected. Because last year Q2'24 versus this half year, initially we had no empanelment's, government and other tie-ups. Mainly the cash patients were more. As we grow, and get the empanelment slowly, you know that government panels will always drag margins. This is more about the volume, and I think this is not the right time to look at it, probably in a few more quarters I think the colors will be better in Jhansi Hospital. I think we would be in an average of anything between 15,000 to 20,000 ARPOB. That's what our expectations are from the Jhansi Hospital, and we will be able to achieve it.
Moderator: The next question is from the line of Gaurang from HSBC. Please go ahead.
Gaurang: Sir, I just wanted to understand the evolving competition dynamics in your Noida Hospital. Given that Max has opened its new hospital, as in the acquired hospital just near your Noida hospital, it's just 2.5 kilometers away, so do you feel that there is a very meaningful change in terms of how our business model is affected by Max opening the hospital there?
Yatharth Tyagi: See, as far as the Noida region is concerned, our Noida Extension Hospital is the first hospital in the region ever to get JCI accreditation and it just got JCI accreditation a few weeks back. JCI accreditation, not just in Noida, it's the first hospital in UP, Uttarakhand and then nearby vicinity to get that. And that was the thought process. As we feel that as there are new players who are entering into the market we also need to constantly upgrade ourselves in terms of our quality standards, in terms of getting more doctors is concerned. So, it's something that we are doing regardless of who are entering, who's not. We also feel that it's actually good for the Noida market that other big players are now entering there. If you look at what is happening in Gurgaon, this is probably now Noida's time to happen because all players started this journey in Gurgaon probably 7, 8 years back, and if you look at the market which is developed for all hospitals being present there, doctor talent move there, more ecosystem for international patients develop, more facilities develop and this mindset of Noida patients going to Delhi, going to Gurgaon shift as more hospitals open up in Noida. So, we feel it's good for the Noida market. We are constantly evolving, upgrading our accreditations, quality standards and we feel that in the long run, it will be beneficial for everyone.
Gaurang: Just an extension to this, and sorry if I'm being ignorant about your comments on doctors cost going up, but are our doctor cost going up related to this incremental competition, is it like a defense mechanism that we have employed to sort of curtail any poaching that may or may not happen?

Yatharth Tyagi: No, our doctor cost is going up intentionally and that's the strategy that we have deployed. See, a few years back, we used to not have star doctors. When we started with a small brand, we used to have doctors who were second in line to the star doctors who were equally experienced, who were equally skilled, who were driving volumes, but today I think the brand is able to attract all the leading star doctors and HODs who are a bit heavy cost model, but then they bring their own benefit and that is also a reason why we are seeing our cash and private insurance segment of

the patients increasing. It
is actually because of these star doctors and the cost that we have incurred. It is just a starting phase. Complete return will come in the quarters to come when they will be even justifying their cost and doing much more volume than the cost layout to them. So, it is a strategic call that we have taken. It has nothing to do keeping in mind with the competition.

Moderator: The next question is from the line of Prashant Nair from Ambit Capital. Please go ahead.

Prashant Nair: Just one question on the seasonality that usually plays out. This year we haven't seen much difference in revenue between the first and 2nd Quarter. In fact, if you take Faridabad out, it's probably flat and it appears that the Noida Hospitals have actually seen dip in revenues quarter-on-quarter. So, what has changed in the normal pattern? My understanding was that 2nd Quarter is usually seasonally stronger.

Yatharth Tyagi: So, this time it's not that Q2 had some higher seasonality. We see at least in the markets that we are operating there wasn't any breakout of some major dengue or certain other flu which usually happens during Q2. So, yes, it wasn't there this time at least in the regions we are there. And second thing as we mentioned earlier in the call, it is a strategic decision where certain percentage of the government business that we have reduced specifically in the mature hospital of Noida in order to optimize better the growth in the incremental occupancy from here on for a higher ARPOB growth in the future to come is the call that we have taken. If you look at the private insurance business and cash business across all our hospitals, they have increased quarter-on-quarter. It is that the government business that we have intentionally kept a check on, as well as seasonality and that is concerned I think that's the way.

Prashant Nair: How much share of government scheme has come down?

Yatharth Tyagi: So, we have reduced government business close to 4% quarter-on-quarter. So, our thought process was that at the H1 level we are almost down 6% compared to the last year government business is concerned and we feel that if we are able to reduce government business close to 8% for the whole year, that is what we are targeting. So, we feel that when we say in the next 2.5-3 years we see a government business reducing to 25%. That's how we're able to do it. So, we have to start now in order to achieve that in 2.5 years' time. But more importantly, as I said, it's not that we're just reducing government business, we have also increased private insurance and cash segment. That is more important to us simultaneously.

Moderator: The next question is from the line of Arpit Shah from Stallion Asset. Please go ahead.

Arpit Shah: I just wanted to understand a little bit on the doctor cost which we have seen a substantial jump. I do understand that we might have hired start doctors from other hospitals for the newer therapy that we are starting at Noida Extension or Faridabad or at any hospital. So, when should we see a significant or subsequent jump in ARPOB going ahead because star doctors would actually take your ARPOBs higher and that should also help you to expand your margins going ahead?

Amit Singh: If you see, we have probably a 10% increase in ARPOB. As earlier mentioned, the treatment modalities have been added in the systems and Faridabad has new infrastructure. So, that's why you see the increase in other costs. Once these doctors actually start kicking the departments, you will see the impact on ARPOB and revenue as well. So, we are very comfortable with this as of now in terms of the doctor cost and are keeping an eye on it, but you will see the impact in the coming quarters.

Arpit Sha

h: How should we see the revenues, let's say for FY'25 and FY'26 given that some seasonality was weaker in Q2 this year, how should we see that number for FY'25 and FY'26 and despite that in '26, we're going to see addition of two new hospitals getting added to the P&L.

Yatharth Tyagi: I think the percentage growth that we are seeing year-on-year growth, this time it will certainly surpass that and we do expect a higher year-on-year growth, even though we are already delivering industry-leading year-on-year growth, but from '25-26, we do expect this growth to be much more, at the same time we feel that EBITDA margin should be sustainable to what they are in this quarter.

Moderator: The next question is from the line of Rishad Shah from BFLY India. Please go ahead.

Rishad Shah: My question is regarding the occupancy drop during the quarter-on-quarter to the management.

Amit Singh: So, I think we have already answered it. If you look at it, the drop in occupancy of 1% or 2% is a deliberate management call. We do government business in a completely controlled way, to increase contribution from other payers. So, this call will have an impact in the coming quarters.

Moderator: The next question is from the line of Nirali Shah from Ashika Stockbroking. Please go ahead.
Nirali Shah: So, I just wanted to ask on this recent acquisition of Model Town and Faridabad. Can you outline what is the plan in terms of how it will be contributing over the two to three years horizon?
Yatharth Tyagi: So, I think if you look at the contribution that the Greater Faridabad Hospital has started off, I think the Model Town Hospital will be on the similar line, but the Delhi Hospital will obviously have a much more contribution to our overall ARPOB is concerned and we feel that the case mix will be very optimized here. And also it's important to note that from now on, all the new hospitals that we

are adding, whether it's Greater Faridabad which is already showing or the Model Town, Delhi or the newer hospital in Faridabad, will have a very high share of private insurance and cash segment, and the government business will be close to 20%, 25% from day one of those new hospitals. So, this is why that's more important for us to work on our KPIs as far as the new hospitals is concerned.
Amit Singh: So, when we take any hospital, where the bed capacity is already there, to fulfill that we tend to take more empanelment government business initially. But as the other payers increases and capacity inches towards the optimum level, we automatically reduce it. So, this is how I think the pattern one should operate.
Yatharth Tyagi: For the newer hospitals, as we said that it will not be even required for us to take a lot of government business in the beginning. So, we will be keeping that in check for these Delhi Hospitals and Faridabad Hospitals.
Moderator: The next question is from the line of Ritika from Perpetuity Ventures LLP. Please go ahead.
Ritika: So, I just wanted to know who is the minority shareholder for the Faridabad hospital?
Yatharth Tyagi: So, the majority shareholder is a family who have a company called MGS. They do not operate any other hospital, they are into other businesses, and they made this hospital because one of their sons is a cardiologist doctor, so they thought of constructing a hospital, starting it. But then they feel it will be better for them to partner with the healthcare specialist firm in order to drive the maximum value from that hospital. So, they would be 40% partner in this hospital.
Moderator: The next question is from the line of Shubham from Perpetual Capital Advisor. Please go ahead.
Shubham: Can you please share what kind of EBITDA margins you generate in different categories le

t's say,
insurance, government?
Amit Singh: So, you are asking a forward-looking number. We don't want to give you any forward-looking number as of now. But yes, as you do more complex and high end surgeries, the EBITDA margin might be slightly compromise. But this is what I think you do business and what you want. As of now there is no forward-looking number I can give you.
Moderator: The next question is from the line of Priyank Parekh from Abakkus Asset Managers LLP. Please go ahead.
Priyank Parekh: My only question is about the maintenance CAPEX that we have to do for our mature hospitals. Any per bed number you would like to give?

Yatharth Tyagi: As I already mentioned, the maintenance CAPEX is usually very low unless and until we upgrade or add a facility, and we feel that in our all matured hospitals now every facility has been added. We have already done actually a lot of CAPEX in the last quarter if you look at but that is for the upgradation of new services. So, we added robots, we added new equipment. Maintenance CAPEX for the whole thing as I mentioned will not be close to even more than 15 crores for the whole year.
Moderator: The next follow up question is from the line of Sumit Gupta from Centrum Broking. Please go ahead.
Sumit Gupta: Sir. Just want to understand on the Model Town Hospital as a whole. So, basically like we have one competitor, Shalimar Bagh is also near to the Model Town Hospital, which is just nearly 15, 20 minutes time. So, how do you see the competitive dynamics panning out? And apart from that, what is the strategy for this hospital over the next four to five years?
Amit Singh: Yes, there are good hospitals nearby, but if you look at it, the Delhi population tends to move within 2-3 kilometers. So, the civil line is absolutely the old area, and very high-end residents live there. We believe this is a strategically wonderful location and as far as the strategy is concerned, we are very much aware of the competition there and will enter by keeping all things in mind. And one thing is sure that we're going to provide very high intensive quaternary care in that particular hospital.

Yatharth Tyagi: See, I think what's also important is being an old area and there are a lot of leading doctors who are already practicing in their own clinic and our thought process will be to add those doctors who have a very high individual practice as their clinic to our hospital. Some of them have actually started contacting us already because that area as we mentioned that people tend to move to hospitals where their own family GP or their family doctor is. So, our strategy would be to again add those doctors in our hospital, and we have a huge growth potential from the Model Town Hospital.
Moderator: The next follow up question is from the line of Arpit Shah from Stallion Assets. Please go ahead.
Arpit Shah: I just want to understand margins in FY'26. Given that we'll be starting in at least with new 700 beds, will you be still able to sustain when you are at 26% margin on the consolidated level, or do you see a dip on that.
Yatharth Tyagi: So, we feel that we will be able to sustain the EBITDA margins. The reason being that, yes, there are, two new hospitals that will be starting and there would be certain drag down due to that. But because the hospitals of Noida Extension and Greater Noida will also expand in margin, there's still occupancy ramp up that is yet to happen in Noida Extension Hospital which is close to around 60% of occupancy in this quarter and even Greater Noida Hospital will further grow because of the airport coming in. Now when the occupancy ramp up in these two hospitals happen, EBITDA margin will further increase. So, at the group level, they will compens

ate for the drag down from a newer hospital.
So, that's why we are confident of sustaining EBITDA margins like we have quarter-on-quarter.

Moderator: The next question is from the line of Naman from Nine Rivers Capital. Please go ahead.
Naman: I have two questions. First is on the employee cost. Can we provide the split between the professional fee which is paid to the doctors, like every other peer in our industry provides? Second is the occupancy ramp up number which we have planned in terms of the newer acquisitions which you have done recently as well as Jhansi and the recently incorporated Faridabad Hospital in May over the next couple of years?
Amit Singh: So, coming back to your second question first, I think again just I stated, there is no forward-looking number, but I think Jhansi and Faridabad are doing really good. If you look at quarter-on-quarter, Jhansi Hospital has done around 20% to 48%, which is a very good jump, similarly, Faridabad also. As far as the new hospital is concerned, I think as of now there's no forward-looking number. First question, I missed it.
Naman: It was related to the employee cost as in the professional fee we pay to the doctors?
Amit Singh: So, as we are a growing organization, you may initially see some higher employee cost because the group is growing, with the two hospitals coming up, and the existing hospital already adding lots of new specialties. But as far as this is concerned, we understand that we're keeping around 14% to 16% of the professionals and 18% to 22% kind of other employee cost. These are the percentages we have in mind, and I think in the coming quarters, it will settle.
Moderator: The next question is from the line of Prerana from PNARS Partnership. Please go ahead.
Prerana: So, I have a couple of questions. The first thing is regarding the new acquisition that you have done in Delhi as well as Faridabad. What is the empanelment status with the government and various other medical insurance companies? And the second is you have done these acquisitions, of course a little bit EBITDA dip could be there. So, going forward for this FY'25 year, what is your PAT expectation or even if you could guide on telling what is the PAT margin, will you be able to sustain the FY'24 PAT margin?
Yatharth Tyagi: As far as the first question is concerned the empanelment’s for government, private insurance happens once you start a hospital. They can't happen without the hospital being operationalized. So, because we are operationalizing the hospital from the new fiscal and it is something that we were not running hospital and it's been quite a while, so there have to be fresh empanelment’s and they will take few months to happen. Unlike Faridabad, Greater Faridabad Hospital where there were a lot of private insurance that were already empaneled, in these two new hospitals, it will take some time for us to empanel both private insurances as well as government empanels. As far as second question is concerned, we said the EBITDA margins should be sustainable because of the increase in the margins in our matured hospitals even though that will compensate from the drag down for the newer hospitals. So, I think that's what we can tell us.

Moderator: The next question is from the line of Akshaya Shinde from SMIFS Limited. Please go ahead.
Akshaya Shinde: I just wanted to understand as we are hiring star and leading doctors in the region, so does that mean we are shifting to the doctor-led model and how we are going to derisk in the long term?
Yatharth Tyagi: So, having star do

ctors is not that you are fully focusing on doctor-led model. Ultimately it's always a mix. A patient chooses where to get his surgery done not just based on the doctor's reputation but also the reputation of a hospital. So, it always goes hand-in-hand just because we are attracting leading doctors. Simultaneously, we are also doing our own separate marketing of the hospital reaching out to the communities, showcasing and building on our quality standards like the JCI accreditation that we have done. So, we are simultaneously doing a holistic approach and that's how we will be always deleveraged from choosing this one single model.
Moderator: The next question is from the line of Siddharth from Caprice Holding. Please go ahead.
Siddharth: So, in Q1 con call you said that we could end up close to 1,000 crores but looking at the current run rate I think it's a bit difficult. So, just wanted to understand if still that 1,000 crores guidance which you unofficially gave in the last quarter still holds true or not?
Yatharth Tyagi: I think what we said is we will be closer to that mark. It's not that we said that we will surpass or touch that mark. So, I think we still maintain that we will be very close to that mark, and I think the company is on track with that and we do expect Q3 and Q4 to be good growth quarters, typically Q4 should be even higher than the previous quarters and because of certain new therapeutic areas maturing, certain new doctors coming up. So, I think we do expect quarter-on-quarter growth going forward and we will be very close to that mark.
Siddharth: You said that margins of 28%, 29%, I mean last year that is roughly. Is that sustainable or this quarter margins are sustainable?
Yatharth Tyagi: It's not that we had 28%, 29% margins. I think our EBITDA margins were somewhere between 26% and 27%. If you talk about quarter-on-quarter we have been close to 26% and this is what we will maintain somewhere between 25% to 26%. That will be the target going forward also.
Nitin Gupta: Even if you compare with the last year's numbers, previously in the last year, there was no new unit which has been added there. So, when we see the current quarter numbers for the EBITDA margins, so that being diluted by nearly around 1.5%. If we add that, then we will be clear to around 26% to 27% to reach the same numbers.
Moderator: Ladies and gentlemen, due to time constraint, we will take that as the last question. I now hand the conference over to Mr. Aman for closing comments.

Aman: On behalf of PhillipCapital India, we thank all the participants for your valuable time and especially the entire team of the Yatharth Hospital and Trauma Care Services Limited. I would now hand over the conference to. Mr. Tyagi for his closing comments. Over to you, Mr. Tyagi
Yatharth Tyagi: Thank you, everyone. Thank you for all your questions and thank you for attending the earnings call of Yatharth Group of Hospitals.
Moderator: On behalf of PhillipCapital, (India) Private Limited, that concludes this conference. Thank you for joining us and you may now disconnect your lines.

(This document has been edited for readability purpose)

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February 03, 2025

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Sub: Earnings Call Transcripts Q3FY25

Dear Sir/Madam,

Pursuant to Regulation 46(2)(oa) of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed the transcript of Investors' conference call organized on January 28, 2025 post declaration of Financial Results (Standalone & Consolidated) for the quarter ended December 31, 2024.

The transcript is also available on the website of the Company:
https://www.yatharthhospitals.com/investors in the section of Corporate Announcements.

Kindly take the same on record.

Your faithfully,
For Yatharth Hospital and Trauma Care Services Limited

Ritesh Mishra
Company Secretary & Compliance Officer
Mem. No. A51166
Enclosed as above.

"Yatharth Hospitals and Trauma Care Services Limited
Q3 FY'25 Earnings Conference Call"

January 28, 2025

MANAGEMENT : MR. YATHARTH TYAGI – WHOLE-TIME DIRECTOR
MR. AMIT KUMAR SINGH, GROUP CHIEF EXECUTIVE OFFICER
MR. NITIN GUPTA – CHIEF OPERATING OFFICER & PRESIDENT (FINANCE)
MR. PANKAJ PRABHAKAR – CHIEF FINANCIAL OFFICER
MR. NEERAJ VINAYAK – HEAD, STRATEGY & INVESTOR RELATIONS
MR. SONU GOYAL – GROUP FINANCIAL CONTROLLER

MODERATOR : MS. AASHITA JAIN – NUVAMA WEALTH MANAGEMENT LIMITED

Moderator: Ladies and gentlemen, good day and welcome to Q3 FY'25 Earnings Conference Call of Yatharth Hospitals and Trauma Care Services Limited hosted by Nuvama Wealth Management Limited.
As a reminder, all participants' line will be in listen-only mode and there will be an opportunity for you to ask question after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing "*" then "0" on your touchtone phone. Please note that this conference is being recorded.
Let me draw your attention to the fact that on this call, the Company's discussion will include certain forward-looking statements which are predictions, projections or other estimates about future events. These estimates reflect management's current expectation about the future performance of the company. Please note that these estimates involve several risks and uncertainties that could cause company's actual results to differ materially from what is expressed or implied.
I now hand the conference over to Ms. Aashita Jain from Nuvama Wealth Management Limited. Thank you. And over to you.
Aashita Jain: Thank you, Steve, and good day everyone. I am Aashita Jain from Nuvama Wealth Management. Welcome you all to Q3 FY'25 Earnings Conference Call of Yatharth Hospitals & Trauma Care Services.
From the management we have with us today, Mr. Yatharth Tyagi – Whole-Time Director, Mr. Amit Kumar Singh – Group Chief Executive Officer, Mr. Nitin Gupta – Chief Operating Officer and President, Finance, Mr. Pankaj Prabhakar – Chief Financial Officer, Mr. Neeraj Vinayak – Head, Strategy & Investor Relations and Mr. Sonu Goyal – Group Financial Controller.
I now hand over the conference call to Mr. Yatharth Tyagi for his Opening Remarks, and we will then open the floor for Question and Answer Session. Over to you, Mr. Tyagi. Thank you.
Yatharth Tyagi: Good afternoon and a very warm welcome to Yatharth Hospitals & Trauma Care Services Limited Earnings Conference Call for the Quarter ended December 31, 2024. We've uploaded our presentation on the exchange and the company website, and we hope you all might have received and have had an opportunity to go through it.
I am pleased to report that Yatharth Hospitals has achieved robust growth in revenue, surpassing 30% year-on-year and EBITDA surpassing 20% year-on-year growth for the nine months ended FY'25. Our expanding network of super specialty services alongside the successful integration of the Greater Faridabad Hospital has been instrumental in driving this momentum.
I am glad to share that we have completed payments for two newly acquired hospitals in New Delhi and Faridabad, adding approximately 300 and 400 beds respectively. Our focus now remains on operationalizing these hospitals by the next quarter. We are intensifying our efforts towards the

procurement of state-of-the-art, medical equipment's and advanced robotics systems. This strategic initiative is designed to elevate the quality of care provided and to ensure that our facilities remain at the forefront of medical innovation. In tandem with these advancements and with the addition of new beds, Yatharth brand is now attracting leading doctors and specialists across NCR region, reinforcing our position as a trusted healthcare provider.
A key notable achievement this quarter with our hospitals was a successful kidney transplant facilitated through one-of-its-kind transfer of a kidney from Faridabad to our hospital in Greater Noida in just 20 minutes and 40 seconds by creation of a 46-kilometer green corr

idor.
Our commitment to clinical excellence is demonstrated by several other noteworthy cases.
1. We performed the first Cochlear Implant Surgery on a 5.5-year-old girl, with a case of bilateral congenital hearing loss from Turkmenistan.
2. A female patient from Africa with complex heart conditions, including severe mitral stenosis, severe tricuspid regurgitation, a sinus venosus atrial septal defect, and a partial anomalous pulmonary venous connection, where severe pulmonary artery hypertension and a right heart failure. After intense medical operation, she successfully underwent surgery which involved replacing the mitral valve, repairing the valve as well as closing the atrial septal defect, and rerouting the whole right pulmonary vein, through the expertise of surgeons.
3. In another scenario, our team conducted a Coronary Artery Bypass Grafting Procedure, where three bypasses were put on the heart targeting LAD, PDA and the ramus intermedius in case of a Dressler Syndrome.
These cases reflect the high standard of care and the hospital's advanced capabilities in handling intricate medical cases at Yatharth Hospitals.
In line with our vision to continuously introduce the most cutting-edge technology for our hospitals,

we have this quarter added Interlaminar Spinal Endoscopy in Noida Extension and started Therapeutic Nuclear Medicine and Radiotheranostics – Lutetium Therapy at Yatharth Hospitals. This cutting-edge therapy integrates diagnostic imaging and targets therapeutic solutions, providing a highly effective treatment for certain cancers such as neuroendocrine tumors and prostate cancer, where all therapy fails. These therapies will enable Yatharth Hospital's oncology department to offer high end oncology specialization services.

I am happy to share that our focus on patient-centric, high-quality healthcare combined with strategic acquisitions and investments have helped us achieve an ARPOB of Rs.30,614 in nine months financial year '25, with Noida Extension Hospital registering the highest ARPOB at Rs.37,608, up 12% YoY, while Greater Noida Hospital's Rs. 34,584, up 21% YoY.

Our newest Greater Faridabad Hospital has ramped up very well and has been successfully integrated. In just seven months of being operationalized, it has the revenue growth contribution to 5% of our overall group's revenue and has an ARPOB Rs. 34,365 for Q3 FY'25 in the initial days of hospital's commensuration.

Finally, I would like to take this opportunity to provide an update on the Income Tax matter and addressing the recent market rumors. IT department had conducted a search operation in 2023, and as part of their standard procedure, a certain amount was provisionally blocked by the department. Till last quarter, a majority of those funds, which were provisionally blocked, have been released and have been unblocked. In the recent development, a new Assistant Commissioner assumed responsibility of the case, which was pending for 2-3 months in between and has in the similar line provisionally attached some of company's assets. The issue was informed to the stock exchanges after receiving notice from the authorities. Further to our intimation, during the QIP, an amount of Rs. 6.23 crores from the attached FDs were indeed allowed to be utilized by the company from the Income Tax authorities. We are actively cooperating with the authorities to expedite the resolution of this matter and do not anticipate any significant material, financial or operational liabilities arising from these proceedings.

Furthermore, Deloitte has already been appointed as an internal auditor since last quarter and our teams have been actively working towards their suggestions for strengthening our financial systems and risk controls. Moreover, in

line with the previous communication regarding the appointment of
a leading auditing firm as statutory auditor, I am pleased to inform you that we are on track to finalize the top six auditing firm from the upcoming new financial year and the company will be having the new top six auditors.

We believe that Yatharth Hospitals is well poised and a focus on patient-centric, high-quality healthcare, combined with strategic acquisitions and investments in medical field will drive long-term value for our stakeholders. The company remains on track into fulfilling all the parameters and targets that is set for itself in the field of delivering high-end patient care in the regions that we operate.

I will now hand over the call to our CFO, Mr. Pankaj, for Financial Updates.

Pankaj Prabhakar: Good afternoon, everyone.

Yatharth Hospitals has delivered growth in both revenues and profitability for the quarter and nine months ended December 31st, 2024. Let me first take you all through the results for the quarter. During the quarter, Yatharth Hospitals achieved a revenue of Rs. 2,192 million, reflecting a substantial growth of 31% year-over-year and a growth of 1% quarter-over-quarter, driven by growth in our occupancy and ARPOB. Notably, our Noida Extension Hospital registered highest growth with a remarkable 43% YoY increase in revenue, now contributing 36% to our top line. Our in-patient volumes were up by 36%, while our out-patient volumes were up by 12% YoY in the quarter.

We have also made significant strides in advancing our tertiary and quaternary care services. Earlier this year, we introduced a New Radiation PET Line at our Noida Extension Hospital and robotics across our Noida Extension, Greater Noida and Greater Faridabad Hospital. These investments have significantly boosted our oncology revenue, with oncology now contributing to 21% to Noida Extension's revenue and 10% to group's overall revenue, a 150% increase from last year. The shift towards high value super specialty services have driven a 4% YoY increase in our ARPOB which now stands at Rs. 30,652 for Q3 FY'25. Notably, Noida Extension achieved an ARPOB of Rs. 37,886, driven by its 70% contribution from our Super Specialty Services, reflecting our focus on high-end healthcare solutions. We remain optimistic about our Greater Faridabad facility, which now contributes about 5% to our top line and it's ARPOB contribution evidenced from Q3 ARPOB number of 34,365.

Our EBITDA for the quarter improved by 18% YoY to Rs. 549 million; however, our EBITDA margin this quarter were at 25.1% compared to 27.8% last year. The reduction in EBITDA margin for the quarter was on account of operational losses at our Greater Faridabad unit, which become operational in mid-May. Adjusting for Faridabad losses, our EBITDA margin would have been in the range of 26%-27% this quarter. Overall, our profits grew by 3% YoY at Rs. 305 million, a relatively slower profit growth recorded due to an increase in depreciation expense in the quarter in line with our ambitious bed capacity expansion plan, and addition of state-of-the-art medical equipment at our leading hospitals.

Coming to Nine Months number, our revenue grew by 32% YoY to Rs. 6,487 million. Our in-patient volume were up by 32%, while our out-patient volume were up by 12% YoY in nine months. Our overall group occupancy also increased to 61% in nine months FY'25, compared to 53% in the same period last year. Notably, our Noida Extension and Jhansi-Orchha have shown impressive occupancy growth, reaching 60% and 50% respectively, compared to 42% and 20% in the same period last year. Our EBITDA for the nine months period stood at Rs. 1,632 million, up 22% YoY, while EBITDA margin stood at 25.2%. Our profit for nine months p

eriod stood at Rs. 911 million, up 21% YoY.

Our balance sheet continues to remain strong with the net cash position of Rs. 5,605 million with the fund infusion from the QIP. We remain confident to not only sustain the growth momentum in revenue and profitability, but also towards capitalizing opportunities ahead to achieve operational excellence and expanding our presence in North India.

Thank you. With this, I would like to hand it over to the moderator for questions-and-answers.

Moderator: We will now begin the question-and-answer session. First question is from the line of Ritika from Perpetuity Ventures. Please go ahead.

Ritika: Sir, I had majorly two questions. One was that the ARPOB was 4% YoY this quarter. So, how is this projected to grow? And even in Noida cluster I've seen a drop in revenue. So, what is the major

reason for this? And around the income tax thing, my understanding is that if you have a provisional attachment, you can release it through bank guarantees. So, is there any plans of that and what is your maximum valuation of your provisional attachment?

Yatharth Tyagi: Right. So, the first question is on the ARPOB. As the company has demonstrated good ARPOB growth, year-on-year, similarly, this nine months have shown good ARPOB growth. This is primarily due to increase in our super specialty business and specifically oncology is now contributing to a larger share driving this good ARPOB growth and also reduction in our government business which we have selectively chosen which is leading to a good ARPOB growth, and we feel the 10% ARPOB growth that the company has seen YoY for last year should be sustainable for the years to come ahead.

Similarly, your second question is on the Noida cluster. So, the Noida Extension Hospital actually has shown good growth and continues to show good growth. Similarly, Noida, Great Noida have been increasing. If you look at Q2 compared to Q3, Q3 has always been a leaner season for us in the past across the industry. So, it is in line with that, but even then, we have sustained and in fact increased a bit in revenue, that is due to the addition of our newer hospital that is in Greater Faridabad as well as growth in some of our existing hospitals is concerned.

Coming to your third question on the Income Tax matter, so the provisional attachment of the assets, we do not require to submit any amount to get them released. We have been assured by the Income Tax department that they will be anyways being released very soon and we are in constant conversation with them regarding this provisionally unblock of the assets. Just like our provisioning block of the amount a year ago had been released, we have been assured by the department that this provisional blocking of assets will also be released and we do expect a very positive outcome very soon from the department and there are no plans to give any FDs in exchange of this.

Ritika: So, what is the lien amount currently?

Yatharth Tyagi: Sorry, what is the ?

Ritika: Lien amount with the income tax department.

Yatharth Tyagi: It is close to 60 crores.

Moderator: The next question is from the line of Parth Kotak from Plus91 Asset Management. Please go ahead.

Parth Kotak: I have a couple of questions. The first is regarding recent acquisition model from Delhi and Faridabad. Could you elaborate on the projected timelines for achieving breakeven and payback for these hospitals and how do you plan to ensure operational efficiency given the significant CAPEX outlay of about 220 crores for Delhi and about more than 150 crores of Faridabad?

Yatharth Tyagi: So, this is in line with our strategy of expanding in our areas where we have a strong presence. The decision for acquiring another hospital in Faridabad was the fact that this is the area that we have studied very well, we have already started a hospital in Greater Faridabad last quarter. So, we understand the market and just trying to replicate that, we have already done in Noida, we expand from one part to another within the same city and become a leading player in that region. So, that's why the Faridabad upcoming hospital, which is a 400-bed hospital which will have cancer treatment, something a Greater Faridabad Hospital couldn't have. So, we will be leveraging the brand completely in that area. Similarly, the Delhi hospital also, which is located strategically very well in Model Town. Both these two hospitals are planning to start from the 1st April, that is the 1st April of the new financial year and in the past, our hospitals have been able to break even somewhere around 18 months to two years' time and I think these two hospitals will also be in a similar timeline as far as that is concerned. And as far as the payback is concerned, usually takes four to five years for our hospitals in the past to have that payback. So, we would assume that both these two hospitals will be similarly on those lines.

Parth Kotak: Sir, just one follow up on the steps being taken to address the initial operating losses at Greater Faridabad Hospital and when can we expect that to turn profitable?

Yatharth Tyagi: So, I think Greater Faridabad Hospital should be able to turn profitable in the next financial year, somewhere between nine months ending of next financial year. In fact, we're quite happy with the progress of that hospital. Already, not just in Q3, but as we speak in Q4, that hospital is looking very promising because now we have all the empanelment that are required. We have recently received certain government empanelments in that hospital within the last few weeks, which are even adding a more occupancy growth to it, and even the NABH inspection has been completed in the past few weeks, which will empanel us to tie up with more government empanels. So, I am quite happy with the progress of the Greater Faridabad Hospital. and it should be able to break even close to nine months of next financial year.

Parth Kotak: Sir, last question rather a basic one. First of all, thank you for giving the beds capacity and census beds in the current presentation. If you could just help us understand the difference between the two, that would be really helpful?

Amit Kumar Singh: In a hospital, we call it day care beds, it's like all emergency beds, all dialysis where patients comes and have a short stay or probably only a day stay. So, those beds are not considered in a census beds. So, census beds are those beds in a midnight occupancy, you take where patients are admitted is completely IPD. So, any hospital takes a midnight occupancy, that patient is staying overnight, right. So, that is a difference between the total beds and census beds.

Moderator: The next question is from the line of Hiten Boricha from Sequent Investments. Please go ahead.

Hiten Boricha: I have a couple questions related to this income tax matter. So, the first question is what was the total amount blocked and you mentioned key majority of this fund has been blocked. If you can quantify

what was the total amount blocked? Second question is can you also share the subsidiaries attached, so what is the value of the asset?

Yatharth Tyagi: So, at the time when the income tax audit started, this is around 12 to 13 months back, because the company had recently raised funds from the IPO, there was amount with the company at that time

in

the form of FDs, so there was an amount, that's why provisionally blocked. Even after repaying of the bank loan at that time, the company's book had an amount very close to 250 crores. Because it was the amount that the company has raised on the books at that time in the forms of FDs, so there were provisional attachment on that and slowly that all amount has been released and only a matter of 50 to 60 crores right now it's provisionally blocked. And second, as far as the blocking of assets is concerned, right now, again because. when the new officer took over our case and reopened a file, at that time, there was no FDs with the company in the tune of what it was a year back. So, that's why they provisionally blocked the assets and I think the assets valuation is to see and can be seen from the NSE and the BSE websites depending upon the number of shares, because we have earlier disclosed also the exact number of shares that are being attached, so the valuation can be taken from

that.
Hiten Boricha: So, now only 60 crores is blocked, right?
Yatharth Tyagi: Yes.
Hiten Boricha: And what was actual number? Actually, you are not audible properly sir. What was the initial number when it was actually blocked?
Yatharth Tyagi: Close to 250 crores, because that amount were available on the company's balance sheet at that time as the FDs, so the whole amount was provisionally blocked at that time. It was not that there was a certain measure of this is what they want to block. There was no thought process that they would block this based on certain calculations, nothing. It was just whatever FDs was there at that time at the company's balance sheet was blocked and we had that FD because we just came out from the IPO fund raise. It was all provisionally attached, it was not frozen or seized.
Hiten Boricha: So, sir, can you quantify what is the total monetary demand or money which is demanded by the authority, just to understand this matter clearly?
Yatharth Tyagi: There is no demand of any money or authority so far. See, for a company, it would have been the easiest thing for us to do. We would have just paid that penalty; it would have been a minute penalty and closed this matter as early as possible. But because the case is going on and usually in any income tax matter in India, takes two years of time. We are 13-14 months into it and we are quite hopeful and this is what we have been told that before the end of this calendar year our matter will also be will be closed. But before that, even the provisional attachment of the property, we are expecting that to be revoked very soon. We are not waiting for that for many months. What we have been assured

is by the department that it would be released very soon. So, we don't need to wait for that for the end of the calendar year.
Hiten Boricha: And just last question is around this matter. So, you mentioned something about QIP money. They have allowed us to utilize some crores something from QIP money. So, just for the sake of a clarification, is our QIP money also freezed by the authority or we can utilize that money at any point of time?
Yatharth Tyagi: It is not at all freeze or provisionally block. In fact, when the company went to QIP, we even discussed with the IT department that we are planning a QIP route because we are acquiring new hospitals, we want to expand and the IT department is completely okay with that; they gave ahead the permission to you go ahead, raise QIP and utilize it as per the objective of the company. If there was any issue from that side, they would have not allowed the company to take the QIP route, or when the money was raised from the QIP, they could have also provisionally blocked it, but they have not blocked it. In fact, fr

om the QIP amount that the company has raised, company has already utilized maximum of that amount. The bank loan has been repaid, the acquisition cost has been paid for both the two hospitals from the QIP money and we still have that money free to be utilized.
Hiten Boricha: No, sir, the reason I ask this question is you clearly mentioned something that 6 crores was allowed to be used by authorities, so that was -
Yatharth Tyagi: That is from the 60 crores amount that was earlier been blocked. So, post QIP, even certain amount of that 60 million has also been released.
Moderator: The next question is from the line of Ranvir Singh from Nuvama Wealth. Please go ahead.
Ranvir Singh: My question was related to IT related issues. I think mostly has been clarified. Another question is during this quarter, the EBITDA includes anything related to acquisition cost or in one-off cost in this quarter?
Yatharth Tyagi: No.
Ranvir Singh: And secondly, that 50 crores tax liability which currently is as per their assessment, so how confident you are, whether ultimately you have to pay it or you are fighting it to reduce it to minimum level or how is your confidence level there?
Yatharth Tyagi: So, just a clarification. The 50 to 60 crores that is provisionally blocked is not the type liability that has been asked from the company, it is provisionally blocked as early stated, but we do expect this also and the assets to be released very soon. The ultimate amount would be much, much less, which would be insignificant for the company's operational performance and the financial aspect, and this amount is not the liability that the tax has asked for the company to pay.

Ranvir Singh: Secondly, currently we have a fairly good level of cash, 560 crores something. So, how we are going

to utilize it because two acquisitions we have already done, so within a year do you have any timeline in your mind when you are looking for an acquisition and what kind of acquisition you are looking up?
Yatharth Tyagi: So, certain of this amount is already marked to the medical equipment's that we will be putting into the two newer hospitals. So, even though the acquisition costing of both two hospitals has been paid, but new medical equipment's will be deployed in both two hospitals in a matter of next few months as we are planning to launch them very soon. So, oncology machines, robots, so a lot of amount will go into those medical equipment's, and the remaining amount is for the company to utilize general corporate expenses which we could utilize if there's an upcoming acquisition of another hospital or if there's a Greenfield land acquisition that we want to do. But we have already marked all objects of the money that we will be utilizing from the amount raised in the QIP.
Nitin Gupta: As on December, we have an earmarked amount out of QIP fund proceeds as reflecting in the balance sheet; which has been subsequently utilized for the objects as defined in the QIP prospectus.
Ranvir Singh: So, even after paying all amount because after the QIP also your balance sheet has been very strong, so after utilizing all what you have planned still, I think you will have some cash surplus. Now, here the question is that if we acquire a running hospital, obviously, the cost would be much, much higher. And if we acquire the hospital like Faridabad we have acquired, then it will take time to actually reach a breakeven or to optimize this. If you just acquire just a building, then a lot of investment of that also is required. There are some uncertainties are there in our mind. Now what we understand is that after Q1 FY'26, when we will be integrating with these acquired hospitals, what would be the EBITDA look like to your assessment?
Yatharth Tya

gi: See, as far as your previous questions are concerned, the cash is also being utilized to fund our Brownfield expansion, right. So, our Noida Extension Hospital, the Greater Noida Hospital are also undergoing capacity increase, the Greater Noida Hospital work has just recently started also. So, the company also do from the money that the company already has in the books, we will not be requiring any new funding or debt for that. And second, yes, when the newer hospital starts, there would be certain EBITDA dip that the hospitals would be making. But then we do expect our existing running hospitals, especially of Noida Extension, Greater Noida to increase as far as EBITDA is concerned. So, at the group level, you know, we feel somewhere what we have delivered, right now in this quarter of EBITDA margins should be sustainable in the coming years also.
Moderator: The next question is from the line of Mohit Mehra from Guardian Capital. Please go ahead.
Mohit Mehra: First question is what are the number of operational beds of Fidelis?
Yatharth Tyagi: It is a 200-bedded hospital and we have operationalized close to 170 beds, that is the census bed in that.

Mohit Mehra: So, the census beds are fully operational, right, almost?
Yatharth Tyagi: Yes.
Mohit Mehra: And the second thing is on this income tax matter, in 2Q note said the amount attached was 33-odd crores and now it's almost double and also the assets have also been attached. So, what's going on there?
Yatharth Tyagi: No. So, 33 and 50, these amounts refer to separate FDs. So, when we earlier mentioned that it was close to 250 at the time of the IPO, that has come down and it has come down since then. Similarly, as we earlier mentioned that now, because the case was pending for a few months, there was no officer to preside over our case. And when the new officer came, he reopened a file and had provisionally blocked the assets. See, what's more important is that as we earlier mentioned that the company is free to utilize the money that we have on our books, whether it's through QIP or the internal accruals and we earlier stated we are quite hopeful that within a few weeks this attachment of the assets will also be released. This is by our constant communication to the income tax department. They are well aligned with the company, they have supported our QIP growth strategy and we are quite hopeful that this matter of attachment will be finished within a few weeks, but even the whole income tax matter will be closed before the end of this calendar year.
Mohit Mehra: I don't understand this sir. The money that was initially attached was the 250 crores or 70 crores, because in the 4Q results I think you mentioned was 70-odd crores.
Yatharth Tyagi: Yes, but that 70 was already after the release from the initial amount. See, again, at the time of the income tax audit, it was not that it was frozen or seized. When that audit happened, whatever amount was in the balance sheet was provisionally blocked, it was not calculated, it was not measured. So, with time, as the investigation goes on, as the paperwork happens, they keep on releasing a certain amount, which has been reflected by them in the last few quarters. And a more important aspect that I mentioned that whatever provisional blocking is there, whether on the amount or on the asset will be provisionally unblocked within a matter of few weeks.

Moderator: The next question is from the line of Sumit Gupta from Centrum. Please go ahead.
Sumit Gupta: Just one thing on the other expenses. So, if I look at this nine-month data, so it was close to around 80 crores in nine months FY'25 versus around 60 crores in nine months FY'24. So, what has led to that increase?
Amit Kumar Singh: So, basically

this includes a major increase in the doctor payout. So, we have added several doctors. As already mentioned by Mr. Yatharth, we have added a new facility in Faridabad. So, doctor share also increases in Faridabad also. Overall, we have added specialty and doctors in all our hospitals. That is the major impact. If you see in quarter-on-quarter, it's 178 crores for last year and now coming to 234 crores, majorly due to the doctors.

Sumit Gupta: So, going forward also we can expect considering we are adding new beds?
Yatharth Tyagi: Your voice is not very clear. Can you please repeat the question?
Sumit Gupta: Going forward also we can expect this to increase considering we are -?
Amit Kumar Singh: Not at all. No, no. Even at the Faridabad Hospital and all this, initially the important is about acquiring new doctors, right. So, that homework has already been done. We started the Faridabad Hospital in the month of May. So, suddenly in that one month all doctors have joined. So, all the doctor’s recruitment is completed. Yes, the new hospitals definitely for the new another Faridabad and Delhi one, we will start recruitments very soon, but as far as existing is concerned, I think this is I think optimally we are doing it.
Sumit Gupta: No, I am talking from a new hospital which is for Delhi and Faridabad only. So, from that point of view I was asking that there might be some.
Amit Kumar Singh: Faridabad and Delhi, the doctor recruitment will start but at the same time see, our existing one is also growing right. The Greater Faridabad will start giving us the income and probably whatever the investment in terms of doctor’s recruitment, which we have done at 2-3 months back, that will start giving us results, right. But yes, overall, I would say, so we understand what has to be the percentage in terms of the doctor’s payout and other this thing. So, it's going to be in line with that. Since we are a growing organization 1% or 2% here and there, but I think it will be very much in line with what industry practices are.
Sumit Gupta: Sir, what is the doctor strategy for the Delhi hospital just want to understand because it's a high growth market and a lot of competition in that particular market?
Amit Kumar Singh: The Delhi one, as we had mentioned in the last earning call as well, this hospital is a 300-bed facility going to be absolutely tertiary or quaternary care facility we are going to provide. So, definitely we will have those skill sets doctors in our facility, because we believe that this area where we are in that's very high-end societies, it's a very old area of the Delhi and around 10-15 kilometers, yes, there are other big players are there. So, we will have to be definitely in line with that and we are going ahead accordingly.
Sumit Gupta: So, like for the empanelment point of view, so how do you see that panning out over the next one year or two years?
Amit Kumar Singh: Initially the empanelment is going to take time, initially it's like six months we spend on that empanelment, but there's a process. See, this hospital was temporarily shut for 3-4 months only, right. So, we don't have to do much of it. So, all the paperwork has been done and then the respective wherever the empanelment, we have to get it done, the processes are being done now. So, as Yatharth mentioned it from the April onwards, our OPD and IPD going to start from this hospital.

Moderator: The next question is from the line of Runit Kapoor from Elara Capital. Please go ahead.
Runit Kapoor: So, I had two questions. So, your top line growth has b

een quite strong, but I think the bottom line has been muted relatively because I think depreciation has more than doubled on a year-on-year basis. So, can you give guidance on the future depreciation as I think a significant CAPEX is still expected in your medical equipment and you are using the written down value depreciation, right, so lot of depreciation will go ahead in the initial years, can you give guidance on that?
Yatharth Tyagi: The reason why this quarter the bottom line shows not that much of a growth, but then going forward yes, even though there would be new medical equipment’s that are being planned, but the depreciation increase will not be that much significant as it was specially this quarter.

Sonu Goyal: Further, basically our existing hospital has been stabilized with the capital expansion for the medical equipment. So, yes, for the future expansion, there is an increase in depreciation, but not that too much and also the current depreciation if you see there is an increase by 2.5% on a counter depreciation that leads to a decline in the PBT percentage by 3%.

Runit Kapoor: So, what would be like a quarterly depreciation run rate like if I say like it's 17, 18 crores or significantly higher like from the current levels like?

Sonu Goyal: Quarterly depreciation will be something around 17 crores.

Runit Kapoor: And another question was like, what's your payor mix for this quarter? And another thing is your ARPOB gap is significant as compared to your peers. So, that gives us a strong growth potential in ARPOB. I understand there would be a discount compared to your peers who are much more established. But can you give guidance on how much ARPOB can grow year-on-year so the gap can be narrowed to large extent?

Amit Kumar Singh: As we had mentioned last time also, so there is a certain strategic call which we have taken in reducing our government business. And that's what you should see and the result of it particularly Noida and Greater Noida Hospital those occupancy will see a bit lesser because of that, but you see both the hospitals have done really good in terms of ARPOB and also reducing the LOS. That's a result of it, right. And coming to the second question, as we had mentioned that 10% growth, that's what we are expecting in a couple of years, year-on-year we will be maintaining 10% growth in ARPOB. And see, the group barring the Ram Raja Hospital, our ARPOB in all these hospitals is close to 34000, 35000. So, this is quite remarkable.

Yatharth Tyagi: As far as your first question on the payor mix percentage, so we feel that in 2.5-years' time our government payer mix should be close to not more than 25%. That's what we are targeting and moving towards and even ARPOB growth as Mr. Amit mentioned should continue to be 10% yearly of at least three to four years and bridge the gap between some of our peers. Another reason was certain new therapeutic areas which are high ARPOB booster were not having much volumes for us

earlier. So, oncology, we started recently, liver transplant program, bone marrow transplant, these are high ARPOB therapeutic areas. So, as their volume mature, our ARPOB is bound to increase much more and that gap will be bridged in the years to come.

Runit Kapoor: So, your current government exposure is how much like?

Yatharth Tyagi: It was close to 40% last year, for the nine months I think it should be close to 35% and that's where we hope to end this year.

Moderator: The next question is from the line of Arpit Shah from Stallion Asset. Please go ahead.

Arpit Shah: I wanted to congratulate the management team on a 31% growth, highest in the space. Excluding internal medicine, it would be a 47% growth which is superb execution from the management team.
I j

ust had a couple of questions. What has been our receivables number for the end of '24 December if you can share that absolute number?

Sonu Goyal: We are at around 270 crores and with days around 110.

Arpit Shah: Has the government revenue in this quarter declined on a YoY basis if you can share any insights on that?

Amit Kumar Singh: Yes, so as I said, this was our strategic call, which we've taken it. So, that is what particularly in Noida and Greater Noida Hospitals we had reduced a bit of dependency on government. So, definitely it has been reduced which you can figure it out with our occupancy as well.

Yatharth Tyagi: Also, just to add here, the newer hospitals that we have started, government business is not even 15% in those hospitals and that's how at the overall group level is how we will achieve 25% in next two and half years, because now all these new hospitals that we are starting, so Greater Faridabad Hospital or the new hospital in Delhi and Faridabad from day one because we have super specialties now, we will be having star doctors. The requirement or dependence on the government business, which is sometimes good to have in the beginning is very less for us now. We can go all out and do the expenses from day one unlike a few years back. So, that's why the government business is even very less in the new hospital that we have started or we will be starting.

Arpit Shah: If you can give any kind of guidance for FY'26, because you're going to be operationalizing almost 50% more capacity in FY'26, going from almost 1,600 beds to 2,300 beds, what kind of revenue numbers you're targeting by FY'26?

Yatharth Tyagi: I mean as we said that we have delivered in the past few quarters industry-leading growth as far as YoY is concerned, I think I think it should be on track for that and the company is on track, has always delivered upwards of 30% YoY growth for the last few quarters and we are quite hopeful for sustaining it and increasing it also in the future.

Arpit Shah: Do you expect any change in seasonality in your business given you're adding a lot of capacity in newer areas?
Amit Kumar Singh: As we said, the Northern India particularly, you see the Q1 and Q3 the leanest quarter, Q2 is better, particularly Q4 is the strongest quarter. So, this was a seasonality impact. That's all.
Arpit Shah: That's a good thing, right, because Q3 sequentially has been higher for us like Q2 was lower?
Amit Kumar Singh: Yes, the weakest Q3, still we had maintain our numbers which is quite significant for us and definitely we are quite hopeful for the last quarter.
Moderator: The next question is from the line of Prolin from Edelweiss Public Alternatives. Please go ahead.
Prolin: So, few questions, right? On this census bed, so when you calculate the denominator for utilization, is it fair to assume that census bed is the denominator number that you take? And is there any regulation which you have to follow as to how much day care and non-day care beds you have to keep?
Yatharth Tyagi: Yes, the denominator is always on the census bed as far as occupancy is concerned across the hospital industry.
Amit Kumar Singh: There's no such legal norms, but that's when you start a hospital you do understand, you plan your beds accordingly, you understand these many beds you needed for a day care and these are for IPD and these many beds for your critical care. So, this all goes with the business sense. You do the excel and mathematics and depends on which area you are, what type of population you'd be catering. So, that's a bit of percentage change here and there, but generally, these are the norms in the hospital industry everyone practices.
Yatharth Tyagi: Occupancy is always calculated on the census.
Prolin: An

d now coming to the acquisitions that you made in Delhi and Faridabad, you mentioned that 300 and 400 beds in Delhi and Faridabad will become operational from Q1 FY'26. So, in this, how would be the mix between census, non-census and also how will the cash flow happens, right, would it be similar to how it happened in Greater Faridabad and what could be the average ARPOB that one can expect in Delhi and Faridabad?
Yatharth Tyagi: So, for both those two hospitals, the 400 beds and the 300 beds, census beds would be close to 90% of both those two hospitals beds are concerned. And as far as the ramp up is concerned, we are quite hopeful that what we have replicated in Greater Faridabad, we can also replicate in Faridabad, because we understand that market is better now, we are able to attract more star doctors, with this being bigger hospitals, now there are a lot of doctors who want to join us and become heads for both the Greater Faridabad and the Faridabad Hospital; because they would be somewhere around 15

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kilometers away from each other. So, very different markets of Faridabad region catering to the nearby villages and cities connecting the Faridabad highway from both the side as well as allied. So, the ramp up that we have seen in the Greater Faridabad, we expect it to be even better in both these two hospitals because these two are bigger hospitals having radiation oncology, the Delhi Hospital also have liver transplant, something the Greater Faridabad Hospital was not having. So, we do expect even a better ramp up in these two hospitals than the Greater Faridabad Hospital. And also, the ARPOB for both these two hospitals is also expected to even be better than the Greater Faridabad hospital because the Delhi area pricing as well as their focus on much more super specialties and low government business, so ARPOB will also be even quite good for both these two hospitals.
Prolin: So, then it will be upwards of at least 31000, 32000, that you're reporting for Greater Faridabad for both these hospitals?
Yatharth Tyagi: No, for both these hospitals, it could be even upwards of 35,000.
Prolin: When we look at the nine months margin, right, you mentioned that had there not been the ramp up of Greater Faridabad, the margins could have been between 26% and 27%, which means that there is a 120-bps odd drag due to Greater Faridabad ramping up. Now, when we enter next year, while this drag not will not be there, but there would be a ramp up drag of the acquisitions that we have made, right? So, should the margins from let's say where you end this year FY'25 be higher in FY'26 or this drag will probably pull the margin lower, how should one think about margin?
Yatharth Tyagi: It will not be lower because what also happens then the Noida Extension Hospital and the Greater Noida hospital will also be contributing next year to a larger margins, because the expenses will also ramp up here much more given the increase in the oncology volume, given the increase in the international patients in Greater Noida as well as transplant program. So, the drag will not be lower and the EBITDA margins will be sustainable next year from where we end this financial year.

Moderator: The next question is from the line of Abhishek Maheshwari from SkyRidge. Please go ahead.
Abhishek Maheshwari: Almost all questions are answered. Just one thing. What has January been like considering that you're coming out of a lean season?
Yatharth Tyagi: So, January has been very promising. In fact, December is one of the most leanest months as far as North India and Delhi NCR is concerned. There was a lot of fog visibility, OPD volumes were quite less in December. And as soon as January starts, school reopens there, patients come back from outstat

ions and we do have seen increase in the OPD volumes and this is why in the past also we have seen Q4 has been the strongest quarter in company's history and we have every reason to believe that same would be the scenario this financial year.
Moderator: The next question is from the line of Devang Patel from Sameeksha Capital. Please go ahead.

Devang Patel: What is the target for receivable days although it has been inching down, the absolute amount keeps rising quarter-on-quarter? And what are we doing to reduce receivable days other than reduce government share?
Yatharth Tyagi: So, I think somewhere at the end of this financial year, if we close it, close to 110 days, that's what I think would be a good reduction from the start of the year even though we know we are quarter-on-quarter increasing the revenue, so government business is also increasing. So, on a larger scale, the receivable days is being reduced is what we are expecting, so close to 110 days. And it's just not that government business reduction is the only way we are able to do it, quarter-on-quarter we have done it this year, there are other aspects also. So, last year was anyways an exception. This year we were anyways expecting a larger receivable from the government and we have been receiving. There have been certain recovery teams that have been established headed by the group CEO himself who's constantly being engaged in recovery of these amounts as well as we have outsourced certain of the recovery channels to certain specialized agencies deal with recovery from not just even the government but even the private insurances. That's what the company has been deployed and we are hopeful of closing this year close to 110 days.
Devang Patel: A quick follow up. What is the receivable days you're targeting on a steady state basis? And for Jhansi, what is the share of government business?
Yatharth Tyagi: The share in government business in Jhansi because of Ayushman Card having there is higher than the group average. So, it is upwards of 36% there. But as far as your first question was concerned, as far as the steady state, I think see, in 2.5 years' time, we expect our receivable days to be somewhere close to 75 to 80 days. And this is what we're working on. And quarter-on-quarter, we are seeing a decline in our receivable days and we're quite hopeful with the reduction of government business, this is where we should be in 2.5 years' time.
Moderator: Ladies and gentlemen, due to time constraints, this was the last question. I now hand the conference over to the management for the closing comments.
Yatharth Tyagi: Thank you, everyone for taking out your time and joining our earnings call. As discussed, we hope that our team is able to answer and clarify all your queries and thank you, Nuvama, for hosting this call.
Moderator: On behalf of Nuvama Wealth Management Limited, that concludes this conference. Thank you for joining us and you may now disconnect your lines.

(This document has been edited for readability purpose)
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Subject: Earnings Call Transcripts Q4 & FY25.

Dear Sir/Madam,

Pursuant to Regulation 46(2)(oa) of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed the transcript of Investors' conference call organized on May 27, 2025 post declaration of Audited Financial Results (Standalone & Consolidated) of the Company for the fourth quarter and financial year ended March 31, 2025.

The transcript is also available on the website of the Company at https://www.yatharthhospitals.com/investors in the section of Corporate Announcements.

This is for your kind information and records.

Thanking You

Yours Faithfully,
For Yatharth Hospital and Trauma Care Services Limited

Ritesh Mishra
Company Secretary & Compliance Officer
M. No.: A51166

Encl.: A/a

"Yatharth Hospitals and Trauma Care Services Limited
Q4 & FY '25 Earnings Conference Call"
May 27, 2025

MANAGEMENT : MR. YATHARTH TYAGI – WHOLE-TIME DIRECTOR
M R. AMIT KUMAR SINGH – GROUP CHIEF EXECUTIVE
OFFICER
M R. PANKAJ PRABHAKAR – GROUP CHIEF FINANCIAL
OFFICER
M R. NITIN GUPTA – PRESIDENT , FINANCE AND GROUP
CHIEF OPERATING OFFICER

M R. SONU GOYAL – GROUP FINANCE CONTROLLER

MODERATOR : MS. AASHITA JAIN -- NUVAMA WEALTH MANAGEMENT

Moderator: Ladies and gentlemen, good day, and welcome to Yatharth Hospital Q4 and FY '25 Earnings Call hosted by Nuvama Wealth Management Limited. As a reminder, all participant lines will be in the listen-only mode, and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touchtone phone. Please note that this conference is being recorded.

Let me draw your attention to the fact that on this call, discussions will include certain forward-looking statements, which are predictions, projections or other estimates about the future events. These estimates reflect management's current expectations about the future performance of the company. Please note that these estimates involve several risks and uncertainties that could cause our actual results to differ materially from what is expressed or implied.

I now hand the conference over to Ms. Aashita Jain from Nuvama Wealth Management. Thank you, and over to you, ma'am.

Aashita Jain: Thank you, Avirath, and good day, everyone. On behalf of Nuvama Wealth Management, we welcome you all to the Q4 and FY '25 Earnings Conference Call of Yatharth Hospital & Trauma Care Services Limited.

From the management side, we have with us today Mr. Yatharth Tyagi, Whole-Time Director; Mr. Amit Kumar Singh, Group Chief Executive Officer; Mr. Nitin Gupta, Group Chief Operating Officer and President, Finance; Mr. Pankaj Prabhakar, Group Chief Financial Officer; Mr. Sonu Goyal, Group Financial Controller.

I now hand over the conference call to Mr. Yatharth Tyagi for his opening remarks. Over to you, Yatharth, and thank you.

Yatharth Tyagi: Good morning, and a very warm welcome to Yatharth Hospital & Trauma Care Services Limited Earnings Conference Call for the quarter and year ended March 31, 2025. Joining me today are Mr. Amit Kumar Singh, Group CEO; Mr. Pankaj Prabhakar, Group CFO; Mr. Nitin Gupta, Group COO and President, Finance; and Mr. Sonu Goyal, Group Finance Controller.

Our earnings presentation has been uploaded to the stock exchange and is also available on our website. We hope you have had a chance to review it.

I'm pleased to share a significant clinical milestone. Our Noida Extension Hospital became the first in the region to initiate CAR T-cell therapy, a groundbreaking treatment for cancer patients. This achievement underscores our commitment to advanced care. We also successfully managed several other complex cases this quarter including:

1. A craniopharyngioma in a 3-year-old child with a 52*53*51 mm cystic mass. The case required a high-risk pterional craniotomy, resulting in near total tumour excision.
2. In another case, we operated a large occipital glioma resection in a 45-year-old Iraqi patient using neuro navigation technology to ensure maximal safe removal with no damage to brain tissues.

3. In another high-risk cardiac surgery involving mitral valve replacement, tricuspid valve annuloplasty and ASD closure was performed on an international patient with multiple comorbidities, including CKD and complex cardiac anatomy.

These cases reflects our clinical depth and advanced infrastructure, positioning us on the top tertiary care providers in the region.

I'm also delighted to report another quarter and year of robust performance. Our revenue for the financial year grew over 30% year-on-year, while our EBITDA increased by over 20% year-on-year. Notably, this year, the company generated an operating cash flow of INR1,496
m

illion with a cash conversion ratio of around 70%, reflecting a strong focus on translating profitability into cash generation. This performance is a testament to our relentless focus on operational excellence, strategic expansion and a commitment to strengthening our leadership in the Delhi NCR healthcare market and in North India.

During the year, we made significant strides in expanding our footprints with operationalization of our Greater Faridabad facility in May 2024, adding 200 beds to our network. I'm pleased to inform that within just 10 months, this facility contributed INR436 million in revenues and accounted for 9% of our total revenue in quarter 4, underlining its strong potential.

Furthering our expansion, 2 new hospitals, 1 each in New Delhi of around 300 beds and Faridabad of around 400 beds have been successfully registered, and we have received possession of the site in this quarter. We expect both hospitals to be operationalized by next month, significantly enhancing our presence in Delhi NCR. Our strategic investments in medical infrastructure and super specialty services continue to drive strong patient inflows and value growth across our network.

Further, the proportion of government business in our newer hospitals remains significantly lower than the group average, with Greater Faridabad hospital seeing only 20% of its business coming from government schemes. We expect this trend to continue with our upcoming hospitals in Faridabad and New Delhi, which are also anticipated to have a lower government mix. This shift will drive meaningful improvement in our revenue mix, as well as ARPOB for the whole group.

We have intensified our focus on attracting medical value travel through strategic engagements with key government and private hospitals in Nigeria, health camps and local doctors meeting in Uzbekistan and Tajikistan. Additionally, we have launched information and branding initiatives targeting the African Students Union. We are also setting up information centres in Uzbekistan, Tajikistan, Mauritius, Tanzania and Kenya, and we are actively engaging in partnership discussions with the Ministries of Health in Iraq and Congo. These initiatives are expected, not only to expand our medical value travel business while benefiting from the upcoming airport, but also to significantly contribute to improving ARPOB in the coming years.

On the ongoing income tax matter, I am pleased to highlight a major positive development involving all our shareholders. Our subsidiary, AKS Medical, which operates the Noida

Extension Hospital has recently deposited INR12.38 million as additional tax in response to assessments by the income tax authorities related to previous years. Given that this facility accounts for nearly 40% of the group's total revenue, this marks a substantial step forward in the resolution process. With this development, the assessment now remains pending largely for the parent entity. The resolution at AKS Medical involving a relatively insignificant financial liability reinforces our confidence in bringing this matter to a close soon with no material impact expected on the group's financials.

Furthermore, in line with our commitment to the best corporate governance practices, we have received written consent from BDO India, a leading auditing firm for their proposed appointment as statutory auditor effective from FY2026. This proposal will be presented to the Board and the upcoming AGM for necessary approvals. Meanwhile, Deloitte continues to serve as an internal auditor, supporting us in strengthening our financial systems and internal controls more.

I would also like to take a moment to appreciate the contributions of Mr. Neeraj Vinayak as he moves on to pursue a new career op

portunity outside NCR and wish him continued success in his future endeavours. Looking ahead, we are pleased to share that a highly seasoned professional with strong background in M&A, Strategy, Investor Relations and having previously worked with leading hospitals will be stepping into the role. This transition underscores our continued commitment to strengthening our M&A initiatives and deepening our investor engagement.

As we move forward, Yatharth Hospitals is well positioned for sustained growth, driven by a commitment to clinical excellence, strategic expansion and investment in advanced medical technologies. We remain focused on delivering long-term value to our stakeholders.

With that, I would now like to hand over the call to Mr. Pankaj Prabhakar, our CFO, for a detailed financial update.

Pankaj Prabhakar: Thank you, sir. Good morning, everyone. I am pleased to report Yatharth Hospitals has continued its strong growth trajectory, delivering robust results for both the quarter and the full year.

Let me first take you all through the results for the quarter. During the quarter, Yatharth Hospitals achieved a revenue of INR2,318 million, marking a substantial growth of 30% year-over-year, driven by growth in our occupancy, as well as ARPOB. Our Jhansi-Orchha hospital led the way with an impressive 53% year-over-year growth, while Noida Extension posted a 34% year-over-year increase in revenue, now contributing 7% and 36% to our top line, respectively.

Inpatient volumes surged by 41% and outpatient volumes by 31% Y-o-Y during the quarter. We have also made significant strides in advancing our tertiary and quaternary care services. Earlier this year, we introduced a new radiation PET line at our Noida Extension Hospital and robotics across our Noida Extension, Greater Noida and Greater Faridabad Hospitals. These investments have significantly boosted our oncology revenue with oncology now contributing

18% to Noida Extension's revenue and 10% to the group's overall revenue, a 161% increase from last year.

Our strategic investment in medical and infrastructure and specialty services continue to drive strong patient inflows and value growth across our hospital network. In quarter 4 FY2025, our group ARPOB rose by 7% to INR31,441. Notably, our Noida Extension facility reported the highest ARPOB at INR38,806, reflecting an 11% Y-o-Y increase driven by approximately 70% contribution from super-specialty services.

Our Greater Noida facility achieved an ARPOB of INR34,960, up by 16% Y-o-Y, while our newest facility in Greater Faridabad posted an ARPOB of INR30,384 within its first year of operation, reflecting our focus on high-end health care solutions.

EBITDA for the quarter rose by 23% Y-o-Y to INR570 million. EBITDA margin stood at 24.6% compared to 26.2% last year. The reduction in EBITDA margin for the quarter was on account of operational losses at our Greater Faridabad unit, which became operational in mid-May.

Adjusting for Faridabad Hospital, our EBITDA margin would have been in the range of 26% to 27% this quarter. Overall, our profit grew by 1% Y-o-Y at INR387 million, a relatively slower profit growth, primarily on account of the increase in depreciation expense in the quarter in line with our ongoing expansion and investment in advanced medical equipment.

Now, the highlight for financial year '25. For the full year, revenue grew by 31% Y-o-Y to INR8,805 million, inpatient volume were up by 34%, while outpatient volume were up by 16% Y-o-Y in FY '25. Our overall group occupancy also increased to 61% in FY '25 compared to 54% in the same period last year.

Notably, our Noida Extension and Jhansi-Orchha have shown impressive occupancy gain, reaching 60% and 50%, respectively. EBITDA for

the financial year '25 stood at INR2,202
million, up 22% Y-o-Y with an EBITDA margin of 25%. Net profit for the period stood at INR1,306 million, up 14% Y-o-Y.

Our balance sheet continues to remain strong with a net cash position of INR5,032 million, bolstered by the recent QIP. Our ROCE for the year stood at 19%, impacted by operational losses at the Greater Faridabad facility and expanded capital base and acquisitions and investments in advanced technologies. However, we are confident that this is a temporary blip. And once the new facilities ramp up, ROCE will return to the earlier level.

We remain optimistic about sustaining our growth momentum in both revenue and profitability. Our focus will continue to be on operational excellence, expanding our presence in North India and capitalizing on emerging opportunities in the healthcare sector.

Thank you for your attention. I would now like to hand over the call to the moderator for question-and-answer session, please.

Moderator: The first question is from the line of Ritika from Perpetuity Ventures.

Ritika: Yes. Sir, I just wanted to know that how do you plan to phase the beds in Faridabad and Delhi? And due to the operationalization of the new hospitals, what will be the impact on the EBITDA?

Yatharth Tyagi: So, both the second hospital in Faridabad region and the New Delhi hospitals, we are planning to launch these hospitals before the end of next month. So, their full operations should be

coming up fully from the first day of quarter 2. We feel that the Faridabad hospital will be -- which is a 400 bedded and Delhi is 300 bedded.
So, from day 1, more than 70% beds of both these hospitals would be operational. And as and when the occupancy increases, the beds can be utilized further. But we are quite confident that before the 6 months of both these hospitals starting, we will be fully operationalizing 400 beds, as well as 300 beds in the New Delhi hospital.
As far as your second question on the impact on the EBITDA, we do expect EBITDA losses in both of these hospitals for at least 12 to 15 months of getting operationalized. If you see our Greater Faridabad hospital, which started in this financial year. So, after the full financial year, we have reached a status there where in the latest month, that hospital has become EBITDA breakeven.
So, I think with a similar trend going forward, we do expect the New Delhi and Faridabad hospitals to also be EBITDA breakeven somewhere after 12 to 15 months of operationalizing.
Moderator: The next question is from the line of Akshat Mehta from Seven Rivers Holding.
Akshat Mehta: On your working capital, sir, it's still quite high at the end of FY '25. What is the plan there going forward? And why are the receivable is so high?
Moderator: Sorry to interrupt, Mr. Akshat voice is not audible. Could you move a bit closer to the phone?
Akshat Mehta: My question was I just wanted to understand what is the plan on reducing the receivable days going forward? And why are they so high at the end of FY '25 as well? That's my first question.
Yatharth Tyagi: Yes. So, our receivable days are higher -- on the higher side because our government business is also on the higher side compared to some of the listed peers. Also, if you look at it, however, this year, even with the similar days that we have last year, the company has been able to generate high cash. In fact, our OCF to EBITDA conversion this year is around 68%, almost 70%.
So, the focus is on -- I mean, the money that was struck last year, that amount has now been majoritively recovered by the company. Yes, the new amount has also gone into outstanding receivables, which will also be received in the coming quarters like we have for this whole year.
In fact, if you look

at the H1 of this year, our days were close to 110 days. So, it is only the H2 where the days have increased, which is a similar trend that we do observe sometimes. Quarter 4 is usually the receivables are less. However, as we have shown for the full financial year, the

company has recovered a huge amount of receivables, and we will continue to do that next year.
As far as your question of in the long-term, how to reduce the total days we have, we feel when our newer hospitals get into full scale running where the government business is already quite less, that's where you will actually see the difference in the receivable days. So, the Greater Faridabad hospital has just 20% of government business.
Also, the New Delhi hospitals and the main Faridabad hospital when that starts, the government business will also be around 20% there. And that is the only solution for this long-term receivable days to come down. However, we are quite happy with the receivable that we have received for the full year. And this is the trend that we will be continuing next year also.
Akshat Mehta: Sir, can you -- the follow up on that, sir, can you share the payer mix for the full year?
Yatharth Tyagi: Yes. So, I think our government business is close to 37% and remaining is the breakup is between the private insurance, as well as the cash system.
Akshat Mehta: Again, equal split or --. Hello? Would that be equal split?
Management: So yes, cash TPA contribute to around 63% of the overall business, okay? And the split in cash and TPA, where our cash business is around 31% and rest is our TPA business, 32%. And the cash includes the international business also.
Akshat Mehta: Okay…
Moderator: Sorry to interrupt again, Mr. Akshat, your voice is not audible.
Akshat Mehta: Sorry. So, I just wanted to ask, there was a pledge that was created in quarter 4 FY '25. What is the reason for that pledge on the promoter holding?
Yatharth Tyagi: Yes. So that pledge was nothing to do with the company Yatharth. That pledge was done for the certain personal investments that the promoters made, and it is only in the tune of 8% share of the overall company work pledge. That was a settlement that happened between the cousin brothers of the promoters because a promoter's extended family member, Krishna Tyagi, who was holding those amount of equity had passed away.
So, her shares had to be transferred to her family members. And because of that settlement, the

pledge was done, which was only 8%. So, because the amount that you receive by pledging the pledge has to be 3.5x of that amount, that's why it appears 8%. However, the amount that was raised was only in the tune of INR130 crores that was raised by pledging these shares. And the promoters are quite confident that within a year or so, once the other properties get sold, they would be also unpledging this amount.

Moderator: The next question is from the line of Pawan Bhatia from Nuvama Wealth.

Pawan Bhatia: Congratulations on a good set of numbers. I just had a few questions. Firstly, regarding the tax rate. So, I think this year, we saw a 24% effective tax rate compared to 27% last year. And in

Q4, I think you saw about 20%. So, I just want to understand what level can we see going forward?

Sonu Goyal: So, if you see, last year, we reported around 27% as effective tax rate. And this year, we are at around 24%. I'm talking for the year-on-year basis. And this is the same trend we will continue for the near future. 24%.

Pawan Bhatia: Okay. Okay. Understood. And secondly, in terms of occupancy rate, I think our Noida facility, w

e have seen a drop from 89% last year to 79% this year. But I think earlier, you had mentioned that this is a move which you all have planned to do. So, I just wanted to know that since ARPOB has grown about 10% here, but the revenue has been flat. So, what can we see going forward in terms of occupancy rate at this facility?

Amit Kumar Singh: Yes. So, you're absolutely right. I think if you remember our earnings calls of quarter 1 and quarter 2, that time we had mentioned that we wanted to sanitize the payers of -- payer mix of Noida. And that's what the deliberate attempt we took to minimize the institutional business. So, the result, what you say, yes, this year, you may see as numbers are flat, but the good side of it, if you see the ARPOB has increased by more than 10%, right? So, I think that operational efficiency has increased by doing so. And a similar kind of thought process, we would be having it even for our existing hospital also, we'll try to sanitize the institutional business. But yes, coming year, next financial year, if you see this -- whatever the steps which we have taken, that impact will start coming from this quarter or probably next quarter. So, our specialty mix will be better. The revenue mix will also be see more towards the super-specialty mix. So, we are very hopeful as far as whatever the steps which we have taken as far as Noida Hospital is concerned.

Pawan Bhatia: Sir, even the occupancy rate could see an increase from here.

Amit Kumar Singh: Occupancy it's close to 80%, which we believe is a good occupancy. I mean, there's a 4%, 5% we may see. So, anything between 80% to 85% occupancy, which is a very good occupancy. If you go more than -- see, I mean, 1 quarter, probably it was closer to 90%, which we believe that's difficult to manage. So, I think 85% is a fairly okay occupancy. And I think we are okay with this.

Pawan Bhatia: Okay. Just one last question in terms of the ARPOB by Jhansi. So, I think we have seen about INR13,200 this year in the terms of ARPOB. So, what trend do we look at going forward from here?

Amit Kumar Singh: So, Jhansi will be more or less similar. If you remember earlier, the Jhansi had reported a higher the ARPOB because of the -- initially when we were not having the institutional business, initially, we were not having the PSUs business, cash business was more, right? But now if you see Jhansi is doing a very good institutional business because that area has -- nature of business is more of the PSUs and institutional, right? So that is the reason we believe that I think this is INR13,000, INR14,000 -- I think anything closer to INR15,000 would be a good ARPOB for Jhansi, and that was as per expected line.

Moderator: The next question is from the line of Devarsh Shah from Sunidhi Securities.

Devarsh Shah: So, like the voice wasn't clear. So, can you repeat the payor mix for this financial year for the company?

Sonu Goyal: So, sir, out of 100%, our 37% business is for the institutional business. And there 63% is for the cash and TPA and out of 63%, 31% for the cash and last 32% for the TPA business.

Moderator: The next question is from the line of Sumit Gupta from Centrum.

Sumit Gupta: So just want to ask on the working capital cycle. So, it was around 120 days. So, going

forward over the next 2 to 3 years, how do you see this panning out?
Yatharth Tyagi: As I mentioned, with now the newer hospitals starting up, the Greater Faridabad, where the institutional business is just 20% and New Delhi and Faridabad hospitals, which are getting operationalized very soon, where similar government business is expected. So somewhere -- when those 2 hospitals start contributing. So, I think we do feel that in 2 to 3 years' time, these days should be reducing som

ewhere around 20, 25 days and that's the plan forward.
However, as earlier mentioned, we are quite happy with this year because the company has been able to recover a lot of amount that was stuck in receivables. So that's equally important for us to also focus. Yes, receivable days will come down in the long run. But equivalently, the company will be getting good cash flows from the amount that has been outstanding in the receivables.
Sumit Gupta: Okay. And on a like-to-like basis, so if we exclude these 2 new facilities, so on a like-to-like basis, how has the working capital cycle fared?
Yatharth Tyagi: Yes. So, I think we would expect the 3 newer facilities probably in a year's time would be having receivable days of close to around 60 to 80 to begin off with. And that's how in the overall group level, even though their contribution will be lower in terms of the absolute volume, but then overall at the group level, that's how we would expect the receivables to also come down.
Sumit Gupta: Okay. And next question is on the capex. So how much capex do you plan to invest over the next 2 to 3 years?
Yatharth Tyagi: So, I think last year, we have done a capex of close to INR300 crores, INR310 crores. Similarly, is the road map ahead for this year and the next year. And any other inorganic acquisitions would be separate. So INR300 crores would be done on the existing 7 hospitals, which includes the New Delhi and the Faridabad for this year. And if we look for any other inorganic acquisition or a greenfield land project, that would be a separate capex that the company would be doing.
Sumit Gupta: Okay. So basically, capex per bed comes out -- usually comes out to INR7 million to INR8 million per bed, if I'm not wrong?
Yatharth Tyagi: For the existing hospitals.

Sumit Gupta: For the existing or for the acquisition that you are looking at, if there is any...
Yatharth Tyagi: For the acquisition.
Sumit Gupta: Like, it would be more than INR1 crores.
Yatharth Tyagi: Yes, acquisitions would be somewhere around INR80 lakhs to INR1 crores ex of land for the inorganic acquisition, you are correct.
Moderator: The next question is from the line of Umakant Sharma from Vanj Venture.
Umakant Sharma: Could you just touch a little bit on how should we see the ARPOB evolving given that we are adding new capacities? And similarly, as with the new existing capacity as well, so how should we be seeing the ARPOB for the current year, as well as going ahead for '27? And similarly, what should be the baseline which should be setting up for the margins for the current year, as well as for the next year?
Amit Kumar Singh: So, if you see, we -- last year, the ARPOB growth, be it in Noida Extension Hospital or even in a Greater Noida Hospital, that's more than 10%, right? So, I mean, this would be in an expected line. So, as far as new hospital is concerned, as Yatharth already mentioned, that's where the very first day, we're going to start with a very tertiary and quaternary nature of the business. And also, the institutional business would be significantly low. So there, you will see a very good guidance for ARPOB concern.
I don't want to give you any particular number. But yes, this will be much better than what probably as of now what we are having it. And you see the trend is suggesting the same. I mean, more than 10% ARPOB year-on-year, we are growing. So, you can even draw the inferences accordingly.
Umakant Sharma: So, would it be fair to say it should be in the range of INR30,000, INR33,000 ARPOB for the -- at least for the initial ramp-up phase for the newer hospitals?
Amit Kumar Singh: Yes, I think that -- yes, that start with -- in fact, to be very honest, it would be closer to what probably today we have Noida Extension

and Greater Noida. So closer to INR35,000 kind of line start with.
Umakant Sharma: Okay. Perfect. Perfect. And on the margin side, what should be the baseline we should be setting given that there will be a lot of incremental costs that will be getting added for the

current year, as well as for '27. If you could just throw some color around that as well?
Yatharth Tyagi: So, I think overall margins, we feel of what we have reported in quarter 4, the overall margins should be somewhere around that for the whole financial year '26 and '27 individually, maybe 0.5 percentage up or down. But I think the Q4 margins would be a right estimate to take for the financial years going forward.
Umakant Sharma: So, you don't think there could be further dilution given that next quarter is when a lot of the capacities will be getting started. You don't see further downside in the margin?

Yatharth Tyagi: That's why we said that quarter 4 was a bit lower than the full financial year FY '25. That's why we have taken quarter 4 as the right assumption. And yes, we do expect the new facility to start, but also let's not forget that our existing facilities will also be expanding in margins because there's still occupancy ramp-up that is still remaining in Greater Noida and Noida Extension Hospital and also the Greater Faridabad hospital will be EBITDA positive from the coming quarters. So that is why we feel they will also help in the sustaining the EBITDA margins that we have in quarter 4.
Umakant Sharma: Got it. Got it. And sir, 1 question on the occupancy side. Normally, in a typical new setup when you put up a new hospital, especially when you have acquired a hospital. What is the time that it takes? How many -- from a year standpoint, how much time does it take typically for it to ramp up to a good level of occupancy?
Amit Kumar Singh: So technically, if you see our Faridabad and Delhi hospital is in a way greenfield hospital, right? We are starting business from the scratch, right? So, it takes around 12 to 18 months it will take for us to reach the operational breakeven, right? And then from there, then we'll see. But sometimes it's early as well, but we don't want to commit as such any this thing. But anything it's fair assumption is 12 to 18 months would be the right assumptions to have reaching an operational breakeven.
Umakant Sharma: Okay. So -- and what occupancy do we say that we are operating -- operational breakeven be, about 30%?
Amit Kumar Singh: It will be closer to -- anything closer to 35% -- 30% kind of occupancy, which we may consider will be, but depends on size as Faridabad hospital is a much bigger hospital, right? Whereas Delhi is a 300-bed hospital, Faridabad is 400 beds. So, there is a 1% or 2%, 3% here and there depends on the specialty and in which location you are and what the clinical mix you are starting with. So that varies, but 30% would be a fair assumption.
Umakant Sharma: Got it. Got it. And just, sir, 1 last question from the one of the previous participants, right? If you could just throw some color around this capex guidance which you have given? So, you said that there's going to be INR300 crores of capex, which will be incurring on the existing hospitals, plus there will be some acquisition and all of that. So, this INR300 crores, where you will be deploying it? If you could just throw some color on that?
Yatharth Tyagi: Yes. So, the breakup of INR300 crores roughly, you can see that the Faridabad hospital, the 400 bedded. So, around INR100 crores would still be spent there for the medical equipment, as well as construction because that is still an ongoing capex. It has not started, and we are at the final stage of now procuring the equipment. So that will technically lie for this financi

al year.
So INR100 crores will go into the 400 bedded Faridabad hospital. Around, I would say, 60 -- around INR55 crores will go to the New Delhi hospital, medical equipment’s also. That's also the procurement is still ongoing there. So close to around INR150 crores in these 2 hospitals as far as medical equipment’s are concerned.
And the remaining would also be spent certain amount on our existing 5 hospitals that is the Noida Extension, Greater Noida, as well as the Greater Faridabad Hospital. And then the remaining capex will also be spent on the brownfield expansion that we would be doing for the

Greater Noida and the Noida Extension hospitals, where we are adding 200 beds and 250 beds each. So that will also be there, and that is how a total of INR300 crores amount you reach.
Umakant Sharma: Got it. Got it. And any color on the acquisition front? Are we evaluating something on the greenfield side, any thoughts for the current year?
Yatharth Tyagi: We would be looking to add at least 1 facility for this financial year. I think in the coming quarters, hopefully, we should be able to give more clarity on that.

Moderator: The next question is from the line of Anuj Kashyap from A3 Capital.
Anuj Kashyap: Just wanted to know that what is the margin differential between our international patients and the domestic patients? Is there any differential?
Amit Kumar Singh: Yes. So typically, if you see institutional business and the insurance business and international business and cash business, there is a work on various percentage. In international business will give you probably 1.3x kind of, right, whereas it's INR100, if you take as in a cash business. TPA insurance business will be probably 90% of it. The institutional business will be closer to 60% to 65% of it. So, this is how it goes.
Anuj Kashyap: And sir, add on to this 1 question is, like, sir, in NCR, there are a lot of MNCs and there are a lot of institutions. So, is there any strategy from our side to capture that market like wellness stuff, which the institutions are focusing on like companies and all?
Amit Kumar Singh: So, I think -- yes, I think that for every hospitals, if you see, there is a very separate institutional business when I talk about just, not only the government, it's about the corporate and PSU. So, there are a whole lot of the corporate and PSUs existing in this region. And, of course, we have a separate team who are already working on that and most of them would be on our -- already been empanelled and some of them would be ongoing. So, it's ongoing activity, and we have a separate team, a very aggressive team who are working on it.
Anuj Kashyap: And sir, last one from my side. What I wanted to know is that, from the past experience as a team, Yatharth team, what we have experienced is, which is -- especially we have to focus on the capex side, which is beneficial for us, greenfield or brownfield expansion, as your past experience?
Yatharth Tyagi: We have -- today, if you see, we have always worked on a mix philosophy. Yes, we started from greenfield, the Greater Noida, Noida Extension, Noida Sector 110 have been greenfield expansions. And then going forward, we have acquired hospitals. When the company grew, we were able to raise capital. We were able to acquire greater Faridabad hospital, now Delhi and Faridabad hospital. So, we have seen a mix of greenfield acquisitions and now brownfield, obviously, we are expanding the Noida Extension and Greater Noida. We have had a good fair mix that we are working alongside 3 strategies.
But one thing that we are clear on is that, so far, we have always own 100% equity, including the land and the buildings with us. And there has been no hospital on O&M lease model. So, I think so far

, that has been our strength. And that is also somewhere helped in our margins as well that having no cost for O&M to share. So, I think that has been a strong reason for a good

margin visibility also. So that's what we have so far believed in. But as far as brownfield, greenfield acquisition is concerned, we have been doing a mix of the philosophy there.
Anuj Kashyap: Sir, just a little more. So, like the hospitality sector, so we are different -- the 2 sectors are totally different, but the asset-light model for hotels will not work -- will not be more beneficial for our balance sheet -- on the balance sheet side.
Yatharth Tyagi: See, I think so far, we have -- we feel that for us, the location and the asset is very important. And sometimes what we can get by creating a whole infrastructure or by acquiring certain assets which are available for 100% equities are not always available for O&M model. And technically also, if you look, so far, even the acquisitions we have done have been sort of hospitals which are non-operational.
So, we have started them up fresh from the scratch. We have not acquired assets which were running. So, we feel because we do not want to compromise on the quality of the asset, the geographic location, infrastructure, minimum size of beds is very important to us.
And we don't mind acquiring those assets and starting afresh as a clean slate. And in fact, that has worked for us, both the Greater Faridabad facility, as well as Jhansi-Orchha hospitals were non-operational for some time before we acquired it. Similarly, how the Delhi hospital, Model Town hospital was non-operational, used to function a year ago. So, I think because of these factors, we are okay with acquiring assets rather than trying to get them on lease.
Anuj Kashyap: And sir, last one, sir, from my side is, sir, is there any -- like you told you were looking for the expansion. So, sir, how strategically, geographically, we can add some different geography to our book so that our portfolio as a group become balanced? Are you looking for a geographical differential in this?
Yatharth Tyagi: So, we don't want to go much out of our geography areas that we are currently present in. So, we see Delhi NCR, North India market of UP, Haryana, probably nearby states like Punjab, Rajasthan, Bihar, these are strong areas. So, this is where we want to be in the coming years. We don't want to come to Southwest. We want to be in the areas where we have strong presence.

In fact, if you look there are hospitals, we have more than 1 hospital within the same city now. We have 2 hospitals in Faridabad, 1 in Faridabad and Greater Faridabad, and then in Noida we have 3 hospitals. So, we believe in being the biggest player in the cities and the region where we operate. And that really gives us a much more brand visibility rather than going to areas which are 5,000 kilometres away where we started off from.

So at least for the next 3 to 4 years, we want to solidify our presence. And these are the areas which we understand very well. Today, to attract doctors in NCR is not an issue for us because we have created a brand here. If we go to other geographies across India, it will take us some time to develop this brand equity yet.

So that's why for the next at least 3 to 4 years, we are very clear, we want to be in North India, the region we are very well understand. And we feel it's the easiest growth for us because

there's so much potential still remaining in the areas where we operate, and we will be leveraging that potential, as well as the brand there.

Modera

tor: The next question is from the line of Vicky Waghwani from Guardian Capital Partners.

Vicky Waghwani: Sir, my first question is on depreciation. It has declined quarter-on-quarter from INR169 million to INR129 million.

Sonu Goyal: So yes, basically, depreciation declined quarter-on-quarter. Main reason is because of the deferred tax. So, there's a deferred tax asset has been created for this quarter to the tune of INR3 crores that leads to the decline in the overall number in current tax.

Vicky Waghwani: Okay. And sir, what would be the quarterly run rate going forward as we operationalize the hospitals?

Sonu Goyal: For taxes? Or for depreciation?

Vicky Waghwani: Yes, quarterly depreciation from next quarter.

Sonu Goyal: The amount quarter-on-quarter will be same, what we have reported in Q4, excluding the 2 new assets. So, this is for the 5 assets as of now. So, for all the 5 hospitals, the depreciation, what we have reported in quarter 4 will be the same from next quarter onwards.

Vicky Waghwani: Okay. Sir, can you please repeat the update on income tax about the subsidiary? And also, you highlighted that there is ongoing case about parent company. Please, if you can give the details on the same.

Yatharth Tyagi: Yes. So, what I mentioned earlier in my commentary was we see a huge development in the ongoing income tax matter. The assessment for the AKS, which is the Noida Extension hospital, which contributes to -- that subsidy contributes to 40% to the parent Yatharth company.

The assessment has there resulted in a requirement for the company to deposit an additional tax of INR12.38 million. So, we feel it is as per our stance and expectation that it will be a very insignificant financial liability for the company. And this is the main subsidiary of 40% is contributing.

The other subsidiaries that is the Ramraja, as well as the Greater Faridabad hospitals are very small contributors to the revenue. Greater Faridabad is just INR40 crores. Ramraja would be around INR60 crores. So now the matter would be for the parent company Yatharth, which operates in Noida, as well as the Greater Noida hospital. So, we are quite confident also that going forward, these 2 specific hospitals will also be resolved and ultimately, hoping to close the complete income matter before the end of this financial year.

Moderator: The next question is from the line of Anand B from KSEMA Wealth Private Limited.

Anand B.: Congratulations on a good set of numbers. I just want to know what would be the segment revenue breakup in terms of geography standpoint for the next financial year?

Yatharth Tyagi: So, I think we expect how we look at is hospital-wise. So, the total growth that the Noida region has shown, which includes the Noida hospital, Great Noida and Noida Extension will still continue at a similar trend because there's still a lot of occupancy ramp-up that is yet to be seen in Noida Extension and Greater Noida. And in fact, Noida 110 also that we spoke about earlier is increasing in ARPOB. So, we expect a similar trend of growth there.

As far as the Faridabad region is concerned, now 2 hospitals will be working there. So earlier only Greater Faridabad was there. But now the main Faridabad hospital, which will also have

cancer treatment, which is a much bigger hospital, will start. So the Faridabad contribution of what it was for this financial year of FY '25 will, in fact, actually double, if I talk about in the financial year '26 because 2 hospitals starting there and also the Greater Faridabad ramping up the numbers after the full financial year of working.
And as far as Delhi is concerned, it's a new territory, in fact, in the sense that it is the first hospital we are starting there in the region of Model Town, which

is in North Delhi. So
probably what the numbers that we have seen in Greater Faridabad for the first financial year could be the number that we can see coming up from the Delhi region. So, this is roughly the overall analysis that we see geography-wise.
Anand B.: Sorry, can you just repeat on the Delhi portion, please?
Yatharth Tyagi: Yes. So, Delhi hospital, we do expect a similar performance of what our Greater Faridabad hospital has shown in the first financial year. So, I think the numbers coming from the Delhi geographies would be something similar to what our Greater Faridabad has shown for this year.
Moderator: The next question is from the line of Pratik Bora from Share India.
Pratik Bora: Yes. Sir, I missed the earlier minutes of the call. So, can you just guide me about what impacted the EBITDA margin on Y-o-Y basis that it has been 29% to 25%? So, can you give me some light on that?
Yatharth Tyagi: So, we were mentioning that somewhere the EBITDA margins that we have shown in quarter 4 should be the right basis for the financial years moving forward, maybe 0.5% up and down, but something similar to what the EBITDA margins in quarter 4 are.
Pratik Bora: Okay, sir. And sir, my 1 more question was that right now, we are having an working capital of -- we have increased by 17 days. So, can you give me a reference to how -- what will we do better?
Yatharth Tyagi: As the newer hospitals get operationalized, that is the Noida, the Faridabad hospital and also the Greater Faridabad contribution increases for -- because it was just its first year of working.
There, we just see government business contribution coming to only around 20%, and that will be the real value addition that will be coming on our working days, as well as working capital and debtor days.
So, I think in the long run, it is expected to go down for sure. However, what's important is that we continue to get the receivables and each year. And this year, we have seen a good

amount of cash flow coming in the company because of the receivables that we are able to recover. So that's equally important for us going forward also. But yes, we do expect the working capital and receivable days to come down in the long run.
Pratik Bora: Any expected working capital days for the next 1 year?
Yatharth Tyagi: I think in the long run, after 2 to 3 years, we should be closer to the days of 80 to for the overall company. And yes, in the newer hospitals, it would be much lesser.
Pratik Bora: Any future guidance on top line basis for the next financial year, FY '26?
Yatharth Tyagi: I think we will continue to do well. That's all we can say. I mean, the company has shown growth of 30% yearly Y-o-Y for last few years now, in fact, so not just FY '25, but even before that, we have continued this growth momentum. And we are quite confident in the future also; we would like to continue this momentum going forward.
Pratik Bora: So, while evaluating the valuation, we can expect the same number -- same percentage of growth, correct?
Yatharth Tyagi: I mean, hopefully, that is the way forward. This is what company has done in the past. Maybe...
Pratik Bora: Yes, I was asking about the numbers that we are expecting for the future of next 1 year.
Yatharth Tyagi: We feel the numbers that we have delivered in the past year-on-year should be a sustainable number even for the coming few financial years ahead.
Pratik Bora: Okay. Sir, can you just throw some light on the reserves? So, reserve has been doubled for this year from March '24 to '25. So, what we see for the next 1 to 2 years overall? So, we are having any growth.
Nitin Gupta: So basically, reserve has been doubled because there was additional profitability, which has been arrived in this year by INR1,305 milli

on. Along with that, we have done the QIP, there
was a security premium, which has been there of INR6,145 million. and there are certain expenditure related to fresh issue of shares expenditure, which has been dealt in this year.
Hence, this way we have arrived to the reserve and surplus, which has grown to INR15,091

million.
Pratik Bora: So, we will continue this or we will have a stagnant growth just because of QIP?
Nitin Gupta: No, It was onetime which has been done with the QIP in this FY, and when the share has been allotted there was security premium which has been done in December 2024, which has added to the reserve of INR 6,145 million additionally in this year, particularly.
Moderator: The next question is from the line of Kunal Tokas from FVC.
Kunal Tokas: Okay. Just two quick questions, sir. First is, you have laid out the capacity growth plan. But what is your preference in terms of what would the specialty mix look like after these capacities come online?

Yatharth Tyagi: So far, we have always maintained a good specialty mix. We are a super-specialty hospital. We do not specifically focus on a single specialty. We like to have leading clinicians and high-end treatments across all the specialties. But as the transition we have seen in the last few years and the current trend is our super-specialties are growing much faster. In fact, the specialties like oncology has increased a lot and the departments like high-end neurosurgery, cardiac surgery, these are departments which are increasing.
And basic general specialties like internal medicine, paediatrics, gynaecology, these are reducing in terms of share of pie. So going forward also with these new hospitals, we would be doing a high-end super-specialty work and expecting our super specialty contribution to be somewhere around 60% to 70% in each hospital and probably around 30% would be from the general specialties.
Kunal Tokas: Okay. Got it, sir. And second question was, you mentioned this in passing earlier that your brand helps you get doctors easily. But for the capacity expansion plans that the industry as a whole has and that you have, do you think talent availability can be a challenge for the industry or for you?
Yatharth Tyagi: See, that is why we -- I earlier mentioned that we like to be in the areas where we already have a good brand presence. So, when we're going to Delhi, we are located -- our previous hospitals are already in Noida and Greater Faridabad. So, there are a lot of doctors even today that come to us from Delhi and a lot of doctors known our brand there. So, it's easier for us to attract star doctors in that region. Similarly, for the Faridabad hospital because already we have a hospital in Greater Faridabad. Today, a lot of doctors in Faridabad wants to join us.
And in fact, in both of these hospitals, the Faridabad and the New Delhi hospitals that are opening up, we have already onboarded certain leading specialists and that process is constantly going on right now. So, because we are going in the areas where we understand the market and NCR is not shortage of doctors there.
There's still a lot of doctors that are available in Tier 1 cities. If we try to go to Tier 3 cities, let's say, in the past, from my experience in Jhansi-Orchha hospital, it did take us time there to onboard super-specialist doctors. But in our NCR hospitals, it is not a challenge for us.
Moderator: The next question is from the line of Vijay Chauhan from RH PMS.
Vijay Chauhan: Yes. So, I would like to understand when do we take the price hike? And what is the range for the annual price hike that we take?
Amit Kumar Singh: So typically, that depends on where we are entering into it. So, we look at that -- we study the market. And generally, if yo

u -- if I tell you that year-on-year, we see the price hike, but it's not that on a blanket this side. So, we try to look at some specialty, we try to look at some of the procedures wise. So, every year, there are some corrections happens, right?
And also, on top of it, every 2 years, there is a correction from the GIPSA insurance pricing, right? So, this is typically which you see. But like when we set up a new hospital coming in Faridabad or Delhi, we'll try to look at our existing market there in a computer, what all the

pricing and accordingly, we'll put our pricing strategy. So typically, year-on-year, you may see.
Vijay Chauhan: And when does it happen typically? Any particular quarter like financial year or calendar year?
Amit Kumar Singh: Yes, typically, it's a new financial year. But in-between also it's required that some corrections can happen, but that too, you'll get advantage of in cash because insurance you can always -- we can do at one. You can't do in-between.

Vijay Chauhan: Okay. Okay. And what is the seasonality in terms of quarter, like which are the stronger ones and which are the weaker ones for the hospitality space? Can you just throw some light?
Amit Kumar Singh: Yes. So Northern India has a seasonality impact. But see, seasonality is like it has an impact on disease mix, that's the clinical mix. Because if you see Q1 and Q4, you may see a strong quarter, right, in terms of the revenue or something.
But in terms of the volume, you may see Q2, Q3 is a good volume because of the dengue and other things comes, right? So, this has an impact. But when you come to the like Q3, you tend to get a good number of super-specialty cases, particularly cardiac and neuro, right? So, it has seasonal variations are there, but it changes with the qualitative and quantitative both as well.
Moderator: The next question is from the line of Aakash Shah from Asian Wealth.
Aakash Shah: My question has already been answered.
Moderator: The next question is from the line of Yash Poddar from Viansh Ventures.
Yash Poddar: Yes. So, I have 2 questions on the business front. So, the first question is with regards to that going forward, as we have already seen a shift in revenue mix from the general internal medicine side towards the specialty segments, which has already come into play. So futuristically, going ahead, how much does that change the total cost structure and the fund application?
And in turn, how will that affect the capex amount per bed, does that significantly inch up that amount because of higher capex's in specialty equipment's or specialty expansions as such? Or does it keep the cost of operation roughly the same?
Amit Kumar Singh: So, the answer cannot be like a 1 answer. So, if you look at it, if it's an existing hospital, right, where we are having operation for the last couple of years where all the super specialty will be there, right? So, we don't require much of the additional expenditure on it because you've already spent on equipment and you've already onboarded those super specialist doctors, right?
But yes, if you want to start any super specialty high-end treatment like a transplant and onco or something, it has a cost in terms of the equipment and infrastructure or a doctor side as well.
So -- but for the new hospital, let's say, Faridabad and Delhi, we want to start very first day, absolutely very high-end treatment procedures our quaternary care. So, we would like to have a cancer, we'd like to have a transplant program there. So there that it totally depends on what type of the clinical mix you want to start with that particular facility. So, this has this thing.

And then it's all tailor-made totally with you.
Yatharth Tyagi: So, it's -- see, I think it would be right to take it on a year-on-year basis. Ultimately, the capex and operational efficiencies do not have to start from literally the first quarter when the hospital gets started because it also takes time to get even after operating to get licenses as far as, let's say, transplant program is concerned. And oncology also takes time.
The machines usually -- so let's say, in our Faridabad hospital, the oncology machines will not be there from first quarter. They will be there somewhere around quarter 3 because it takes time for the machine even after order to come to India.
So overall, the whole year, yes, the operational cost spread is there, but it still is done quarter-on-quarter for the company. So, the whole loss doesn't come straightaway from quarter 1. And by the time you ramp up to these super-specialty treatments with specialty machines, basic hospital working also starts. So ultimately, that's how the overall year-end is the right way to look for the first year operating of a hospital rather than looking at the first year from quarter-on-quarter basis.
Yash Poddar: Okay. Okay. That's helpful. And the second question that I have is, with -- can you shed some light on the doctors' attrition rates within the company? And also, you mentioned that because of our current brand, there's -- we've been pulling doctors in for the specialty segment. So how does that empanelment look like for the new specialties that we are now -- which are now contributing important parts of our revenue base? And what are some of the ways that we are doing these empanelment's and also the attrition numbers of doctors?
Yatharth Tyagi: Yes. So firstly, as far as empanelment of doctors are concerned, if you look across our number of doctors across the group have rapidly increased. Even within the same specialties that we were having earlier, we are having multiple doctors within the same specialties. So, it's not just the newer specialties, the new doctors are being onboarded, but we are constantly engaging with leading doctors of this region outside NCR also who are joining us in our existing specialties also.
So, in fact, today, with these 2 hospitals getting operationalized, our total doctor count could even touch upwards of around 800 doctors. That includes consultants, senior consultants, junior consultants, overall doctor numbers if I talk about. And attrition has really come in

control for our group.
If I talk about 3 years ago, it used to be much higher, but that was because of certain programs that we were not having as far as the educational institutions within the hospitals are concerned.
So DNB, as soon as we started DNB students who are coming to us, now we do not require to hire many junior doctors, and that is where the attrition is highest because now we get junior doctors from the DNB students who join us remain at least for 3 years and then pass out with a post-graduate equivalent degree. So that is why attrition has come down a lot. If I talk about in super specialty and senior doctors, our attrition is not even 20%. And as far as junior doctors are concerned, it has really come down after the start of DNB program.

In fact, 1 place where attrition is still a challenge for a lot of hospitals in North India is the nursing attrition. Nursing attrition is very high compared to even South India. North India and Delhi-NCR have had this problem much more. So that is what even today, we are trying to take a lot of steps as far as doctor retention -- nursing retentions are concerned.
One of our hospitals that is a Greater Noida hospital will very soon be star

ting nursing courses
where GNM and courses of nursing, BSc nursing students can come and get nursing degree and then we can place them directly placements into our hospitals. So, there are a lot of steps that we're constantly taking. And today, because we have accreditations like JCI, it is much easier for us to attract these doctors, these nurses because ultimately, quality of a hospital does help to attract these talent when it comes to it.
Moderator: The next question is from the line of Akshat Mehta from Seven Rivers Holding.
Akshat Mehta: Yes, sir. So, I just had 1 question. I just wanted to understand, have we started anything on the brownfield expansions that we were going to do? And what is the exact date that they will be operationalized? Is there any update on that?
Yatharth Tyagi: Yes. So Greater Noida Hospital, the brownfield expansion, the basement working has started there. That is the whole architecture plans has been approved. All the compliance work is going on. And as far as Noida Extension is concerned, we are in final stages of finalizing the whole architectural drawing of the brownfield expansion and the addition of the floors are concerned. So, we are quite confident that within a timeline of 1.5 years from today, we should be having these capacity to use. Greater Noida will be much faster because it is a step further ahead.
Moderator: The next question is from the line of Saurabh Dole from True Beacon Investment Advisors.
Saurabh Dole: I just had 1 question. The expansion that you did in Jhansi-Orchha, did ARPOBs there are obviously distinctly lower than the rest of the hospitals. So going forward and especially given the fact that you obviously have a plan of adding some hospitals, at least 1 hospital this financial year, does it mean that you will not go out of New Delhi and perhaps Jhansi-Orchha was a lesson that you should not be venturing out into such areas where ARPOB acceleration can be a challenge?
Yatharth Tyagi: No. I think it is nothing specific to Jhansi-Orchha only. But in general, we feel it is better to stay in Tier-1 cities to at least stay in NCR region. If not NCR, then even there are big cities across UP like Lucknow, Kanpur and Rajasthan, like Jaipur, Punjab has some really good cities that are coming up where the health care market is quite attractive.
So, I think we feel that this is where we would like to be in the years going forward. And it's not that we learned anything different from Jhansi. It was as per expectations. We expect the hospital to perform on what it is expecting. If you look at it, the investment we have done, we are quite happy with the return that we are getting from Jhansi. Look at the ROCE and the ROE numbers that, that hospital has.
Yes, the ARPOB is not very high. So, the scale in terms of total contribution to the hospital revenue per month or to the whole group is less. But in terms of the amount that we paid to get

that property, it is breakeven within our expected timeline. We are generating profits from there. So, we are happy with the way the bus is running there.
As far as going forward, we do feel that the market of NCR and big cities in the nearby states are much attractive, and we can make a much bigger impact to the group if we expand in those

areas.
Saurabh Dole: So, if you look at the ROCE of perhaps the Jhansi hospital, the ARPOBs are nowhere close to what the other hospitals are generating. So, the kicker on the ROCE comes from what? Is it the lower initial capital that you put out to buy that hospital? Or is there any other metric there?
Yatharth Tyagi: Yes. Yes, the lower capital that was initially done to buy that hospital.
Saurabh Dole: Okay. And does it also mean that you will not be adding all sorts of other special

ties that you
have in the other hospitals at Jhansi-Orchha or will it be at par?
Yatharth Tyagi: No. I mean, we are doing almost all major specialties that we have in NCR, except oncology and transplant program. Because see, if you look at transplant program like liver, kidney and bone marrow transplant, majority of the patients who come for these treatments are international patients. So, we don't -- we feel in NCR, there's much scope for these international patients to come. And as far as also oncology is concerned, we feel that it is much better to have those in this region rather than in a place like Jhansi-Orchha. But in other hospitals going forward that we will be acquiring, we would be doing all specialties, including cancer, as well as the transplant program.
Amit Kumar Singh: Just to add what Yatharth said, if you ignore the numbers, see, Jhansi hospital adds lots of goodwill to our brand. So, if you look at the existing numbers in Jhansi, we are doing a very good super-specialty work, neurosurgery, orthopaedic, neurosurgery is doing a fantastic number.
Yes, the ARPOB is low because we know that the kind of -- even if you won't get an incidence rate, what you are getting in Delhi NCR, right, then the institutional business is much more. But it adds lots of value goodwill around 200, 250 kilometres that area, right? And that we're also getting so many super-specialty cases from those regions coming directly to Greater Noida side. So that's the advantage we have.
Moderator: Thank you. Ladies and gentlemen, due to time constraints, that was the last question. I now hand the conference over to the management for closing comments.
Yatharth Tyagi: Thank you, everybody, for your questions, and we hope we were able to take most of the questions and answer. On the whole management of Yatharth Hospital's behalf, I thank you.
Thank you to the moderator and Nuvama Wealth for organizing this earnings call. Thank you.
Moderator: Thank you so much. On behalf of Nuvama Wealth Management Limited, that concludes this conference. Thank you for joining us, and you may now disconnect your lines.

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Call: Aug 2025

YH/SE/39/2025-26
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Subject: Earnings Call Transcripts Q1 & FY26

Dear Sir/Madam,

Pursuant to Regulation 46(2)(oa) of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed the transcript of Investors' conference call organized on August 06, 2025 post declaration of Unaudited Financial Results (Standalone & Consolidated) of the Company for the first quarter ended June 30, 2025.

The transcript is also available on the website of the Company at https://www.yatharthhospitals.com/investors in the section of Corporate Announcements.

This is for your kind information and records.
Thanking You

Yours Faithfully,
For Yatharth Hospital & Trauma Care Services Limited

Ritesh Mishra
Company Secretary & Compliance Officer
M. No. A51166

Encl.: A/a

"Yatharth Hospitals and Trauma Care Services Limited
Q1 FY '26 Earnings Conference Call"

August 06, 2025

MANAGEMENT : MR. YATHARTH TYAGI – WHOLE TIME DIRECTOR
MR. AMIT KUMAR SINGH – GROUP CHIEF EXECUTIVE

OFFICER
MR. NITIN GUPTA – PRESIDENT (FINANCE) & CHIEF
OPERATING OFFICER
MR. PANKAJ PRABHAKAR – GROUP CHIEF FINANCIAL
OFFICER
MR. ASHUTOSH KUMAR JHA – GROUP CHIEF –
STRATEGY , M&A AND INVESTOR RELATIONS
MR. SONU GOYAL – GROUP FINANCE CONTROLLER
MODERATOR : MR. AMAN -- PHILLIPCAPITAL (INDIA) PRIVATE
LIMITED

Moderator: Ladies and gentlemen, good day, and welcome to the Q1 FY ’26 Earnings Call of Yatharth Hospitals and Trauma Care Services Limited hosted by PhillipCapital PCG Desk.

As a reminder, all participant lines will be in the listen-only mode, and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing “*” then “0” on your touchtone phone. Please note that this conference is being recorded.

Let me draw your attention to the fact that on this call, discussions will include certain forward-looking statements, which are predictions, projections or other estimates about the future events. These estimates reflect management’s current expectations about the future performance of the company. Please note that these estimates involve several risks and uncertainties that could cause company’s actual results to differ materially from what is expressed or implied.

I now hand the conference over to Mr. Aman from PhillipCapital (India) Private Limited. Thank you, and over to you, sir.

Aman: Thank you, Shruti. Good afternoon, everyone. On behalf of PhillipCapital Private Client Group, I welcome you all to the Q1 FY ’26 Earnings Conference Call of Yatharth Hospitals and Trauma Care Services Limited.

From the management side, we have with us today Mr. Yatharth Tyagi – Whole Time Director, Mr. Amit Kumar Singh – Group Chief Executive Officer, Mr. Nitin Gupta – President (Finance) and Chief Operating Officer; Mr. Pankaj Prabhakar – Group Chief Financial Officer, Mr. Ashutosh Kumar Jha – Group Chief – Strategy, M&A and IR, and Mr. Sonu Goyal – Group Finance Controller.

I now hand over the conference call to Mr. Tyagi for his opening remarks and we will then open the floor for the question-and-answer session. Over to you, Mr. Tyagi.

Yatharth Tyagi: Good afternoon, and a very warm welcome to Yatharth Hospitals and Trauma Care Services Limited earnings conference call for the quarter ended June 30, 2025.

Joining me today are Mr. Amit Kumar Singh – Group CEO, Mr. Pankaj Prabhakar – Group CFO; Mr. Nitin Gupta – Group COO and President (Finance), Mr. Ashutosh Kumar Jha – Group Chief - Strategy, M&A and Investor Relations, and Mr. Sonu Goyal – Group Finance Controller.

Our Earnings Presentation has been uploaded to the Stock Exchange and is also available on our website. We hope you have had a chance to review it.

I am pleased to report another quarter of stellar performance, achieving our highest ever revenue and profitability. Our revenue grew 22% year-on-year, while net profit surged by 38% year-on-year, reflecting the strength of our strategic initiatives and operational excellence.

A key highlight, this quarter was the Greater Faridabad facility, which turned net profit positive in Quarter 1, within just one year of operations. It contributed Rs. 234 million in revenue, accounting for approximately 9% of our total revenue. This rapid turnaround underscores our capability to ramp up and optimize new assets effectively.

Looking ahead, our new hospitals in New Delhi and Faridabad with a combined capacity of ~700 beds will further accelerate growth from Quarter 2 FY ‘26 onwards. The New Delhi Facility was inaugurated in mid July 2025, and the Faridabad facility is scheduled for inauguration in late August 2025. Both the facilities are being equipped with cutting-edge technologies, including robotic surgery, equipment and state-of-the-art diagnostic infrastructure

.
We are also onboarding, leading super-specialists, which will enhance our clinical strength and brand equity in these regions. The latest of which is the pediatric cardiology team, which will be the first in Noida region, which is already taking great strides in the treatment of complicated pediatric cases.
Our commitment to clinical excellence was demonstrated when several complex cases handled this quarter, such as:
■ An 11-year-old girl from Tajikistan, initially misdiagnosed with lymphoma, was correctly diagnosed with thymoma at our facility. Our team successfully removed a 2.5 kg tumor through a combined sternotomy and thoracotomy.
■ In another case, a 33-year-old woman with advanced lymphoma and multiple organ complications was treated with intensive care, dialysis, chemotherapy by our oncology team. She achieved complete remission and is now on maintenance therapy with no signs of the occurrence of tumor.
These cases highlight our clinical capabilities, advanced infrastructure and growing reputation as a leading quaternary care provider in the region. We continue to focus on improving our case mix and payer mix, supported by strategic investments in super speciality services.
We have made significant progress in our medical value travel initiatives. Recently, we have collaborated with the Children's Heart Fund and Black Lion Hospital, the largest government hospital in Ethiopia, paving the way for a patient transfer program. We also conducted continued medical education program on IVF in Mauritius, along with patients' screening to identify potential treatable cases. We are preparing to open an information center in Baghdad as well.
Additionally, we are working with Chinor Medical Center in Tashkent to establish a representative office serving as a hub for medical value travel for countries such as Uzbekistan, Kazakhstan, Turkmenistan, and Kazakhstan. These initiatives are expected to drive medical value travel and contribute meaningfully to overall growth in the coming years, along with improvement in operating and financial metrics.

We are also pleased to report the resolution of the Jhansi land issue during the quarter. The Commissionerate of Sagar Sambhag, Madhya Pradesh, following the direction of Honorable High Court of Madhya Pradesh has set aside its earlier order challenging our ownership over the land. This outcome brings legal clarity, reaffirming our ownership and ensuring operational continuity.
We are also in the process of transitioning our statutory auditors with our current auditors resigning in this Board Meeting. We will propose an appointment of the well reputed firm MSKA & Associates, a member of BDO International at the upcoming AGM. This in line with a commitment to strong governance practices underscores our vision for a strong and transparency to all our stakeholders.
As we move forward, we remain focused on strategic expansion to strengthen our presence in North India, clinical excellence, and continued investment in advanced medical technologies.
We are confident that these efforts will continue to deliver superior healthcare outcomes and create a long-term value for all our stakeholders.
With that, I would now like to hand over the call to Mr. Pankaj Prabhakar – our CFO, for a detailed financial update.
Pankaj Prabhakar: Good afternoon, everyone. I am pleased to report that Yatharth Hospital has continued its journey of accelerated growth, delivering a strong result for the quarter ended June 2025.
During Q1 FY '26, we achieved a revenue of Rs. 2,578 million reflecting a 22% Y-o-Y and 11% quarter-over-quarter growth. This performance was driven by higher occupancy rates as well as improvement in ARPOB supported by our strategic focu

s on super speciality services and operational excellence.
Our newer hospital leads the growth with Jhansi-Orchha Hospital, reporting a significant 63% year-over-year increase contributing 7% of total revenue. The Greater Faridabad Facility which got operationalized in May 2024 has ramped up well and now, accounts for 9% of total revenue.
We witnessed a strong patient volume growth in the quarter. Inpatient volume surged 27% year-over-year and outpatient volumes rose 20% year-over-year, reflecting the growing trust in our services and the effectiveness of our outreach and engagement initiatives.
Our focus on high value super speciality services has resulted in a 6% year-over-year increase in the ARPOB reaching a record of Rs. 32,395 for Quarter 1 FY '26. Notably, Noida Extension achieved an ARPOB of Rs. 39,830 with ~70% contribution from super speciality services.
Our Greater Noida Facility reported an ARPOB of Rs. 38,377, up 9% year-over-year. Notably, oncology now contributes 17% of Noida Extension's revenue and 10% of the group's revenue

making a 49% year-over-year increase.

On the profitability front, EBITDA rose 20% year-over-year and 13% quarter-over-quarter to Rs. 645 million demonstrating 13 consecutive quarters of growth. EBITDA margin improved by 41 bps sequentially and stood at 25%.
Despite higher depreciation due to ongoing expansion and investment in advanced medical equipment, our net profit has grown 38% year-over-year and 9% quarter-over-quarter, reaching Rs. 420 million.
Our balance sheet position has stood healthy with a strong net cash position, providing us with the flexibility to capitalize on future growth opportunities as they arise. Looking ahead, we remain optimistic about sustaining our growth momentum in both revenue and profitability. Our strategic priorities will continue to focus on operational excellence, sustainable growth, and capitalizing on emerging opportunities in the healthcare sector.
Thank you for your attention. I would now like to hand over the call to the moderator for the question-and-answer session. Thank you.
Moderator: Thank you very much. We will now begin the question-and-answer session. The first question is from the line of Amay Jain from Lucky Investments. Please proceed.
Saumil Just a small correction. This is Saumil not Amay. But thank you for the opportunity and congrats on a good set of numbers, sir. Can you lay out ARPOB in the new hospitals and the status on the upcoming hospitals?
Amit Kumar Singh: So, for upcoming hospitals, Delhi, as we have stated earlier, ARPOB will be better than probably what at the current ARPOB of our hospitals in Noida and Greater Noida, which we are operating as of now. So, that you can figure it out. For Faridabad, I would say that is going to be in a line of what Noida Extension is operating as of now. So, this is what as far as ARPOB is concerned.
Yatharth Tyagi: And as far as for the already started new hospitals, which was your earlier question, that is for the Greater Faridabad, the ARPOB that we currently have is around Rs. 31,393 for Quarter 1 and as Mr. Amit Singh mentioned the upcoming two new hospitals which would be starting, the Delhi hospital has actually started last month, and the Faridabad hospital will be starting late of this month. We do expect ARPOB there to be more than our group average ARPOBs.
Saumil Okay. And what about Jhansi, sir, you mentioned there was 60% year-on-year growth, how much is the ARPOB rise here?
Amit Kumar Singh: As we have stated earlier as well, so Jhansi is going to remain on the ARPOB, it will not change much. Jhansi has a volume growth, right, Jhansi will be in a line of close to Rs. 15,000, I think that is what we are reaching, Rs. 14,000, A

RPOB. Yes, Rs. 13,500 something. So, anything between, closer to Rs. 15,000 to the best-case scenario of Rs. 16,000 - Rs. 17,000, that will be, I think, it will be a pleasing number for us.

Saumil: Right, okay. And two more questions sir, one is on the timeline for the new hospitals, are we on track?
Yatharth Tyagi: Yes. As I just mentioned that --
Saumil And secondly, on the margins for the full year, you mentioned you expected to remain around the same levels as quarter 4 for the entire FY ‘26, are we on track for that as well?
Yatharth Tyagi: Yes. So, the upcoming new hospitals, as I mentioned Delhi has already started, got operationalized, so it’s revenue will start flowing from the month of August as we speak. And the bigger hospital in Faridabad, the 400-bed hospital, we are planning to inaugurate it late August. So, the revenue for these two hospitals, we will start seeing from Quarter 2 onwards. So, we are pretty much on track for that.
As far as the margins are concerned with the operation of these two hospitals, yes, we do expect, these two hospitals, there would be certain drag on the EBITDA margin compared to the whole of FY ‘25. But quarter four of FY ‘25 would be the right analysis to compare the margins, and probably 1% up and down from Q4 would be a right estimation for these two hospitals now starting for the overall group.
Saumil Okay. When do you expect these two new hospitals to reach breakeven, sir?
Yatharth Tyagi: See, if we talk about, if you look at Greater Faridabad Hospital that got operationalized 12 months ago, that hospital has in the latest quarter now become profit positive, so which is, we

feel is a good achievement for any hospital starting operations within 12 months of it. So, we do expect probably these two hospitals to follow a trend somewhere around 15 months would be a right estimation for these two hospitals also.

Saumil: Okay. Got it. That is it from my side. Thank you so much and all the best.

Moderator: Thank you. The next question is from the line of Himanshu Mali from Wallfort PMS. Please proceed.

Himanshu Mali: Hello, sir. Congratulations for the good set of numbers. I have two questions. So, my first question is, what is the share of MVT now and how it will look in future? And the second question is the share of dialysis and chemotherapy now and how it will look in further quarters?

Yatharth Tyagi: Okay. So, as far as your first question is for medical value-added tourism. So, international patients is something that we have recently started catering to, because if you look at Delhi NCR, international patients come for high end treatments like oncology, radiation oncology, liver transplant, bone marrow transplants. These are the programs that we have just started over the course of last 1.5 years.

As soon as we have started these treatments, we have seen a good flow of these patients now. As I mentioned in the commentary itself that we have tied up and opened up, various offices across CIS countries. There are some hospitals that we have tied up there. So, we are seeing good step-up in the volume as well as the contribution of medical tourism is concerned.

And your second question on the chemotherapy aspect, I think, in a large way to generalize it, how we look at it in terms of oncology. Because not just chemo but the whole radiation oncology, the surgical oncology as well as the bone marrow transplant, we have placed strong emphasis on that, and we are quite happy to see that year-on-year we have shown 50% growth in our onco

logy business. And oncology is now contributing close to 10% for the overall group.

We do expect this number to even go higher because remind you that our upcoming hospitals both the Faridabad as well as Model Town Hospital, within few months will also be having radiation oncology. So, these two hospitals will also act as oncology treaters for the whole group and the 10% radiation oncology that is for the complete oncology that is today for the whole group can even grow around 15% in next couple of years as far as the total payer mix is concerned.

Himanshu Mali: Okay. Thanks. That is it from my side.

Moderator: Thank you. The next question is from the line of Dhruv Maheshwari. Please proceed.

Ritika: Hi. Ritika here from Perpetuity. So, just wanted to understand that the Greater Noida Hospital, the occupancy for years has been around 65% and Noida Extension also seems to be plateauing at an occupancy of 61%. So, do you see a breakthrough in this? That would be my first question. Secondly, I wanted to understand that the Noida Hospital had a 79% occupancy in quarter four and has gone up to 86% so quite a huge jump. So, like what were the factors contributing to this? And have we been improving the payer mix as well? And the third would be on the other income and the tax increase.

Amit Kumar Singh: So, as far as your first question is concerned, Greater Noida and the Noida extension has an occupancy almost similar to what we had actually. But what you need to look at it as Yatharth mentioned in the commentary, the IPD numbers if you see has increased. So, your ALOS has decreased. So, we have treated more patients. However, because of the ALOS and that is a deliberate attempt.

As we had said in the previous quarters, we are trying to sanitize the payer mixes, right. So, these are the impact of it. However, I think that this percentage, we are very much satisfied with the Noida, Greater Noida with 67% of occupancy and Noida extension with 61%. The totality numbers, if you see, the IPD numbers, we have treated more. That is the answer of your first question.

Second question is about Noida. Yes, Noida occupancy has increased, significantly increased. But the numbers, if you see, as we had mentioned earlier, the earlier number, if you see now, we see the cash and TPA, Noida is doing better. This was a deliberate attempt, as we had said earlier. So, we are reducing a bit of dependency on the government. So, the cash, TPA and then the Speciality mix also has gone better. So, this is what this thing.

Yatharth Tyagi: Yes. As far as your third question on the other income is concerned, So, the other income that you see for this year is basically the interest the company has received on certain FDs that we were having, both from the QIP funds that we raised as well as certain internal accruals we had. So, over the course of upcoming quarters, that will also be generalized. But for this quarter, yes, there was other income received due to the interest that we have received on the FDs.

Ritika: Okay. So, what is your payer mix currently?

Yatharth Tyagi: So, as far as the payer mix is concerned, as we have always maintained, we are reducing our government business. Our government business, which, 1.5 year ago used to be somewhere around 40%. As we speak today, in the latest quarter of Q1, it is around 35%. And the remaining is, equally divided between the cash as well as private insurance , with private insurance being a bit more in terms of the volumes, but in terms of the revenue, cash component might be a bit more. And we are seeing good growth in our international patient flow as well. We are quite confident that maybe with all these steps that we have taken in medical value tourism, wit

h the

airport opening up, with now, these new leading star doctors onboard, we feel international payer mix can even touch close to double-digit numbers in a couple of years for the whole group.

Ritika: That is good to hear. And just lastly, what is your net cash on the book?

Yatharth Tyagi: So, today, we would be upwards of around Rs. 300 crores, and when we closed the financial, we were close to Rs. 500 crores. So, the amount that has been spent has been on the equipment front in the upcoming two new hospitals of the Delhi Hospital as well as the Faridabad Hospital.

Ritika: Okay. Thank you.

Moderator: Thank you. The next question is from the line of Harsh Shah from Seven Rivers Holding. Please proceed.

Harsh Shah: Hi, sir. Good afternoon. And a strong start to Q1. Sir, I just wanted to reiterate, you mentioned that the impact on EBITDA from the two new hospitals, that is Delhi and Faridabad, will be 100 bps, right? And it will get neutralized by Q4. So, Q4 FY '26, they should be back to 25% EBITDA margin?

Yatharth Tyagi: See, I think rather than seeing quarter-on-quarter, how we see it for the full-year, we feel as earlier mentioned, EBITDA margin would be somewhere to what we had in Q4, maybe 1% up and down. That is how we are looking at it. And Q1, we also saw a good EBITDA percentage as well as good EBITDA growth because the Greater Faridabad Hospital has become also

EBITDA positive. So, the drag that the Greater Faridabad Hospital had last year will not be having this year. So, even though we have two hospitals operationalizing, we also have one hospital who is now turning EBITDA positive for the year.

Harsh Shah: Okay. And sir, how should we look at these two new hospitals in terms of, if I look at Q4 FY '26 exit run rate, what kind of occupancy do we expect? And what kind of payer mix will these new hospitals have? Because I believe the government dependency might be low in this newly commenced hospital. So, what kind of payer mix do we expect? What kind of occupancy do we expect, say, on Q4 FY '26 exit run rate basis?

Yatharth Tyagi: So, again, rather than quarter-on-quarter, how we feel is that after first year of operations, so technically, they are starting in Q2. So, the right estimation would be a year from Q2. We do expect both these two hospitals to have an occupancy somewhere around close to 30% to 35%. And that is what our existing other new hospitals have also had. As far as the payer mix is concerned, yes, you are right. There would be very little government business, similarly to what we have already proven in the Greater Faridabad Hospital, because that is also a reason why, the hospital has achieved good numbers, because their dependency on government business is very low, we have hired a lot of star doctors also in that region. So, we do expect for these two hospitals on a similar line, maybe government business should not be more than 20% - 24% in these two hospitals going forward.

Harsh Shah: And sir, on FY '26 as a whole, do we still expect our ARPOB to increase by 10% say for FY '26 as well as FY '27 on a blended basis?

Amit Kumar Singh: Yes. So, see, as we have said, anything between 8% to 10% that is the ARPOB growth which we are looking at. So, we have all the drivers. So, I mean we believe that I think this will be easily achievable.

Harsh Shah: And sir, last question, any update on the income tax investigation that you would expect?

Yatharth Tyagi: So, as we were in Q4, the company is quite happy with the progress in the income tax matter. It is the status quo from Q4, that is AKS which is a very large subsidiary of the company, contributing close to around 40% of the revenue. The income tax audit is largely being completed and there is a minis

cule demand that has been arise and in the coming time we would be looking to pay that and settle that off.
As well as for the whole remaining company is concerned, we do expect somewhere close to the end of this calendar year that also will be completed. And as we have always maintained, before this closing of the financial year or somewhere close to the end of the calendar year, we would be completely looking to settle off the complete matter and whatever amount that would be required at all, the company would be well capable of handling that and it would be a very small amount that we are anticipating in that matter is concerned.

Harsh Shah: And if I can squeeze in just one last question, if you can share the status and timeline on the Brownfield expansion at our Greater Noida and Noida Extension Hospital?
Yatharth Tyagi: Yes. So, we have just started two new hospitals, that is the New Delhi Hospital and the Faridabad Hospital that we have just recently starting now this month. So, core focus was there in the ramping up of these two hospitals. Now that these two hospitals are started, we are shifting back our focus to finish up our Brownfield expansion. Greater Noida Hospital is already at a commencing infra stage, that means the construction is about to start for the digging of the basement, creating all the floors up.
As far as Noida Extension is concerned, we are at finalizing stage for the maps for the whole building, both 200 beds and 250 beds will be added in these two hospitals. And the timelines remain largely similar, maybe two months - three months up and down. But however, for last quarter, we were more focusing on the Delhi and the Faridabad Hospitals. The Brownfield expansion is now again back in focus for Quarter 2.
Harsh Shah: All right, sir. Thank you so much and all the best.
Moderator: Thank you. The next question is from the line of Sumit Gupta from Centrum. Please proceed.
Sumit Gupta: Hey. Hi, good afternoon. So, sir, first question is on the case mix, so just want to understand, so like what is the trajectory over the next two years to three years where we can see onco share rising from current 10% and medicine share declining. So, can we expect that to increase?
Amit Kumar Singh: Yes, so in fact, Sumit, you see the future trends going to be as onco definitely going to increase that. You see there are other transplants and high-end procedures will increase, right? So, definitely in terms of the percentage, the internal medicine and conservative will be a down, but not the absolute number, right? Because as we are growing these things. But yes, this is a trend that more super speciality, particularly leading by the oncology, going to be the driver. And as I said earlier all these departments are not mature Sumit as of now. So, as we said that quarter-on-quarters, in fact, every super speciality department is growing, right? But yes, the faster growth coming from oncology.
Sumit Gupta: Yes, I am talking from two years to three years down the line. So, a bigger picture. So, in medicine, let us say as of now, it is contributing around 20%, 18% to 20%. So, can it go to around 10% - 11% in the next three years as well?
Amit Kumar Singh: But I think that is internal medicine. Medicine is very, very important because medicine can feed all super speciality. So, reducing medicine is also not a smart move, right? So, there has to be a good balance. So, I believe that if medicine is contributing around close to 15% - 17%, I think that is a fairly good number, right? Because that works as a feeder for super speciality.
Sumit Gupta: Understood.

Ashutosh Kumar Jha: And down the line, what we are focusing is on improving on all the CONGO specialities, cardiac, onco, ortho and neurosciences.
Sumit Gupta: Understood. Thanks, Ashutosh. And with respect to the depreciation, how should we look at it? Shall we expect these two to be at the same trend? Or should we see a loss for the next two years?
Sonu Goyal: So, for the next two years, we are adding two more facilities. So, that will definitely impact, but the impact is not so much. So, we can presume around Rs. 20 million - Rs. 30 million impact quarter-on-quarter on a total basis.
Sumit Gupta: Okay. And for the next three years, how much of cumulative CAPEX for the next three years?
Yatharth Tyagi: Can you repeat the question?
Sumit Gupta: Cumulative CAPEX for the next three years?
Yatharth Tyagi: Yes. So, cumulative CAPEX would be, for the next three years, would be in the tune of, if you

include both Brownfield as well as Greenfield, it would be in the tune of around Rs. 1,400 crores to Rs. 1,500 crores over the next three years.

Sumit Gupta: And how much would be maintenance?

Yatharth Tyagi: Maintenance is a low CAPEX for us. Maintenance is not included in this. Maintenance for the whole year should be around Rs. 20 crores to Rs. 25 crores for us.

Nitin Gupta: We have already ramped-up our hospitals in the recent past and all the kind of new equipments have already been added. Just a few civil work has to be there, as we mentioned about the Brownfield expansion as well. So, that is the only thing which will be coming up. Rest of the thing has been already completed in all the centers.

Sumit Gupta: Understood. Thank you. All the best.

Moderator: Thank you. The next question is from the line of Nirali Shah from Ashika Stock Services. Please proceed.

Nirali Shah: Yes, thank you for the opportunity. I had a couple of questions. I just wanted to know, is there any update on the Ghaziabad Greenfield opportunity? And we were also evaluating two, three more cities. Has there been any progress on those fronts?

Yatharth Tyagi: You know, as you are right, we are evaluating not just Ghaziabad, we are evaluating other opportunities within NCR, Ghaziabad, Gurgaon, outside NCR also, big cities basically. And the company would be looking to add at least one acquisition for this year. Maybe it could be a Greenfield land, it could be a stressed asset. The company is evaluating certain opportunities.

And we remain confident for the remaining of FY 26, we will be adding and closing one of these opportunities.

Nirali Shah: And what kind of investment are we looking at this number, I mean the CAPEX?

Yatharth Tyagi: Yes. So, for Greenfield, including the land, we are looking to add around 300 beds to 350 beds and including land, I think the total CAPEX outlay that we see is around Rs. 300 crores.

Nirali Shah: Okay. And just wanted to know, the effective tax rate for the quarter is higher than usual. I guess it is around 28%. Are there any specific adjustments this quarter or should we still assume a full year ETR as guided earlier to be around 24%?

Sonu Goyal: If you see the current tax rate is coming around 28.32%, okay. Earlier there were deferred tax assets in our Ramraja Multispeciality Hospital that is located in Orchha. Right now, there is a deferred tax liability. That to total 28% of the overall percentage in the tax rate.

Nirali Shah: So, on a full year basis, can you expect it to be?

Sonu Goyal: No, it will come down to 25%, 26%.

Nirali Shah: Okay. Understood.

Nirali Shah: And just a last one on the ARPOB, so this quarter is around Rs. 32,395 and that is likely aided by Faridabad breakeven. The two new hospitals we were expecting to be around in the range of Rs. 33,000 to Rs. 35,000, correct?

Yatharth Tyagi: It would be a bit more for the two new hospitals. It would be somewhere around Rs. 38,000 for those two new hospitals, that New Delhi as well as the Faridabad Hospital, the bigger Faridabad.

Nirali Shah: Understood. So, will that come from 2nd Quarter?

Yatharth Tyagi: Yes. Big impact you would start seeing from Q3 onwards, because we are midway of Q2 already and Faridabad is yet to start. So, the big impact you should start seeing from Q3 itself.

Nirali Shah: So, on a blended basis, say for ‘26, what could be the ARPOB? What is the number that we are building in?

Amit Kumar Singh: See, it is no, no forward-looking number but I can tell you, you can just correlate. As of now, you see the Greater Noida, Noida Extension is in a line of Rs. 40,000 close to Rs. 40,000 ARPOB, right? As you said that that Faridabad and Delhi, Delhi would do a bit more and Faridabad would be in a line of this thing. So, this is what I think I can tell you as of now.

Nirali Shah: Understood. That is it. Thank you.

Moderator: Thank you. The next question is from the line of Aditya Sen from Findoc. Please proceed.

Aditya Sen: Hello, hi. Thanks for the opportunity. I am audible, right?

Amit Kumar Singh: Yes, please go ahead.

Aditya Sen: Yes. So, this is regarding the international patients that we are targeting. So, as we added to the double-digits as a percentage of our total patient mix, will there also be any additional cost

associated with the procurement of international customers?
Amit Kumar Singh: So, typically, as of now, we are a single-digit number. And then so what do you need? You need basically there is a good team, a team has already been added, right? So, when you start doing your various information centers, you are setting up various information centers on different place. So, that is all the OPEX, right? And then, definitely, if you are doing international and you understand that the ARPOB is better. So, right, I think that is only the OPEX will be this thing. There is no other CAPEX as such, neither even a manpower cost. It has already been taken care of now.
Yatharth Tyagi: Yes. And we do not see any impact on our profitability or EBITDA due to increase in the international patients. So, technically your question --
Aditya Sen: All right.
Yatharth Tyagi: It will not increase or change any of our parameters once the international business increases. In fact, it will be better our ARPOB numbers, because all the expenses mostly have been done in terms of hiring the doctors, getting those equipment's in place and starting those treatments. Now, just the volume has to grow.
Aditya Sen: Understood. So, I was asking this question particularly because there is a competitor of ours in Gurgaon who has a higher ARPOB because of international patients, but the EBITDA margin is way low than our EBITDA margins. So, I was just afraid that if we are adding international, we might not fall back to their level of EBITDA margin?
Yatharth Tyagi: The difference between that competitor and us is that we are not just single-handedly focusing on international patients. Ours still one-third of the revenue comes from cash patients, one-third comes from private insurance and certain still comes from government. We will not make international patients' contribution to one-third of our revenue, so that is the difference. It will still be close to around 10%, which is how a lot of hospitals, other hospitals also operate on.
Aditya Sen: Okay. Fair enough. Got my answer. Thank you.
Moderator: Thank you. The next question is from the line of Bhagwat from Prosperity Wealth Management. Please proceed.

Bhagwat: Thank you for the opportunity. Actually, in the operationalization of two new hospitals, what is the estimated revenue growth on a consolidated level for current year and next year FY '27?
Yatharth Tyagi: See, our revenue growth expectations for the whole year remain the same. Our guidance remains the same, as we have always maintained that last few years the company has grown close to around 30%. I think in the upcoming years also, including this year, we remain on track for that.
Bhagwat: Okay. Similar range of 30% we can expect.
Yatharth Tyagi: Yes.
Bhagwat: Okay. Thank you. Thank you so much.
Moderator: Thank you. The next question is from the line of Surya Narayan Nayak from Sunidhi Securities. Please proceed.
Surya Narayan Nayak: Yes, thank you for giving me the opportunity. Congratulations for the great set of numbers. So, sir, just one question is that, how are you going to phase out the addition of new beds in the new Faridabad 400 and the Delhi 300 units because presently we are seeing that, we have added 300 beds. So, can we assume around 250 census beds from Q2 onwards? Likewise, can you please guide for the Faridabad unit also?
Yatharth Tyagi: Yes. So, whenever we are starting a hospital, the beds become official on a scale-wise, but it is not that we have restricted to any beds. We might operationalize like the Delhi Hospital, which is inaugurated. Today, we can have operationalized close to 150 beds. But if we get 151 patients today, it is not that that patient would be refused for admission. So, the scalability of the beds is there as and when required. The infra is ready, the nurses are there, the equipment is there. As and when the patients are coming, we will look to add up and scale up those flows.
So, even from day one, we are starting the hospital with complete infra. It is not that, certain floors are not ready or certain construction is going on in the Delhi Hospital, everything is complete. So, similarly to what we did in the Greater Faridabad Hospital, we would be ramping up our census beds and operational beds on a similar manner.
Surya Narayan Nayak: Okay. And sir, for the Noida Extension, Greater Noida Extension, the current occupancy is, you said is similar to that. So, can we expect what would be the expectation for the peak occupancy in these regions in the NCR, especially, can it go to, let us say, 75% or 80%?
Amit Kumar Singh: Noida Extension, Greater Noida is above 400 bed facility. So, if anything, 75 to 80, it will be a very, very ideal and very optimal utilization. I think beyond that, it is difficult. So, I think that is we are inching towards it. So, anything 75 to 80 can be a guidance.
Yatharth Tyagi: And certain drivers are there for this, which we are quite confident that in the coming years, in

the coming quarters, you will see an increase in occupancy in these two hospitals. Specifically,

we have added a lot of star doctors in even these two hospitals. We started certain new treatments like the transplant program, the radiation oncology, where the volumes is bound to increase, as well as increase in international patients. So, these three are good drivers for the growth of occupancy also. And we are quite confident that, in the coming quarters, you will see that rise in occupancy.

Surya Narayan Nayak: And sir, because, you are even offering so, can we consider that, some of the areas where we are not actually matured or not established, and often establishing in our cases, maybe in radiation oncology and other areas, we will be g

etting higher occupancy and not before that.

Yatharth Tyagi: Already, it is not just the new therapeutic areas we have added, even in the existing areas, like, pediatrics, ortho, gyne, there is constant upgradation of departments and the new doctors that are added. New doctors are not just added in the new specialities, but even our existing specialities. So, there also, we do expect an increase in the volume, including general surgeries, including certain other super specialities, like gastro, like neuro, that we have been doing since a long time. So, we do expect occupancy to grow from all the super specialities, but yes, highest share will increase of the super specialities like so cardiology, oncology and neurosurgery.

Surya Narayan Nayak: Okay. Can it be fair to assume that we will be beating the inflation in all the NCR region hospitals at least for the next three years?

Sonu Goyal: Yes, because twice in a year we keep on changing our pricing, so by considering the inflation we keep on changing the pricing of the overall patient charging, so we are pretty much sure to beat the inflation.

Surya Narayan Nayak: Okay. Thanks.

Moderator: Thank you. The next question is from the line of Prerana from Equity Research Program. Please proceed.

Prerana: So, what is the status of empanelment in your New Delhi Hospital that you recently inaugurated? Like how is empanelment going with insurance and various other government departments?

Yatharth Tyagi: So, we have started taking all the private cash patients as well as for the private insurances now the process is ongoing because we are getting fast-tracked our NABHs and other QCRE permissions. As soon as that happens, private insurances will be empaneled and then the number comes for the government empanelment's.

However, already from day one, nowadays there are a lot of agencies like MediBuddy, GMoney and other aggregators which does provide cashless facility to any private insurance holder who is walking to a hospital. So, he is able to utilize his insurance through those third-party aggregators, and they are able to get cashless treatments.

Similarly, we have done that in the past in our Greater Faridabad Hospital and we are doing that today in our New Delhi Hospital. So, already we are treating private insurance patients there. As far as the empanelment's of these private insurance and government empanelment's are concerned, I think within the first year, we would be able to get 90% of all private insurance as well as the government empanels. But it will not hamper our progress of taking those patients.

Prerana: Okay, sir. And the last question is on EBITDA margin. You told that because of the starting of these two new hospitals, the EBITDA margin will be similar to Q4 of FY '25. So, can we expect around 24% or is it going to be lower than that?

Yatharth Tyagi: Around that would be a right estimation. 1% up and down, but around 24% would be a right estimation.

Prerana: Okay, sir. Thank you so much. Thank you.

Moderator: Thank you. The next question is from the line of Vicky from Guardian Capital Partners. Please proceed.

Vicky: Thank you for the opportunity, sir. Sir, my question is, what was the EBITDA and PAT for Greater Faridabad Hospital for this quarter?

Sonu Goyal: So, EBITDA for this quarter in Greater Faridabad, that contributes to 1.7% of the overall group EBITDA and PAT is 3.4% of the revenue in Greater Faridabad.

Vicky: Okay. And sir, one more thing I wanted to ask. We have been seeing increase in employee cost as we are onboarding new super doctors. When do we expect this to settle down? Because earlier our employee costs as a percentage of sales was around 17% - 17.5%, which has inched up to around 19%. So, when sho

uld we expect this to normalize going forward?

Amit Kumar Singh: So, see, as you said, these hospitals like Greater Faridabad, the new hospitals, right? So, you see the employee costs will be higher. But yes, when they are mature hospitals, I think it should be anything between 15% to 17%, 18%. It is an ideal percentage. I think we should get settled in it.

Vicky: Okay. And sir, one more question. These are very early days, but how is the response of the new hospital, new hospital that we inaugurated?

Amit Kumar Singh: We started just on the 14th. So, I think that the excitement in the market, I think the medical fraternity, they are excited. So many people are coming, joining these things. And I just said that we are in that area, which is a very old, but very, very prominent area of Delhi, right? So, there we see that I think this hospital will do a much, much faster ramp up in our group.

Vicky: Thank you, sir. Thank you, sir.

Moderator: Thank you. The next question is from the line of Yash Modani from Purnartha Investment Advisors. Please proceed.

Yash Modani: So, sir, I just wanted to ask you, what are the drivers when the increase in occupancy rate in Jhansi-Orchha Hospitals? And what are your projections for next quarters also, in terms of ARPOB?

Amit Kumar Singh: So, Jhansi-Orchha, if you see, I mean, it has made very good progress. Y-on-Y last year it was 45%. Now, it is around close to 60%, right? So, Jhansi-Orchha is doing good. As that is a hospital delivering good quality care, all super specialities, the name is getting spread in that particular market. And then we have an advantage of, there is no other big hospital in the range of 200 kilometers in that region.

So, I think, as time is moving, yes, a bit of empanelment is also playing a big role. There are slowly the more government empanelment are happening in that particular hospital. So, within the two years of operation, it is a 60% occupancy. I believe it is a very good occupancy and if it is around close to 70% - 75%, that is, optimally we will utilizeJhansi.

Participant: Okay. And my next question, sir, how do you anticipate the ramp up of the two new hospitals? And how will this influence the overall occupancy rate group level?

Yatharth Tyagi: So, with the addition of even 700 beds, and they will take time to ramp up, because as I said that after first year of both these two hospitals operationalized, we feel that both these two hospitals can have an occupancy close to 30% - 35%. So, at a group level, yes, even though with the addition of 700 significant beds, we expect even at the group level occupancy should increase, because occupancy is bound to increase in the existing hospitals of Faridabad as well as the Greater Noida and the Noida Extension.

Participant: Okay. Thank you, sir.

Moderator: Thank you. The next question is from the line of Aman from PhillipCapital. Please proceed.

Aman: So, my question is on the CAPEX outlay that you mentioned. So, we are currently at 2,300 beds and we are about 700 beds from our targeted 3,000 number, right? Now, if I look at the CAPEX guidance that you gave earlier of about Rs. 1,400 crores - Rs. 1,500 crores, is that number right just to begin with?

Yatharth Tyagi: Yes, but approximately around that.

Aman: Okay. Now, if we just roughly do the math, the CAPEX per bed comes out to about Rs. 1.52 crores per bed almost, right? And this is relatively higher than what our historical numbers have been. So, is this majorly because do we plan on doing more on Greenfield CAPEX that might drive our cost up? Is my understanding correct?

Yatharth Tyagi: No. So, just to correct you there, when I have said Rs. 1,500 crores, it is not just to take a bed from 2,300 to 3,000. In next three years, we are looking to add around 1,200 beds more. That is why, the CAPEX per bed will come down in the tune of Rs. 1 crores and not in the tune of that. Because there is no benchmark that we have taken that, we will just stop at 3,000 once the growth is there.

We have accelerated our growth, we will continue to grow at the same level in terms of adding

of the beds are concerned. Because there are a lot of good opportunities that we have seen and we are evaluating them. It is not that we are chasing a number of beds that we want to be. If a good opportunity is there, we would like to add to it. So, that is why we are evaluating few land parcels, we are evaluating few stressed assets. So, over the course of next, a 1.5 years or to two years, we will be adding more beds than just 3,000 number.
Aman: Okay. Noted. I just wanted to clarify that. Thank you.
Moderator: Thank you. The next question is from the line of Anand from KSEMA Wealth Private Limited. Please proceed.
Anand: Sir, good afternoon, sir. Yes, sir, congratulations on the good set of numbers. I just want to understand on the Brownfield expansion of Greater Faridabad Hospital. Would you see a Brownfield expansion by how many beds, let us say, in the next four years - five years for Greater Faridabad?
Yatharth Tyagi: So, Greater Faridabad does not have any Brownfield expansion possibilities. That hospital has currently 200 beds, and in fact that is the reason why we have acquired another hospital in Faridabad which is now a 400-bed hospital, which is going to start at the end of this month. Maybe the hospital you are talking about is Greater Noida, which currently is 400 beds, and we are doing a Brownfield expansion of 200 beds there, which will be coming to utilize that capacity within two years.
Anand: Okay. But there is no Brownfield expansion for Greater Faridabad at all?
Amit Kumar Singh: No.
Yatharth Tyagi: No.
Anand: Okay. Fine. And I just want to like get an understanding of what will be the census, as a percentage of operational beds, what will be the census percentage of the new hospitals Faridabad and Delhi?
Yatharth Tyagi: 80% to 85%, yes.
Anand: Sorry?

Yatharth Tyagi: 80% to 85%.
Anand: Sorry, I am sorry, can you repeat it again? Not very clear.
Yatharth Tyagi: It is going to be 80% to 85%.
Anand: 80% to 85%. Okay. And what would be the occupancy rate for these two hospitals as well?
Yatharth Tyagi: As we have already mentioned, after a year or so, somewhere around 30% - 35% would be the right estimation.
Anand: That is all the questions I have. Thank you.
Moderator: Thank you. The next question is from the line of Mohammed Patel from Edelweiss Public Alternative. Please proceed.
Mohammed Patel: Yes, hi. I just wanted to have one clarity. So, can you break up the CAPEX of Rs. 1,500 crores?
Yatharth Tyagi: Right. So, if you look at it, we are already doing a Brownfield expansion of Greater Noida and Noida Extension Hospital. So, that comes to around 450 beds. Now, Brownfield expansion that we are doing would be doing at a CAPEX of around Rs. 60 lakh to Rs. 70 lakh CAPEX per bed. So, that is the cost of the Brownfield expansion we are doing. Plus, I have talked about that when we are acquiring a Greenfield project, as earlier mentioned, maybe this year, around 300 beds - 350 beds, we will be spending around Rs. 300 crores, including land for that hospital. And similarly, for any expansion of 300 beds to 350 beds in the corresponding years also, we would be spending Rs. 1 crore CAPEX per b

ed. So, that is roughly how it is, probably within Rs. 1,500 crores, we would be able to add at around 1,200 beds additional including the Greenfield and the Brownfield.
Mohammed Patel: Okay. So, this includes inorganic acquisition?
Yatharth Tyagi: Yes.
Mohammed Patel: Thank you.
Moderator: Thank you. The next question is from the line of Vicky from Guardian Capital Partners. Please proceed.
Vicky: Sir, thank you again for the opportunity. I just wanted to ask one question. What was the debtor days for the last quarter?
Yatharth Tyagi: So, debtor days for the whole year in FY ‘25 was around 123 days . And I think for the latest quarter also, I think we are looking to reduce that. Somewhere I think we feel by H1 we should

be reaching a number of around 118 days to 117 days, and I think that is if we are on that because even our scale is increasing, the volume is increasing, but probably H1 we would be looking to close around 117 days for the debtor days.

Vicky: Okay, sir. Thank you so much.

Moderator: Thank you. The next question is from the line of Om Kant Sharma from Vaishan Ventures. Please proceed.

Om Kant Sharma: Yes. Hi. Thank you for the opportunity, sir. Most of my questions are answered. I just have one quick question. If you could just touch down on your new hospitals, which is your Model Town and the Faridabad hospital. Could you just talk a little bit about what is the competitive scenario situation in those two areas as to which are the hospitals that we would be competing, which are the hospitals which are there in the catchment area, if you could just throw some color?

Amit Kumar Singh: Yes. So, as far as Delhi hospital is concerned within 10-kilometer radius, we have a Fortis and Max, but yes if you see, that that hospital is very, in the region where there is a very high density of the population. right. So, I think the 3 kilometers - 4 kilometers, 5 kilometers is enough for you to feed largely 60% - 70% of the bed capacity. And that is what since our competitions are with Fortis and Max and a few smaller hospital there, we are making that hospital as absolutely on a quaternary care hospital. So, that is there, but I believe that more and more hospitals and then in particular region more and more market develops. I think we are nowhere inferior in that particular market. As far as Faridabad is concerned, I think --

Yatharth Tyagi: Yes. And also, just to add for the Delhi Hospital, which just operationalized a few weeks ago, or the name of the competitors that we just mentioned, we have taken on Board some of the star doctors who have been in those hospitals for last 10 years - last 15 years. So, that is how we have positioned that hospital. And in fact, how we are trying to compete is also, we are trying to create a very boutique hospital culture where every patient who is entering a hospital is getting care and addressed to.

The other hospitals in the region, which might be 7 kilometers – 8 kilometers within the radius, the patient volume and the patient turnover is so huge in those hospitals that patients are sometimes not getting the personal attention that is required. So, how we have positioned ourselves is each and every patient that is coming to the hospital will be highly focused on the patient care experience by the same star doctors who have been previously practicing in those regions.

As far as Faridabad is concerned, Faridabad is a market we very much understand. We already have a hospital 200 bed in Greater Faridabad, we operationalized that a 1.5 year ago. So, this new hospital in Faridabad that we have is also, there are one published hospital chain, it has many big local hospitals, which are still not listed. So, those hospitals have already b

een doing

well in that region. In fact, Faridabad, if you see, is an older city with much more bigger healthy market than even Noida as of today we speak.

So, there are a lot of opportunities for, again, similarly from star doctors that we are onboarding in Faridabad. And the good part is we already have a feeder from a Greater Faridabad hospital to feed certain of the specialities that we will be doing here, which we could not do there, like oncology, like transplant program, like certain of the bone marrow transplants. We already have a feeder from an existing hospital to feed a bigger 400 bed hospital in Faridabad also.

Om Kant Sharma: Got it. Perfect. And sir, just one, from a numbers' perspective, right, we see that the employee costs have jumped by about Rs. 4 crore Q-on-Q basis. Can this be related with the operationalization of this new hospital? And also, a corollary to the question would be, let us say, by year end, what would this on a quarterly run rate basis, what are the numbers that we are talking about on the employee cost by the year end?

Amit Kumar Singh: See, I think I have already answered this question. So, as the new hospital is coming up, the percentage wise, if you see, it looks a bit higher side of it, but we understand that this is going to be the well remain within the industry practice. Anything between 15% to 17% - 18%, that is what I think that the employee cost and that is going to settle down.

As far as doctor cost is concerned, I mean, that is also you see, because the new hospital is coming up, the percentage will increase, but this will be well within the range and we are very much on to it, these numbers. So, once this gets matured, I think that is the ideal time when you can, questions on this percentage, but the hospitals are not matured yet I mean they have just started it. So, 1% or 2% range maybe 3% --

Om Kant Sharma: No, what I am trying to get a sense, is this 48 number we are seeing for the quarter, does that

reflect the entire cost of the greater, for the Delhi hospital at least?
Yatharth Tyagi: No, even in the Delhi hospital, because it has actually been operationalized just a few weeks back. So, in Quarter 2 is when you will see those numbers and they are not reflecting in Quarter 1.
Yatharth Tyagi: Yes. July itself is when the hospital got inaugurated.
Om Kant Sharma: Okay. Got it. And just one last question, I do not know if you mentioned that, but if you could just talk a little bit on the operationalization of the Brownfield CAPEX that you are planning, when are we seeing that actually getting operationalize?
Yatharth Tyagi: The greater Noida hospital should be before two years, and Noida Extension should be around two years from today.
Om Kant Sharma: Okay. Perfect. Thank you. Thank you very much.

Moderator: Thank you. Our next question is from the line of Urmi Khania from Ledge Advisors. Please proceed.
Urmi Khania: Hello, this is Urmi from Ledge Advisors. My question is that any revision on the guidance of achieving a bed capacity of 3,000 beds by FY ‘28, since you have managed to add new beds faster. Do we see this target to be achieved sooner than this? And also beyond this 3,000 beds target, in the long-term, say five years ahead, what do we see going ahead? Thank you.
Yatharth Tyagi: So, definitely, we are looking to expand, because as earlier mentioned that we are not just looking to expand at any cost, we are looking to expand because there are some good opportunities that we have identified which are present in t

he geographies where we operate, where we feel that we can have even first mover advantages by being present in those geographies. So, 3,000 target, yes, definitely looks like it might be met before the FY ‘28 target. But even beyond that, for a five year horizon, we would be looking to scale up at a similar trend to what we have scaled up in the last two years. So, we would like to continue with the same momentum as to how many beds it will leave after five years, time will tell. But the momentum should be continuing at least within the North India market.
Amit Kumar Singh: Yes. We are not chasing any, particular targets in terms of number of beds. So, as and when good opportunities come, we will be scaling ourselves.
Urmi Khania: Okay. Thank you.
Moderator: Thank you. The next question is from the line of Anand Kulkarni from Front Wave Research. Please proceed.
Anand Kulkarni: Hi, thank you for the opportunity and congrats on a great set of numbers. Most of my questions have already been answered. So, just one on the speciality charges. As you know, the share of CONGO in total revenue continues to rise, how are we looking at our speciality charges as a percentage of CONGO revenue, a couple of years down the line? Thank you and all the best.
Amit Kumar Singh: Yes, Anand, can you please repeat the question, please?
Anand Kulkarni: Yes. So, as I was saying, just one on the speciality charges part. As the share of CONGO in total revenue continues to rise, how are we looking at our speciality charges as a percentage of CONGO revenue a couple of years down the line?
Amit Kumar Singh: See, we have not seen that way, but if you see that there is a 70% revenue being contributed by this CONGO, I believe, I think that is and will be having probably the higher expenditure. I mean, that is there, right? But as such, if you ask me, we have not seen that way, that is how much percentage of these specialities and how much percentage of the expenditure of our CONGO. We have not seen it. But yes, you can extrapolate in that way because if 70% revenue contributed by this thing and their ARPOB is higher where the in high-end specialities you do,

right, where we do transplant and others you do. I mean, there are a good percentage of the expenditure. So, higher percentage of expenditure goes in those super specialities.
Anand Kulkarni: Okay. Right sir. For the last five years, if I take it, it is in the range of 40% to 45%. So, do we expect to be in similar range or on the high end?
Yatharth Tyagi: I think some 5% higher would be a right estimation, but not significantly higher than that.
Anand Kulkarni: Okay. Great. Thank you and all the very best.
Amit Kumar Singh: Thank you.
Moderator: Thank you. The next question is from the line of Ankur from Anived. Please proceed.

Ankur: Yes, I have just one question on the trade receivable side. Our trade receivables have been higher. So, anything you can just guide us on that?
Yatharth Tyagi: See, I think last year we were quite happy with the OCF conversion the company has done. We had a cash conversion of around 68% we have said. So, even with high receivables, the important part is that we are receiving that money. Yes, the new business that was generated has also gone into receivables, but the way it happened last year, this year we are on the similar trend. So, Q1 itself we have seen good receivable position as far as the money that is incurring to the company. And over the course of next few years, when the government business comes down, in the tune of 25%, that is where you could expect the days and absolute number of the receivables to also come down. But more important for us is that the money that is being, generated has been received by the

company and we are happy with that progress for the quarter as well.
Ankur: Understood. Like so for FY '26, like anything that we can assume?
Yatharth Tyagi: I think I have already mentioned for H1 the number of receivables days that we do expect, and I think for the whole year could be on a similar trend also.
Ankur: Understood. Yes. So, that is it from mine. Thank you so much.
Moderator: Thank you. The next question is from the line of Sumit Gupta from Centrum. Please proceed.
Sumit Gupta: Hi. Just one clarification on the tax rate. So, you guided for 25% to 26% or is it still 24%?
Sonu Goyal: It is 25.67% effective tax rate.
Sumit Gupta: No, how much do you expect for '26 on a full year basis basically?
Sonu Goyal: It is coming down 26% on a full year basis.

Sumit Gupta: Okay. Cool. Thank you.
Moderator: Thank you. The next question is from the line of Rudraksh from MB Investments. Please proceed.
Rudraksh: Hi, my question is, what steps is the Board taking to improve the corporate governance to your peers like Max or Apollo? And my question to you is what is your outlook for expansion strategy in niche markets, in niche cities like you have in Jhansi, Gwalior, Agra? What is your expansion strategy there? If you could share some light on that, that would be great.
Yatharth Tyagi: Your first question is concerned, as I already talked about in the last commentary, we have onboarded BDO as a statutory auditor, who would be taking over, and existing auditors, we have accepted the resignation. So, BDO being appointed as statutory auditor as well as Deloitte has been appointed as an internal auditor from last few quarters, and we have been gaining a lot of good insights and improving our operational efficiency through Deloitte is concerned.
As for your second question in terms of our expansion geographies are concerned, so I think Jhansi-Orchha for us was good, because they were one of our, good ROI, ROCE opportunity that we saw. But going forward, we would like to be in bigger cities. We would not like to experiment much with Tier III cities, maybe, NCR and, capital cities of the big states is where we would like to be, because we see good growth in terms of paying potential and ARPOB in those cities and that is what we would like to explore more.
That is why if you see post-Jhansi, we have acquired three hospitals that are in New Delhi, Greater Faridabad and Faridabad. So, this sort of reflects as an example of a strategy that the one which we are approaching forward with.
Rudraksh: All right. Just to follow-up, by when do you think that on an average level, the group can achieve an average occupancy of upwards of 75%?Barring the new one.
Yatharth Tyagi: Yes. That is what I was about to say because we are constantly adding up new beds, right, so that is why at a group level only we can talk about is barring the new hospitals. So, I think in couple of years, our existing hospitals should be at a group level occupancy of close to 75%.
Rudraksh: All right. Thank you. That answers my question.
Moderator: Thank you. The next question is from the line of Mohammed Patel from Edelweiss Public Alternative. Please proceed.
Mohammed Patel: Sir, how are we going to fund this CAPEX?
Yatharth Tyagi: So, we already have a cash position as we talked about of around Rs. 300 crores as we speak. Today, the company is sitting on zero debt. So, that is an option the company will explore in the future. And thirdly, we will be having good internal accruals. So, the CAPEX that we have talked

abou

t is spread across the years, right? It is not that immediately we do require something. So, I think the company is quite capable of funding it through the internal accruals, the debt, as well as the cash position that we currently sit on.
Mohammed Patel: Okay. Thank you.
Moderator: Due to time constraint, that was the last question. I now hand the conference over to Mr. Aman. Over to you, sir.
Aman: Thank you. On behalf of PhillipCapital Private Client Group, we thank all the participants for your valuable time and especially, the entire team of Yatharth Hospitals and Trauma Care Services Limited.
Before we close the call, I would like to hand it over to Mr. Tyagi for his closing comment. Over to you, Mr. Tyagi. Thank you.
Yatharth Tyagi: Thank you, everyone for participating in our earning calls for Quarter 1 FY '26. And thanks for your questions.
Moderator: Thank you. On behalf of PhillipCapital (India) Private Limited, that concludes this conference. Thank you for joining us and you may now disconnect your lines.

(This document has been edited for readability purpose)

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Call: Nov 2025

YH/SE/70/2025-26
November 20, 2025

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Subject: Earnings Call Transcripts Q2 FY26

Dear Sir/Madam,

Pursuant to Regulation 46(2)(oa) of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed the transcript of Investors' conference call organized on November 14, 2025 post declaration of Unaudited Financial Results (Standalone & Consolidated) of the Company for the second quarter and half year ended September 30, 2025.

The transcript will also be available on the website of the Company at https://www.yatharthhospitals.com/investors

This is for your kind information and records.

Thanking You

Yours Faithfully,
For Yatharth Hospital & Trauma Care Services Limited

Ritesh Mishra
Company Secretary & Compliance Officer
M. No. A51166

Encl.: A/a

"Yatharth Hospitals and Trauma Care Services Limited
Q2 FY '26 Earnings Conference Call"
November 14, 2025

MANAGEMENT : MR. YATHARTH TYAGI – WHOLE-TIME DIRECTOR
MR. AMIT KUMAR SINGH – GROUP CHIEF EXECUTIVE OFFICER
MR. NITIN GUPTA – PRESIDENT (FINANCE) AND CHIEF OPERATING OFFICER
MR. PANKAJ PRABHAKAR – GROUP CHIEF FINANCIAL OFFICER
MR. ASHUTOSH KUMAR JHA – GROUP CHIEF - STRATEGY , M&A AND INVESTOR RELATIONS
MR. SONU GOYAL – GROUP FINANCE CONTROLLER

MODERATOR : MR. KASHISH THAKUR – ELARA CAPITAL

Moderator: Ladies and gentlemen, good day, and welcome to the Yatharth Hospital & Trauma Care Services Limited Q2 FY '26 Earnings Call hosted by Elara Capital. As a reminder, all participant lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Kashish Thakur from Elara Capital. Thank you, and over to you, sir.

Kashish Thakur: Thank you, Yousuf. Good afternoon, everyone on behalf of Elara Capital. I would welcome you all to Q2 FY '26 Earnings Conference Call of Yatharth Hospital & Trauma Care Services Limited. From the management, we have Mr. Yatharth Tyagi, Whole-Time Director; Mr. Amit Kumar Singh, Group CEO; Mr. Nitin Gupta, President (Finance) and Chief Operating Officer; Mr. Pankaj Prabhakar, Group CFO; and Mr. Ashutosh Kumar Jha, Group Chief - Strategy, Merger and Acquisition, IR; and Mr. Sonu Goyal, Group Finance Controller.

I now hand over the call to Mr. Tyagi for his opening remarks, and then we will open the floor for question and answer. Over to you, Mr. Tyagi. Thank you.

Yatharth Tyagi: Good afternoon, and welcome to Yatharth Hospital & Trauma Care Services Limited's Earnings Conference Call for the quarter ended September 30, 2025. Joining me today are Mr. Amit Kumar Singh, Group CEO; Mr. Pankaj Prabhakar, Group CFO; Mr. Nitin Gupta, Group COO and President (Finance); Mr. Ashutosh Kumar Jha, Group Chief - Strategy, M&A and Investor Relations; and Mr. Sonu Goyal, Group Finance Controller. Our earnings presentation is available on the stock exchange and on our website. We hope you have had the opportunity to review it.

This quarter marks a significant phase of operational and strategic progress for Yatharth Hospitals, delivering industry-leading performance with the highest ever revenue and EBITDA. A key positive milestone this quarter has been the order from income tax authorities unfreezing the provisional attached properties and fixed assets of the company, restoring full financial flexibility for expansion initiatives. This milestone marks an important step towards the resolution of the long pending income tax issue.

Upholding highest standard of governance, we are happy to announce the appointment of Mr. Ramesh Krishnan as Independent Director. He is a healthcare veteran, with 3 decades of experience leading hospitals and private equity in the healthcare space. In our latest Annual General Meeting, we have also appointed MSKA & Associates, a member firm of BDO International, as our Statutory Auditor with effect from quarter 2 itself, further reinforcing highest standard of transparency and governance for all our stakeholders.

We are also pleased to share that CRISIL has upgraded our credit rating to CRISIL A/Stable, reflecting the strength of our business prospects. During the quarter, we operationalised Model Town Hospital in New Delhi during mid-July, adding 300 beds to the capacity. Moreover, we also commenced operations at our Faridabad Sector 20 Hospital, adding 400 beds to the capacity.

We're already seeing increasing footfalls at both these new hospitals. With our advanced infrastructure and clinical team, we expect both these facilities to be a key growth driver in the upcoming quarters.

Continuing our expansion strategy, we also, t

his quarter acquired Shantived Hospital in Agra, having an expanded capacity of 250 beds. This hospital is already running and hence will contribute meaningfully to both our revenue and profitability from day 1 of its integration. We expect this integration to be completed by H2 of this fiscal itself, and we should be adding revenues and EBITDA of this hospital from quarter 4 of this financial year.

Our commitment to clinical excellence is also demonstrated by several technological adoptions and complex procedures conducted this quarter:

1. We utilised the latest navigation system and Intelli Joint software to perform Delhi's first-ever Robotic Intelli Hip Revision surgery at our newly started Model Town hospital
2. We also adopted Rapid AI, an advanced AI-enabled neuroimaging technology for fast and accurate stroke detection at our Greater Noida facility, enhancing diagnosis and treatment timelines

These advanced procedures position us among a select group of hospitals in North India providing these cutting-edge clinical care.

We've also made significant strides on the medical value travel front. In Q2 FY '26, we established Yatharth representative offices in Baghdad and Tashkent in partnership with Chinor Medical Centre and expanding into Cameroon through both institutional and private partnerships. We are also in the process of being the exclusive healthcare partner for the upcoming Jewar Airport, which is expected to drive international patient volumes from these regions and presence of Yatharth Hospital's clinical medical examination room at the Jewar Airport will also enhance our visibility to the passengers, both domestic and international using the airport.

I would also like to take this opportunity to highlight a key industry-wide development this quarter with the long-awaited central government health scheme (CGHS) rate revision after more than a decade, effective October 13, 2025. We welcome this move and the new framework introduces rationalised tier base rates with higher reimbursement for accredited super speciality providers in metros. This structural change will significantly enhance our revenue realisation and profitability across all our hospitals.

Looking ahead, our focus remains on ramping up the new facilities, expanding super speciality offerings and driving operational efficiencies across the network. We are confident that these initiatives, combined with a robust governance practices and strong management team will help us deliver sustained value to all our stakeholders.

With that, I would now like to hand over the call to Mr. Pankaj Prabhakar for a detailed financial update.

Pankaj Prabhakar: Good afternoon, everyone. I am pleased to say that Yatharth Hospital has delivered another quarter of record revenue and EBITDA. During quarter 2 FY '26, we recorded revenue of INR2,794 million, reflecting a healthy 28% year-over-year and 8% quarter-over-quarter growth. Our newer hospital were key growth drivers, reporting a significant 110% Y-o-Y increase, while our mature hospitals sustained the upward trajectory, reporting a 19% year-over-year increase. Notably, Greater Faridabad just in its first full year of operation now contributes 10% of our overall group revenue, reflecting the strength of our expansion strategy.

During the quarter, we have added 700 beds with commencement of our New Delhi and Faridabad hospital. However, despite the bed addition, our occupancy stood healthy at 66% during the quarter. As far as, hospital-wise occupancy is concerned, our Noida facility achieved an occupancy of 89%, Greater Noida at 69%, Noida Extension at 64% and Jhansi-Orchha at 71% during the quarter.

Our strategic focus on high-value super speciality care and

continued investment in cutting-edge
medical infrastructure has resulted in an increase in consolidated ARPOB to INR32,015 in quarter 2 FY '26. Notably, mature hospital, which includes Noida, Greater Noida and Noida Extension, achieved an ARPOB growth of 9% year-over-year, while our newer hospital registered an ARPOB growth of 19% year-over-year, reflecting our continuing focus on improving the mix.

Our Noida Extension Hospital achieved its highest-ever ARPOB of INR40,800, up by 7% year-over-year, given its approximately 70% contribution from super speciality services. Our Noida Hospital reported ARPOB of INR31,100, Greater Noida at INR38,300 and Greater Faridabad at INR31,900. Our Model Town New Delhi facility began operation in the quarter with an initial ARPOB of INR33,600. As we ramp up our Model Town and Faridabad Sector 20 hospitals, we expect these hospitals to generate ARPOB much higher than our Group ARPOB.

On the profitability front, EBITDA stood at a record level of INR645 million, up by 18% year-over-year. Given the initial operational losses at our newly operationalised Delhi and Faridabad Sector 20 hospital, our adjusted EBITDA was INR737 million, up by 35% year-over-year, while our adjusted EBITDA margin stood at healthy 26.7%. Accordingly, our adjusted profit after tax grew by 63% year-over-year.

Our balance sheet position continues to remain robust with a strong net cash position of INR3,692 million, providing ample financial headroom to capitalize on suitable growth opportunities.

Looking ahead, we remain optimistic of sustaining our growth trajectory. Our strategic priorities will focus on expanding our footprint across North India, optimizing existing clinical infrastructure, enhancing clinical excellence to drive higher ARPOB and capitalizing on emerging opportunities in the sector.

Thank you for your attention. I would now like to hand over the call to the moderator for question-and-answer session. Thank you.

Moderator: Thank you very much. The is from the line of Ritika Khandelwal from Perpetuity Ventures. Please go ahead.
Ritika Khandelwal: I just wanted a bit of clarity on Faridabad and Delhi hospitals, the amount of losses that you would be looking at? And since Faridabad has been operationalised right now and the minority share has been positive, past minority share, so are the Faridabad losses being booked? And what are the expectations going forward on the 2 facilities?
Sonu Goyal: So if you see in Q2, we have around 3.3% as overall dilution in EBITDA because of the 2 new facilities, that is, Faridabad Sector 20 Hospital and New Delhi Model Town hospital. And we are pretty much sure with the next 15 months to 17 months, both hospitals will be EBITDA breakeven.
Yatharth Tyagi: Also, even though the Faridabad 20 facility was operationalised fully, i.e., the IPD started on 1st of November, but prior to that for a month or so, almost a couple of months, the staffing was already there. So that's why a bit of the loss is already there, including some of the doctors, nurses and admin staff all stationed and getting ready for the hospital. So yes, certain of the expenses have been booked for the FY '26 and will also fully be operationalised and full expense will be booked from quarter 3 itself.
Ritika Khandelwal: What kind of -- if you can quantify what kind of losses are you looking at?
Yatharth Tyagi: Similar of what the EBITDA loss for the Q2 would be a bit similar also in Q3 because of these 2 hospitals because Delhi hospital will be then probably 4, 5 months into the operations. So the EBITDA drag there will reduce in Q3, but EBITDA drag of Sector 20 will increase a bit. So I think similar trend of the EBITDA loss of Q2 should be seen in Q3 fro

m these 2 specific hospitals.
Ritika Khandelwal: Also like for Delhi, what is the run rate currently? And what kind of occupancy are you seeing?
Amit Kumar Singh: So it's a very -- I mean, it's very early to tell you that occupancy in just 2, 3 months of the operations. But I think 13%, 14% of already Delhi is operating as of now. Faridabad, we just started. But as said earlier, we are expecting both these hospitals to have a breakeven reaching in 15, 16 months kind of. So I think we are very much up to it, right.
Yes, more empanelments and then insurances are yet to come. So once it will be done in next 4, 5 months, I think we would be -- and as we had said earlier as well, around anything between 30%, 35% of occupancy, we reach an EBITDA breakeven. So that's what we are expecting in the next 15, 16 months.
Ritika Khandelwal: For the Agra facility, when is it going to start?
Yatharth Tyagi: Agra facility will be on our books from quarter 4 fully. So somewhere from the first week of the new year, we should be starting the facility. It's already a running facility, but it will be integrated on our balance sheet from the first day of quarter 4.

Ritika Khandelwal: Okay. And lastly, can you tell us the payor mix, and what is the percentage of professional fees you are paying to the doctors?
Yatharth Tyagi: Payor mix is somewhere around 37% roughly is the government business. And the rest is equally divided between private insurance, TPA as well as self that is the cash payor. As far as your second question of...
Sonu Goyal: So overall, we are at 21.5% in the doctor share -- doctor cost percentage of revenue.
Moderator: Sorry to interrupt, Ms. Ritika, may we please request you to rejoin the queue, ma'am. Several participants are waiting for their turn. Next question is from the line of Nirali Shah from Ashika Stock Service.
Nirali Shah: I had a question on ARPOB. So your slide shows that Group ARPOB is moving from INR32,395 in 1Q to INR32,015 in 2Q. So despite the mature and new hospitals reporting ARPOB growth, this is what we are seeing difference. Can you reconcile the math like which hospital or the payor segment for the consolidated ARPOB from Q-o-Q and why did that happen?
Yatharth Tyagi: So the reason for that is the very strong increase in the occupancy of our Jhansi hospital. We are quite happy that Jhansi-Orchha Hospital has reached an occupancy of 70% as per our target, and it's now become a leading service provider in that area. So because the ARPOB in Jhansi Hospital is almost half of that of the group average.

So increase in the occupancy in Jhansi Hospital led to the decrease -- not decrease, but muted ARPOB growth. However, with the Delhi and the Faridabad 20 hospital ramping up in the coming quarters, which will have the ARPOB much higher than the group average, we will still be on the track for our ARPOB growth of for the whole year somewhere around 8% to 9% from last year.

Nirali Shah: Okay. Got it. Also on the income tax matter, you've reported that assets have been unfreezed and the FDs have been unfreezed. Are there any residual contingencies or appeal windows or any potential future claims? Like any clarity on this matter?

Yatharth Tyagi: Yes. So I think this was a major development. As far as the whole matter is concerned, we expect the whole matter also to be resolved somewhere, I think, before the end of the financial year, probably before the end of the calendar year, it will also significantly move towards closure, but I think the right timeline should be before the end of this financial year.

Nirali Shah: And my last one is on the capex plan. So we had announced in the previous con call that somewhere around INR1,400 crores to INR1,500 crores for 3 ye

ars. I just wanted to know how much additional debt do you plan to take in the next 12 months to fund the brownfield? And what will be the highest cost of borrowing you could consider as an acceptable level?

Yatharth Tyagi: Yes. So just a correction here. The INR1,500 crores to INR1,400 crores capex for the 3 years means that we will end up spending it technically in the next 5 years, somewhere around 4.5 to 5 years because, yes, deals would be announced for that capex within the next 3 years. But let's say, after 3 years, there is a greenfield expansion that we're doing. The complete utilization of

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that fund would take somewhere around 4.5 to 5 years. So technically, we will be spending INR1,500 crores over the next 4.5 years. That's first correction.

Second is, as far as the next 12 months are concerned, so obviously, Agra's 250 beds will be with us in Q4. And then we also remain committed post that. That means somewhere around H1 of the upcoming next financial year '27, we should also be looking to add somewhere around 300 to 400 beds through an acquisition or an upcoming project that we are working on.

Nirali Shah: I have one more question, if I may.

Moderator: Ms. Nirali, may we request you to rejoin the queue Ma'am. Next question is from the line of Param Vora from Trinetra Asset Managers.

Param Vora: Congratulations for a great set of numbers. So sir, your Q2 revenue growth is strong. So can you break down the quarter's revenue growth like what did it led to the growth? Like was it the patient footfall that has increased? Or is it the mix and shift like speciality like towards higher-margin speciality or maybe the pricing?

Amit Kumar Singh: Yes. So it is a mix. So a, definitely, the -- as you had said that we have a still -- in existing hospitals, we have drivers, be it change of mix or bit of fine-tuning our existing mix. Second is very good growth, as mentioned in the commentary, that's 10% contribution is from the new hospital Faridabad 88, right. So all these things, we believe that adding Delhi and new -- I mean Faridabad Sector 20 will add up much more growth. However, there is still good room for our existing hospital be it a Greater Noida and Noida Extension for upward growth.

Moderator: Next question is from the line of Akhilesh Rawat from Ridhanta Vision Private Limited.

Akhilesh Rawat: So first of all, I want to congratulate you on the good set of numbers. And I have basically a question regarding this international like medical value tourism. So like what's your plan regarding this for upcoming financial '26 and financial '27?

Amit Kumar Singh: So we have a strong team now and then we have already rolled out our strategy. Recently, we had opened 2 information centre and OPD centre One is in Tashkent and other is Naira. A few more are going to come in African market. So I think we have identified these pockets. And in this financial year, I think that the strategy is very, very clear that we will open around 5 to 6 information centres.

And we are developing -- we are working on the channel partners be it in that particular respective country or the local. So we believe that this financial year or probably the H1 of the next financial year, we will have a very good percentage contribution from the international business and airport is also going to start by towards the end of this financial year.

Akhilesh Rawat: Okay. And what kind of ARPOB lift you are seeing from this, could you quantify on that?

Amit Kumar Singh: See, I think there is always upward of around anything between 45% to 55% that's up from your domestic. So even you can extrapolate from there.

Akhilesh Rawat: And my last question is regarding the Agra hospital, like when will we see the results in our books regarding that hospital.
Yatharth Tyagi: As earlier mentioned from quarter 4 of this financial year.
Akhilesh Rawat: Okay. Okay. And what kind of ARPOB are we expecting from that hospital ARPOB?
Yatharth Tyagi: I think upwards of INR30,000, somewhere around that to begin of, slowly as we ramp up more super specialities that this has the potential to be somewhere as the group average ARPOB and also a bit higher also in the future if possible.
Moderator: Next question is from the line of Akshat Mehta from Seven Rivers Holding.
Akshat Mehta: Congrats on a great set of results. I have a few questions here. One is that in the Delhi hospital that we just started, we had earlier guided for INR39,000, INR40,000 kind of an ARPOB number. But in this quarter, you've seen a INR34,000 roughly ARPOB. Can you help us explain why -- what is the difference?
Amit Kumar Singh: Delhi, I would say -- I mean it's too early to judge Delhi. Still there is no insurance, even cash patients-- initially, there is a very early rates which we have put in market, but in the next 2, 3 months, if you see when the tariffs and everything is frozen as per the competitive, insurance negotiation happen and more and more international flow in that particular hospital, you see this ARPOB will be much more than what you are seeing as of now. So what we have said earlier, it is absolutely realistic.
Yatharth Tyagi: Whenever a hospital start, usually first few months it takes for ARPOB building. In fact, we are quite happy that the first month itself, it has started with INR33,000 ARPOB. That gives us confidence that within 12 months or even earlier, this ARPOB can cross INR40,000. And that's the trend across other hospitals also that we witnessed when we started them.
Akshat Mehta: Okay, sir. Next question is that I want to understand with the losses that we've seen from the new hospitals and that will continue in Q3 and probably in Q4 because Shantived will also be adding. What kind of margins can we look at on a full year basis? And with the new hospitals being added, I think we can overshoot a number of 30% revenue growth also. So if you can provide a guidance on that as well.
Yatharth Tyagi: So Agra will not be EBITDA drag because when it comes to Q4, it is already an EBITDA positive hospital and even profitable. So there will be no drag due to Agra. That's one. Second, as far as the margins are concerned, I think what we closed the H1 margin for, I think somewhere around that would be a realistic number for the whole of the end year also, maybe 0.5% up and down here.
But I think that these margins that we have already will not be dragged further. We are quite confident even for the quarter 2 margins, which are here, it will be only upside from here for the whole financial year. And Agra will be EBITDA positive from day 1. Lastly, your question on the 30% revenue growth. Yes, I think that's easily visible. I think we are being actually a bit

conservative when we're seeing 30% revenue growth. We feel that this number should be easily overachieved.
Akshat Mehta: My next question is on the overall capex side as well. So this year, sir, what is the kind of -- this and next year, what is the kind of capex that we are seeing in the actual spend?
Yatharth Tyagi: I think we have already given the guidance for the next 4, 4.5 years of INR1,500 crores of capex. I think we'll just stick to that. We have a good cash position. We are debt free, and we are generating good internal accruals. So whatever capex is r

equired, the company will be able to very well fund it also.
Akshat Mehta: Sir, one bookkeeping question...
Moderator: Sorry, Mr. Akshat, may we please request you to rejoin the queue, sir? Next question is from the line of Surya Narayan Nayak from Sunidhi Securities and Finance Limited.
Surya Nayak: Congratulations on the good set of numbers. So just want the -- the understanding about the Agra hospital. When we took over, I believe it is operating at around 40% occupancy. And we have plan to expand it to 250. So over what period of time the hospitals will be ramped up? I understand that it will be acting as a feeder hospital. Then what will be -- when we will be able to incorporate all kind of super specialities in it? And when we will be able to get a kind of ARPOB at least of INR35,000 plus?
Yatharth Tyagi: So Agra, we are not just seeing it as a feeder hospital. Agra is a big stand-alone 250-bed super speciality hospital. Agra as a market is very fertile. It is -- we are the first corporate hospital in

that region, and it has huge population of not just Agra, but nearby places like Hathras, Saifai, Etawah, Shikohabad and all these patients drain to Agra. And the combined at this level of facility is not there in that whole region.

So Agra will be a full stand-alone super speciality hospital where we will be providing all the super specialities. And it will be on our books from January itself of the next calendar year. We will start with 150 beds, but we do not need to do any more capex to reach that 250 bed expansion, only these beds need to be added. The whole infra is almost ready there for 250 beds. So as and when required, as per occupancy, we will see and we will just keep on opening floors and putting in beds there. And as far as the ARPOB full mature is concerned, so remind that because it's already a running hospital, it will not take time for us to develop super specialities much time because already there's a cath lab, there is cardiac surgery is happening there, there angioplasty is happening there. A lot of super specialities are already existing in that hospital. So we will see a very good and a very quick ramp-up of both the ARPOB, super specialities as well as occupancy for the Agra hospital is concerned.

Surya Nayak: Okay. And just in the previous participants' answers, I just missed the year ending EBITDA margin considering the 2 quarters hit on the new hospitals cost absorption. So what would be the guided EBITDA margin for FY '26? And whether that will be fully absorbed and will not be incurring more loss in the FY '27?

Yatharth Tyagi: No. So as we earlier said, the EBITDA margin what we have for this quarter there's only upside from here that will happen for the coming quarter and the whole financial year also. And it will be fully absorbed because yes, there would be -- next year also, we would be adding capacity. But this year, if you look at it technically, we have added -- including Agra, we will be adding -- the whole financial year, we would be adding 1,000 beds. So that's 3 new hospitals that have come up. And even with the 3 new hospitals in a financial year, we are able to maintain margins upwards of 23% of EBITDA. So whatever in the future is going to be upside from here only.

Surya Nayak: Sir, regarding...

Moderator: Sorry to interrupt, Mr. Surya, may we please request you to rejoin the queue, sir?

Surya Nayak: Just one follow-up question, sir. Can I?

Moderator: Yes, please go ahead, sir.

Surya Nayak: Okay. Sir, with the long-term ambition of INR1,500 crores capex over 4.5 years, so can we assume that we will first see that the consolidation of this additional 1,000 beds happens and maybe then we will think of having inorganic -- because

we have brownfield plants as well going ahead. So can we assume that we will expect some kind of consolidation at the existing new facilities, then we will think of expansion or this inorganic expansion? And whether inorganic expansion will limit to NCR region or periphery of NCR, that is what I just want to understand?

Yatharth Tyagi: So at least for the remaining of this financial year, there will be no additional capacity that would be coming to be utilised. We might take a greenfield project upcoming soon, but that will take time for the capacity to come into play. And if these hospitals as we are targeting that in 15 months, they're getting breakeven. So it's not that we have to wait long to first consolidate and then add capacity.

I think we are very well in a position right now also, not this financial year, but in maybe the next financial year, to add more acquisitions, more hospitals in H1 itself, and that's the plan. And second, to your question as far as the locations and geographies, we believe that the markets where we are present, that is the NCR market, the whole North India market of like big cities in UP, Haryana, Punjab, Rajasthan.

I think we want to be in metro cities. We want to be in big cities. It will technically not be just peripheral hospitals. We -- all our hospitals so far, if you look at it in the last few quarters, we have added in the hardcore, Delhi and Faridabad market, which are big cities. And this is what we also want to continue going forward. Agra is also a big city in UP. So these are the big metro cities where we want to be in the coming years also.

Surya Nayak: So what is the cut-off ARPOB you have set?

Yatharth Tyagi: Sorry, can you repeat that?

Surya Nayak: What is the cut-off ARPOB you have planned for new acquisitions? I mean below a certain

threshold, you will not want to look at, I mean, location-wise. I mean is it earning INR35,000.
Yatharth Tyagi: Yes, I think upwards of that, I mean, that should be the minimum. That's criteria.
Moderator: Next question is from the line of Dhaval Sangoi from Canara HSBC Life Insurance.
Dhaval Sangoi: So, just wanted to check your view on this CGHS rate revision that has happened from October. Have we seen any benefit flowing in? And does it any way impact us positively?
Nitin Gupta: CGHS rate has been implemented recently in the month of October. So we are looking to the upside of CGHS revenue that will be around 1.5% in this FY '26 with the benefit on EBITDA to around 1% in this financial year, which may go up to 2.5% in the next year on the revenue front and nearly around 1.75% on the EBITDA front.
Dhaval Sangoi: Okay. So say, on a base of 37% of government revenues that you have, we will see an incremental benefit coming in on that.
Nitin Gupta: Price benefit, yes. It will be benefit to the company. The rate revision will be adding to the ARPOB as well that will benefit to the company.
Moderator: Next question is from the line of Naveen Baid from Nuvama Asset Management.
Naveen Baid: Great set of numbers. If I look at your trend for the past 6, 7 quarters, your ALOS has been coming down and your occupancy ratios have also been going up. So in a way, your throughput has been increasing pretty significantly. What explains the drop in ALOS over the last 6 quarters from, say, 4.8, 4.9 to where this quarter we are reporting this at 4.06?
Amit Kumar Singh: Introduction of the new sets of technology like robotics, that's reduced our ALOS significantly, right, high-end speciality, state-of-the-art infrastructure. So that's anything which is earlier -- I'll give you an example, for any Lap surgery where patient used to stay 3, 3.5 days or 4 days kind of, now you can discharge that patient in up to 2

days, right. So adding of the new technology has played a big role in reducing the ALOS and very, very new sets of treatment modality. So this has a role to play.
Naveen Baid: But if that is the case, then should your ARPOB not have increased by much more than what it has actually even. If I step off the government segment, which is about, say, 37% your ARPOB has gone up by just about, say 5%, 6% over the last 6 quarters despite the significant improvement in the case mix. But like what explains this? Or do you think there's a significant upside in your ARPOB to go up over the next 4, 6 quarters?
Amit Kumar Singh: So, you need to see the data of that flagship hospital, particularly the Noida extensions where we have all those sets of modalities and new technology. So there, the ARPOB has significantly increased, right. So it's not like directly you see the INR1 increase and INR1 increase there. But I think this is how it should play. So we believe that more and more the capacity gets ramped up in those super speciality, we believe that -- I really believe that the ARPOB will also increase.

Yatharth Tyagi: Also, for last 2, 3 years, clearly, we have shown 8% to 10% ARPOB growth. And I think for next 3 years also, we do expect 8% to 10% ARPOB growth clearly. And especially now with the Delhi market and the Faridabad bigger hospital having oncology transplant, increase in international share, that's ARPOB will be a huge lever for us also going forward.
If we continue to be in, as I talked about, big in metro cities, obviously, a market like Jhansi-Orchha wouldn't have been a driver on the ARPOB. But if we are expanding our facilities here in Noida extension, in Delhi, so ARPOB should significantly increase at a faster pace as the capacity happens in these new hospitals.
Naveen Baid: Sure. Just one more question. So this is continuing on the previous participant's question that if your government share of revenue is roughly about say 37-odd percent, if I take the full year impact for next year, wouldn't 2% be too conservative? Or this is what you are seeing from your kind of case that the blended improve is going to 2%. Also if you could kind of split the government revenues into what is derived through CGHS and what is non-CGHS?
Yatharth Tyagi: Yes, so 2.5% is the minimum that we have said conservatively. Whatever upside we get from there, we will take it. But I think it's realistic to take a 2.5% number because at a 37% revenue, it's not that 100% of that revenue will fall under the new rate revision scheme. There is, for example, we do a bit of Ayushman in Jhansi-Orchha Hospital and Ayushman doesn't fall and benefit from this scheme. So overall, I think it's -- when we have calculated, I think minimum to minimum 2.5% gain we should be seeing next year on the overall revenue not the... So overall revenue upside is 2.5%. So the whole company will be benefiting significantly from this rate revision. 2.5% of the whole revenue, you can imagine is a big quantum that we'll be benefiting next year and in the coming years also.
Moderator: Next question is from the line of Sumit Gupta from Centrum Broking.
Sumit Gupta: So 2 questions. First is, is there any scope of further margin improvement in the mature facilities?
Yatharth Tyagi: There is, yes, because there is still capacity left to utilise in the mature capacities when we mature

hospitals. When we say they're mature, that doesn't mean that they are saturated in terms of occupancy. For example, both Noida extension and Greater Noida can reach optimised utilization of 75% of occupancy. So still a 10% room way for occupancy to grow, which will directly flow to the EBITDA. So yes, there is scope for EBITDA expansion in our mature facil

ities also.
Sumit Gupta: Okay. So, over the last 1.5, 2 years, I think it is at 26% to 27% kind of range and majority of the drag obviously is coming from the new hospitals. So this 27% can reach around 28.5%, 29%.
Yatharth Tyagi: It can in the coming years.
Sumit Gupta: So basically, Noida extension would be running at 30%, 33% kind of margin, if I'm sure -- basically I just want to ask on Noida extension as well as Greater Noida.
Yatharth Tyagi: Yes. I mean not that high. But as I said, there are also a lot of recent additions in terms of doctors and star doctors expenditure that we have done in last 1 year. Going forward, a lot of those

expenses are being done now that happened over the course of 1 year. But going forward, we will see an upside from those expenses done, which should be able to drive the numbers that you are just mentioning.
Sumit Gupta: Understood. Okay. And the additional 300 to 400 bed M&A that you plan to do over the next 12 to 15 months, so is that already mentioned, like is it additional to the capex plan mentioned in the presentation?
Yatharth Tyagi: It is inclusive of that INR1,500 crores capex.
Moderator: Next question is from the line of Gaatha Jain from Anantnath Skycon Private Limited.
Gaatha Jain: I just had 2 questions. First one is, so what is the cost to set up a bed in a mature or a developing hospital? And how many days does it take for it to breakeven?
Yatharth Tyagi: So if you are doing a greenfield today in NCR, it would take you roughly around INR1 crore capex per bed at least for our brand. That's the threshold we would be spending. And it takes almost, I would say, 15 to 18 months, which we personally target as a brand to achieve breakeven after operation -- after operationalizing of that hospital. If you're doing brownfield, then that capex per bed can be somewhere around INR75 lakhs per bed because a lot of medical equipments are already present in one tower. So brownfield expansion is a bit less than the greenfield expansion of INR1 crore capex per bed.
Gaatha Jain: And what is the time period for breakeven for brownfield?
Nitin Gupta: We generally have - EBITDA breakeven to around 15 months for the new brownfield projects.
Gaatha Jain: All right. Got it. And how many total number of...
Moderator: Sorry to interrupt, Ms. Jain. May we please request you to rejoin. Next question is from the line of Tripti Shukla from Kedia Securities Private Limited.
Tripti Shukla: My question is related to working capital like your receivable has increased meaningfully this quarter and cash balance have declined due to capex and working capital. So could you elaborate on which specific receivable bucket are stretching the cycle? And by when the debtor days should normalise? Like Also when do we expect the operating cash flow to EBITDA return to the historical level of 70% to 80%?
Sonu Goyal: So if you see our debtor days has been reduced. So what we have given the projections earlier also in our earnings call. So this time also, we have reduced the debtor days by 8 to 10 days. And we are very much sure we will follow the same trend, and we will clock the same 8 to 10 days reduction as on March 2026. Regarding cash flows. So OCF to EBITDA currently we are 58%. So we have changed the policy. Earlier we have paid salaries on net cumulative of a month, now we are matching the salaries payment as per the competition, as per the peers, okay. Now we are making a bit on the same -- last day of a month, okay. If we neutralise that effect, so our OCF to EBITDA will be around 78%. It is better than what we reported earlier.

Moderator: Next question is from the line of Vidhi Shah from CRK.
Vidhi Shah: I wanted to ask, on the EBITDA margin, you said it will only go upwards henceforth. But I want to know with the new capacities coming in the future, how do you see the EBITDA margins improving further? And wouldn't there be any margin dilution?

Yatharth Tyagi: No, there will be no margin dilution because as we earlier mentioned that we still have added 700 beds in this one quarter and still have this base margin. Going forward, there's no plan to add 700 beds in 1 quarter at least for next 4, 5 quarters. So there's only an upside on the margin. In fact, if you look at it, we have clearly mentioned that excluding the capacity expansion that we have done, our EBITDA margins would have been close to 26.5%. So that is the reason why we feel confident that when these new facilities start ramping up on occupancy, there is a big margin expansion that should happen in the coming years.
Vidhi Shah: All right. And can you give me...
Moderator: Sorry to interrupt, Ms. Shah, may we please request you to rejoin the queue. Many participants are waiting for their turn.
Vidhi Shah: my second question, sir...
Moderator: Ma'am, we restrict to one question only. Please rejoin the queue. Next question is from the line of Anand B from Ksema Wealth Private Limited.
Anand B.: I just want to know the capex -- sorry, not capex, census beds of each of the hospitals in this quarter.
Nitin Gupta: We have a census of around 1,750 as of date for all the capacity we are operating.
Anand B.: Can you just like break it up from the older annual capacity as well?
Nitin Gupta: So you need unit-wise breakup?
Anand B.: Yes, yes, breakup by hospital to hospital.
Nitin Gupta: In the Greater Noida facility we have 330 beds -- census beds, Noida 110 -- we have 215 beds. Noida Extension we have 390 beds. Jhansi-Orchha, we have around 250 beds. Greater Faridabad Sector 88, we have 150 beds and the rest of the new facility we just added in this quarter in Delhi and Sector 20, Faridabad as well. We have been in ramp-up stage of around 180 each. So that comes to around 1,750.
Moderator: Next question is from the line of Santosh Singh from Asha Investments Fund.
Santosh Singh: I have one question. Sir, could you please explain the reason behind the reclassifying loan amounting to INR516 crores in this quarter? Additionally, which subsidiary are we seeing this loan? What does the subsidy do? And what is the intended purpose of the loan like working capital or the expansion or any specific project?

Nitin Gupta: So basically, it's a subsidiary we have just added the new hospital in Sector 20 Faridabad as one of the subsidiary for which we have given the capital outlay as per the defined object for which we have raised the funds in the QIP. So basically, there is no loan for anything, but it's a deployment for the capex as well as the expansion of the hospital as per defined objects.
Santosh Singh: Okay, sir. Got it.
Yatharth Tyagi: Our current debt levels are standing at almost 0. So the company is actually debt free. So there is no current debt that any of the holding company or its subsidiaries have taken as of now.
Moderator: Next question is from the line of Abhay Sahukar from Convergence Capital.
Abhay Sahukar: Congratulations on a good set of numbers. So I just wanted to know that what are the initiatives taken to improve clinical programme? Are there any specific speciality lines, for instance, cardiac, neuro or oncology or any other seeing faster ramp-up in any of the units?
Amit Kumar Singh: So we will be going into the unit-wise you see that what is in the market an

d the facility we are
lacking in super speciality and quaternary care. So 1.5 years back, we started in Noida Extension, an entire spectrum of cancer treatment and all the transplant programmes. Similarly, Greater Noida, we started our kidney and liver transplant programme.
Delhi definitely going to be a full-fledged quaternary care of hospital, including the transplant. Similarly Faridabad as well. So what clinical treatment modalities would be added, that depends on which area we are operating and what are the competitive advantage what we'll have by adding those Super Speciality.
Similarly Agra also, we're seeing that definitely we will be having the entire speciality care of the facilities in that particular, including probably long-term cancer, right. So this is how we analyze which are the gray areas where we need to invest and what would be the ROI and all these things metrics works.
Abhay Sahukar: Perfect. And just a follow-up on that....
Moderator: Sorry to interrupt Mr. Sahukar. We please request you to rejoin the queue for the follow-up question, sir. Next question is from the line of Prerana Amanna from Equity Research Programs.
Prerana Amanna All my questions have been answered.
Moderator: Next question is from the line of Anand Kulkarni from Front Wave Research.
Anand Kulkarni: Just one question from my end. Can you share the status of the brownfield expansion of Greater Noida and Noida Extension Hospitals? Like when can we expect this to be operational? And

post that, post FY '28, after we achieve our target of 3,000 beds, how are we looking at our bed expansion strategy?
Yatharth Tyagi: Yes. So I think both these 2 hospitals capacity should be available for us to be utilised somewhere 15 months from the date today. So 15 should be Greater Noida and maybe 17, 18 months from today should be the Noida extension. Because we will -- and the 3,000 bed capacity

by '28, I think we should cross it much more even earlier because there's something planned next year in terms of acquisition is concerned. So that's -- in fact, we have revised our bed capacity target rather than now talking about 3,000 beds in '28, the bed capacity would be much more for next 3 years from today.
With the Agra addition, we would be around 2,550 beds. We feel that probably in the next 3 to 4 years, there is still scope in the company to even close to double this capacity in next 3 to 4 years, including all these brownfield expansions that are already in place, the upcoming acquisitions and 1 more greenfield that the company is planning very soon within the NCR region. So I think we are looking at a much bigger bed capacity after 4 years from today.
Moderator: Next question is from the line of Abhijeet from PI Asset.
Abhijeet: Just one question. There is a GST rate cut on consumables, reagents and chemicals. So will it be safe to assume that we can retain the benefit?
Sonu Goyal: So there are change in GST that will definitely help us. So that will lead to around 0.3% to 0.5% of the overall revenue. That will change only 0.3 to 0.5% in EBITDA.
Moderator: Next question is from the line of Nirali Shah from Ashika Stock Service.
Nirali Shah: Yes. I just had a follow-up on the cost of borrowing. So what is the highest cost of borrowing that you can consider acceptable?
Nitin Gupta: As on date, we do not have any kind of debt in the books. However, if in future we take up the debt, then generally it comes to around 7.5% to 8% on a rate coupon from the PSU banks.
Nirali Shah: Okay. And we are guiding INR40,000 ARPOB for FY '28, we are consistent on that?
Yatharth Tyagi: So 10% -- 8% to 10% growth clearly from now. I think we are quite confident of reaching there.
Moderator: Next follow-up question

is from the line of Akshat Mehta from Seven Rivers Holding.
Akshat Mehta: Just one bookkeeping question, sir. In this quarter or in this half year, we've seen a slight reduction in our cash flow from operations. There is a bump up in other financial liabilities. If you can help us understand what is that?
Sonu Goyal: So basically -- so as I already mentioned, there is a change in policy. The company has taken a call to match the salary payment as per the peers. So now we have started making salaries on the last day of a month. That's why there is a change in other financial liability.
Yatharth Tyagi: Earlier we used to make on the 1st day of the month.
Moderator: Ladies and gentlemen, we will take this as the last question for the day. I would now like to hand the conference over to the management for the closing comments.
Yatharth Tyagi: Thank you, everyone, for tuning into the earnings call of Yatharth Hospital & Trauma Care Services Limited for the Q2 FY 2026. Thank you for your presence.

Moderator: Thank you, sir. On behalf of Elara Capital, that concludes this conference. Thank you all for joining us, and you may now disconnect your lines.

(This document has been edited for readability purpose)

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Call: Feb 2026

YH/SE/87/2025-26
February 13, 2026

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Scrip Code: 543950
ISIN: INE0JO301016
Subject: Earnings Call Transcripts Q3 FY26

Dear Sir/Madam,

Pursuant to Regulation 46(2)(oa) of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed the transcript of Investors' conference call organized on February 06, 2026 post declaration of Unaudited Financial Results (Standalone & Consolidated) of the Company for the quarter ended December 31, 2025.

The transcript will also be available on the website of the Company at https://www.yatharthhospitals.com/investors

This is for your kind information and records.

Thanking You

Yours Faithfully,
For Yatharth Hospital & Trauma Care Services Limited

Ritesh Mishra
Company Secretary & Compliance Officer
M. No. A51166

Encl.: A/a

"Yatharth Hospitals & Trauma Care Services Limited
Q3 FY '26 Earnings Conference Call "
February 06, 202 6

MANAGEMENT : MR. YATHARTH TYAGI – WHOLE -TIME DIRECTOR
MR. AMIT KUMAR SINGH – GROUP CHIEF EXECUTIVE
OFFICER
MR. NITIN GUPTA – PRESIDENT , FINANCE AND GROUP
CHIEF OPERATING OFFICER
MR. PANKAJ PRABHAKAR – GROUP CHIEF FINANCIAL
OFFICER
MR. ASHUTOSH KUMAR JHA – GROUP CHIEF -

STRATEGY , M&A AND INVESTOR RELATIONS
MR. SONU GOYAL – GROUP CHIEF FINANCE
CONTROLLER

MODERATOR : MR. SHRIKANT AKOLKAR – NUVAMA WEALTH
MANAGEMENT LIMITED

Corporate Office: Second Floor, Sovereign Capital Gate, FC -12 Sector 16A, Noida - 201301
CIN: L85110DL2008PLC174706

Moderator: Ladies and gentlemen, good day, and welcome to Yatharth Hospital's Q3 FY '26 Earnings Conference Call hosted by Nuvama Wealth Management Limited. As a reminder, all participant lines will be in the listen only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during this conference call, you can signal an operator by pressing star then zero on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Shrikant Akolkar from Nuvama Wealth Management Limited. Thank you, and over to you, sir.

Shrikant Akolkar: Hi. Good day, everyone, and thank you, Bhumi. On behalf of Nuvama Wealth Management, we welcome you all to the Q3 and 9 Months FY '26 Earnings Conference Call of Yatharth Hospital & Trauma Care Services Limited. From the management side, we have with us to day Mr. Yatharth Tyagi, Whole -Time Director; Mr. Amit Kumar Singh, Group Chief Executive Officer; Mr. Nitin Gupta, President, Finance and Group Chief Operating Officer; Mr. Pankaj Prabhakar, Group Chief Financial Officer; Mr. Ashutosh Kum ar Jha, Group Chief Strategy, M&A and Investor Relations; and Mr. Sonu Goyal, Group Chief Financial Controller.

I now hand over the conference call to Mr. Yatharth Tyagi for his opening remarks. Thank you, and over to you, Yatharth.

Yatharth Tyagi: Good afternoon, and welcome to Yatharth Hospitals & Trauma Care Services Limited's Earnings Conference Call for the quarter ended December 31, 2025. Our earnings presentation is available on the stock exchange and on our website. We hope you have had the o pportunity to review it.

Quarter 3 FY '26 marked yet another quarter of industry -leading performance for Yatharth Hospitals with a robust 46% Y -o-Y revenue growth trajectory, delivering our highest -ever quarterly revenue and profitability. A standout highlight this quarter has been the rapid scale -up of our newly operational New Delhi and Faridabad Sector -20 hospitals.

In the first full quarter of operations, these hospitals generated INR279 million in revenue, contributing 9% to group's revenues. Faridabad Sector -20 has already reached a monthly revenue run rate of INR7 crores to INR8 crores within just 3 months of launch, while New Delhi is delivering close to INR5 cror es of monthly revenue run rate. Both these hospitals derived 100% of their revenues from cash and TPA, that is private insurances, reflecting our focus towards payer mix from day 1 of operations. On the operating metrics, they demonstrated strong initial ARPOB performance also with New Delhi at approx. INR40,000 and Faridabad Sector -20 at INR36,000. These levers are higher than group average, underscoring the quality of case -mix and the clinical offerings from day 1 on our new hospitals.

With the addition of Faridabad Sector -20, alongside our earlier started Greater Faridabad hospital that commenced operations last fiscal, Yatharth has now emerged as the most preferred health care chain in the Faridabad region. This has been achieved through strategic investments in star clinicians, high -end and complex super specialties and state -of-the-art technology, including robotics -enabled care from the inception.

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Our Greater Faridabad facility, which achieved breakeven in quarter 1 FY '26, continues to contribute meaningfully, not only to revenue but also to profitability, with its EBITDA margin now approaching the group average. These

outcomes reinforce the strong target identification, integration and scale -up of capabilities as well as our focus on achieving profitability in new facilities within short time frames.

Moving to our recent expansion, the Agra hospital has now been fully integrated into the Yatharth network effective February 1, 2026. The hospital is already a fully operational facility with strong visibility in its micro -market, and we are confident that it will contribute

meaningfully to revenue and EBITDA from this quarter.
Moving on, clinical excellence remains at the core of our value proposition. During the quarter, our teams delivered several advanced and complex interventions, including a successful management of pan-brachial plexus injury using high -precision intercostal -to-musculocutaneous nerve transfer to restore key motor functions. In another treatment of a rare congenital urological condition, left ureterocele with duplex collection system complicated by ureteric stone and severe hydronephrosis through advanced minimal invasive endourological surgery was performed. We also completed a 13 Peroral Endoscopic Myotomy, POEM procedure, in 2 months, demonstrating our leader ship in scar -free endoscopic therapies across the region.
These outcomes highlight the depth of clinical expertise across our network and strengthen our positioning as a leading quaternary care provider. Some of our key clinicians were also recognized with marquee national awards this quarter, further strengthening our clinical reputation.
With the Jewar Airport expected to begin operations soon, we have accelerated our medical value travel initiatives and continue to gain traction. During the quarter, we expanded international outreach through OPD operations in Mauri tius, Nigeria and Turkmenistan. We also hosted delegation visits and hospital tours from Afghanistan, Uzbekistan, Kazakhstan and strengthening our diplomatic and health care partnerships through this. We further conducted partner engagement events with stakeholders across Africa and Iraq.
Looking ahead, our focus will remain on accelerating the ramp -up of new facilities like New Delhi, Faridabad Sector -20 and Agra hospitals, while continuing to drive operational efficiencies across the network. We are already seeing encouraging signs from the recent CGHS price revision, benefiting bot h our top line and bottom line.
And we further remain confident and committed to clinical excellence, disciplined execution and strong governance and are confident in our ability to deliver sustained value to all our stakeholders. Quarter 3 has been an exceptional performance by the comp any, and we further expect due to the integration of the new hospitals in our network that quarter 4 would even be better than the quarter 3 numbers.
With that, I would now like to hand over the call to Mr. Pankaj Prabhakar for a detailed financial update.

Pankaj Prabhakar: Good afternoon, everyone. I am pleased to share that Yatharth Hospital has delivered a stellar performance this quarter. During quarter 3 FY '26, we achieved a revenue of INR3,205 million, reflecting a strong growth of 46% year -over-year and 1 5% growth quarter -over-quarter.
This strong growth was driven by our existing hospitals, which sustained a robust growth trajectory of 33% year -over-year as well as our newly operational hospital, which contributed to INR279 million in revenue, 9% to group's revenue in their first full quarter of operations.
Occupancy across our hospital network stood healthy at 67% during quarter 3, with our key hospitals, Noida operating at 91%, Greater Noida at 74%, Noida Extension at 61% and Jhansi -Orchha at 72% during the quarter. Our newer facili

ties saw promising adoption with healthy
volume at our Greater Faridabad, New Delhi and Faridabad Sector -20 hospitals, demonstrating Yatharth position as a preferred health care provider in the region.
Our ARPOB was up by 10% year -over-year to INR33,744 in quarter 3, driving by our continued emphasis on improving our mix of high -value super specialties and investment in the state -of-the-art medical infrastructure. Our Noida Extension hospital achieved its highest -ever ARPOB of approx . INR44,000, up 16% year -over-year, supported by approximately 70% contribution from super -specialty services, of which 18% came from oncology treatment.
Even in initial days of their operation, our New Delhi hospital achieved an ARPOB of approx . INR40,000, while Faridabad Sector -20 achieved an ARPOB of INR36,000, which are higher than the group average. Our other hospitals in Greater Noida, Noida and Jhansi -Orchha, registered an ARPOB of approx INR39,000 +14% year -over-year, INR31,000, that is 8% year -over-year and INR14,000, respectively.
On the profitability front, we have achieved our highest -ever EBITDA at INR742 million, up by 35% year -over-year. Adjusting for the initial ramp -up losses at our newly operationalized New Delhi and Faridabad Sector -20 hospitals, our adjusted EBITDA margin stood strong at 29.2%, led by operating leverage, mix improvement and positive impact of price revision in government business. Net profit after tax stood at INR431 million, up by 41% year -over-year, while adjusted PAT up by 80% year -over-year.
With a strong execution engine in place, we remain confident of sustaining the accelerated

growth momentum while enhancing operational efficiencies and exploring new avenues for growth. Our robust balance sheet position with a strong net cash position prov ide us ample financial headrooms to capitalize on sustained growth opportunities as it comes.

Thank you for your attention. I would now like to hand over the call to the moderator for question -and-answer session. Thank you.

Moderator: Thank you very much. We will now begin the question and answer session. Our first question comes from the line of Priyanshu Jain from Infinity.

Priyanshu Jain: Sir, my first question is on the revenue mix side. So as of today, by the end of Q3, what is the revenue mix from the cash and TPA and from government side?

Yatharth Tyagi: So the current revenue mix from the government side is close to 35% and the remaining is equally divided with cash and private insurances. The recent quarter with the promises the new hospitals have shown on the cash and private insurances segment side, we remain quite confident that in the coming quarters, we should be inching towards reduction in our government mix, like our initial target. And I feel in 2, 2.5 years' time, our government mix should be reduced somewhere close to 25% to 28%.

Priyanshu Jain: So 2 years down the line, we are aiming for 25% to 30% like approx?

Yatharth Tyagi: Two years down the line, government business should be lesser than 30%.

Priyanshu Jain: Okay. Sir, my second question is on the brownfield expansion as we are going for 3,000 beds going forward. So , like till what time we can think that this might be operational?

Ashutosh Kumar Jha : Yes. So we are talking of the addition of almost 3,000 beds to take the total cumulative beds to 5,000 in the next 3 years, where we have mentioned that the deals will be announced by the next 3 years and the operationalization of the hospitals will be in 4 to 5 years over a period of time.

Priyanshu Jain: Okay. And sir, like are we -- as of today, what's the net cash on the balance sheet, like if we are aiming for any acquisition going forwa

rd? And are we like...

Sonu Goyal : So current cash and bank position as on 31st December at a consol level is coming close to INR200 crores.

Priyanshu Jain: Okay. And like we are aiming for any acquisition as of today because we have like acquired many hospitals in last few years. So like are we still like looking for it?

Yatharth Tyagi: See, acquisitions will definitely happen over the course of coming years. So we do have a good, strong cash position as well as we can always take certain debt. However, our internal accrual is also quite strong. So I think using all these 3 mixes, we are well in position to fund our capex for the next good 3 years.

Priyanshu Jain: Okay. Sir, just last 2 questions. What is the right now capex per bed, like as of today, if we go for it?

Yatharth Tyagi: So as of today, capex per bed should be around INR60 lakhs per bed because there were a lot of hospitals that we started 10 years ago where the construction as well as the land prices were quite cheap that time. And slowly, we had upgraded to medical equipments.

But if you look at our newer hospitals, that is the Delhi, the Faridabad -- both the Faridabad hospitals, there the capex per bed is much higher. I think there, the capex per bed would be in the tune of upwards of INR80 to INR90 lakhs per bed. So that's the current capex per bed plan.

Priyanshu Jain: Okay. Sir, last question is on the -- sir, I just wanted to know that there is a recent incident where one of the relatives have go for a knee replacement surgery. And like suppose it's -- take an example, it's around INR2 lakh in cash. Is it possible that an insurance -- like hospitals do practices these kind of activities where like in cash segment, if they go for INR2 lakh, but like

if the patient is having an insurance policy, they make it to up to INR2.5 lakhs or INR3 lakhs like?

Amit Kumar Singh : No, it's absolutely not. I mean that this pricing is already fixed. So if the cash is, let's say, INR2 lakh -- in fact, for the insurance, it will be lesser effectively to us, I mean, what is the landing price to us because that agreement is sig ned, rates are already freezed.

So you can't make a change. And this is not manual that if I wish, I can change it. So that's not the practice. Always the cash will be the higher pricing , then there will be insurance and then the government business. This is the practice.

Moderator: Our next question comes from the line of Surya Narayan Nayak from Sunidhi Securities.

Surya Narayan Nayak: So a couple of questions. One is that sir if you can throw some light on or give the numbers regarding the newer hospitals, I mean, census beds and the ramp -up strategy going forward for FY '27? And regarding the Agra unit also same, if you can give the AR POB currently we are enjoying and what is the status of census beds there? And what is the plan in FY '27?

Nitin Gupta : So basically, as of now, we have a capacity of around 2,550 beds, including the Agra of 250 beds. In totality, we have a plan to go around 5,000 to 6,000 beds, as we just told in the earlier question as well. So in the ramp -up stage, we may come with around additional 3,000 beds in the coming 3 to 4 years down the line. Now on the occupancy front, we are at 67% on a blended numbers and the ARPOB is ~33K.

Surya Narayan Nayak: So sir, my limited question was related to the newer hospitals, newer facilities, newer facilities in the NCR region and Agra? Yes.

Nitin Gupta : Model Town, which is in Delhi, we have an occupancy of 38% on the 100 census beds with a capacity of around 300 beds which we will be ramping up as the demands come in the coming quarters. We have another new hospital in the Sector -20 Farid

abad, in which we have an occupancy of 43% in this quarter with 125 beds on the operational levels, which has a capacity of aroun d 400 beds.

Agra, we have just started in the month of February 2026 . Just w e have started it out, having current operational beds of around 150, which we will be going to expand to around 250 beds in the coming quarters.

Surya Narayan Nayak: So newer hospitals, can we expect 50% occupancy next year?

Amit Kumar Singh : Yes, definitely. See, Faridabad, if you see just started 3 months back and 2.5, 3 months back, it is presently with the 125 beds as operational 43% occupancy. And without having any panel, in fact, not full -fledged even insurance and TPAs, right .

So definitely, that is very promising these numbers as of now. Even the same goes for Delhi as well. Still 100% insurances are yet to come. I mean the all government channels or even corporate and PSUs, which are dependent on the various states, still nego tiation going on.

So I think by end of quarter 4, or probably the beginning of quarter 4, sorry, the next 2 months, I think we should be full -fledged with all empanelment, then we expect more than 50% occupancy in the coming quarters.

Coming to the Agra, which we started just 1st of February, current ARPOB, because that's running hospital, it's presently at around INR26,000 kind of ARPOB, by having us over there with the -- having -- starting with the super -specialties and others, as we believe that we can easily take it up by next quarter, I think anything between INR30,000, INR32,000 kind of ARPOB.

Surya Narayan Nayak: Okay. So -- and regarding the Agra unit compared to the other facilities with -- our understanding is that the acquisition cost is little bit higher side. So how do you plan to make it at par with the other NCR region facilities? What is the overall case mix and all?

Yatharth Tyagi: Yes. So see, Agra Hospital, it is one of the -- and I would say, the best infrastructure hospital within not just Agra, but even up till a radius of 100 kilometer in that whole region. So it's a fully super specialty hospital. It has all the machines, incl uding MRI, Cath Lab. And there's also space available for further expans ion, it's a full NABH hospital.

So there were multiple ways why we felt the value is right. It is already a running hospital and the hospital, in last, if I talk about 12 months, has had a revenue of close to INR45 crores to INR50 crores and is almost now EBITDA positive and P& L positive also.

So from February, when it is on our books, we do not need to incur any more losses to make it profitable. So we also add that sort of to justify that value because otherwise, when we start a new hospital, it takes 15 months and more cash losses to get that into profitability. But it is already from Fe bruary when it is on our books.

Not just it is contributing meaningfully to the revenue, but also contributing to EBITDA as well as profitability. So we are quite confident. We have huge plans for Agra. We already are in the process of a ppointing leading star doctors.

In fact, just this week itself, Agra hospital has been placed with Da Vinci Surgical Robot, which is not just Agra, but in that regions, one of the first robotic surgery is going to happen. So we feel Agra will contribute meaningfully to our coming quarter s.

Surya Narayan Nayak: And in terms of payer mix, I just want to understand, even in case of TPA, whether -- how much -- is it possible to give a breakup of group policy versus retail policy because retail policy happens to be a little profitable than the group policy? So is it possible to give the bifurcation of that?

Amit Kumar Singh : I think largely -- yes, it's -- grou

p is mainly about the corporate insurances and all you're talking
about, I believe. So the retail business, yes, it's always higher in terms of the number, but I think it's probably going to the 60%, 40% kind of because that also depends o n which area you are operating.

Let's say, if you are operating in Delhi NCR, where there are more corporates are there, right, so that percentage will be different. If you are operating in the Agra or probably the Meerut or some other region where the corporate spaces are not that much, that numbers will be lesser. So that won't be any standard answer for it. That depends on unit to unit and the areas where you are operating.

Surya Narayan Nayak: No, because structurally, due to the GST benefits, the retail policies should rise. I mean, the quantum should rise. That is my understanding. And structurally, it is positive. And thirdly, oncology, I saw that the -- our quantum of percentage is remaining same. So is there any facility constraint in our offerings? Or is there any scope of increase going ahead?

Yatharth Tyagi: So first of all, it is not same. Our pie has increased, right . A percentage might look similar, but there is an absolute increase in the volume and the revenue coming from oncology this quarter and year -on-year. In fact, it's a significant increase. Also, it has to be noted that in the upcoming Faridabad and New Delh i hospitals, we will also be offering full scale of oncology service, which still have not started in this quarter.

So going forward in these 2 hospitals, a new hospital -- because oncology takes 5, 6 months to start, that is machines and complete bone marrow transplants and other high -end cancer surgeries. So in the coming quarters in the new hospitals, oncology will a lso begin.

And that's where you'll also see an increase in the percentage, but also in an absolute number.

Oncology, if I talk about, has significantly increased. In fact, if we compare year -on-year, oncology has increased from INR63 crores to almost INR85 crores, which is a significant jump.

Surya Narayan Nayak: So any guidance going ahead for oncology alone? I mean for couple of years?

Yatharth Tyagi: Oncology today is contributing close to 10% of our overall specialty pie, which is -- if you compare 2, 3 years ago, it was less than 4%. So we are quite confident that going forward with the new -- 2 new hospitals now being fully fledged oncology services providers, we expect oncology to touch around 15% in less than 2 years' time. Maybe in 1.5 years' time, we should be there.

Surya Narayan Nayak: And any influx in the medical tourism this quarter?

Amit Kumar Singh : Yes, absolutely. So the numbers has definitely increased. Percentage wise, it still is in a single -digit, I would say, but it's quite encouraging the kind of the initiatives which international team has taken. Various OPD centers and the channel partners w ill be started. So we believe that's -- in fact, Jewar Airport is going to start probably by within 2 months or something, tha t's a new deadline had come up.

So we are quite confident on these numbers. Absolute number definitely increased. And rather just only the numbers, you see the geography which we touched upon in last entire -- I mean, whatever 8, 9 months, 10 months which we have worked on international, so we have touched various geography, and it's quite encouraging that patient has come from the all those geographies. So I think it's a very good base has been created. Now we just have to capitalize it.

Surya Narayan Nayak: And case mix, oncology -- apart from oncology, more or less the case mix will be remaining same? Or will there be any change in neuroscience or something?

Yatharth Tyagi: So as a totality, we are focusing on increase in our super specialty services versus specialties. So in the case mix also, you will see all the super specialties. So cardiology, neurosurgery, gastroenterology and a couple of others combined together would be higher than, let's say, specialties like gynecology, internal medicine, basic orthopedic works. So that's where the shift you will see in the coming years, which we are already reflecting quarter -on-quarter.

Surya Narayan Nayak: Any guidance on the ALOS going forward?

Yatharth Tyagi: I think ALOS should be as per industry standard.

Surya Narayan Nayak: It should be at 3 going forward?

Amit Kumar Singh : See, it should be around -- because that's the -- There's a couple of new hospitals coming up, there are certain business call we need to take. So I think it will be anything around close to 4, 4.3, that's quite acceptable, within 4, 4.5, I think that's ac ceptable number, which we see that as you grow, probably the 2 years down the line, if you say, definitely there's going to be a lesser.

Moderator: Our next question is from the line of Ishika from Perpetuity Ventures.
Ishika: I hope I'm audible. On the payer mix, your new hospitals seem to be driven largely by cash and TPA insurance with minimal government scheme contribution. Is this mix structural and expected to continue? Or do you anticipate a meaningful shift towards gover nment empanelment over the next 12 to 24 months?
Yatharth Tyagi: See, this is very much strategically planned and expected. We, as a group, have always talked about in the past to reduce government mix, and that is what we are doing. I wouldn't say that it will remain 0 after 1 year. Definitely, it will increase, but I don't see in the new er hospitals government mix being more than somewhere 1 5% to 18% or maybe max for 20%.
So ultimately, this is going forward what you will see. And ultimately, this is what will drive our debtor days and receivables to come down. So that is a strategic call that we have done, and we are able to showcase it with a strong attraction of cash and private insurance patients.
Ishika: Okay. Understood. That was helpful. And could you just please repeat your hospital wise occupancy and ARPOB for Q3 FY '26 for all your operating units?
Nitin Gupta : Yes, So the total occupancy is 67% on the blended basis. The Greater Noida Hospital, we have an occupancy of 74% ; Noida, we have 91% ; Noida Extension, we have 61% ; 72% in Jhansi - Orchha and we have 60% in Greater Faridabad . In new hospita ls, we have 38% at Model Town and we have 43 % at new hospital Faridabad . In totality, we have ARPOB ~INR33K, out of which we have ~INR39K at Greater Noida ; ~INR32K we have at Noida ; At Noida Extension, we have ~INR44K; Jhansi -Orchha, we have ~INR14K; At Greater Faridabad, we have ARPOB ~INR35K; Model Town, we have ~INR 40K and Faridabad our new hospital , we have ~INR 36K.

Ishika: Okay. That was very helpful. Can I just squeeze in one last question? You had brownfield beds coming in for Greater Noida and Noida Extension. What is the current status?
Yatharth Tyagi: 1.5 years from now, I think we should be close to commissioning them.
Moderator: Our next question is from the line of Shreya Chatterjee from Ageless Capital .
Shreya Chatterjee: My question was more about if you could give an EBITDA level breakdown for all the hospitals? And where do you see your EBITDA ramp -up in the next 2 to 3 years? Would it be around 29%, 30%? Or

would it continue in the range of 24%, 25%?
Yatharth Tyagi: See, I think unit -wise EBITDA is something right now it would not be a right estimate because a lot of new hospitals and old hospitals are starting. So we are always calculating as a consolidated level. And also, we have certain subsidi aries and certain hospitals are within the parent company.
So in totality, 2, 3 hospitals are within the Yatharth holding company and then there are 2, 3 subsidiaries comprising them. So I can talk about the group level EBITDA. So even if you look at it, with the 2 new hospitals starting, even then we are almost a bit, in fact, higher than the last quarter 2 EBITDA margins.
And without these 2 new hospitals, we would have been somewhere close to 28%, 29% of EBITDA. So I think the company is very well on track for our targeted EBITDA. We feel in the coming quarters, EBITDA margins at consolidated level will definitely increase because the major reason is that in the first 2 quarters when a hospital starts, like a New Delhi hospital and Faridabad -20, it is expected to be EBITDA drag. But by the third or the fourth quarter of their operations, that is reduced.
Also very positive is the Agra hospital that we have started is not a EBITDA drag at all. So that is also another reason whatever contribution it will have, will add to the EBITDA. So we are quite confident in the coming quarters , EBITDA margins will increase.
And no, we are not targeting 28%, 29% EBITDA even in, let's say, in 2, 3 years because there will constantly be new additions, new hospitals. As we earlier mentioned, we want to be somewhere around 5,000 bed capacity within the next 3, 3.5 years. So that w ill mean lot of EBITDA drag from the upcoming hospitals.
But at a consolidated level, we feel somewhere around 24%, 25% EBITDA margin would be a right estimation. Yes, if you remove all the new hospitals that we'll be starting, the EBITDA margins would be somewhere around 3%, 3.5% more than the consolidated aver age.
Shreya Chatterjee: Got it, sir. And also with your current payer mix, what is the receivable days as of Q3 '26, and where do you project it to go maybe in FY '27 to '28? Also on ARPOB, like right now, you have really ramped up well. But maybe in 2 years' time at a consol level, where do you see your ARPOB going up with the oncology picking up and everything else?

Sonu Goyal : So in September 2025, we have reported around 116 receivable days. In December also, we are very, very much close to the same receivable days, and we are very much hopeful to close March 2027 with receivable days less than 110.
Shreya Chatterjee: And what about going forward, like in the next 2 years when your government portion will -- like CGHS portion will fall down to around 25%, 28%, what receivable days can we expect to observe in that time?
Sonu Goyal : So after 2 years down the line, as we already mentioned, we will reduce our government mix, we will increase our cash and TPA business. And as we already mentioned, our both the hospitals, we have started with the cash and TPA business only. After 2 years of 3 years down the line, we will very much close to 80 or 82 days.
Shreya Chatterjee: Got it, sir. And what about the ARPOB guidance? It was just an extension. it was just an extension of the question. What about the ARPOB guidance in maybe FY '27 to '28?
Amit Kumar Singh : See, if you look at it, that's even the last numbers which you see that year -on-year, we are growing more than 10% of our ARPOB. So I think that's -- you can easily guess over there. And even the current numbers, which you see the kind of Noida Extension, Greater Noida hospital has grown. So 10% ARPOB year -on-year, I think th

at's quite easily achievable for us. That's what our guidance.
Moderator: Our next question is from the line of Nirali Shah from Ashika Stock Services.
Nirali Shah: Congratulations on the great set of numbers. I had a couple of questions. We had expected some margin pressure given the contribution from newly opened hospitals. But like you said EBITDA margins have broadly remained stable Q -o-Q also. So the question is that what helped offset this drag in this particular quarter? Were there any specific factors that we should not be assuming that will repeat in Q4?
Yatharth Tyagi: Sorry, there's a lot of background noise to your line. So there were a lot of good EBITDA traction we gained from our existing hospitals. So other than the new hospitals, our existing hospitals are ramping up good oncology business. They're ramping up good , even ARPOB, right .
So ARPOB of if you see Noida Extension has increased upwards of 15% year -on-year, similarly Greater Noida. So even our existing hospitals, older hospitals are growing at a good rate. So that has led to this margin being sustainable.
Sonu Goyal : Further if you see our Model Town hospital also, we started in Q2 and the losses has been reduced from Q2 to Q3. So in Q3, there is a 50% reduction in losses in Model Town Hospital itself.
Yatharth Tyagi: Yes. And also going forward, with the new CGHS rate revision, which has been the -- we saw in December and now we will be seeing from 1st January in the whole quarter 4, that will also help going forward to contribute to our EBITDA margins significantly.

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Nirali Shah: So is it fair to assume that the older hospitals -- the margins from older hospitals were much higher than the drag that could have come from the newly opened hospitals?
Yatharth Tyagi: Yes, that's true. That's true. And also like as we mentioned that quarter 2, quarter 3, the new hospitals drag was also less because of the increase in the business.
Nirali Shah: But then do we expect that in Q4 or Q1 maybe?
Yatharth Tyagi: Yes, definitely, definitely. As I mentioned, the EBITDA margins should be better from here on in the coming quarters for sure.
Nirali Shah: Understood. And could you also help us understand how much of the increase in employee expenses is attributable to the labor code change versus the normal hiring and the ramp -up at the new hospitals?
Nitin Gupta : See, based on the recent changes, which is done by the Labo ur Code, we don't have any significant impact on us. We are mostly aligned to our current stature as per the proposed codes. However, minor impact may come as a change of the basic pay, which has been factored to the gratuity.
To elaborate, we don't have any major impact on the other fronts like bonus or the leave encashment . We perceive that we have a complete assessment in place, by March -end, in wh ich we don't expect any major impact in this financial year due to this New Labo ur Codes.
Nirali Shah: Okay. And in the -- so if it is not coming in this financial year, it won't be coming in the next financial year too?
Nitin Gupta : We are already in line to our current stature. Our stature is already in line. So there is no gaps to

have any major financial impact in this financial year. Obviously, in the next financial year, it
will be aligned as per proposed laws and we d on't foresee any major financial impact in coming
years in this place in the financials.
Nirali Shah: Fantastic, fantastic. And just the last one, I wanted to know on the MVT business. So because
there was a mention about the benefits and the union budget mentioned about the promoting
medical tourism. So do you think any incremental benefit would be coming to us?
Yatharth Tyagi: Yes

. So we would see around 5% to 6% increase in the cost that we would be benefiting from
that. That's our estimation on that specific business.
Moderator: Our next question is from the line of Abhijeet from PI Asset.
Abhijeet : Sir, we can notice a sharp increase in the other expense. Was it because of one time marketing
or empanelment of star doctors? And are these costs expected to taper down in FY '27? Or should
we assume the same run rate?
Sonu Goyal : If you see there is a quarter -on-quarter increase at a consol level majorly because of the new
hospital additions. And in both the new hospitals, we have added the doctors . That's the only
reason. If you see now, there's no major change.

Abhijeet : We should assume this as your new normal, right?
Sonu Goyal : Yes. This is now normal, and there will not be any major increase. No w.
Abhijeet : Understood. And sir, what is the capex for FY '27? I missed that figure.
Nitin Gupta : Please pardon?
Abhijeet : What is the capex for FY '27?
Yatharth Tyagi: So we have discussed our overall capex plan for next 5 years. For it, d eals will be announced
within, let's say, in next 3, 3.5 years, but the fund deployment of the capex will take 5 years'
time, and that is around INR1,500 crores for getting us a bed capacity of close to 5,000 beds.
Moderator: Our next question comes from the line of Akshat Mehta from Seven Rivers Holding.
Akshat Mehta: Yes. So the first question that I have, maybe for Yatharth himself is that if you assume that the
overall trade receivable days in the business is 110, then that would -- and assuming that 35%
of the business is from your government and 65% is split evenly between cash and TPA, that
would imply that your government business receivable days are north of 245, 250 days. Is that
the right way to look at it?
Yatharth Tyagi: So I mean, there are certain payers within, let's say, TPAs also, which does drag because majority
of the TPA business is received -- the payment is received within time, but then there are certain
exceptions and with the certain things. So that also contr ibutes a bit. But yes, I would still say
what you've mentioned, it would be close to that, but not exactly to that amount. So it varies day
by day -- even government also...
Akshat Mehta: But there are other listed peers who have indicated that their receivable on the government
business is sub -130, 140 days. So why is it that we are at closer to 250? And what are we doing
to bring that down specifically?
Yatharth Tyagi: Yes. So I don't think it's closer to 250. I think it's closer to 200, first. And second is, as we've
also stated in the past, there was a certain reason that this led to, especially if you look at 1 year
ago, when we got increased with the infrastructure, there were a lot of government business that
we onboarded, right .
So our systems and processes took time to upgrade ourselves, right . It's not that it's all 100% on
the government only the receivables. There are certain framework and systems within the
hospital, which allows faster recovery. So we were not having it initially.
If I talk about in last 1 year, the reason why we have generated close to 60% to 70% OCF to
EBITDA is a way where we've optimized the process. In fact, recently, there have been certain
teams that have been outsourced for recovery to get faster recovery, w hich are already working
for certain other hos pitals that you have mentioned.
So we have learned from their example and deployed the same network of facilities within our
hospital. So that also has led to this reduction. And other than just the pure decrease in the

government share, which will ultimately bring this down, a better recovery and better follow -

ups from ours is also helping us to achieve this.
Akshat Mehta: Got it. My second question is around the trade receivables...
Yatharth Tyagi: Sorry, just one last thing I would also add to you. A lot of -- when you mentioned other hospital chains, some of them also don't do the exact same pair of government mix that we do, right . There's a government payer called ESI that many other hospital chains, they don't do. ESI has been shown typically in the pas t to have a slow payment cycle.
And I would say around 30% to 35% of our outstanding with the government is specifically from ESI channels. So that has also led to an increase, which is not standard across other hospitals chains.
Akshat Mehta: Okay. That clarifies it. My second question is around your trade receivables again. In your annual report, there was a reduction in overall trade receivables to the tune of INR20 crores. And in FY '24, the number was around INR35 crores. So implying almost a 6% and 11% reduction, respectively. What was the reason for that reduction? And how should we look at it moving forward?
Sonu Goyal : This is on the basis of the collections, as we already mentioned. So from last more than 1 year, we are working on the rigorous collections. We have implemented all the protocols to have better billing processing, better dispatch processes and all the reco very processes.
Akshat Mehta: No, I'm talking -- I'm talking about a write -off in trade receivables.
Nitin Gupta : Write off. See, as a prudent policy, we generally have the provision for the doubtful debts. But certainly, very miniscule of the bad debts generally comes -- we don't have any bad debts as of now. We generally make a provision as a prudence based on the k ind of principle of the accounting. But based on the trends, we see that the -- our deductions is coming down. And generally, our provision is being reversing over the period. So...
Yatharth Tyagi: So basically, what you're mentioning to deduction is basically not a write -off of any debt, but basically, it is certain bills which are contested by the payer mix and then that's what the deduction comes to. Just like, let's say, any private insurance com pany, there are certain deductions happening billed on bill basis. So that was that case.
Akshat Mehta: And how should we look at it moving forward? Should we assume a similar percentage in the coming years, quarters as well?
Sonu Goyal : Our deduction overall at a group level is reducing from last 2 years. You can see from our reported results also. And the same trend we will follow, but it is not 1% or 2%. Yes, it will decrease. And we are taking the provisions for the non -collected amount als o.
Akshat Mehta: Congratulations on a good set of numbers.
Moderator: Our next question is from the line of Shubham Harne from Purnartha Investment Advisers .

Shubham Harne: What risk do you foresee due to entry of new chain hospitals in Noida region?
Amit Kumar Singh : See I think rather than risk, we feel that it's good for our business because when any territory, when more and more hospital comes, that territory gets developed. So you attract, your catchment gets increase, people come. So I think everyone will have a p ie. I'll give you example. I mean, in our territory, 2, 3 new hospital has come with th e new brands and the new entry. But still, I think we have grown more than 30%. And with even the existing hospital, I see that growth was more than 25%. So I mean, it's all how you

look at it, right . We personally believe that -- yes, you need to be aware with your competitor, what they do it.
But if you are working with your core strength and more and more players coming in, I mean, we focus -- I mean, beyond limelight, people will always -- I will come to you, that's fine. So we don't see, I mean, reduction in a pie to be very honest.
Shubham Harne: And do you foresee any poaching of doctors from your institute to other institute?
Amit Kumar Singh : Yes, it's a part and parcel. Believe you me, any hospitals entering into any territory, they try to look at it which one is doing good, who are the good doctors. We also do it, right . When we enter in Delhi, we did the same thing, entered in Faridabad, we did the same thing. So it's a part and parcel. And we as a hospital, we are prepared for this, right . So one go, other can come. So this is how it is.
Moderator: Our next question is from the line of Akhilesh Rawat from Redhanta Vision Private Limited.
Akhilesh Rawat: So first of all, I would like to congratulate management on good sets of numbers. So I have just one quick question and all my questions are answered. So like I just want to understand what is our current receivable days as of now, group -- all group hospi tals?
Sonu Goyal : So current receivable days as a group, as a consol level is coming around 115 days. And as I already mentioned in the earlier question, this will be reduced and come down between 105 to 110 in the -- by March 2027.
Moderator: Our next question is from the line of Anand B from Ksema Wealth Private Limited.

Anand B .: Congratulations on good set of numbers. I just have 2 questions. One is the -- the IT issue has been completely resolved right now?
Yatharth Tyagi: Yes. So all the attachments and the freezing of any asset or everything has been removed. And we are in the final stages. We are expecting very soon that matter to be resolved. So yes, it's going as per plan.
Anand B .: But when do you think the matter will be completely resolved, by -- can you give a time line as quarterly or something?
Yatharth Tyagi: See, I mean, these are industry standard. I think sometime in the coming quarters, definitely, it will, even though the matter which was on the emphasis has already been resolved in the last quarter. So for us, it's more of a formality now which is remaini ng.

Anand B .: Okay. My final question is, can you just repeat the ARPOB numbers for each of the hospitals again? And what are your expectations of the ARPOB, let's say, in FY '27, FY '28 for each of the hospitals?
Nitin Gupta : So yes, on the blended basis, our ARPOB is INR33K, out of which at Greater Noida having an ARPOB of nearly around INR39K. Noida Hospital, we have ARPOB of nearly INR32K. Noida Extension, we have ARPOB of nearly around INR44K and at Jhansi -Orchha, we have ARPOB of INR14 K and at Greater Faridabad, we have a hospital ARPOB of nearly INR34K. At our new hospitals, at Model Town, we have ARP OB of INR40K and new Faridabad hospital we have ARPOB INR36K -- so you can see that the trends we are on rise or growth of ~10% every year on a year basis. And the same we can expect that we have a growth of around another 10% in the coming period.
Anand B .: Got it. You mentioned for Agra, ARPOB expectation is around INR30,000 to INR32,000. But for the new hospitals like Delhi and Faridabad, do you have like similar expectations also for FY '27, FY '28?
Nitin Gu pta: Yes, The territory we are following in Delhi and the Faridabad is different from the Agra. The Agra we are believing that it will be ARPOB having touch around INR30,000 the same way, and it's a good ARPOB in terms of the territory of the Tier 2 city.
Moder

ator: Our next question is from the line of Umakant from Viansh Ventures Private Limited.
Umakant: Congratulations on a good set of numbers. Most of my questions have been answered. I've just got just one quick question. Could you just touch down on a little bit about the margin profile at where do we stand currently on your Greater Faridabad hosp ital as well as the Model Town? I just -- the reason I asked this question is because I just want to get a clarity in terms of how much juice do we further have to scale up the mature hospital margin profile from 29% can it go to, let's say, a further X percentage? So if you could just throw some color around that. And even on the Jhansi, how much -- what is the margin profile currently?
Yatharth Tyagi: So I think as we mentioned that if we remove the newer hospitals of Model Town and Sector -20, right, Sector -20 and Greater Faridabad, they are 2 different hospitals. So the Sector -20 and Model Town hospitals removing them, the group margins comes to 29%. But in this, the major contributor are from our the Greater Noida, the Sector -110 Noida and Noida Extension hospitals because these are earlier started hospitals. If I talk about some hospitals like Jhansi or even the Greater Faridabad because they were started almost 2 years and 1.5 years back time, there the margin would be still reaching close to 20% upwards. But it is basically the 29% margin figure that you are getting at a consolidated level without the new hospitals is primarily because of the Noida hospitals.
However, going forward, we feel even without the newer hospitals, the margins, we do not expect them to rapidly rise above these levels because there are certain -- even though occupancy ramp -up will happen, but there is high -quality growth that is happening, right . We're also growing ARPOB wise. We're also changing our payer mix. We are reducing government pay, not just in the new hospitals, but even our earlier hospitals. So for that, there are certain cost

expenditures that we have to do. So the margins for other new hospitals would roughly remain in the same line and will not increase significantly.
Umakant: Okay. Got it. Just a follow -up on this one -- on the first question. When we say that I think we have incurred a loss of about INR10 crores in our newer hospital, that is Sector -20 and Model

Town, ab out we've run losses of about 20 at operational level. Do we -- now that we -- at one point we will be ramping it up? So do we expect these losses to widen before we start seeing the margins -- this thing turning positive? And secondly, what is the occupancy that we, let's say, see -- when do we see a healthy level of occupancy in these 2 hospitals?

Amit Kumar Singh : No. So the losses, as we mentioned earlier, so all the cost centers, which you see, so all the expenditure has been done, mainly for new hospitals, the doctor cost and the employee costs already placed. Now the costs related to the business. So coming quar ters, these losses should come. And what we -- I can tell you things that's Faridabad probably we will do a breakeven probably much earlier. In fact, probably well within the 12 months, which will be really a very good set of -- I think that's what expectation is coming and the kind of b usiness trend coming in the last 3 months. Similarly, Model Town, which we see, I think within the 15 months, which we should have operational breakeven. So we don't see any losses coming up. It's whatever losses would be, that's related to the directly proportional to the whatever business we acqu ire. What was the second question of yours? I missed that second one.

Umakant: Regarding the occupancy in these 2, when do we see a healthy

level of occupancy?

Amit Kumar Singh : So when we do a full -fledged, as of now, I think Faridabad is operating with around 125 operational bed and Model Town is around 120 beds. But when we have a full -fledged as a Model Town 300 bed and Faridabad is a 400 bed, I think anything between 30%, 35% of the full operational b ed, I think we'll be a t breakeven. And next -- probably, as I said it, within the 12 months or 15 months, I think we will be very much achieve those targets.

Umakant: Got it. Perfect. And then my second question was, if you could just provide some -- any further updates on your future expansion plans? Would it be led by any greenfield expansion or brownfield and if it's a brownfield, what kind of a size are we looking a t? And what will be the -- what's -- let's say, what capex per bed we are comfortable with? And also just some color on the geography?

Ashutosh Kumar Jha : So as we have mentioned, we are targeting to reach to close to 5,000 beds in the next 3 years in terms of announced deals. And of course, the deployment can be over the next 4 to 5 years because there will be a mix of greenfield, brownfield and asset -light expansion. And the combined capex that we are planning for this is around INR1,500 crores. So this would be coming to around INR60 lakhs per bed because there is a mix of greenfield and asset -light. And this will be in the geography of first priority as NCR and second priority as major cities in North India.

Umakant Sharma: So just one clarification. When you say we are targeting a INR1,500 crore s capex, I'm assuming we are talking only about the 2,000 incremental beds because we've already planned for 500 bed brownfield, right? At Noida and Noida Extension, we've already planned it. So 2,000 for

INR1,500 crores. So we are saying that we're going to be just at INR60 lakhs, that's quite a less number, right? Like I mean if you get it that's great, fantastic …

Ashutosh Kumar Jha : That’s because it will be asset -light models also. So there are asset light models where we are not spending on land and building. We will be only bringing in the equipment. That kind of models also we are exploring.

Moderator: Our next question is from the line of Subhanu Bangal from 3 Head Capital.

Subhanu Bangal: Sir, as you mentioned, Q4 will be better than Q3 and EBITDA margin will be -- going forward will be improved. What kind of EBITDA margin we are targeting in FY '27? This is my first question?

Ashutosh Kumar Jha : So as we have indicated that since we will -- we are in a growth phase, we will keep on adding more hospitals, and we will ramp up the latest additions of hospitals. So our guidance for the blended EBITDA margin is in the range of 24% to 25% on a consolidated level.

Moderator: As there are no further questions, I would now like to hand the conference over to management for closing comments.

Yatharth Tyagi: Thank you, everyone. Thank you for tuning to Yatharth Hospitals & Trauma Care Services Limited's Earnings Conference Call for the quarter ended December 31, 2025. We hope we are able to answer all your queries, and thank you for your questions.

Moderator: Thank you. On behalf of Nuvama Wealth Management Limited, that concludes this conference. Thank you for joining us, and you may now disconnect .

(This document has been edited for readability purpose)

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Call: Jun 2026

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June 03, 2026

The Listing Department
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Scrip Code: 543950
ISIN: INE0JO301016
Subject: Earnings Call Transcripts Q4 FY26

Dear Sir/Madam,

Pursuant to Regulation 46(2)(oa) of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed the transcript of Investors' conference call organized on May 26, 2026 post declaration of Audited Financial Results (Standalone & Consolidated) of the Company for the quarter and financial year ended March 31, 2026.

The transcript will also be available on the website of the Company at https://www.yatharthhospitals.com/investors/corporate-announcements

This is for your kind information and records.

Thanking You

Yours Faithfully,
For Yatharth Hospital & Trauma Care Services Limited

Ritesh Mishra
Company Secretary & Compliance Officer
M. No. A51166

Encl.: A/a

"Yatharth Hospital & Trauma Care Services Limited
Q4 FY '26 Earnings Conference Call"

May 26, 2026

MANAGEMENT : MR. YATHARTH TYAGI - WHOLE-TIME DIRECTOR
MR. AMIT KUMAR SINGH - GROUP CHIEF EXECUTIVE
OFFICER
MR. NITIN GUPTA - PRESIDENT , FINANCE AND GROUP

CHIEF OPERATING OFFICER
MR. PANKAJ PRABHAKAR - GROUP CHIEF FINANCIAL
OFFICER
MR. ASHUTOSH KUMAR JHA – GROUP CHIEF -
STRATEGY, M&A AND INVESTOR RELATIONS
MR. SONU GOYAL – GROUP CHIEF FINANCE
CONTROLLER
MODERATOR : MR. SUMIT GUPTA - SENIOR VP, INSTITUTIONAL
EQUITY RESEARCH HEALTHCARE , ANTIQUE STOCK
BROKING LIMITED

Moderator: Ladies and gentlemen, good day and welcome to Yatharth Hospital & Trauma Care Services Limited Q4 FY '26 Earnings Conference Call hosted by Antique Stock Broking Limited.

Let me draw your attention to the fact that on this call discussions will include certain forward-looking statements which are predictions, projections or other estimates about the future events. These estimates reflect management's current expectations about the future performance of the company. Please note that these estimates involve several risks and uncertainties that could cause company's actual results to differ materially from what is expressed or implied.

As a reminder, all participant lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing "*" then "0" on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Sumit Gupta – Senior VP, Institutional Equity Research Healthcare from Antique Stock Broking Limited. Thank you and over to you Mr. Gupta.

Sumit Gupta: Thank you. Hi, good day, everyone. On behalf of Antique Stock Broking, we welcome you all to the Q4 and FY '26 Earnings Conference Call of Yatharth Hospital & Trauma Care Services Limited.

From the management side, we have with us today Mr. Yatharth Tyagi – Whole-Time Director, Mr. Amit Kumar Singh – Group Chief Executive Officer, Mr. Nitin Gupta – President, Finance and Group Chief Operating Officer, Mr. Pankaj Prabhakar – Group Chief Financial Officer, Mr. Ashutosh Kumar Jha – Group Chief Strategy, M&A and Investor Relations, and Mr. Sonu Goyal – Group Chief Financial Controller.

I now hand over the call to Mr. Yatharth Tyagi for his opening remarks. Thank you and over to you, Yatharth.

Yatharth Tyagi: Good afternoon and welcome to Yatharth Hospital & Trauma Care Services Limited's Earnings Conference Call for the quarter and year ended March 31, 2026. Our Earnings Presentation has been uploaded on the Stock Exchanges and on our website and we hope you have had the opportunity to review it.

FY '26 has been an exceptional and a transformative year for Yatharth Hospitals, marked by strong operational execution, strategic expansion into high potential healthcare markets, accelerated ramp-up of new facilities, and continued strengthening of clinical excellence across our network.

During the year, the company reported consolidated revenue of approximately INR 12,072 million reflecting a robust growth of 36% YoY, while EBITDA increased by 30% YoY to INR 2,921 million.

On the clinical front, we continue to strengthen our position as a leading quaternary care provider through ongoing investments in advanced medical technologies, expansion of high-end specialities, and onboarding of reputed clinicians.

During the quarter, our hospitals delivered several noteworthy clinical milestones, reinforcing the depth of our tertiary and quaternary care capabilities. Some of them are:

- At Greater Noida Hospital, the neurosurgical team managed complex pan-brachial plexus injury using a precision intercostal nerve-to-nerve musculocutaneous nerve transfer to restore motor function.
- In another case, reinforcing our neuroscience leadership in Noida Sector 110 hospital performed a navigation-guided awake craniotomy for an intracranial lesion in the speech

area, followed by continuous intraoperative speech monitoring enabling t

he patient to
recover without neurological speech deficits.

■ In Sector 20, Faridabad facility, which has also emerged as a strong center for advanced GI interventions, performing 13 POEM procedures within two months of its starting and further strengthening our minimally invasive gastro-sciences capabilities.

We are also pleased to witness growing recognition of our clinical leadership. Our Dr. Viresh Mahajan was honored as “Pediatric Cardiologist of the Year” at National Cardiovascular Summit 2025, while Dr. Dhansukh received the prestigious “Desh Ratan Award 2025” for his contribution to Pediatrics and Neonatology. Dr. Kunal Bahrani was recognized with the “Leadership in Neurological Services” Award at VOH National Healthcare Awards 2025. And also Dr. Supradip Ghosh received the “Chairman Award for the Lifetime Contribution to Critical Care Medicine” from ISCCM Delhi-Noida-Gurugram. These recognitions underscore the strength of our growing clinical talent and a continued commitment to delivering world-class patient care.

In line with our cluster-based expansion strategy and following the early success of our Faridabad cluster, we have seeded our next high potential cluster in the Gurugram market through the acquisition of an ultra-modern under-construction super-speciality hospital in Sector 40, Central Gurugram. The upfront consideration for this project is approximately INR 100 crores, along with an additional proposed investment of nearly INR 100 crores towards finishing and providing medical infrastructure. Once operational, the hospital is expected to deliver ARPOB in the excess of INR 50,000, positioning it among premium healthcare assets in NCR. Strategy located near IGI Airport and key residential and corporate catchments, the facility is well-placed to strengthen our premium healthcare positioning and expand medical value travel opportunities. We expect the hospital to become operational by April 2027.

During the year, our newly operational hospitals at New Delhi and Faridabad Sector 20 witnessed a strong ramp-up. These facilities scaled up ahead of internal expectations and have emerged as meaningful contributor to the growth within a short period of commencement. Together, these two new hospitals contributed approximately 11% to revenue in Q4 FY '26 and continue to deliver healthy operating metrics, including superior ARPOB levels and a favorable payer mix, largely comprising cash and TPA patients. We also successfully integrated 250-bedded Agra hospital, which strengthens our presence across the NCR-Agra healthcare corridor via the Yamuna Expressway. Since its integration effective February 2026, the hospital has demonstrated encouraging traction, achieving a monthly revenue run rate of approximately INR 7 crore, along with double-digit EBITDA margins, reflecting strong patient inflows and a healthy operating start. Establishing our clinical excellence, we installed Agra's first Da Vinci robot and have successfully completed 50 robotic surgeries till date in the cities nearby Agra. Robotic surgery is something which has been provided for the first time and Yatharth Group has taken initiative by completing such a large number of robotic surgeries in that region. With strong connectivity to our Noida and Faridabad facilities, we expect the Agra hospital to evolve into a key feeder hub for oncology and other high-value specialities over the medium term.

Our Greater Faridabad hospital, which commenced operations last year, delivered strong performance and achieved profitability during the year, demonstrating our ability to efficiently scale and turn around new assets.

During the year, we further strengthen our focus on international collaborations and global outreach. A

key milestone was our association to become the exclusive hospital partner for the now-operational Noida International Airport at Jewar, which is expected to significantly enhance international patient access and regional healthcare community and connectivity. As part of our medical value travel initiatives, we undertook multiple international outreach programs across the Middle East and the CIS regions, hosted international delegations, conducted OPD initiatives, and participated in strategic healthcare forum, strengthening long-term partnerships and global presence.

Looking ahead, we remain focused on scaling up our newly added facilities, driving operational efficiencies, enhancing speciality mix, and strengthening our presence across high-growth healthcare markets in North India. With the upcoming Gurugram facility and brownfield expansions planned at Noida Extension and Greater Noida, our total bed capacity has reached over 3,200 beds. We remain confident of achieving our target of 5,000 beds over the next three years. With newer hospitals demonstrating higher ARPOB levels, we expect a meaningful uplift in overall group realization as these facilities continue to scale up. FY '26 has been a transformative year for the group, and the early success of new facilities gives us confidence in sustaining and eventually potentially surpassing the YoY growth that we have shown for this year, for the year to come, with this growth trajectory in the coming fiscal.

Thank you, and now I will pass on to Mr. Pankaj Prabhakar, Group CFO, for his management commentary on the financials.

Pankaj Prabhakar: Good afternoon, everyone. I am pleased to share that Yatharth Hospitals has delivered a strong performance in Q4 FY '26, concluding the year on a robust note. During the quarter, we reported a revenue of INR 3,416 million, reflecting a growth of 47% year-over-year and 6% quarter-over-quarter. For FY '26, the company reported a revenue of INR 12,072 million, registering a 36% year-over-year growth.

This performance was driven by sustained momentum across our existing hospitals, along with increasing contribution from our newly operational facilities. Our established hospitals maintained a healthy growth trajectory during the quarter and throughout the year, supported by higher occupancies, strong patient footfalls, increasing surgical volume, and a rising contribution from super-speciality treatments. Occupancy across the network stood at 71% in Q4 FY'26, while FY '26 occupancy stood at 68%. Our Noida Hospital operated at 86%, Greater Noida at 76%, Noida Extension at 61%, Jhansi-Orchha at 86%, and Greater Faridabad at 69% during the quarter. Our ARPOB improved to INR 33,282 in Q4 FY '26, up 5% year-over-year, while FY '26 ARPOB stood at INR 33,124, up 7% year-over-year.

Notably, during Q4, our Noida Extension Hospital reported its highest ever ARPOB of INR 47.8k, up 23% year-over-year, Greater Noida Hospital at INR 40.3k, up 15% year-over-year, while our newer hospitals reported encouraging initial ARPOBs with New Delhi at approximately INR 40k, Faridabad Sector 20 at 38k, and Greater Faridabad at 31k, reflecting sustained improvement in mix.

On the profitability front, we achieved our highest ever quarterly EBITDA of INR 799 million, reflecting a growth of 37% year-over-year, with EBITDA margin at 23.4%. For FY '26, EBITDA stood at INR 2,921 million, with margins at 24.2%. Adjusted for initial ramp-up losses at our new hospitals, adjusted EBITDA margin remains strong at 30.4% in Q4, and 28.5% for FY '26, reflecting operating leverage and improved mix. Profit after tax for Q4 FY '26 stood at INR 447 million, up 15% year-over-year, while FY '26 PAT stood at INR 1,703 million, reflecting a growth of 30% year-over-year. The improvement in profitability was driven by strong revenue growth, operating efficiencies, better realizations, and a scale-up of newer hospitals.

Our balance sheet remains robust, supported by a strong cash generation and disciplined capital deployment. During the year, pre-tax operating cash flows stood at INR 2,866 million, with a cash conversion ratio of 98%, reflecting significant improvement in working capital efficiency. We ended the year with a healthy total cash position of INR 3,931 million, a net cash position of INR 1,160 million, providing us with ample financial flexibility to pursue growth opportunities, while continuing investment infrastructure, technology, and clinical excellence.

With a strong execution engine, improving our occupancy trajectory, ramp up of new hospitals, including Agra, and continuous focus on operational efficiencies and speciality expansion, we remain confident of sustaining healthy growth momentum in the coming quarters.

Thank you for your attention. I would like to hand over the call to the moderator for question-and-answer session. Thank you.

Moderator: Thank you very much, sir. Ladies and gentlemen, we will now begin with the question-and-answer session. We will take the first question from the line of Ashutosh Adsare from Universal Sompo. Please go ahead.

Ashutosh Adsare: Good morning all and thanks for the opportunity. I just wanted to understand about the new hospitals, Agra, Model Town, Faridabad and Jhansi and Gurugram also. Could you just tell in the operational parameters, how those things are performing for these newer hospitals? And one is that Jhansi, what is the specific in Jhansi? What is the condition? How much occupancy is currently running at? You said 86%. Is it that the ARPOB has increased in that because we have been seeing that the ARPOB for this particular Jhansi was not up to the mark or not up to the

company level? One is that. And second question on the interest cost. We have seen a significant rise on a sequential basis on the interest. We just wanted to understand how is your debt position and what would be the debt position in the coming year?
Nitin Gupta: Yes. So, basically on the occupancy front, , our new hospital, primarily opened in Delhi, Faridabad Sector 20 and Agra. we have enough headroom in the occupancy level. We have a current occupancy level of 32% in the Model Town, that is Delhi, with the census capacity of 100 beds., with capacity of 300 beds in Delhi. Similarly, if we say about the Faridabad, the newer hospital, we have a current occupancy of 52% on 100 beds with the capacity of 400 beds . In Agra, we have a current occupancy level of 52% with the census bed of 110 with the overall capacity of 250 beds. So, the overall bed expansion and the capacity, we have enough headroom to grow with the occupancy level. And on Jhansi, the Jhansi is currently operating around 85% of the occupancy on the 250 bed census level with the bed capacity of around 305. So, we have enough headroom there as well also to have a increase complete network occupancy and expansions.
Yatharth Tyagi: As far as your question on the impact of Jhansi on the overall group is concerned, even though Jhansi ARPOB is lower than the group average, the impact at a group level is not significant because the whole contribution to Jhansi in terms of our revenue pie chart is very less. So, if you look at it, in fact, as we mentioned in the commentary, the ARPOB has been growing really at a good pace for even our matured hospitals of Greater Noida, Noida Extension which has touched upwards of 43,000 and 48,000 as far as the ARPOB is concerned.
Sonu Goyal: And in relation to the interest cost, that was your third qu

estion. So, as we know, we have invested in Agra. So, we have taken certain loan to fund the Agra unit. That's the main reason there is an upside in the interest cost what you were asking for.
Yatharth Tyagi: And as far as the debt levels are concerned, somewhere our debt today is at INR 230 crores. But going forward, there would be still certain debt, but we will also be generating strong internal accruals as well as strong cash flow conversion cycles, which will be further leading to higher

internal accruals. So, going forward, yes, we will also take certain debt. But even if you look today, as far as our net debt cash position is concerned, so we stand, our net debt today stands at INR 116 crores. Net cash position is INR 115 crores.
Ashutosh Adsare: Okay. And any levels for increasing the ARPOB going forward?
Yatharth Tyagi: So, you know, if you look at the strategy we have had for the new hospitals, even the Delhi Hospital, the Sector 20 Faridabad Hospital, these two have just recently started, but then also showing a starting ARPOB of upwards of 40,000. So, as volumes and cases, specialities mature in these hospitals, these will be significant ARPOB levers for further growth. As far as Gurugram Hospital is concerned, we expect ARPOB in Gurugram to be upwards of INR 50,000. That has to do primarily because of three reasons. First is, the optimized case mix there, you know, having international patients, a high pricing of private insurance and cash tariffs in that Gurgaon region would also significantly help us for the growth levers in ARPOB. And, you know, constantly we have been increasing the pie of our private insurance, as well as cash and international patients, which has also led us this year to a YoY growth of around 7%. As far as ARPOB is concerned, going forward, also this growth should be close to 10% for upcoming years is concerned.
Ashutosh Kumar Jha: So, to add, we are also improving our speciality mix across all our hospitals. So, while Noida Extension has a large share of super speciality hospitals, we are also improving the share of super speciality cases in all the other hospitals. That will also help improve the ARPOB.
Ashutosh Adsare: Okay. That's it from my side. If I have any further questions, I will join back in the queue.
Moderator: Thank you. We will take the next question from the line of Satyam Kumar from JM Group - Family Office. Please go ahead.
Satyam Kumar: Thanks for the opportunity. I have a couple of questions. First is, can you talk us through how, what steps has companies taken since listing for strengthening the corporate governance? And how do you see going forward, what steps company would take? Or is it now things are more or less settled? So, first question is with regards to overall corporate governance. How do you see evolving it going further?
Yatharth Tyagi: So, for us, since listing corporate governance is one angle that has been a top priority for the management. And if you look at our journey since listing, and especially in last one and a half years, we have taken multiple steps to further strengthen our corporate governance. It all starts with appointment of top six audit firm, BDO International, the MSKA & Associates. They have been appointed as company's statutory auditor a couple of quarters back. In fact, this year's financial results were the first results with the BDO International as a statutory auditor. Second

step we took was appointment of Mr. Ramesh Krishan as an Independent Director. So, we have expanded our board, we have added a strong Independent Director. So, today we have him and his expertise. He's a professional seasoned healthcare leader who has worked across different

organizations and helping and strengthening our corporate governance as far as the board level is concerned. Another step that we took was appointment of Deloitte as an internal auditor. This also happened during the course of last one year, which has further helped us to fast track our financial systems, our processes, and strengthening governance mechanisms through the help of Deloitte. So, these are the few steps that we have taken as far as improving and strengthening our corporate governance is concerned.

Amit Kumar Singh: Going forward also, just to add on a qualitative side of business, we are adding a couple of very key positions as far as operations are concerned for taking care of our medical operations as well as in the quality side of it.

Satyam Kumar: Understood. Thanks for the answer. And another question which I have that is more on the expansion side, as you have shared that companies aim to have a 5,000 bed count in the next three years. So, first I would like to acknowledge that I literally like the way you have expanded your portfolio, the kind of acquisitions you have done. So, going forward, do you see acquiring those hospitals or you will be building hospitals from scratch also? Like any vision, if you'd like to share. I know it's far-fetched, but anything in your mind, and will that increase the debt levels as well? And how it will impact, profitability is the kind of thing, but how you are looking to expand basically?

Yatharth Tyagi: So, as far as the target of 5,000 beds is concerned over the next three years, so 3,200 beds have been already announced, over and above the 2,500 beds that we currently operate, the Noida extension, the Greater Noida and the Gurgaon hospital upcoming, that adds to the 3,200 beds. As far as the target of 5,000 beds over the next three years is concerned, personally, we feel that we might even reach there a bit earlier rather than the three years. We have always surpassed our bed capacity targets in the past, and it might be a possibility going forward also. As far as the question on the Greenfield, Brownfield and the acquisition split is concerned, of these upcoming beds, we feel around 70% would be acquisitions, mostly, and 30% beds would be through Greenfield that we will be adding. And these would primarily be in the cities that we very well understand and know. We have not just gone to any outside area just because there is a hospital to acquire. We have been selectively choosing an asset and then going forward for an asset, because for us, geography matters a lot. There is a reason why all the recent acquisitions have done very well, because they have been strategically located in the well-established markets of NCR, bigger cities of North India, capital cities of the states and Uttar Pradesh, Haryana, Punjab. These have major cities and metro cities where we would also like to be going forward. As far as the capex plan for these bed expansions is considered, we feel that we have a good cash position as of today, and there is still room way for certain debt is concerned. And as we said, because of high cash conversion percentage, we would be generating higher internal accruals going forward. So, we are well in the position to fund this capex. There is no plan to raise any fund going forward for these specific 5,000-bed capacities concerned.

Satyam Kumar: Just a small follow-up on this. As you said, the geography is important for you, and also the 70% might come through acquisitions. So, I just wanted to specifically understand what Yatharth

Hospital does diff

erently. Like we have a M&A team, or how we do so that we come up with such good acquisitions. Just an operational thing I would like to understand. How company is different from any other company when it comes to acquisitions? Because I believe we are at the forefront of acquisitions. So, operationally, if you can share what difference we do?

Yatharth Tyagi: So, I think there is a strong team which is led by Ashutosh, who is sitting with us today. He heads the group M&A strategy. So, he has been instrumental in some of the acquisitions that we have done recently. Also, we have a team who is constantly looking out for assets. But we also have a very straightaway, clear-cut guideline as far as acquisitions are concerned. It is not that we need to frame guidelines for every asset that we see. For us, the capex per bed needs to make sense. For us, if the geography is big enough, if it is a proven healthcare market within the micro-market also. And what has been instrumental is the cluster approach that we have taken. If you look, we started with one hospital in Noida and we have three hospitals there. We started with one hospital in Faridabad, we have two there. Similarly, we have just seeded a new cluster in Gurugram with one hospital. But each of these clusters, and again the UP cluster that we operate, which has the Agra and the Jhansi Orchha Hospital, has the potential for a cluster-based approach, which really helps us to ease into the acquisitions while identifying the clusters that we want to operate in and then we choose the assets which are available in those clusters.

Satyam Kumar: Understood. Thanks for patiently answering my question. I will get back into the queue if I have any question.

Moderator: Thank you. The next question is from the line of Akshat Mehta from Seven Rivers Holdings. Please go ahead.

Akshat Mehta: Hello. My first question is on, understanding this big jump in Noida Extension, Greater Noida ARPOB, from 8% and 15%. Is this purely because of case mix jump, or is there something else behind this increase?

Amit Kumar Singh: More with this jump on OPD, which is translated into the IPD. And plus, yes, all this, as we have mentioned in our previous comments, these super specialities are not matured. So, I think quarter-on-quarter they are getting matured. New line of treatments getting added within the super specialities . So, I think these are the factors which has improved ARPOB significantly.

Yatharth Tyagi: There's also a growing increase in our international patients as far as Greater Noida and Noida Extension Hospital is concerned. The pie of the international patients specifically in these two hospitals have significantly increased, which has hugely contributed and even surpassed ARPOB expectations for these two hospitals growth is concerned. And especially with the, now the new Noida airport starting, which very soon international flights will be taking off from there. The ecosystem around the whole international tourism and medical tourism within the Noida zone is really at an increasing trend. And we feel that both these two hospitals are benefiting from the industry trend.

Akshat Mehta: Just to follow up, can you share with us what the oncology share in Noida Extension now? It was 19%-20% earlier.

Yatharth Tyagi: It is somewhere around 30% today as far as Noida Extension Hospital is concerned. So, the oncology share is growing close to, I would say close to 30% from 20%. And this is primarily due to increase in key areas within oncology, that is the surgical oncology and the bone marrow transplants is concerned. So, we have been performing very good numbers of bone marrow transplants, which is also contributing. We have also added a new o

ncology team of doctors within the Noida Extension Hospital, which is further helping us to drive this forward.

Akshat Mehta: My second question is on the revenue and margins. How should we look at those in FY '27? Because we have done an acquisition, which will come in April 2027. All the acquisitions that we completed in '26 are just going to ramp up in '27. So, how should we look at the margins and the revenue?

Yatharth Tyagi: So, as far as margins are concerned, we have always maintained that at a consolidated level for the full year. Our margin guidance has always been somewhere around 24% to 25%. And we have been delivering on that. And going forward also, we do not see any variation from our margin guidance is concerned. Even yes, a new hospital might be coming in '27, but we have had three new hospitals that have come up in '26 also, we have added around 1,000 bed capacity in the financial year of '26. And even then, we have been at a steady margin of upwards of 24% for the whole year is concerned. So, we feel that in fact, from this financial year, EBITDA margins, next financial year EBITDA margins should actually be better. And similar story would also be seen as far as our revenue growth is concerned. This full financial year, we have grown somewhere close to 36% in the revenue YoY. We feel that in '27, this we would surpass this 36% YoY revenue growth.

Akshat Mehta: Follow up on that, sir, when do we expect a breakeven for this New Delhi and Faridabad hospitals approximately?

Amit Kumar Singh: So, both these hospitals are, I mean, doing fantastically well. To be very honest, it has gone beyond our expectations. So, probably the Faridabad unit Sector 20, I think that's probably the first hospital would be in our group, which will be obviously breakeven in a month of probably 10th or 11th month maximum. As Delhi is concerned, Delhi will take probably close to around 14 to 15 months.

Yatharth Tyagi: So, I think both these two hospitals combined together, you will see in this FY '27 H2, that both these two hospitals would be EBITDA breakeven.

Akshat Mehta: Okay. If I may ask one last question, I just want to understand where are we in the cycle of our brownfield additions? Have we started construction or exactly where are we? And why is it taking kind of, is that taken a backseat with all the new beds coming in with the brownfield additions or where is it exactly?

Yatharth Tyagi: So, we have commissioned the Greater Noida Hospital's brownfield expansion plans. just finalized the structural drawings and the basic construction work has also started. As far as the Noida Extension is concerned, similar phases there. It's not that we have deliberately slowed it down. It's just that we feel that when we were adding new beds in the other parts of NCR that became a first priority. However, the legal formalities and everything have now been completed for both these hospitals. And we feel that at the right time, because these both hospitals still have some occupancy ramp up yet to be seen. So, by the time we will have these brownfield capacity with us is when we will actually be requiring them. So, as far as those two brownfield hospitals plans are concerned, it's pretty much on track.
Akshat Mehta: Okay. Thank you. I will come back in queue.
Moderator: Thank you. The next question is from the line of Nilay Parekh from Perpetuity Ventures LLP. Please go ahead.
Nilay Parekh: Hello, everyone. So, I have two questions. First is, can I get ARPOB and the occupancy percentage across all the units? And my second question is, what is the outlook for the debtor days by the end of FY '27? Thank you.
Nitin Gupta: Yes. So, we have an overall blended occupancy of 71% in the Quarter 4 wit

h the kind of a new
hospital adding, having enough headroom there, as we explained in the earlier question as well. So, the Model Town Hospital, which has been recently opened in this financial year, having an occupancy of 32% on the census bed of 100 with a capacity of 300 beds. On the Faridabad new one having an occupancy of 52% with the census bed of 100, having a capacity of 400 beds. Agra having an occupancy of 52% on the census of 110 with the capacity of 250 beds. Greater Noida having an occupancy of 76% with the census of 330 and the capacity of 400 beds with the further expanded as we explained in the Brownfield expansion. Noida one we have an occupancy of 86% with the census of 215 with the capacity of 250 beds. Noida Extension we have an occupancy of 61% with the census of 390 beds with the total capacity of 450 beds with the further expanded capacity in the Brownfield project. Jhansi-Orchha we are occupancy of 86% with the census bed of 250 with the full capacity of 305 beds. And the Greater Faridabad, the old Faridabad hospital having an occupancy of 69% with the census bed of 150 with the capacity of 200 beds. The overall occupancy is around 71% in Q4 with the year end of 68% in totality.
Nilay Parekh: Okay, thank you sir. Also, my second question was regarding this outlook for the debtor days by the end of FY '27. Thank you.
Sonu Goyal: Yes, so as we informed earlier for FY2025 we clocked debtor days of 124 debtor days and this year that is financial 2026 we had a debtor days of 112 debtor days. This is by driving process efficiency and controls and by reducing time to dispatch the bills and upload through outsourcing the same. But for FY '27 the outlook tend to around 90 to 95 days. What we are aiming for 90 to 95 days for FY2027.

Nilay Parekh: Okay, thank you.
Sonu Goyal: Thank you.
Moderator: Thank you. The next question is from the line of Deekshant Gupta from Geojit PMS. Please go ahead.
Deekshant Gupta: Good morning. Just one question. So, what is the payer for FY '26 versus FY '25?
Amit Kumar Singh: So, the payer mix is it's almost the same. Only thing is it's a 2%-3% down. So, the government payer is around close to 35% and remaining is the cash and TPA. But as we had mentioned, previous efforts are going on to reduce it by and close it to around 25% in next probably two financial years. And I will just add you the effort that if you look at it in any new of our hospitals, the government business percentages are very, very low. And that's what we are keeping control on it. And then existing one also, we are very selectively choosing it.
Yatharth Tyagi: So, when the Sector 20 Faridabad have started and the Model Town Hospital has started, and,

the Gurugram Hospital is starting in few quarters, the government business in those hospitals, we are not expecting more than 10%-12% within two years. So, this is where the overall pie of the government business will come down when the impact of these newer hospitals increases as far as the revenue contribution is concerned. We are able to do it because today we are being joined by most reputed and clinical star doctors within those cities who are bringing huge clientage of private insurance and self-pay patients, as well as international patients, which now we can cater to at a large scale, because we have those treatments, which are required for international patients, something which we are not having 4-5 years back. This is how we are very confident and already being reflected on the numbers of new hospitals, that in two years down the line, the government business should be somewhere around 25%.

Deekshant Gupta: Okay, thank you so much. And in the 35% government mix that you said, majority CGHS?

Yatharth

Tyagi: So, it's basically three themes, it's CGHS, ECHS and ESI. So, the rates of all these are sort of CGHS rates.

Amit Kumar Singh: It varies hospital-to-hospital, but I mean, it's difficult to say overall, what it is. It has, some of the hospital as CGHS is higher percentage. One of the hospital, which is just across the largest ECHS society has the ECHS patients are more. So, largely on a similar front.

Deekshant Gupta: But that means most of the rates are linked to CGHS patients.

Yatharth Tyagi: Yes.

Deekshant Gupta: Okay, and last question for me, are you seeing any dip in international patients in Q1 FY'27 due to the West Asia crisis?

Amit Kumar Singh: Yes, so it's overall, we saw that whatever had happened in the last 3-4 months, there was a dip across the industry. But we believe that some of the particularly for India, like Bangladesh was closed, Afghanistan was closed, Middle East has got disrupted. But we believe that the coming quarter, I think these things will get over and we will be benefited largely. Our efforts are very much into it. In fact, we are very strongly focused on the African market. We have sent our very senior resources in those countries. Two, three medical centers are getting operated, started very soon. Some of the initiatives which we took in the last financial years, I think these are getting matured. So, we are getting a substantial inflow from those regions. So, yes, we feel that the next quarter coming quarter, I think the numbers will be much better.

Deekshant Gupta: Okay, thank you so much and all the best.

Moderator: Thank you. The next question is from the line of Dhaval Sangoi from Canora HSBC Life. Please go ahead.

Dhaval Sangoi: Hi, good morning. Thanks for taking my question. So, could you quantify the CGHS rate benefit which we would have booked in FY '26 and Q4?

Sonu Goyal: So, there is upside of around 5% in our overall business with the revised guidelines by the government, which was come in the month of December.

Yatharth Tyagi: So, from December onwards, if we see, each month on month, we have benefited 5% in the overall revenue for the increase in the rates is concerned and around more than 3% of that has flown to the EBITDA as far as that is also concerned.

Dhaval Sangoi: Okay, so suffice to say, say on a base of about INR 1,200 crores, maybe INR 60 crores to INR 70 crores would be the benefit in terms of the overall revenues for FY '27?

Yatharth Tyagi: So, that happened from that happened from December onwards. So, the real impact you will see would be from this year. So, I think from Q3 and Q4 majorly, Q3 second half and Q4 majorly is where the 5% increase you would see. So, complete effect of 5% was come only in the Q4 quarter of FY2026. For the complete effect of 5% will come in the next financial by FY2027.

Dhaval Sangoi: Okay, and my second question is on the oncology side. So, some of our peers have sort of reported disruption with regards to the chemotherapy drugs, there has been some price control and some conflict. So, have we seen any impact at all in terms of volumes or numbers?

Yatharth Tyagi: Yes, there has been an impact, but it's not a huge impact as far as we are concerned. So, if you see at a group level, oncology contributes to 10% of our overall revenue. Now, what we have measured is certain oncology drugs that were sort of the pricing had been capped, as far as those impact is concerned within that 10% oncology revenue, we see somewhere an impact of close to 20% odd to 30% of the pricing within that 10% of oncology. So, it's not that we have stopped those drugs completely. We do also understand that the substitutes of drugs are not

straightforward and not that easily available. So, yes, we have been seeing a marginal impact. However, for that also we feel that going forward this year as far as because of revision of the CGHS rates, still the pie of oncology revenue will still continue to grow for us. And still for oncology, if you see there are certain of our hospitals that we have recently started oncology and oncology machines like the radiation oncology is still going to start soon in the Sector-20 Faridabad is concerned, for the Model Town New Delhi is concerned. So, for us, the oncology revenue will still continue to increase. However, there is a very marginal impact that we have seen because of the new government policy that has come up.
Sonu Goyal: So, further, as already mentioned by Mr. Yatharth, we have not stopped it. Some of the patients bringing medicines from CGHS, ECHS dispensary. We are also doing parallel exercise to minimize the impact. There are also other streams in oncology like radiation oncology, surgical oncology and bone marrow transplant. We are focusing more.
Dhaval Sangoi: Okay. Thank you.
Moderator: Thank you. We will take the next question from the line of Surya Narayan Nayak from Sunidhi Securities. Please go ahead.
Surya Narayan Nayak: Hi. Hello, everyone. So, I want to ask a question with the acquisition of the hospital in Gurugram. What kind of speciality mix are you targeting for this facility?
Amit Kumar Singh: So, Gurgaon, see, if you see any of our hospital, completely the tertiary and quaternary care services which you would be offering in the same Gurgaon. Because we believe that the Gurgaon is a very competitive market as well, but it has the same time it has a huge potential catchment, right. So, we have to have a very tertiary and quaternary care facility in Gurgaon.
Surya Narayan Nayak: Okay. And could you elaborate on the estimated timeline you are targeting for this Gurugram facility to reach break-even? Additionally, it would be helpful to understand the expected mix both from a speciality perspective and from a pure mix catchment.
Amit Kumar Singh: So, I think that the Gurugram will be up in the next 13 months' to 15 months' time, right. And as far as mix is concerned, there definitely will be more on the self-pay and the insurance business. As we mentioned just now, that's we want to have a bit control on the government's business.
Yatharth Tyagi: And, similar expectations would be to break-even would be somewhere around 15 months from the date it gets operationalized. We are quite confident it should be on our books from the first day of the very new financial year is concerned.
Surya Narayan Nayak: Okay.
Moderator: Thank you. The next question is on the line of Vidhi Shah from CR Kothari & Sons. Please go ahead.

Vidhi Shah: Good morning, sir. I would like to know what is the current loss that we are incurring from the new hospital? And when do we expect the Agra Hospital to break-even?
Nitin Gupta: Among the entire pie at the Model Town New Delhi, so we have a EBITDA loss of INR 21 Cr. Similarly, in the Faridabad new hospital, which has started in this year having a EBITDA loss of 9 Cr. Overall, the percentage of EBITDA drag is around 2% in the overall pie in the net losses. But as we said about it, in the H1 next year ,that is nearly around 12-15 months, we will be able to have the break-even of the EBITDA which will support on the entire one.
Sonu Goyal: Yes, for Agra, Agra itself is contributing around 18% itself.
Yatharth Tyagi: It is a profitable hospital and it is having an EBITDA margin of already 18%

. So, when we acquired the hospital, it was already break-even. So, it's not a EBITDA drag for us.
Vidhi Shah: That's wonderful. Is there any revenue and margin guidance for FY '27-'28?
Yatharth Tyagi: See, we already have said that we will surpass the growth that we have shown this year as far as both the EBITDA and as far as the topline is concerned.
Vidhi Shah: Thank you, sir.
Moderator: Thank you. The next question is from the line of Rushikesh Bhoi from Purnartha Investment Advisors. Please go ahead.
Rushikesh Bhoi: So, I have one follow-up question which is towards the brownfield expansion. So, what is the cost per bed for Greater Noida and Noida Extension expansion? Is there any cost escalation down the line due to uncertainty in current energy prices?
Yatharth Tyagi: So, I think the cost for both these two hospitals, brownfield expansion is concerned, is somewhere around 75 lakhs capex per bed. That is because we have already acquired land. So, that does not include the land. And also, because certain machines and certain things are already there in the towers which are already running. So, that's why the capex per bed would be around

75 lakhs per bed and there is no escalation in the cost that we expect there.
Rushikesh Bhoi: Okay, thank you.
Moderator: Thank you. The next question is from the line of Satyam Kumar from GM Group Family Office. Please go ahead.
Satyam Kumar: Thanks for the opportunity. Sir, just wanted to know like what's the current status of the income tax issue like where we are sitting right now?

Yatharth Tyagi: Income tax issue is almost at its final leg of conclusion. If you go through the order notes of this fiscal year's balance sheet also, we have closed certain years, certain years order has come. However, the company has looked into that and do not see any major financial liability or any large cost as far as that is concerned. For the complete case of order to be out there, we feel that before the end of Quarter 2 for this financial year is somewhere when the whole matter would be resolved. And, today, as far as we are concerned, there's no financial liability or any impact as far as our operations or any financial operations are concerned. And there has been no FDs or any assets that has been provisionally blocked or frozen. Everything is free for the company to be utilized as per our wishes.
Satyam Kumar: I understood. And just one last thing. So, is the margin profile of procedures conducted under different government schemes are like different, like margin profile? Is it the case?
Yatharth Tyagi: So, if you look at the CGHS, ECHS and ESI, the margin profile is sort of similar as far as these three schemes are concerned, because the pricing is same. They all follow CGHS rate. As far as Ayushman is concerned, that is where the margin profile differs. It tends to be much lower. However, Ayushman is something that we.
Satyam Kumar: No, sir, I was actually asking the margin profile of the different procedures, like the margin profile, which a cardiac procedure will give is the same for neurology or the nephrology under government scheme. So, is the margin profile different and for the different procedure under the same government scheme? So, do we focus on conducting high margin procedures under government scheme? Is it the case? Just wanted to, and that's all the questions I have.
Amit Kumar Singh4: Yes, the margin profile definitely varies within the government scheme as well. Depends on what procedures you are operating. It's a surgical procedure. Margins are different if it's a conservative. The margins are very different there. And what is the second question?
Yatharth Tyagi: So, for example, oncology tends to have a better margi

n than compared to, let's say, any other procedures within the government scheme as well. So, does cardiac procedures. So, yes, we also selectively sometimes try to focus on high margin specialities, even within the government profile.
Amit Kumar Singh: But again, that depends on, let's say we are starting a hospital in particular regions. When we have a bed capacity, maybe you need those numbers to fill the beds. So, I mean, that's a journey, right. So, as you get matured, as your occupancy gets filled, you try to be a bit more selective and the controls over the business.
Satyam Kumar: Understood. And sir, ARPOB for Agra hospital?
Amit Kumar Singh: Agra is giving around close to around 26,000-27,000 ARPOB, which is from that region, it's quite good.

Satyam Kumar: Okay. Got it. Thank you for patiently answering that question. Thanks.
Moderator: Thank you. The next question is from the line of Akshat Mehta from Seven Rivers Holding. Please go ahead.
Akshat Mehta: Just one confirmation, you just said, ARPOB, what was that?
Amit Kumar Singh: 26,000.
Akshat Mehta: Okay. My other question was on the overall, I just wanted to understand on the CGHS benefit as well. You said that 5% on the revenue front and 3% on the EBITDA front. A couple of calls back, in FY '27, we had estimated that the revenue impact will be 2.5% and it will be 1.75% on the EBITDA front. Has the benefit kind of gone up or is this, or because 5% and 3% are for Q3 and Q4, we should accordingly take it for the full year?
Yatharth Tyagi: The benefit always as far as the CGHS thing is on the overall business has been similar. The benefit of CGHS on the purely government business, if you compare on that percentage tends to differ. So, probably, that is where the different numbers might be.

Ashutosh Kumar Jha: In your numbers that you were quoting, 4.5% and 1.7%. So, 4.5% was for the full quarter. And that's the same that we are quoting now also. 1.7% was in the quarter when the scheme, the price increase was implemented. So, we did not get the benefit for the full quarter. That's why the benefit for that quarter was 1.7%.
Yatharth Tyagi: And, the benefit also happened in a stage wise. Certain schemes were first implemented the rate revisions. Certain schemes took more time. So, now as far as the full implementation of the rate revision has been done across CGHS, ECHS, and ESI. That's where, in our overall calculation, we feel it should contribute 5% to our overall revenue.
Akshat Mehta: Okay. And, sir, my next question is, if you look at a balance sheet, sir, there's been a big jump in your other assets, your financial assets, and other current assets. Why is that?
Sonu Goyal: So, there is a FD. So, there is a presentation of books of accounts as per the NDA. So, these are the FD parked in the other financial assets. That's the only reason you are getting upside in the number as compared to the last year.
Nitin Gupta: So, in the last week of the financial year, we have good collections in hand from the different peers. So, we have parked the funds centrally into these FDs, so that we can get the best returns from that.
Akshat Mehta: Okay. Thank you.

Moderator: Thank you. The next question is from the line of Gopal Bhatt from Baroda BNP Paribas AMC. Please go ahead.
Gopal Bhatt: Hello. Hi. Thank you for taking my question. Yes, I wanted to check, I was having a look at the strategy slide in the deck. And I see that you have sort of emphasized a cluster-based approach in Delhi. I just want to check that, given

how do you view the competitive dynamics in the overall market, given you have much larger players in the market as well, and something related that, what is the differentiation in the strategy of Yatharth versus other peers?
Yatharth Tyagi: When we talk about cluster-based approach, we sort of tend to become the largest player in the areas of our core operations, so similar to what happened in Noida, we have already proven that with three hospitals, these are sort of different micro markets within the same city. And this is what we are doing with Faridabad. The two hospitals in Faridabad and the three hospitals in Noida are each located sort of 20 kilometers from each other. So, we tend to cater to different micro markets within that same city. However, that gives us the upper hand in that brand building within that city is concerned. So, today, when we are trying to attract star doctors in Faridabad, having two hospitals on different sides of the town, each catering to different micro markets within that city helps us to have that talent on board. Within the Delhi region also, within the Gurugram region also, this is the same approach that we will follow. What makes it sort of different in the sense that we identify micro markets, which are sort of unmet within NCR and the regions beyond, there's still a lot of areas which a city might have a lot of hospitals, but within those cities, the other areas are still upcoming areas which are coming up with population with high-rise urban societies that we are identifying. And sort of that helps us to move early into those regions and build a bigger brand within those cities are concerned. And it's just not that hardcore NCR is what we are looking at. Our UP cluster, which started the seed from Agra still has huge potential. So, does Haryana beyond this Gurugram. And we will be looking at a much bigger market across North India and we feel there are assets available to acquire. Some might not fit certain other players who might be looking for, let's say, a bigger land parcel or bigger size for a hospital. And sometimes for us, it might be okay as far as that size is concerned. Some other players that you mentioned might be already present in those areas. But for us, it could be a good entry to those areas. And these are the things which will sort of help us to make these acquisitions. And that's why we are quite confident of surpassing of target of 5000 beds over the next years. I think we should be there much earlier.
Gopal Bhatt: Okay. And so sorry, the second part, or if I could just follow up that what is the differentiation in our offerings versus other players in Delhi NCR given it is fairly competitive?
Yatharth Tyagi: Healthcare has a huge scope within Delhi NCR is concerned, cities of Noida, Gurugram, Faridabad, and even Ghaziabad and so that other parts of Delhi are still growing massively of population. So, in fact, this is far from saturation. So, there's enough space for quality hospitals for quality clinical talent to establish themselves. Something that sort of helped us in the past that we have been a very doctor friendly organization sort of we have been able to attract clinical talent based on that approach. We have not typically followed the concepts of P&L driven

corporate led organizations where clinicians are given targets to perform certain numbers, we have always sort of provided an easy environment for the clinical talents to join us and sort of that is the reputation that we have created. And that is the reason why a lot of other clinical talent today wants to join us when we are going to the new areas. As far as our infrastructure is concerned, still it's at par with all the other players in the region. As f

ar as certain of our new
hospitals, we are trying to create a sort of a much more boutique hospital environment where sometimes these days when patients are going to big massive hospitals, they feel lost. But with the much more patient centric approach with the same clinical talent, and a much more customized boutique hospital experience is where we feel that we are filling the gap within the metro cities within the regions which might have certain bigger players, 10-20 minutes from us. But this is sort of the gap which we are trying to also fill within the capital and metro cities that we are operating.

Gopal Bhatt: Okay, thank you.

Moderator: Thank you. Ladies and gentlemen, we will take that as the last question for today. I will now hand the conference over to Mr. Yatharth Tyagi for closing comments. Thank you and over to you, sir.

Yatharth Tyagi: Thank you, everyone. Thank you for joining the conference call for this financial year of '26. And we appreciate your questions. Thank you.

Moderator: Thank you, members of the Management. On behalf of Antique Stock Broking Limited, that concludes this conference. We thank you for joining us and you may now disconnect your line. Thank you.

(This document has been edited for readability purpose)

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